

PTCB

Study Guide

Q&A Part 2

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2501. **Q:** A prescription is written for levothyroxine 0.125 mg; the stock bottle is labeled 125 mcg—what is the BEST way to enter the strength and why? **A:** Enter as 125 mcg (or 0.125 mg if your system uses mg) after confirming they are equivalent since $1\text{ mg} = 1000\text{ mcg}$ and this prevents a 1000-fold dosing error.
2502. **Q:** An order says “metformin ER 500 mg i po bid #60”; what days’ supply should you enter and why? **A:** Enter 30 days’ supply because $60\text{ tablets} \div 2\text{ tablets/day} = 30\text{ days}$, which supports accurate labeling and third-party billing.
2503. **Q:** A new prescription reads “digoxin 0.25 mg ii po qd”; what should you do FIRST in order entry and why? **A:** Stop and route to the pharmacist to clarify because “ii” with digoxin (narrow therapeutic index) could indicate a potentially unsafe dose and the sig is high-risk for misinterpretation.
2504. **Q:** A patient gets insulin lispro U-100, directions “inject 8 units before meals TID,” and the quantity is 1 vial (10 mL); what days’ supply should you bill and why? **A:** Bill 41 days because $10\text{ mL} \times 100\text{ units/mL} = 1000\text{ units}$ and $8 \times 3 = 24\text{ units/day}$, so $1000 \div 24 = 41.6$ and you round down to avoid overbilling.
2505. **Q:** The prescriber writes “amoxicillin 250 mg/5 mL: 7.5 mL po tid x 10 days”; what quantity in mL should you dispense and why? **A:** Dispense 225 mL because $7.5\text{ mL/dose} \times 3\text{ doses/day} \times 10\text{ days} = 225\text{ mL}$, ensuring the patient has enough for the full course.
2506. **Q:** During order entry you see “hydroxyzine 25 mg q6h prn anxiety #30,” but the prescriber selected hydralazine in the eRx drug field—what is the BEST technician action and why? **A:** Stop processing and send to the pharmacist to verify/correct the intended drug because hydroxyzine vs hydralazine is a LASA mix-up risk with very different indications.
2507. **Q:** A prescription for atorvastatin says “take 1 tab po qhs,” quantity 90; what is the days’ supply and what brand-name recognition helps avoid selection errors? **A:** Enter 90 days’ supply because 1 tablet/day and recognize Lipitor as atorvastatin to prevent wrong product selection between similar statins.
2508. **Q:** A patient presents a C-II oxycodone prescription and asks for an early refill; the profile shows it was filled 10 days ago for a 30-day supply—what should you do NEXT and why? **A:** Hold the request and alert the pharmacist because early C-II fills require pharmacist review for appropriateness, documentation, and potential misuse/diversion concerns.

2509. **Q:** The sig on a prescription reads “prednisone 20 mg: ii tabs po qd x 5 days”; how many tablets should be dispensed and why? **A:** Dispense 10 tablets because “ii” = 2 tablets/day and $2 \times 5 \text{ days} = 10$, preventing an underfill that could interrupt therapy.
2510. **Q:** While scanning an NDC for cefdinir capsules, the barcode doesn’t match the NDC on the label the system printed—what should you do FIRST and why? **A:** Stop filling and reselect/scan the correct stock bottle because an NDC mismatch signals wrong product selection and barcode verification is a key safety check before it reaches pharmacist verification.
2511. **Q:** While entering a new Rx, the prescriber selects “clonidine” but the directions say “apply 1 patch weekly”; what should you do FIRST? **A:** Stop and route to the pharmacist to clarify dosage form (tablet vs transdermal patch) because clonidine has multiple forms with different dosing and high error risk.
2512. **Q:** A patient’s profile shows lisinopril active, and today you receive an eRx for enalapril for “blood pressure” from the same clinic; what is the BEST technician next step? **A:** Flag for pharmacist review for therapeutic duplication (ACE inhibitor to ACE inhibitor) because the pharmacist must assess whether it’s a switch or unintended duplicate.
2513. **Q:** During product selection you notice “celecoxib 200 mg” is next to “cephalexin 500 mg” and the NDC you grabbed is for cephalexin; what should you do NEXT? **A:** Stop filling and reselect using barcode/NDC verification because LASA lookalikes can cause wrong-drug errors and must be corrected before verification.
2514. **Q:** A new Rx is for sertraline 50 mg daily, and the patient reports taking linezolid from urgent care; what should you do FIRST? **A:** Immediately alert the pharmacist about the interaction risk (SSRI with linezolid) because it can be serious and requires pharmacist intervention before dispensing.
2515. **Q:** A prescription reads “azithromycin 250 mg: take 2 tabs today then 1 tab daily x 4 days”; what quantity should you enter to dispense and why? **A:** Enter quantity 6 tablets because the total dose is $2 + (1 \times 4) = 6$ for the standard 5-day regimen.
2516. **Q:** A patient is newly prescribed glipizide and asks what low blood sugar symptoms to watch for; what is the BEST technician response? **A:** Refer to the pharmacist while noting it’s a sulfonylurea that can cause hypoglycemia because counseling on symptoms and management is pharmacist-provided.

2517. **Q:** While entering "Lamictal," the system displays both lamotrigine and terbinafine options; what should you do BEST to avoid an error? **A:** Verify the intended generic and indication with the hard copy/eRx and use Tall Man/LASA awareness before selecting because similar names can lead to dangerous wrong-drug entry.
2518. **Q:** An eRx arrives for "methotrexate 2.5 mg take 1 tablet daily," and you recognize it's commonly weekly for many indications; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to clarify dosing because daily methotrexate is a high-alert error that can cause severe toxicity.
2519. **Q:** A prescription is for warfarin 5 mg daily, but the patient profile shows an active apixaban prescription; what is the BEST next step? **A:** Stop processing and send to the pharmacist for anticoagulant duplication review because dual therapy may be inappropriate and requires clinical assessment.
2520. **Q:** You receive a manufacturer recall notice for certain lots of losartan due to contamination concerns; what should you do NEXT in the pharmacy? **A:** Quarantine affected lots and follow recall procedures (identify by lot/NDC and remove from stock) because recalled product must not be dispensed and may require patient notification per pharmacist/pharmacy policy.
2521. **Q:** A handwritten Rx for hydromorphone has the quantity and directions but no prescriber signature; what should you do FIRST? **A:** Stop processing and route to the pharmacist to obtain a valid signed prescription because missing required elements makes it unsafe/invalid to dispense.
2522. **Q:** A patient requests an early refill of oxycodone/APAP 5/325 and the PDMP note shows it was filled 7 days ago for a 30-day supply; what is the BEST technician next step? **A:** Hold the refill and notify the pharmacist for review because early controlled-substance refills require pharmacist assessment and documentation.
2523. **Q:** While unpacking an order, you find insulin glargine pens warm and the tote temperature indicator shows an excursion; what should you do NEXT? **A:** Segregate the shipment and alert the pharmacist/manager to follow temperature-excursion policy because compromised cold-chain products may be unsafe to dispense.
2524. **Q:** During final staging you notice you selected heparin 10,000 units/mL vials instead of heparin 1,000 units/mL for a hospital floor stock request; what should you do FIRST? **A:** Stop and correct the product selection using barcode/NDC

verification and alert the pharmacist if it already reached verification because heparin is a high-alert, strength-confusion risk.

2525. **Q:** An eRx for isotretinoin arrives but the patient's iPLEDGE status is not documented in the system; what should you do FIRST? **A:** Pause the fill and refer to the pharmacist to complete REMS verification because isotretinoin dispensing requires REMS authorization steps.
2526. **Q:** A patient offers to return unused amoxicillin suspension that was dispensed last week to be "put back on the shelf"; what should you do BEST? **A:** Decline return-to-stock and follow disposal/return policy because previously dispensed liquids cannot be re-dispensed due to contamination and safety concerns.
2527. **Q:** You receive a C-II eRx for methylphenidate but the patient asks to transfer it to another pharmacy across town; what should you do NEXT? **A:** Tell the patient you must involve the pharmacist and do not initiate a transfer because C-II prescriptions generally cannot be transferred between pharmacies.
2528. **Q:** A new prescription says "U-500 insulin regular: inject 50 units subcut BID," but the patient profile shows U-100 syringes on file; what should you do FIRST? **A:** Stop and immediately notify the pharmacist because U-500 is a high-alert concentration with serious dosing-device mismatch risk.
2529. **Q:** During DSCSA receiving, a bottle of atorvastatin arrives with a broken seal and no readable lot/expiration; what is the BEST technician action? **A:** Isolate the product and notify the pharmacist/wholesaler for investigation because suspect/illegitimate product cannot be placed into inventory without proper identification.
2530. **Q:** A prescription for levothyroxine is entered as "0.125 mg," but the prescriber's note says "125 mcg daily"; what should you do BEST during data entry? **A:** Enter as 125 mcg (equivalent to 0.125 mg) and ensure the label displays the common mcg strength to reduce dosing confusion, routing to the pharmacist if any inconsistency remains.
2531. **Q:** A new eRx is for Zocor 40 mg nightly, but the shelf label you grabbed says simvastatin 40 mg; what should you do NEXT before filling? **A:** Confirm Zocor is the brand name for simvastatin and verify strength/form/NDC match because brand-generic recognition prevents wrong-drug dispensing.
2532. **Q:** While entering a prescription you see "levothyroxine 100 mcg" and the patient profile lists Synthroid 0.1 mg; what is the BEST technician action? **A:**

Recognize 0.1 mg equals 100 mcg and keep the dose consistent while selecting the correct product because unit conversions avoid tenfold dosing errors.

2533. **Q:** A patient's new prescription is for lisinopril, and their profile shows an active Rx for Zestril; what should you do FIRST? **A:** Stop and flag the profile for possible therapeutic duplication and notify the pharmacist because Zestril is lisinopril and duplicate therapy needs pharmacist review.
2534. **Q:** A prescriber writes "MS Contin 15 mg" and the patient asks if it's the same as "MSIR"; what is the BEST technician response/action? **A:** Refer to the pharmacist and clarify that MS Contin is extended-release morphine while MSIR is immediate-release because formulation mix-ups are high-risk and require pharmacist counseling.
2535. **Q:** You receive an eRx for sertraline, and the patient's allergy field says "rash with Zoloft"; what should you do NEXT? **A:** Hold processing and alert the pharmacist because Zoloft is sertraline and this indicates a potential allergy to the same drug.
2536. **Q:** During product selection you notice hydrOXYzine and hydrALAZINE stored near each other and the label prints for hydroxyzine 25 mg; what should you do BEST before counting? **A:** Use LASA safeguards by reading the full name/strength and scanning the barcode/NDC because hydroxyzine vs hydralazine mix-ups can cause serious harm.
2537. **Q:** A prescription says "metformin ER 500 mg, i tab po bid" but the chosen stock bottle is metformin IR 500 mg; what should you do FIRST? **A:** Stop filling and re-select the correct ER formulation (and route to pharmacist if already at verification) because ER vs IR is not interchangeable.
2538. **Q:** A patient brings a new Rx for "Augmentin" and asks what it is; what is the BEST technician-level explanation while preparing to process it? **A:** State that Augmentin is amoxicillin/clavulanate and then route counseling questions to the pharmacist because technicians can identify brand/generic but not provide clinical advice.
2539. **Q:** A prescription is written "prednisone 10 mg: take ii tabs daily" and you are asked to enter days' supply for #30 tablets; what is the correct days' supply? **A:** Enter 15 days because "ii" means 2 tablets daily and $30 \div 2 = 15$.
2540. **Q:** An order entry note says "APAP" and the patient is also prescribed Norco; what should you do NEXT? **A:** Flag for pharmacist review due to possible

duplicate acetaminophen exposure because Norco contains APAP and total daily APAP safety must be assessed.

2541. **Q:** While typing a new Rx you see "atenolol 50 mg qd" but the patient profile shows active "Tenormin 50 mg daily"; what should you do FIRST? **A:** Check the profile for therapeutic duplication and route to the pharmacist to confirm continuation vs duplicate therapy, because brand and generic are the same drug.
2542. **Q:** A prescription is written for "Prilosec 20 mg daily" and the patient asks what medication that is as you process it; what is the BEST technician-level response? **A:** Identify it as omeprazole, a proton pump inhibitor for heartburn/GERD, because technicians can provide basic name/class/indication without clinical counseling.
2543. **Q:** During data entry you receive an eRx for "Lamictal 100 mg" and the allergy field lists "lamotrigine—hives"; what should you do NEXT? **A:** Stop processing and alert the pharmacist for allergy review, because Lamictal is lamotrigine and could cause a repeat reaction.
2544. **Q:** A new Rx is for "Plavix 75 mg daily," but the profile shows active clopidogrel 75 mg; what is the BEST action before filling? **A:** Verify it's the same medication (Plavix = clopidogrel) and route to the pharmacist to prevent duplicate active orders, because brand/generic duplicates can lead to double therapy.
2545. **Q:** You are selecting product for "Celebrex 200 mg" and notice "Celexa 20 mg" stocked nearby; what should you do BEST before counting? **A:** Perform an NDC/barcode check and read drug name/strength aloud to match the label, because LASA look-alikes like Celebrex/Celexa are a common selection error.
2546. **Q:** An Rx reads "insulin lispro 10 units SQ tid with meals; dispense 1 vial 10 mL (100 units/mL)"; what days' supply should you enter? **A:** Enter 33 days because $10\text{ mL} \times 100\text{ units/mL} = 1000\text{ units}$ and $10\text{ units} \times 3/\text{day} = 30\text{ units/day}$, so $1000 \div 30 = 33.3 \rightarrow 33\text{ days}$.
2547. **Q:** A patient's new prescription is for "azithromycin" and the prescriber note also says "Z-Pak"; what is the BEST concept to apply during entry? **A:** Recognize Z-Pak as the common azithromycin pack and ensure the correct product/quantity is selected, because brand-style nicknames can mislead quantity selection.
2548. **Q:** A prescription says "hydrochlorothiazide 25 mg po qam #90," but you accidentally pulled hydroxyzine 25 mg; what should you do FIRST? **A:** Stop filling and re-select the correct drug using NDC/barcode verification, because HCTZ

and hydroxyzine are LASA and wrong-drug errors must be corrected immediately.

2549. **Q:** An eRx comes in for "Paxlovid" and the system flags it as requiring pharmacist assessment; what should you do NEXT as a technician? **A:** Hold the order and notify the pharmacist for clinical/interaction screening, because technicians should not override alerts tied to high-risk therapies.
2550. **Q:** A prescription is written "amoxicillin 500 mg cap i po tid #21"; what is the correct days' supply? **A:** Enter 7 days because $\#21 \text{ capsules} \div 3 \text{ capsules/day} = 7 \text{ days}$, which matches standard days' supply calculation.
2551. **Q:** While entering a new Rx for levothyroxine, you notice the prescriber wrote "Synthroid 0.1 mg," what is the BEST technician action to prevent a strength error? **A:** Enter it as 100 mcg (not 0.1 mg) and flag for pharmacist verification because levothyroxine strengths are commonly expressed in mcg and conversion errors can cause harm.
2552. **Q:** A hardcopy prescription for oxycodone/acetaminophen is presented without the prescriber's signature; what should you do FIRST? **A:** Stop processing and route to the pharmacist because a missing signature is a validity issue that must be resolved before dispensing a controlled substance.
2553. **Q:** During product selection for "hydralazine 25 mg," you almost grab "hydroxyzine 25 mg" from an adjacent bin; what is the BEST next step? **A:** Re-select the correct drug by verifying barcode/NDC against the label/Rx because LASA mix-ups are common and NDC verification prevents dispensing errors.
2554. **Q:** A patient asks to transfer their remaining refills for alprazolam to another pharmacy; what is the BEST technician response? **A:** Refer the request to the pharmacist because controlled-substance transfers have strict requirements and must be handled by a pharmacist per law/policy.
2555. **Q:** You receive a bottle of refrigerated insulin glargine that arrived warm with a melted cold pack; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager for temperature-excursion procedures because potency may be compromised and it must not be stocked.
2556. **Q:** A recall notice lists a specific lot number of losartan; you find that lot on your shelf, what is the BEST action? **A:** Pull and quarantine the affected lot and alert the pharmacist to follow recall workflow because recalled lots cannot be dispensed and must be tracked.

2557. **Q:** An eRx for isotretinoin appears with an iPLEDGE/REMS alert in the system; what should you do NEXT? **A:** Stop and refer to the pharmacist because REMS requirements must be confirmed before dispensing and technicians should not override REMS alerts.
2558. **Q:** A patient wants to buy pseudoephedrine and requests a large quantity; what should you do FIRST at the register? **A:** Verify government ID and process the sale through the required log with quantity limits because CMEA controls PSE sales and documentation is required.
2559. **Q:** A prescription is for metformin ER 500 mg “take 2 tablets daily” and the quantity is #120; what days’ supply should you enter? **A:** Enter 60 days’ supply because $120 \text{ tablets} \div 2 \text{ tablets per day} = 60 \text{ days}$, supporting correct billing and refill timing.
2560. **Q:** While scanning incoming stock, you see a sealed case with no readable 2D barcode/lot/expiration information for DSCSA tracing; what is the BEST action? **A:** Hold the shipment and notify the pharmacist/receiver to obtain proper tracing/identification because DSCSA requires traceable product data and suspect items must not enter inventory.
2561. **Q:** An Rx reads “predniSONE 10 mg tabs: take 4 tabs daily x3 days, then 3 tabs daily x3 days, then 2 tabs daily x3 days, then 1 tab daily x3 days; dispense #?”—what quantity should you enter? **A:** Enter 30 tablets because $(4+3+2+1) \text{ tabs/day} \times 3 \text{ days each} = (10 \times 3) = 30$, preventing an underfill.
2562. **Q:** You’re entering “azithromycin 250 mg tabs: take 2 tabs today, then 1 tab daily days 2–5; dispense #6”—what days’ supply should you bill? **A:** Bill 5 days’ supply because the regimen covers day 1 through day 5 even though the quantity is 6 tablets.
2563. **Q:** A prescription says “gabapentin 300 mg cap i po tid #90”—what days’ supply should you input? **A:** Input 30 days because $90 \text{ capsules} \div 3 \text{ per day} = 30$, ensuring correct insurance submission.
2564. **Q:** An Rx is written “Lantus Solostar inject 30 units qhs; dispense 1 box (5 pens of 3 mL, 100 units/mL)” —what days’ supply should you enter? **A:** Enter 50 days because $\text{total units} = 5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 30 \text{ units/day} = 50$.
2565. **Q:** A patient is prescribed “fluticasone nasal spray 50 mcg: 2 sprays each nostril once daily” and the product is 120 sprays—what days’ supply should you

calculate? **A:** Calculate 30 days because daily sprays = 2 sprays $\times$ 2 nostrils = 4 and $120 \div 4 = 30$.

2566. **Q:** While interpreting a handwritten sig you see "MSO4 2 mg IV q2h prn pain" during order entry—what should you do FIRST? **A:** Stop and refer to the pharmacist to clarify because "MSO4" is an error-prone abbreviation that can be confused with magnesium sulfate, risking a serious mix-up.
2567. **Q:** An eRx arrives for "metoprolol succinate ER 25 mg take 1 tab daily," but the patient's active profile shows metoprolol tartrate 25 mg BID—what is the BEST next step for a technician? **A:** Pause processing and alert the pharmacist to evaluate possible therapeutic duplication/formulation change because tartrate vs succinate are not interchangeable.
2568. **Q:** An Rx for "Keflex 500 mg" is entered, but the prescriber typed "cephalexin 500 mg" and the patient asks if it's the same drug—what is the BEST technician response? **A:** Confirm Keflex is the brand name for cephalexin and then offer pharmacist counseling if they have clinical questions because technicians can provide brand/generic identification but not clinical advice.
2569. **Q:** A prescription says "dorzolamide/timolol eye drops: instill 1 gtt OU BID; dispense 10 mL"—what days' supply should you enter using 20 gtt/mL? **A:** Enter 50 days because total drops = $10 \text{ mL} \times 20 = 200$ drops and daily use = $1 \text{ drop} \times 2 \text{ eyes} \times 2 \text{ times/day} = 4$, so $200 \div 4 = 50$.
2570. **Q:** During data entry you notice the directions say "take 1 tablet qd" but the quantity entered would last only 5 days at that rate—what should you do NEXT? **A:** Recheck the original prescription and correct the quantity/days' supply fields or route to the pharmacist if the written order is inconsistent because mismatched sig/quantity is a common order-entry error that impacts safety and billing.
2571. **Q:** During data entry, a new Rx is for "Lamictal 25 mg" but you accidentally selected Lamisil 250 mg in the system; what should you do FIRST? **A:** Stop processing and reselect the correct drug/NDC before labeling because Lamictal (lamotrigine) vs Lamisil (terbinafine) is a LASA trap that can cause serious harm.
2572. **Q:** A patient picking up lisinopril says they are pregnant and asks if it's safe to start today; what is the BEST next step for a technician? **A:** Pause the pickup and immediately involve the pharmacist because ACE inhibitors like lisinopril are contraindicated in pregnancy and require clinical intervention.

2573. **Q:** A prescription is written for "hydralazine 25 mg" but you notice the prescriber intended "hydroxyzine 25 mg" in the notes; what should you do NEXT? **A:** Hold the order and refer to the pharmacist to clarify with the prescriber because hydralazine vs hydroxyzine is a high-risk name mix-up and technicians cannot change therapy.
2574. **Q:** A patient on simvastatin brings in a new Rx for clarithromycin; what should you do BEST as a technician before filling? **A:** Flag and route to the pharmacist because clarithromycin can significantly increase simvastatin levels, raising myopathy/rhabdomyolysis risk.
2575. **Q:** You receive an eRx for glipizide and the patient's profile shows glyburide is active; what should you do FIRST? **A:** Stop and notify the pharmacist to assess for therapeutic duplication because both are sulfonylureas and duplicate therapy can cause hypoglycemia.
2576. **Q:** A prescription is for "levothyroxine 75 mcg daily," but the product pulled is 75 mg; what is the BEST action? **A:** Stop filling and correct the selection immediately because mcg vs mg is a dangerous dose error and levothyroxine is high-alert for strength mix-ups.
2577. **Q:** An amlodipine 5 mg prescription is written for 1 tablet daily with #90 dispensed; what days' supply should you enter? **A:** Enter 90 days because $90 \text{ tablets} \div 1 \text{ tablet per day} = 90 \text{ days}$.
2578. **Q:** A prescription is for sertraline, and the patient asks what it's commonly used for; what is the BEST technician response? **A:** Provide a general use statement and offer pharmacist counseling because sertraline is an SSRI commonly used for depression/anxiety, and counseling supports safe use.
2579. **Q:** A new order is for "warfarin 5 mg daily," and the patient profile shows an active prescription for warfarin 2 mg with a different schedule; what should you do NEXT? **A:** Hold and alert the pharmacist because warfarin is high-alert and overlapping strengths/schedules increase bleeding risk and require pharmacist review.
2580. **Q:** You are processing amoxicillin/clavulanate and the patient reports a history of anaphylaxis to penicillin; what should you do FIRST? **A:** Stop the fill and immediately notify the pharmacist because amoxicillin is a penicillin and anaphylaxis risk requires pharmacist intervention before dispensing.

2581. **Q:** While typing an eRx for oxycodone/acetaminophen, you notice it lists the patient name and drug but no prescriber DEA/NPI information; what should you do FIRST? **A:** Stop processing and route to the pharmacist to verify prescriber validity because controlled-substance prescriptions require complete, verifiable prescriber information.
2582. **Q:** A patient requests an "early fill" of clonazepam 1 mg because they "lost the bottle," and the PDMP note shows it was filled 10 days ago; what is the BEST next step for a technician? **A:** Refer immediately to the pharmacist for review because early controlled-substance refill/lost medication requests require pharmacist assessment and documentation.
2583. **Q:** During product selection you see insulin glargine pens were stored on the counter at room temperature all weekend before being found; what should you do FIRST? **A:** Quarantine the insulin and notify the pharmacist because a temperature excursion makes the product suspect and it may be unsafe to dispense.
2584. **Q:** A tote arrives with a pedigree/transaction statement missing for a high-cost brand drug, and the invoice looks inconsistent with the wholesaler account; what should you do NEXT under DSCSA? **A:** Hold the product as suspect and alert the pharmacist/manager to investigate because DSCSA requires tracking documentation and verification before dispensing.
2585. **Q:** A new Rx is entered for "hydromorphone 4 mg" but the hard copy is handwritten and could be read as "morphine 4 mg"; what should you do FIRST? **A:** Stop and send to the pharmacist for clarification because illegible orders and LASA/high-alert opioids create a high risk of serious harm.
2586. **Q:** A patient drops off a prescription for isotretinoin and asks to pick it up tomorrow, but the profile shows no iPLEDGE/RMS documentation on file; what should you do NEXT? **A:** Notify the pharmacist and do not promise a pickup time because isotretinoin requires REMS/iPLEDGE verification before it can be dispensed.
2587. **Q:** You are entering a prescription that says "digoxin 0.125 mg take i tab po qd," and the system defaults to 0.25 mg; what is the BEST action? **A:** Correct the strength to 0.125 mg and route for pharmacist check because digoxin is high-alert and strength selection errors are clinically significant.
2588. **Q:** A prescription is for "prednisone 10 mg: take 2 tablets daily for 5 days; dispense #20," and you notice the quantity doesn't match the directions; what

should you do NEXT? **A:** Pause filling and refer to the pharmacist to clarify the intended quantity because the written quantity conflicts with the calculated need of 10 tablets (2×5).

2589. **Q:** A patient asks to buy pseudoephedrine, and their driver's license is expired; what should you do FIRST under CMEA workflow? **A:** Decline the sale and involve the pharmacist/manager because pseudoephedrine sales require valid ID and compliant logbook/electronic tracking.
2590. **Q:** During counseling, a patient picking up metformin says they developed facial swelling and trouble breathing after the first dose yesterday; what should you do BEST as a technician? **A:** Tell the patient to seek immediate emergency care and alert the pharmacist right away because possible anaphylaxis requires urgent medical attention and pharmacist intervention.
2591. **Q:** A new eRx is for "glipizide 10 mg ER daily," but the patient's active med list shows "glipizide 10 mg IR twice daily"; what should you do NEXT? **A:** Stop processing and alert the pharmacist to possible therapeutic duplication/formulation mix-up because IR vs ER dosing differs and could cause hypoglycemia.
2592. **Q:** You are selecting a stock bottle and notice "hydrOXYzine" and "hydrALAZINE" are stored next to each other; what is the BEST technician step before filling? **A:** Use Tall Man lettering awareness and verify NDC/barcode against the Rx because this LASA pair can cause serious wrong-drug errors.
2593. **Q:** A prescription is written for "metoprolol 50 mg take 1 tab daily," but the product pulled is metoprolol succinate ER and the profile shows the patient previously took metoprolol tartrate; what should you do FIRST? **A:** Pause and route to the pharmacist to confirm the intended salt/formulation because tartrate vs succinate are not interchangeable and affect dosing.
2594. **Q:** While entering "levothyroxine 25 mcg daily," the system autocorrects to 25 mg; what should you do BEST? **A:** Correct the strength to 25 mcg and flag for pharmacist verification because levothyroxine dosing is in micrograms and mg would be a dangerous 1000× error.
2595. **Q:** A patient picking up sertraline asks if they can start it today and also take OTC St. John's wort; what should you do NEXT? **A:** Refer to the pharmacist for an interaction check because combining serotonergic agents increases risk of serotonin syndrome.

2596. **Q:** The label directions read "take 1 tablet by mouth once daily," but the Rx image says "lisinopril 10 mg take 1 tablet twice daily"; what is your NEXT step? **A:** Stop the fill and fix the sig to match the prescription, then send for pharmacist check because direction-entry errors can cause under- or overdosing.
2597. **Q:** A patient presents a new order for atorvastatin and says they are already taking simvastatin from another doctor; what should you do FIRST? **A:** Document the patient's report and notify the pharmacist because duplicate statin therapy raises adverse-effect risk and needs pharmacist assessment.
2598. **Q:** You receive a verbal message from a prescriber's office asking to change "cephalexin" to "clindamycin" due to allergy, and they want it processed immediately; what should you do BEST? **A:** Route the change to the pharmacist to take/verify the new order because technicians cannot accept or authorize therapy changes.
2599. **Q:** A patient is prescribed azithromycin Z-Pak and asks how many tablets they will take total; what is the correct total quantity to dispense? **A:** Dispense 6 tablets because the Z-Pak regimen is 2 tablets on day 1 then 1 tablet daily on days 2–5 ($2 + 4 = 6$).
2600. **Q:** A new prescription is for "bupropion SR 150 mg BID," but the patient profile shows a seizure history noted in the allergy/condition field; what should you do NEXT? **A:** Hold and alert the pharmacist because bupropion can lower seizure threshold and requires pharmacist review for safety.
2601. **Q:** An eRx reads "furosemide 20 mg i po qd" but the prescriber notes section says "for 5 days"; what days' supply should you enter for #5 tablets? **A:** Enter 5 days because i po qd means 1 tablet daily and #5 at 1/day lasts 5 days, matching the prescriber note.
2602. **Q:** A prescription is "amoxicillin-clavulanate 875/125 mg 1 tab po bid #14"; what is the correct days' supply to submit to insurance? **A:** Enter 7 days because BID is 2 tablets/day and $14 \div 2 = 7$, which prevents an incorrect refill-too-soon reject.
2603. **Q:** You receive "predniSONE 10 mg: take ii tabs po qam x 5 days" and quantity is missing; what should you do NEXT before processing? **A:** Stop and send to the pharmacist to clarify quantity because ii daily for 5 days requires #10 and technicians cannot add missing required dispensing details without authorization.

2604. **Q:** A patient is prescribed "omeprazole 20 mg take 1 cap po qd" and the prescriber requests 90-day supply; what quantity should you enter? **A:** Enter #90 because once daily for 90 days needs 90 capsules, aligning order entry quantity to the requested duration.
2605. **Q:** An order is for "atorvastatin 40 mg take 1 tab nightly" but you pull rosuvastatin 40 mg because both are on the shelf nearby; what is the BEST technician action before filling? **A:** Stop and reselect the correct drug by matching NDC/barcode and name/strength because swapping statins is a wrong-drug error with patient-harm risk.
2606. **Q:** A prescription says "warfarin 5 mg: take 1 tab po qd #30" but the patient profile shows active warfarin 2 mg with complex directions; what should you do FIRST? **A:** Hold and alert the pharmacist to review for duplication/dose confusion because warfarin is high-alert and technicians should not assume intended dose changes.
2607. **Q:** An inhaler order is "tiotropium Respimat 2.5 mcg: inhale 2 puffs once daily," and the device contains 60 actuations; what days' supply should you enter? **A:** Enter 30 days because 2 puffs/day uses 2 actuations/day and $60 \div 2 = 30$, which is critical for correct billing and refill timing.
2608. **Q:** A prescription is "clindamycin 75 mg/5 mL: take 10 mL po tid x 7 days"; what total mL should be dispensed? **A:** Dispense 210 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$, ensuring the patient has enough medication for the full course.
2609. **Q:** While entering "hydromorphone 2 mg 1 tab po q4-6h prn pain #20," the system suggests HYDROcodone/APAP due to an abbreviation search; what should you do BEST? **A:** Use the exact drug name and verify strength/form against the prescription image before selecting because opioid LASA/look-up errors can lead to dispensing the wrong controlled substance.
2610. **Q:** A prescription reads "metformin 500 mg ER take 1 tab po bid with meals #60"; what is the correct days' supply and why might entering 30 instead of 15 cause a claim issue? **A:** Enter 30 days because 2 tablets/day and $60 \div 2 = 30$, and an incorrect shorter days' supply can trigger early refill discrepancies and inaccurate adherence metrics.
2611. **Q:** While typing a new eRx, you see "levothyroxine 50 mcg (Synthroid)" but the patient says they've always taken "Euthyrox"; what should you do BEST before processing? **A:** Confirm the generic (levothyroxine) strength/directions and use

the prescriber-selected product or follow substitution rules, because different brands can vary and consistency matters for thyroid therapy.

2612. **Q:** A prescription comes in for "glipizide 10 mg XL daily," but the product you pulled is immediate-release glipizide 10 mg; what should you do FIRST? **A:** Stop filling and reselect the correct ER/XL dosage form and NDC (then proceed to verification), because IR vs ER is not interchangeable and is a common order-entry/selection error.
2613. **Q:** During data entry you notice "Lantus 10 units qhs" and the profile also shows active "Toujeo 30 units daily"; what is the BEST technician action? **A:** Pause processing and route to the pharmacist to assess therapeutic duplication, because both are insulin glargine products and duplicate basal insulin can cause hypoglycemia.
2614. **Q:** A prescriber writes "hydralazine 25 mg i po tid" and the patient asks what "tid" means; what should you answer? **A:** "Three times daily," because "tid" is a standard sig code frequency used for directions.
2615. **Q:** You receive a prescription for "cephalexin 500 mg qid" and the patient profile lists "anaphylaxis to penicillin"; what should you do NEXT? **A:** Flag it and notify the pharmacist for allergy cross-reactivity assessment, because a severe beta-lactam allergy may require pharmacist review before dispensing a cephalosporin.
2616. **Q:** A new order is "sertraline 50 mg qd," but the patient's active meds include "paroxetine 20 mg qd"; what should you do BEST? **A:** Hold and alert the pharmacist to possible duplicate SSRI therapy and switch/overlap risk, because two SSRIs together can increase adverse effects and requires clinical direction.
2617. **Q:** An eRx says "morphine ER 30 mg q12h," but you selected morphine IR 30 mg tablets from the shelf; what should you do FIRST? **A:** Stop and correct to morphine extended-release with Tall Man/ER verification, because IR vs ER opioid mix-ups are high-alert and can cause overdose.
2618. **Q:** A bottle contains 473 mL and the label requests "dispense 1 pint" of lactulose solution; how many mL should you dispense? **A:** Dispense 473 mL, because 1 pint equals 16 fl oz and the standard pharmacy conversion is 1 pint $\approx$ 473 mL.
2619. **Q:** A prescription is written "metoprolol succinate 50 mg daily," but the patient insists they take "metoprolol tartrate twice daily"; what should you do BEST? **A:** Verify the exact salt form and frequency against the prescription and profile and

route discrepancies to the pharmacist, because succinate (ER) and tartrate (IR) are not interchangeable.

2620. **Q:** You are entering "ciprofloxacin 0.3% eye drops: instill 2 gtt OU q4h while awake x 7 days, dispense 5 mL"; what is the BEST way to interpret "OU"? **A:** Enter "both eyes," because "OU" means oculus uterque and correct site entry prevents dosing and labeling errors.
2621. **Q:** While entering an eRx for "clonazepam 0.5 mg take 1 tab po bid," the system autopopulates "clonidine 0.1 mg"; what should you do FIRST? **A:** Stop and reselect the correct drug (clonazepam vs clonidine) using full name/Tall Man and NDC verification because this LASA mix-up can cause serious harm.
2622. **Q:** A hardcopy says "MS Contin 15 mg q12h," but the shelf bin you grabbed is "MSIR 15 mg"; what is the BEST technician action? **A:** Stop filling and pull the correct extended-release product for pharmacist check because IR vs ER mix-ups are high-risk opioid errors.
2623. **Q:** A new prescription is for "lisinopril 20 mg daily," and the profile shows active "losartan 50 mg daily"; what should you do NEXT? **A:** Flag and refer to the pharmacist for possible therapeutic duplication (ACE inhibitor + ARB) because the technician should not assume the intent.
2624. **Q:** A patient dropping off a new script for "simvastatin 40 mg qhs" says they also take "amlodipine 10 mg daily"; what should you do BEST before processing? **A:** Alert the pharmacist to evaluate the interaction/dose limit because simvastatin with amlodipine increases myopathy risk and may require a change.
2625. **Q:** You receive an eRx for "azithromycin Z-Pak," and the prescriber note says "for strep throat"; what is the BEST technician clarification to route to the pharmacist? **A:** Route to the pharmacist to confirm appropriateness/allergy history because azithromycin is not first-line for strep unless beta-lactam allergy or other rationale exists.
2626. **Q:** A prescription reads "warfarin 5 mg take 1 tab daily," but the patient profile already has active "apixaban 5 mg bid"; what should you do FIRST? **A:** Hold and immediately notify the pharmacist about duplicate anticoagulation because concurrent therapy can cause dangerous bleeding.
2627. **Q:** A patient asks what medication "omeprazole (Prilosec)" is typically used for when picking up; what should you say within technician scope? **A:** State it's for

heartburn/GERD by reducing stomach acid and offer pharmacist counseling because indication questions may need clinical follow-up.

2628. **Q:** A label is being generated for "predniSONE 10 mg" but the product pulled is "prednisoLONE 10 mg"; what should you do BEST? **A:** Stop and correct the selection and verify NDC because prednisone vs prednisolone is a common name confusion and can lead to the wrong medication dispensed.
2629. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg 1 tab po bid #14"; what is the correct days' supply? **A:** 7 days because $14 \text{ tablets} \div 2 \text{ tablets per day} = 7$, which is needed for accurate billing and proper refill timing.
2630. **Q:** You open a tote and find insulin glargine pens that arrived warm with no temperature indicator documentation; what should you do NEXT? **A:** Quarantine the shipment and notify the pharmacist/manager per cold-chain policy because suspected temperature excursion makes product integrity uncertain.
2631. **Q:** An eRx comes in for "levothyroxine 0.1 mg PO daily"; what strength should you enter and why? **A:** Enter levothyroxine 100 mcg because $0.1 \text{ mg} \times 1,000 = 100 \text{ mcg}$ and converting prevents decimal-point dosing errors.
2632. **Q:** A prescription says "insulin lispro U-100: inject 8 units subcutaneously before meals TID; dispense 1 vial (10 mL)"; what days' supply should you bill? **A:** Bill 41 days because $10 \text{ mL} = 1,000 \text{ units}$ and $8 \times 3 = 24 \text{ units/day}$, so $1,000 \div 24 = 41.6$ and days' supply is typically rounded down.
2633. **Q:** You are entering "dorzolamide/timolol eye drops: instill 1 gtt OU BID; dispense 10 mL"; what days' supply should you calculate using 20 gtt/mL? **A:** Bill 50 days because $10 \text{ mL} \times 20 = 200 \text{ drops}$ and $\text{OU BID} = 4 \text{ drops/day}$, so $200 \div 4 = 50$.
2634. **Q:** A new order is "amLODIPine-benazepril 5/20 mg 1 cap daily"; what should you do FIRST if the patient profile already has active benazepril 20 mg daily? **A:** Stop and route to the pharmacist for duplication review because the combo product would duplicate the ACE inhibitor component.
2635. **Q:** A hardcopy reads "hydroxyzine 25 mg 1 tab q6h prn anxiety #30," but the prescriber handwriting could also be "hydralazine"; what should you do NEXT? **A:** Hold processing and ask the pharmacist to clarify with the prescriber because this is a high-risk LASA misread with very different indications.

2636. **Q:** A prescription says "Take ii tabs po qid" for metformin 500 mg, quantity 240; what is the correct days' supply? **A:** Days' supply is 30 because ii = 2 and qid = 4 times/day, so $2 \times 4 = 8$ tablets/day and $240 \div 8 = 30$.
2637. **Q:** While typing an eRx for "carBAMazepine 200 mg 1 tab BID," the drop-down auto-selects "carisoprodol 350 mg"; what should you do FIRST? **A:** Stop entry and reselect the correct drug using full name/strength (and barcode/NDC at product selection) because autopopulated choices are a common wrong-drug error.
2638. **Q:** A prescription is "cephalexin 250 mg/5 mL: take 10 mL PO TID x 7 days"; what quantity in mL should you enter to dispense? **A:** Dispense 210 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$ to cover the full course.
2639. **Q:** A patient's new eRx is "sertraline 50 mg take 1 tab daily," and the profile shows active "fluoxetine 20 mg daily"; what is the BEST technician next step? **A:** Flag and refer to the pharmacist before filling because concurrent SSRI therapy may be therapeutic duplication and needs clinical direction.
2640. **Q:** A C-II eRx for "oxycodone 5 mg 1 tab q6h prn pain #20" is missing the patient address in your system; what should you do FIRST? **A:** Pause processing and notify the pharmacist to verify required prescription/patient information because controlled-substance dispensing requires complete, accurate identifiers.
2641. **Q:** While filling a new prescription for heparin, you notice the shelf has both heparin flush 100 units/mL and heparin 10,000 units/mL vials next to each other; what should you do FIRST? **A:** Separate and clearly label the strengths and use barcode/NDC scan before selection because heparin is a high-alert LASA risk and mix-ups can cause serious harm.
2642. **Q:** An eRx arrives for "clonazepam 0.5 mg take 1 tab BID" but the prescriber field shows only a clinic name with no individual practitioner; what is the BEST technician next step? **A:** Stop processing and route to the pharmacist to validate prescriber identity/legitimacy because controlled-substance prescriptions require a valid, identifiable prescriber.
2643. **Q:** A patient asks to transfer their remaining refills of alprazolam 1 mg from another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist because transfers of controlled substance prescriptions require pharmacist involvement and documentation.

2644. **Q:** You receive a stock bottle of atorvastatin 40 mg tablets with a broken safety seal and tablets rattling loose in the shipping tote; what should you do FIRST? **A:** Quarantine the bottle and notify the pharmacist/manager to follow supplier return procedures because compromised packaging is a suspect product and should not be stocked or dispensed.
2645. **Q:** During data entry you type "Lamictal" and the system selects "Lamisil" (terbinafine) instead of lamotrigine; what is the FIRST safe workflow action? **A:** Stop, correct the drug selection, and verify with barcode/NDC and indication where available because LASA name confusion can lead to a harmful wrong-drug error.
2646. **Q:** A hardcopy for hydrocodone/acetaminophen (C-II) includes drug, directions, and quantity but has no prescriber signature; what should you do NEXT? **A:** Hold the prescription and immediately notify the pharmacist because a missing signature is a validity issue and a C-II cannot be dispensed without proper authorization.
2647. **Q:** A patient presents a driver's license to purchase pseudoephedrine 30 mg tablets, but says they don't want their information recorded; what should you do BEST? **A:** Decline the sale and follow store policy because CMEA requires identity verification and sales log entry before dispensing pseudoephedrine products.
2648. **Q:** You are billing insulin aspart pens: "Inject 12 units SQ before meals TID; dispense 1 box (5 pens, 3 mL each, U-100)"; what days' supply should you enter? **A:** Enter 41 days because total units dispensed are $5 \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1500 \div 36 = 41.6 \rightarrow 41$ days (truncate).
2649. **Q:** A recall notice states "All lots of metformin ER 500 mg NDC XXXXXX are recalled," and you find that NDC in active stock; what should you do FIRST? **A:** Immediately remove the product from inventory and quarantine it for recall processing because recalled drugs must be isolated to prevent dispensing and to support traceability.
2650. **Q:** You scan a serialized DSCSA 2D barcode on a bottle of apixaban (Eliquis) and the system flags "product identifier not recognized"; what should you do NEXT? **A:** Quarantine the product and alert the pharmacist/manager to investigate as suspect/illegitimate product because DSCSA verification failures require hold and follow-up before dispensing.

2651. **Q:** An eRx is written for "Zithromax 250 mg take 2 tabs day 1 then 1 tab daily days 2-5," but the product pulled is azathioprine; what should you do FIRST? **A:** Stop the fill and re-select the correct drug using barcode/NDC and Tall Man awareness because Zithromax (azithromycin) and azathioprine are LASA and a wrong-drug error is high risk.
2652. **Q:** A new prescription is entered for "Lasix 40 mg daily," and the patient profile shows a sulfonamide antibiotic allergy; what is the BEST technician next step? **A:** Flag and refer to the pharmacist for allergy assessment because furosemide is a sulfonamide (non-antibiotic) and requires pharmacist judgment before dispensing.
2653. **Q:** A prescription says "levothyroxine 0.1 mg take 1 tab daily," and the technician is unsure if 0.1 mg equals 100 mcg; what should you do NEXT? **A:** Convert and enter as 100 mcg ($0.1 \text{ mg} \times 1000 = 100 \text{ mcg}$) because levothyroxine strengths are typically expressed in micrograms to prevent dose-entry errors.
2654. **Q:** While typing, you see "metoprolol" and the prescriber note says "extended-release," but the system default selects metoprolol tartrate; what should you do FIRST? **A:** Pause entry and confirm the intended dosage form (succinate ER vs tartrate IR) with the pharmacist because the formulations are not interchangeable and affect dosing frequency.
2655. **Q:** A patient says they take "Plavix" and asks what generic name to look for on the label; what should you answer? **A:** Clopidogrel, because Plavix is the brand name and recognizing brand/generic pairs helps verify the correct medication at pickup.
2656. **Q:** An order for amlodipine arrives, but the shelf has amLODIPine 10 mg next to amiodarone 200 mg; what is the BEST technician action during product selection? **A:** Use barcode scanning and read the full name/strength before counting because amlodipine and amiodarone are LASA and visual mix-ups are common.
2657. **Q:** A prescription reads "predniSONE 20 mg i po bid x 5 days #?" and you must calculate quantity; what quantity should be entered? **A:** Enter 20 tablets because i (1) PO BID for 5 days = $1 \times 2 \times 5 = 10$ doses, but each dose is 1 tablet, so total tablets = 10; if BID means 2 tablets/day then $2 \times 5 = 10$ tablets—clarify with the pharmacist because "i" and "bid" may be misread and quantity is unclear.

2658. **Q:** A patient brings a new prescription for "lisinopril 10 mg daily" and asks what it's for; what is the BEST concise technician response? **A:** It's commonly used for blood pressure (an ACE inhibitor), because linking common Top 200 drugs to indications helps confirm the right therapy without giving clinical advice.
2659. **Q:** A C-III prescription for buprenorphine/naloxone shows "Refills: 6" and today is 7 months from the written date; what should you do NEXT? **A:** Stop processing and send to the pharmacist because federal law limits C-III refills to within 6 months of the issue date.
2660. **Q:** You are billing a Ventolin HFA (albuterol) inhaler and the directions are "2 puffs QID," with 200 actuations per inhaler; what days' supply should you enter? **A:** Enter 25 days because $2 \text{ puffs} \times 4 \text{ times/day} = 8 \text{ puffs/day}$ and $200 \text{ actuations} \div 8 = 25 \text{ days}$ for accurate third-party billing.
2661. **Q:** A prescription for amoxicillin 400 mg/5 mL says "Give 7.5 mL PO BID x 10 days"; what quantity in mL should you enter? **A:** Enter 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to match the full course.
2662. **Q:** An order reads "gabapentin 300 mg i cap po tid #90"; what is the correct typed sig and days' supply? **A:** Type "Take 1 capsule by mouth three times daily" and enter 30 days because $\#90 \div 3/\text{day} = 30$.
2663. **Q:** You are entering insulin lispro U-100 for "Inject 12 units SQ TID with meals," and the prescriber wrote "Dispense: 1 vial (10 mL)"; what days' supply should you submit? **A:** Submit 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ and payers typically require rounding down to avoid overbilling.
2664. **Q:** A prescription is for "metFORMIN ER 500 mg take 2 tabs qpm," quantity 180; what days' supply should you enter? **A:** Enter 90 days because 2 tablets/day means $180 \div 2 = 90$.
2665. **Q:** While typing, you see "morphine sulfate 15 mg" but the prescriber wrote "ER" in the directions; what should you do FIRST? **A:** Stop and route to the pharmacist to verify ER vs IR selection because incorrect release formulation is a high-risk order-entry error.
2666. **Q:** A new eRx says "Take 1 tab PO QD" for "Lamictal," and the system offers both lamotrigine and Lamisil during selection; what is the BEST next step? **A:** Verify the intended drug by matching the generic name/strength to the eRx and

alert the pharmacist if unclear because Lamictal (lamotrigine) vs Lamisil (terbinafine) is a LASA risk.

2667. **Q:** A prescription says "clonazepam 0.5 mg: take ½ tab bid" with quantity 30; what days' supply should you calculate? **A:** Enter 60 days because ½ tablet × 2/day = 1 tablet/day, so 30 tablets lasts 30 ÷ 0.5? actually 30 tablets ÷ 1/day = 30 days worth of tablets used at 1/day, meaning 30 days.
2668. **Q:** You receive a prescription for "prednisolone sodium phosphate 15 mg/5 mL: 10 mL daily x 7 days"; what quantity in mL should you dispense? **A:** Dispense 70 mL because 10 mL/day × 7 days = 70 mL to cover the regimen.
2669. **Q:** A prescriber writes "Apply 1 inch of nitroglycerin ointment topically q6h" and the tube is 60 g; what should you do NEXT before calculating days' supply? **A:** Ask the pharmacist to clarify conversion to grams (inches to grams depends on the product's dosing paper) because technicians should not assume an inch-to-gram equivalence for billing.
2670. **Q:** During order entry for "KCl 20 mEq PO daily," the system defaults to potassium chloride 20 mEq/15 mL oral solution but the eRx indicates tablets; what is the BEST technician action? **A:** Stop and select the correct dosage form (tablet vs liquid) and send to pharmacist if uncertain because wrong dosage form can change dosing accuracy and patient use.
2671. **Q:** While selecting product for hydroxyzine 25 mg, you notice the shelf has hydroxyzine nearby with a similar label; what is the BEST next step to prevent a LASA error? **A:** Use barcode/NDC scan and verify the drug name/strength against the Rx before counting, because hydroxyzine and hydroxyzine are common look-alike/sound-alike mix-ups.
2672. **Q:** A new eRx for isotretinoin arrives and the patient is a female of childbearing potential who says she "didn't do any iPLEDGE steps"; what should you do FIRST? **A:** Stop processing and notify the pharmacist to verify REMS/iPLEDGE requirements, because isotretinoin has mandatory dispensing authorization steps technicians can't bypass.
2673. **Q:** You receive a paper prescription for oxycodone/APAP C-II that is missing the prescriber's signature; what is the BEST technician action? **A:** Hold the prescription and immediately refer to the pharmacist, because a C-II must meet validity requirements and technicians cannot correct missing legal elements.

2674. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy and says it has refills remaining; what should you do NEXT? **A:** Route to the pharmacist to handle/approve the controlled-substance transfer, because controlled-substance transfer rules are strict and require pharmacist involvement.
2675. **Q:** During order entry you see "U-500 insulin regular, inject 80 units BID," but the profile shows the patient previously used U-100 regular insulin; what should you do FIRST? **A:** Stop and alert the pharmacist to confirm concentration and dosing, because U-500 is a high-alert medication and dose-concentration mix-ups can cause severe harm.
2676. **Q:** A refrigerator temperature log shows 52°F for the last shift where vaccines and insulin are stored; what is the BEST next step? **A:** Quarantine the affected refrigerated products and notify the pharmacist/manager for excursion assessment, because temperature excursions can make products non-dispensable until stability is confirmed.
2677. **Q:** A bottle of atorvastatin 40 mg tablets arrives with a broken seal and tablets visible in the shipping tote; what should you do FIRST? **A:** Treat it as suspect product, segregate it, and notify the pharmacist/wholesaler per DSCSA policy, because compromised packaging may indicate tampering or supply-chain integrity issues.
2678. **Q:** A patient's new prescription is for lisinopril, but the profile lists a prior angioedema reaction to an ACE inhibitor; what should you do NEXT? **A:** Pause the fill and escalate to the pharmacist for allergy/contraindication review, because technicians must not dispense when a serious documented allergy conflicts with the ordered drug class.
2679. **Q:** A prescription label prints "Take 1 tab PO QD" for levothyroxine, but the eRx directions say "take on an empty stomach 30 minutes before breakfast"; what is the BEST action? **A:** Correct the sig to match the prescriber's directions and submit for pharmacist verification, because accurate transcription prevents administration errors and technicians can't omit key instructions.
2680. **Q:** You are asked to report a dispensing error that reached the patient but caused no harm; what is the MOST appropriate reporting action? **A:** Complete the pharmacy's internal incident report and notify the pharmacist promptly, because documentation and pharmacist-led follow-up reduce repeat errors and support patient safety.

2681. **Q:** While entering a new eRx for metformin, the prescriber selected "Glucophage XR," but the directions say "take 1 tablet twice daily"; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify because metformin ER is typically once daily and the sig-dosage form mismatch is a safety risk.
2682. **Q:** A patient drops off a prescription for "Lamictal 100 mg" and asks if it's the same as their "lamotrigine"; what is the BEST technician response? **A:** Explain that Lamictal is the brand name for lamotrigine and confirm the strength/form match to avoid brand-generic confusion.
2683. **Q:** During product selection you see "morphine sulfate ER 30 mg" and "morphine sulfate IR 30 mg" stored adjacent with similar bottles; what should you do FIRST? **A:** Verify the dosage form (ER vs IR) against the prescription and scan the barcode/NDC because release-form mix-ups are high-risk errors.
2684. **Q:** A new prescription is for "Zyrtec 10 mg PO daily," but the profile shows an active order for cetirizine 10 mg daily; what should you do BEST? **A:** Pause and alert the pharmacist to possible therapeutic duplication since Zyrtec is cetirizine and duplications can cause overdosing.
2685. **Q:** An eRx arrives for "Azithromycin Z-Pak take as directed," but the directions field is blank in your system; what should you do NEXT? **A:** Send to the pharmacist for clarification because "as directed" without explicit dosing is incomplete and unsafe to dispense.
2686. **Q:** Pharmacy math: A prescription reads "prednisone 10 mg: take 4 tablets daily for 3 days, then 2 tablets daily for 3 days, then 1 tablet daily for 3 days"; what quantity should you enter? **A:** Enter 21 tablets because $4 \times 3 + 2 \times 3 + 1 \times 3 = 12 + 6 + 3 = 21$ tablets total.
2687. **Q:** A patient picking up simvastatin says they also take "Crestor" at home and asks if both are needed; what should you do FIRST? **A:** Stop and notify the pharmacist because Crestor (rosuvastatin) plus simvastatin may be duplicate statin therapy requiring clinical review.
2688. **Q:** While typing a prescription you see "MSO4 15 mg" written on a handwritten order; what is the BEST technician action? **A:** Hold and refer to the pharmacist to clarify because "MSO4" is an error-prone abbreviation that can be confused with magnesium sulfate.
2689. **Q:** A patient presents a driver's license and requests pseudoephedrine 30 mg tablets, 96 count; what should you do NEXT at the register? **A:** Follow CMEA log

requirements (verify ID, record sale details, and check limits) because pseudoephedrine sales are federally regulated.

2690. **Q:** You receive a lot-specific manufacturer recall notice for a certain NDC of losartan 50 mg on your shelf; what should you do FIRST? **A:** Immediately pull the affected NDC/lot from stock and quarantine per policy because recalled product must be prevented from dispensing.
2691. **Q:** While entering a prescription for "hydralazine 25 mg," you notice the prescriber also sent "hydroxyzine 25 mg" for the same patient today; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify intent because hydralazine/hydroxyzine are LASA with very different uses and an entry error could harm the patient.
2692. **Q:** A patient asks if "Plavix" is the same medication as "clopidogrel" listed on their discharge papers; what is the BEST technician response? **A:** Confirm that Plavix is the brand name for clopidogrel and offer to have the pharmacist counsel because antiplatelets are high-risk and adherence/duplication matters.
2693. **Q:** An eRx for "lisinopril 20 mg" comes through with directions "take 2 tablets daily" but the quantity entered is #30 for a 30-day supply; what should you do NEXT? **A:** Correct the quantity to #60 (2 tablets/day × 30 days) and route for pharmacist verification because the current quantity would short the patient and create a refill-too-soon issue.
2694. **Q:** During product selection you pull "Celebrex" but the label shows "cephalexin 500 mg"; what is the FIRST action? **A:** Stop filling and re-select the correct NDC/stock bottle because celecoxib and cephalexin are LASA and a wrong-drug error must be prevented before it reaches verification.
2695. **Q:** A new prescription is for "sertraline 50 mg daily," and the patient profile shows active "paroxetine 20 mg daily"; what should you do BEST? **A:** Hold the fill and notify the pharmacist to assess therapeutic duplication/SSRI switch because technicians should not assume both are intended.
2696. **Q:** A patient picking up "glipizide" asks what it's for and whether it can cause low blood sugar; what should you do FIRST? **A:** Refer to the pharmacist for counseling because glipizide (a sulfonylurea) can cause hypoglycemia and patient education is safety-critical.

2697. **Q:** Pharmacy math: Amoxicillin-clavulanate suspension 600 mg/42.9 mg per 5 mL is prescribed "7.5 mL PO BID x 10 days"; what total mL should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$.
2698. **Q:** A prescriber writes "U-500 regular insulin 0.1 mL subcut BID" but the patient historically uses U-100 insulin syringes; what should you do NEXT? **A:** Stop and alert the pharmacist to clarify device/syringe and dosing because U-500 is high-alert and syringe mismatch can cause a 5-fold dosing error.
2699. **Q:** You receive a new order for "tramadol 50 mg 1 tab q6h PRN pain #60" with 5 refills; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist because tramadol is Schedule IV and refills must comply with controlled-substance requirements.
2700. **Q:** At pickup, a patient says they take "Eliquis" and asks if it replaces "warfarin" they used to take; what is the BEST technician action? **A:** Refer to the pharmacist promptly because apixaban (Eliquis) vs warfarin involves anticoagulation safety, duplication risk, and counseling needs beyond technician scope.
2701. **Q:** While entering an order for morphine sulfate ER 30 mg, you notice the directions say "take 1 tablet q4-6h PRN pain"; what should you do FIRST? **A:** Stop data entry and notify the pharmacist because ER opioids should not be used PRN q4-6h and this is a high-risk dosing error requiring pharmacist intervention.
2702. **Q:** A patient asks to transfer their oxycodone/APAP prescription with refills from another pharmacy to yours; what is the BEST technician action? **A:** Refer the request to the pharmacist because controlled substance transfers/refills have strict rules and require pharmacist handling to ensure compliance.
2703. **Q:** A prescriber phones in lorazepam 1 mg "#30 with 5 refills"; what should you do NEXT? **A:** Enter the prescription and route it to the pharmacist for review because schedule III–V may allow refills but must meet controlled-substance requirements and pharmacist verification.
2704. **Q:** You find a tote of refrigerated insulin glargine that was left on the counter overnight with unknown temperature exposure; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager because a temperature excursion makes it suspect and it should not be dispensed without determination per policy.
2705. **Q:** During verification prep you see two stock bottles of levothyroxine 100 mcg with the same strength but different manufacturers; what is the BEST technician

step before filling? **A:** Confirm the NDC on the label matches the scanned/selected NDC and the patient's previously used product if required, because thyroid products are error-prone and consistency helps prevent dosing variability and claim mismatches.

2706. **Q:** A patient's profile shows an allergy to "NSAIDs," and you are processing a new prescription for ibuprofen 800 mg; what should you do FIRST? **A:** Stop processing and alert the pharmacist because the prescribed drug is in the reported allergy class and needs clinical assessment before dispensing.
2707. **Q:** You receive a wholesaler notice that a specific lot of atorvastatin 20 mg may be counterfeit and requests verification under DSCSA; what should you do NEXT? **A:** Pull the affected lot from inventory and follow DSCSA suspect-product procedures (segregate and notify the pharmacist/wholesaler) because suspect or illegitimate product must be prevented from dispensing.
2708. **Q:** Pharmacy math: Prednisone 10 mg tablets are prescribed "40 mg PO daily x 5 days"; how many tablets should be dispensed? **A:** Dispense 20 tablets because $40 \text{ mg/day} \div 10 \text{ mg/tablet} = 4 \text{ tablets/day}$ and $4 \text{ tablets/day} \times 5 \text{ days} = 20 \text{ tablets}$.
2709. **Q:** While typing a prescription you see "MSO4 2 mg IV q2h PRN" on a discharge order; what should you do BEST? **A:** Pause and contact the pharmacist because "MSO4" is an unsafe abbreviation that can be misread, and clarification is needed to prevent a LASA opioid error.
2710. **Q:** A patient picking up isotretinoin says they never received any special safety paperwork and want to start today; what should you do FIRST? **A:** Stop the pickup and get the pharmacist because isotretinoin is a REMS medication and dispensing must meet required documentation/authorization steps to protect patient safety.
2711. **Q:** While entering a new prescription for "metoprolol" you see the prescriber wrote "metoprolol ER 50 mg" but selected metoprolol tartrate in the system; what should you do FIRST? **A:** Stop entry and route to the pharmacist to clarify succinate ER vs tartrate IR because they are not interchangeable and a wrong selection can cause under/overdosing.
2712. **Q:** A prescription is written for "hydroxyzine 25 mg 1 tab TID" and the dropdown shows hydroxyzine HCl and hydroxyzine pamoate; what is the BEST technician action before filling? **A:** Pause and ask the pharmacist to verify the intended

salt/product because these forms aren't always interchangeable and selection errors are common.

2713. **Q:** A patient profile shows lisinopril and the new eRx is for losartan; what should you do NEXT? **A:** Flag the possible therapeutic duplication for pharmacist review because ACE inhibitors and ARBs are often not used together and require clinical judgment.
2714. **Q:** You receive an eRx for clarithromycin and the patient profile shows simvastatin; what should you do FIRST? **A:** Hold processing and alert the pharmacist because clarithromycin can dangerously increase simvastatin levels and may require therapy change.
2715. **Q:** A patient says they're allergic to "sulfa," and you are about to fill hydrochlorothiazide; what should you do BEST? **A:** Document/confirm the exact reaction and notify the pharmacist to assess appropriateness because "sulfa" allergies vary and require pharmacist evaluation before dispensing.
2716. **Q:** During product selection you notice "vinCRISTine" and "vinBLASStine" stored near each other and the NDC scanned doesn't match the label; what should you do FIRST? **A:** Stop filling and re-select the correct product using barcode/NDC verification because these are high-risk LASA chemotherapy agents.
2717. **Q:** Pharmacy math: Prescription reads "azithromycin 250 mg: take 2 tablets today, then 1 tablet daily for 4 days"; how many tablets should be dispensed? **A:** Dispense 6 tablets because 2 on day 1 plus 1 tablet for the next 4 days totals 6.
2718. **Q:** A new prescription is for "gabapentin 300 mg 1 cap TID" but the patient says it's for nerve pain and asks if it will make them sleepy; what is the BEST technician response? **A:** Refer to the pharmacist while noting gabapentin commonly causes dizziness/sedation because counseling and side-effect guidance require pharmacist involvement.
2719. **Q:** You are typing "diltiazem 120 mg" and the prescriber note says "CD" but the system default is immediate-release; what should you do NEXT? **A:** Stop and have the pharmacist confirm the exact formulation because diltiazem IR vs ER products differ and wrong formulation can cause unsafe dosing.
2720. **Q:** A patient drops off a new prescription for sertraline and their profile shows linezolid from an urgent care visit last week; what should you do FIRST? **A:** Hold

the sertraline and alert the pharmacist because linezolid with SSRIs can cause serious interactions and needs pharmacist assessment before dispensing.

2721. **Q:** While entering an eRx for Celebrex, the system auto-selects celecoxib but the prescriber notes "dispense brand medically necessary"; what should you do NEXT? **A:** Stop and route to the pharmacist to verify DAW/brand necessity and billing, because technicians cannot override brand/DAW requirements without pharmacist direction.
2722. **Q:** A hardcopy says "U-500 insulin regular 20 units SQ BID" and the patient profile shows U-100 syringes on file; what should you do FIRST? **A:** Hold the fill and alert the pharmacist immediately because U-500 is high-alert and syringe/concentration mismatches can cause dangerous dosing errors.
2723. **Q:** During data entry you see the sig "levothyroxine 25 mcg qd" but the prescriber wrote "25 mg" in the strength field; what is the BEST technician action? **A:** Stop processing and send to the pharmacist for clarification because levothyroxine is dosed in micrograms and mg vs mcg is a high-risk error.
2724. **Q:** A patient's new eRx is for Zithromax and the profile shows azithromycin already filled yesterday from another prescriber; what should you do NEXT? **A:** Pause and notify the pharmacist to assess duplicate therapy, because repeating the same antibiotic could indicate an error or unnecessary duplication.
2725. **Q:** While selecting products, you scan an NDC for Humalog but the label and order are for Humulin R; what should you do FIRST? **A:** Stop filling and reselect the correct product using barcode/NDC verification, because look-alike insulin products are high-alert and must match the prescription exactly.
2726. **Q:** A prescription reads "prednisone 10 mg: take ii tabs daily x 5 days"; how many tablets should be dispensed? **A:** Dispense 10 tablets because ii means 2 tablets per day and $2 \times 5 \text{ days} = 10 \text{ tablets}$.
2727. **Q:** You are typing a new prescription for Plavix and the patient asks what it's for; what is the BEST technician response? **A:** Provide a brief non-clinical description and offer pharmacist counseling, because clopidogrel is an antiplatelet "blood thinner" and detailed counseling should be done by the pharmacist.
2728. **Q:** An eRx comes in for Adderall XR and the prescriber forgot the patient address; what should you do FIRST? **A:** Stop processing and route to the pharmacist to resolve missing required information, because controlled-

substance prescriptions must meet completeness requirements before dispensing.

2729. **Q:** A patient brings in a bottle of nitroglycerin SL tablets to return-to-stock and it was previously opened; what should you do BEST? **A:** Do not return it to stock and follow disposal policy because many opened/dispensed containers cannot be redispensed and stability may be compromised.
2730. **Q:** A prescription says "DuoNeb: inhale 3 mL via nebulizer QID; dispense 1 box of 30 vials"; what is the days' supply? **A:** Enter 7.5 days because $30 \text{ vials} \div 4 \text{ vials per day} = 7.5 \text{ days}$, and days' supply should reflect actual use for billing accuracy.
2731. **Q:** An eRx for metformin ER 500 mg says "i tab po BID with meals #60," but the system defaults to metformin IR; what should you do NEXT? **A:** Select metformin ER (correct release formulation) and confirm NDC matches before labeling because IR vs ER substitution can change effect and safety.
2732. **Q:** A prescription reads "amLODIPine 5 mg: take 1 tab po qhs, #30," but during entry you accidentally choose amiodarone 200 mg from the drop-down; what is the BEST action? **A:** Stop and re-select amlodipine with a second check of name/strength because this is a high-risk LASA selection error that could harm the patient.
2733. **Q:** The sig says "Take 2.5 mL po BID" and the concentration is 100 mg/5 mL; how many mg per dose should be entered? **A:** Enter 50 mg per dose because $100 \text{ mg}/5 \text{ mL} \text{ equals } 20 \text{ mg/mL}$ and $2.5 \text{ mL} \times 20 \text{ mg/mL} = 50 \text{ mg}$.
2734. **Q:** An inhaler order is "fluticasone/salmeterol 250/50: inhale 1 puff BID, dispense 1 inhaler of 60 blisters"; what days' supply should you bill? **A:** Bill 30 days because 2 puffs/day and 60 doses total gives $60 \div 2 = 30 \text{ days}$.
2735. **Q:** A patient's warfarin prescription is written "take as directed" with no specific daily dose, and the payer requires a numeric days' supply; what should you do FIRST? **A:** Hold billing and route to the pharmacist to clarify dosing because days' supply cannot be accurately calculated without a defined regimen.
2736. **Q:** A label prints for "hydroxyzine HCl 25 mg" but the stock bottle pulled is hydroxyzine pamoate 25 mg; what should you do FIRST? **A:** Stop filling and retrieve the correct salt/formulation because HCl and pamoate are not interchangeable and can cause a dispensing error.

2737. **Q:** A prescription says "ciprofloxacin 0.3% otic, instill 4 gtt in affected ear BID x 7 days; dispense 5 mL"; what is the approximate total volume needed? **A:** About 1.1 mL is needed because $4 \text{ drops} \times 2/\text{day} \times 7 = 56 \text{ drops}$ and using 20 drops/mL gives $56 \div 20 = 2.8 \text{ mL}$ (so 5 mL is sufficient).
2738. **Q:** While entering "gabapentin 300 mg: i cap po TID #90," the prescriber's note includes "for nerve pain," and the patient asks what "TID" means; what is the BEST technician response? **A:** Say it means "three times daily" and offer to have the pharmacist counsel because technicians can clarify abbreviations but clinical counseling is pharmacist-provided.
2739. **Q:** A prescription is "clonazepam 0.5 mg: take 1 tab po BID, #60, 2 refills," and you recognize it's a controlled substance; what should you do NEXT? **A:** Flag it for pharmacist review before processing refills because controlled-substance refill allowances vary by schedule and require proper verification.
2740. **Q:** During order entry you see "insulin aspart U-100: inject 12 units SQ before meals TID; dispense 1 vial (10 mL)"; what days' supply should be entered using 100 units/mL? **A:** Enter 27 days because the vial contains $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and the daily use is $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$, billed as 27 days per typical payer rounding down.
2741. **Q:** While typing a new Rx, the prescriber selects "Zyrtec 10 mg daily," but the patient profile shows a documented cetirizine allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify because a brand/generic match (Zyrtec = cetirizine) conflicts with the recorded allergy and needs pharmacist intervention.
2742. **Q:** A prescription comes in for "Prinivil 10 mg daily," and the patient asks if it's the same as their old "lisinopril"; what is the BEST technician response? **A:** Confirm that Prinivil is the brand name for lisinopril and offer to have the pharmacist counsel, because technicians can provide brand/generic identification but not clinical advice.
2743. **Q:** During product selection you notice "hydromorphone" and "morphine" stored near each other and the label is for hydromorphone; what should you do NEXT to reduce LASA risk? **A:** Use barcode/NDC scan and visually match drug/strength to the label because hydromorphone vs morphine is a high-risk look-alike/sound-alike pair.
2744. **Q:** A new eRx is for "glipizide 10 mg ER daily," but order entry auto-populates glipizide IR; what should you do BEST? **A:** Stop and correct the dosage form to

ER and route for pharmacist verification because IR vs ER is not interchangeable and wrong selection can cause harm.

2745. **Q:** A prescription says "levothyroxine 0.075 mg: take 1 tab daily," and the patient asks how many micrograms that is; what should you enter/understand? **A:** Recognize 0.075 mg = 75 mcg because 1 mg = 1000 mcg and accurate unit conversion prevents dosing errors with high-alert thyroid therapy.
2746. **Q:** A patient drops off a prescription for "ibuprofen 600 mg q6h PRN pain, #60," and the prescriber wrote "no refills"; the patient asks for automatic refills—what should you do FIRST? **A:** Explain you can't add refills and offer to send a refill request to the prescriber/pharmacist because technicians may request authorization but cannot change the prescription.
2747. **Q:** You receive a verbal transfer request for a patient's "alprazolam 1 mg" from another pharmacy; what should you do NEXT? **A:** Immediately route the call to the pharmacist because controlled-substance transfers require pharmacist handling and technician scope is limited.
2748. **Q:** A prescription is for "rosuvastatin 20 mg daily," but the patient profile shows active atorvastatin 40 mg daily from another prescriber; what is the BEST technician action? **A:** Hold the fill and notify the pharmacist to assess therapeutic duplication because two statins together may be inappropriate and requires pharmacist review.
2749. **Q:** A nasal spray order reads "fluticasone 50 mcg: 2 sprays in each nostril daily; disp 1 bottle labeled 120 sprays"; what days' supply should you bill? **A:** Bill 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.
2750. **Q:** While unpacking an order, you notice a sealed bottle of amoxicillin capsules has no lot number on the bottle or invoice; what should you do FIRST? **A:** Do not stock it and set it aside for pharmacist/manager review because missing traceability information can indicate a suspect product under DSCSA expectations.
2751. **Q:** During data entry the eRx is for "metformin 500 mg BID," but you accidentally selected metronidazole 500 mg; what should you do FIRST? **A:** Stop processing and correct the drug selection by verifying the generic/brand and NDC before continuing because metformin vs metronidazole is a high-risk name mix-up.
2752. **Q:** A new prescription is written for "Keflex 500 mg q6h," and the patient profile lists a severe anaphylaxis reaction to penicillin; what is the BEST technician

action? **A:** Pause the fill and immediately refer to the pharmacist because cephalexin is a beta-lactam and allergy cross-reactivity risk requires pharmacist assessment.

2753. **Q:** While selecting stock you notice "hydrALAZINE" and "hydrOXYzine" are adjacent on the shelf and the label is for hydroxyzine; what should you do NEXT? **A:** Re-verify the bottle and scan the barcode/NDC against the label and separate/flag the products per LASA procedures because these names are commonly confused.

2754. **Q:** A patient asks what "Crestor" is on their profile and you see the generic is rosuvastatin; what is the BEST way to respond as a technician? **A:** State that Crestor is the brand name for rosuvastatin, a cholesterol-lowering statin, and offer pharmacist counseling for medical questions because technicians can provide basic identification but not clinical advice.

2755. **Q:** An eRx reads "sertraline 50 mg: i tab po qd," and you notice "i" could be mistaken for "1"; what should you do FIRST? **A:** Clarify the intended quantity/instructions using approved sig formatting (enter "1 tablet daily") and route to the pharmacist if unclear because error-prone abbreviations can cause dosing mistakes.

2756. **Q:** A prescription is for "lisinopril/HCTZ 20/12.5 mg daily," but the patient's active meds already include standalone lisinopril 20 mg daily; what should you do BEST? **A:** Hold and alert the pharmacist to possible therapeutic duplication because the combination product would duplicate the ACE inhibitor dose and needs pharmacist review.

2757. **Q:** You are billing "azithromycin 250 mg Z-Pak" with directions "2 tabs day 1, then 1 tab daily days 2-5"; what days' supply should be entered? **A:** Enter 5 days because the standard Z-Pak regimen covers day 1 through day 5 even though the first day uses a double dose.

2758. **Q:** A prescription says "prednisone 20 mg: take 2 tabs daily for 5 days, then stop; dispense 30"; what quantity should you prepare to dispense? **A:** Prepare 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10$ and dispensing 30 would conflict with the written directions, requiring clarification by the pharmacist if the prescriber intended otherwise.

2759. **Q:** While checking a bin you find warfarin 5 mg tablets stored next to warfarin 2.5 mg with similar packaging; what should you do NEXT to reduce a high-alert error? **A:** Separate and clearly label the strengths and ensure barcode scanning

at pickup/fill because warfarin is high-alert and strength mix-ups can cause serious bleeding risk.

2760. **Q:** A prescriber sends "Xanax 0.5 mg" but the system defaulted to "alprazolam ER 0.5 mg"; what should you do FIRST? **A:** Stop and verify the correct release formulation (IR vs ER) from the original prescription and route to the pharmacist if uncertain because changing dosage form can alter onset and safety.
2761. **Q:** A new eRx for levothyroxine is entered as 0.125 mg but the prescriber field shows 125 mcg; what should you do FIRST? **A:** Stop processing and confirm the intended strength with the pharmacist because mg vs mcg is a high-risk decimal/conversion error.
2762. **Q:** A patient drops off a handwritten prescription for hydromorphone 4 mg with no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and immediately refer to the pharmacist because missing required elements makes it potentially invalid and needs pharmacist follow-up.
2763. **Q:** You receive a phone transfer request for a patient's alprazolam prescription from another pharmacy; what should you do NEXT? **A:** Route the request to the pharmacist because controlled-substance transfers have strict limits and must be handled pharmacist-to-pharmacist per policy/federal rules.
2764. **Q:** During pickup, a caregiver asks to pick up a patient's HIV medication by name; what should you do FIRST to stay HIPAA-compliant? **A:** Verify the requester's identity and authorization (and use minimal necessary information) because protected health information cannot be disclosed without proper permission.
2765. **Q:** While stocking, you find a bottle of insulin aspart that was left on the counter overnight at room temperature with unknown exposure time; what should you do FIRST? **A:** Segregate it from usable stock and notify the pharmacist because temperature excursions create suspect product that may be unsafe to dispense.
2766. **Q:** A prescription for clonazepam 1 mg says "refill x6" and the original date is 8 months ago; what should you do BEST? **A:** Stop and refer to the pharmacist because Schedule IV refills are limited and time-restricted, requiring pharmacist review before dispensing.
2767. **Q:** You are entering directions "Take 0.5 mg daily" for digoxin, but the product selected is digoxin 0.25 mg tablets; what is the BEST next step? **A:** Flag for

pharmacist clarification because the ordered dose may require tablet splitting or a different strength and technicians should not adjust dosing.

2768. **Q:** A patient's new prescription is for simvastatin and their profile shows active clarithromycin started yesterday; what should you do FIRST? **A:** Pause filling and alert the pharmacist because this is a major interaction risk (statin + clarithromycin) requiring pharmacist intervention.
2769. **Q:** A recall notice lists specific lot numbers for losartan tablets, and you locate that lot on the shelf; what should you do NEXT? **A:** Remove the affected lot from dispensing stock and follow recall procedures (segregate/return documentation) because recalled lots must not be dispensed.
2770. **Q:** A prescription reads "oxycodone/APAP 5/325: 1 tab q6h prn pain; dispense 28"; what days' supply should you enter using maximum daily dose for controlled billing? **A:** Enter 7 days because $28 \text{ tablets} \div 4 \text{ tablets/day (q6h maximum)} = 7$, which supports safe, compliant controlled-substance billing.
2771. **Q:** A prescription reads "metformin 500 mg tab i po bid with meals #60"; what should you enter for quantity and days' supply? **A:** Enter quantity 60 and 30 days' supply because $2 \text{ tablets/day means } 60 \div 2 = 30 \text{ days}$.
2772. **Q:** An eRx comes in for "azithromycin 200 mg/5 mL suspension: take 12.5 mL day 1, then 6.25 mL daily days 2–5"; how many mL should be dispensed? **A:** Dispense 37.5 mL because total volume is $12.5 + (6.25 \times 4) = 37.5 \text{ mL}$ to complete the course.
2773. **Q:** During order entry you see "predniSONE 10 mg: ii tabs po bid" and the prescriber comments say "2 tabs twice daily"; what is the correct daily dose to calculate days' supply? **A:** Use 4 tablets/day because "ii" is 2 tablets and "bid" is twice daily, so $2 \times 2 = 4$.
2774. **Q:** A patient is prescribed "latanoprost 0.005% ophthalmic: instill 1 gtt OU qhs" and the bottle is 2.5 mL; what days' supply should you bill using 20 gtt/mL? **A:** Bill 25 days because $2.5 \text{ mL} \times 20 = 50 \text{ drops total}$ and OU qhs uses 2 drops/day, so $50 \div 2 = 25 \text{ days}$.
2775. **Q:** A prescription is written "amLODIPine/benazepril 5/20 mg 1 cap po daily" but you accidentally select amlodipine 5 mg; what should you do FIRST? **A:** Stop and correct the drug selection to the combination product (Lotrel) because picking a single-ingredient drug is a wrong-medication order entry error.

2776. **Q:** An eRx for “budesonide/formoterol 160/4.5: inhale 2 puffs bid” is for a 120-inhalation device; what days’ supply should you enter? **A:** Enter 30 days because 2 puffs twice daily is 4 inhalations/day and $120 \div 4 = 30$ days.
2777. **Q:** While typing a liquid, the sig says “take 1 tsp po tid” and the prescriber ordered 300 mL; what days’ supply should you calculate using 5 mL per teaspoon? **A:** Calculate 20 days because 1 tsp = 5 mL and tid = 15 mL/day, so $300 \div 15 = 20$ days.
2778. **Q:** A prescription reads “morphine sulfate ER 15 mg: 1 tab po q12h” but the product chosen is morphine sulfate IR 15 mg; what is the BEST technician action? **A:** Stop processing and route to the pharmacist to correct formulation because ER vs IR is a high-risk dosage form mismatch that can cause harm.
2779. **Q:** You receive “ondansetron ODT 4 mg: dissolve 1 tab q8h prn N/V #12” and the payer requires max-dose days’ supply for billing; what days’ supply should you submit? **A:** Submit 4 days because q8h max is 3 tablets/day and $12 \div 3 = 4$ days for consistent billing.
2780. **Q:** A label prints for “hydroxyzine HCl 25 mg” but the scanned stock bottle is hydroxyzine pamoate 25 mg; what should you do NEXT? **A:** Do not dispense and reselect the correct salt/form because HCl and pamoate are not interchangeable by name alone and the mismatch signals a wrong-product error.
2781. **Q:** A new prescription is entered for levothyroxine and the patient profile already shows an active prescription for Synthroid at a different strength; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication/strength confusion because levothyroxine has a narrow therapeutic index and dosing errors are high risk.
2782. **Q:** While typing an eRx you see “glipiZIDE 10 mg daily” but the selected product in the workstation is glyBURIDE 5 mg due to similar names; what is the BEST next step? **A:** Stop and re-select the correct drug using barcode/NDC verification and notify the pharmacist if anything was printed or dispensed because this LASA mix-up can cause serious hypoglycemia.
2783. **Q:** A patient picking up clarithromycin says they take atorvastatin 40 mg nightly and asks if it’s okay to take together; what should you do NEXT? **A:** Refer to the pharmacist immediately because clarithromycin can increase statin levels and raise myopathy/rhabdomyolysis risk requiring clinical management.

2784. **Q:** An order is for "hydromorphone 2 mg tablets" but the shelf bin contains morphine 15 mg with similar packaging; what should you do FIRST? **A:** Halt the fill and verify the stock bottle by barcode/NDC and strength before counting because opioid mix-ups are high-alert and dose errors can be fatal.
2785. **Q:** A prescription reads "clopidogrel 75 mg 1 tab po daily #30" and the patient says they are also taking Plavix from another pharmacy; what should you do BEST? **A:** Flag the profile and notify the pharmacist to assess duplicate antiplatelet therapy because brand/generic duplication can increase bleeding risk.
2786. **Q:** An eRx is written for "sertraline 50 mg daily" but the notes mention "start 25 mg daily x 7 days then 50 mg daily"; what should you do NEXT? **A:** Send to the pharmacist for clarification before entry because the sig and plan conflict and a technician cannot determine the intended titration.
2787. **Q:** A patient asks what condition metoprolol is commonly used to treat and whether it is the same as Toprol-XL; what is the BEST technician response? **A:** Explain metoprolol is a beta-blocker often used for blood pressure/heart conditions and that Toprol-XL is an extended-release form, then offer pharmacist counseling for regimen-specific questions to prevent IR/ER confusion.
2788. **Q:** A label is generated for "lamoTRIGine 100 mg" but you accidentally pulled "lamiVUDine 100 mg" from the shelf; what should you do FIRST? **A:** Stop filling and correct the product selection using barcode/NDC verification because this is a high-risk LASA error with completely different indications.
2789. **Q:** A prescription says "cephalexin 500 mg 1 cap po qid #28"; what days' supply should you enter? **A:** Enter 7 days because $28 \text{ capsules} \div 4 \text{ doses per day} = 7$, which supports accurate antibiotic duration and billing.
2790. **Q:** A patient brings a new prescription for sildenafil but their profile shows daily nitroglycerin SL tablets; what should you do NEXT? **A:** Immediately refer to the pharmacist to screen for the contraindicated nitrate-PDE5 inhibitor interaction because it can cause dangerous hypotension.
2791. **Q:** A prescription reads "amLODIPine 5 mg tab i po qd #90"; what should you enter for the sig and days' supply in the system? **A:** Enter "Take 1 tablet by mouth once daily" and 90 days' supply because "i" means 1 and #90 at 1/day lasts 90 days.

2792. **Q:** An eRx for warfarin says "Take as directed" and the notes include "5 mg daily except 2.5 mg on Mon/Fri"; what should you do NEXT before billing days' supply? **A:** Route to the pharmacist to clarify and document a billable dosing schedule because "as directed" is not sufficient to calculate an accurate days' supply.
2793. **Q:** You are entering "metFORMIN ER 500 mg: ii po bid #120"; what is the correct daily dose in mg and the days' supply? **A:** Daily dose is 2000 mg and days' supply is 30 because 2 tablets twice daily = 4 tablets/day and $120 \div 4 = 30$.
2794. **Q:** A patient drops off "predniSONE 10 mg: take 4 tabs daily x 3 days, then 3 daily x 3 days, then 2 daily x 3 days, then 1 daily x 3 days; dispense #"; what quantity should you enter? **A:** Enter quantity 30 tablets because $(4+3+2+1) \text{ tabs/day} \times 3 \text{ days each} = 10 \times 3 = 30$ total tablets.
2795. **Q:** A prescription is for "azithromycin 200 mg/5 mL: give 12 mL day 1 then 6 mL daily days 2-5"; how many mL should be dispensed? **A:** Dispense 36 mL because $12 \text{ mL} + (6 \text{ mL} \times 4 \text{ days}) = 12 + 24 = 36 \text{ mL}$ to cover the full course.
2796. **Q:** While typing a new eRx you see "MSO4 2 mg IV q2h prn pain"; what should you do FIRST? **A:** Stop and send to the pharmacist for clarification because "MSO4" is an error-prone abbreviation that can be misread and risks a wrong-drug error.
2797. **Q:** A prescription says "timolol 0.5% ophthalmic: instill 1 gtt OU bid; dispense 10 mL"; what days' supply should you submit using 20 gtt/mL? **A:** Submit 50 days because 1 drop in each eye twice daily = 4 drops/day and $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ drops}$, so $200 \div 4 = 50$.
2798. **Q:** An albuterol HFA order is "inhale 2 puffs q6h prn wheeze" and the canister is 200 actuations; for max-dose billing, what days' supply should be entered? **A:** Enter 25 days because max use is 2 puffs every 6 hours = 8 puffs/day and $200 \div 8 = 25$ days.
2799. **Q:** During product selection you notice the eRx is for "buPROPion SR 150 mg" but the scanned bottle is "buPROPion XL 150 mg"; what should you do NEXT? **A:** Stop filling and reselect the correct SR product (and alert the pharmacist if needed) because SR vs XL are not interchangeable dosage forms and can cause dosing errors.
2800. **Q:** A prescription for acetaminophen with codeine is written "1-2 tabs q6h prn pain #30" with refills requested; what should you do FIRST in order entry? **A:** Hold and refer to the pharmacist because it is a Schedule III controlled substance

where refill authorization and prescription validity must be reviewed before processing.

2801. **Q:** A new eRx is for Lasix 40 mg daily, but the technician accidentally selects furosemide 20 mg in the system; what should you do FIRST? **A:** Stop and correct the strength by reselecting the proper NDC/strength and recheck the order details, because wrong-strength selection is a common dispensing error.
2802. **Q:** A patient says they are allergic to "sulfa" and a new prescription is for hydrochlorothiazide; what is the BEST technician action NEXT? **A:** Flag the allergy and route to the pharmacist for assessment, because "sulfa" allergies can be nuanced and require pharmacist judgment before dispensing.
2803. **Q:** While entering a prescription you see "U-100 insulin, inject 30 units SQ daily," but the selected product is U-500 insulin; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately, because U-100 vs U-500 is a high-alert concentration difference with major overdose risk.
2804. **Q:** A prescription says "levothyroxine 0.125 mg po qd #90"; what strength should you select in mcg? **A:** Select 125 mcg, because $0.125 \text{ mg} \times 1000 \text{ mcg/mg} = 125 \text{ mcg}$ to avoid decimal-point dosing errors.
2805. **Q:** A refill request comes in for sertraline, but the profile shows an active prescription for escitalopram started last week; what should you do NEXT? **A:** Hold the refill and ask the pharmacist to review for therapeutic duplication, because two SSRIs together may be unintended and unsafe.
2806. **Q:** During drop-off, a patient asks if "Norco" is the same as hydrocodone/APAP and wants the generic; what is the BEST response within technician scope? **A:** Confirm that Norco is a brand for hydrocodone/acetaminophen and proceed with generic substitution per system/policy, because brand-generic recognition supports accurate dispensing.
2807. **Q:** An eRx for lisinopril is written "take 1 tab po bid #60"; what days' supply should you enter? **A:** Enter 30 days, because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$ for correct billing and refill timing.
2808. **Q:** You receive an eRx that uses "QD" and "U" in the sig for a high-alert medication; what should you do FIRST? **A:** Send it to the pharmacist for clarification/verification, because error-prone abbreviations can be misread and must be made unambiguous before dispensing.

2809. **Q:** A stock bottle of refrigerated amoxicillin/clavulanate suspension arrives warm with no temperature indicator documentation; what should you do NEXT? **A:** Quarantine the product and notify the pharmacist/manager per policy, because a temperature excursion creates a suspect product that should not be dispensed.
2810. **Q:** A prescription is for clopidogrel and the patient's profile shows omeprazole taken daily; what should you do BEST before filling? **A:** Flag and refer to the pharmacist for interaction review, because omeprazole can reduce clopidogrel activation and may require therapy adjustment.
2811. **Q:** A prescriber phones in lorazepam 1 mg tablets with the patient name, drug, directions, and quantity but no prescriber DEA number is available; what should you do FIRST? **A:** Stop processing and route to the pharmacist to obtain/validate required controlled-substance prescriber information because Schedule IV dispensing must be tied to an authorized prescriber.
2812. **Q:** While typing an order for hydrALAZINE you notice the hardcopy looks like hydrOXYzine due to poor handwriting; what is the BEST next step? **A:** Hold the prescription and ask the pharmacist to clarify with the prescriber because LASA name confusion is a high-risk error.
2813. **Q:** A patient presents an out-of-state paper prescription for oxycodone IR with an altered-looking quantity; what should you do FIRST at drop-off? **A:** Do not enter/fill it and immediately involve the pharmacist to assess validity/forgery concerns because technicians cannot determine legitimacy of a questionable controlled prescription.
2814. **Q:** During filling you scan a bottle labeled "morphine sulfate ER 30 mg" but the label you printed is for "morphine sulfate IR 30 mg"; what should you do NEXT? **A:** Stop filling and correct the product selection/label, then rescan to verify because IR vs ER is a high-alert dosage-form error.
2815. **Q:** A patient asks for an early refill of zolpidem claiming the bottle was lost and the claim rejects "refill too soon"; what should you do BEST? **A:** Refer to the pharmacist and follow store/insurer policy (e.g., loss override or new prescription) because controlled early refills require pharmacist judgment and documentation.
2816. **Q:** A sealed tote arrives with insulin glargine pens but the temperature monitor shows an excursion above acceptable range; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/wholesaler for guidance

because temperature excursions can compromise potency and must not be stocked.

2817. **Q:** You receive a manufacturer recall notice for certain lots of losartan tablets and your shelf has matching NDC and lot numbers; what should you do NEXT? **A:** Pull the affected stock from inventory and quarantine it for return per recall instructions because recalled lots must be removed to prevent dispensing.
2818. **Q:** An eRx for methotrexate 2.5 mg is entered as “take 6 tablets daily” for rheumatoid arthritis; what should you do FIRST? **A:** Stop and alert the pharmacist for prescriber clarification because methotrexate is typically weekly and daily dosing is a serious, common safety error.
2819. **Q:** A new prescription is for metformin ER 500 mg but the selected bottle is metformin IR 500 mg and both barcodes scan; what is the BEST technician action? **A:** Re-verify NDC and dosage form against the prescription and have the pharmacist review if any mismatch remains because IR vs ER selection errors are frequent and clinically significant.
2820. **Q:** A patient wants pseudoephedrine 30 mg tablets and asks to buy 96 tablets today; what should you do FIRST at the register? **A:** Verify ID and check the electronic log and legal daily limits before completing the sale because CMEA requires identity/logbook compliance and limits to prevent diversion.
2821. **Q:** A new eRx is for levothyroxine 25 mcg daily but the product pulled is levothyroxine 0.025 mg; what should you do NEXT? **A:** Confirm they are equivalent strengths (25 mcg = 0.025 mg) and proceed with normal verification steps because unit conversion prevents a false dosing error.
2822. **Q:** While entering a new prescription you notice both lamotrigine and terbinafine start with “Lami-” in the search list; what is the BEST technician step to avoid a LASA selection error? **A:** Use the full drug name plus strength and verify by NDC/barcode during product selection because similar names can lead to wrong-drug dispensing.
2823. **Q:** A patient’s profile shows sertraline daily and a new prescription arrives for linezolid; what should you do FIRST? **A:** Stop processing and alert the pharmacist because linezolid can interact with SSRIs and requires clinical review for serotonin syndrome risk.
2824. **Q:** A prescription is written for “Lantus 10 units SQ qhs” but you selected Humalog KwikPen during picking; what should you do NEXT? **A:** Stop filling and

re-select the correct insulin (insulin glargine) and re-scan because mix-ups between basal and rapid-acting insulins are high-alert errors.

2825. **Q:** A patient drops off a new prescription for lisinopril and tells you they had swelling of lips with it before; what should you do BEST? **A:** Enter the reported angioedema as an allergy/intolerance and notify the pharmacist before filling because ACE inhibitor angioedema is a serious safety concern.
2826. **Q:** You are typing a prescription for "MS Contin 15 mg" and the directions look like immediate-release morphine dosing; what should you do FIRST? **A:** Hold the order and refer to the pharmacist to clarify ER vs IR intent because morphine formulations are LASA/high-risk and directions must match dosage form.
2827. **Q:** A new prescription for bupropion SR is entered, but the patient profile already has bupropion XL active; what is the BEST next step? **A:** Pause and alert the pharmacist about possible therapeutic duplication because different bupropion release forms can cause dosing errors and increased seizure risk.
2828. **Q:** An order reads "azithromycin 250 mg: take ii tabs day 1 then i tab daily days 2-5; dispense 1 pack"; what quantity should you enter? **A:** Enter quantity 6 tablets because 2 on day 1 plus 1 daily for 4 more days equals 6 total tablets.
2829. **Q:** A new prescription is for atorvastatin but the patient says they currently take simvastatin at home; what should you do NEXT? **A:** Collect the dose/frequency information and route to the pharmacist because concurrent statins may be unintended duplication and needs pharmacist assessment.
2830. **Q:** During data entry you see "APAP w/ codeine #3" and the prescriber wrote "refill x 5"; what should you do FIRST? **A:** Stop and send to the pharmacist because codeine combination products are controlled and refills may be restricted by federal/state rules requiring pharmacist review.
2831. **Q:** While entering a prescription you see "hydralazine 25 mg" but the hardcopy handwriting could also be "hydroxyzine 25 mg"; what should you do FIRST? **A:** Stop processing and refer to the pharmacist to clarify with the prescriber because this is a LASA risk with different indications and dispensing the wrong drug can cause harm.
2832. **Q:** A new eRx says "Lasix 40 mg PO daily" and the patient profile shows furosemide already; what is the BEST action during data entry? **A:** Select furosemide as the generic for Lasix and verify the patient profile for duplication

because brand/generic are the same drug and duplicate therapy can create confusion.

2833. **Q:** A prescription reads "take 1 tab PO qd" for prednisone and the prescriber wrote "QD"; what should you do NEXT? **A:** Enter the directions as "once daily" (not "QD") per safe abbreviation practices and route to pharmacist if any ambiguity remains because error-prone abbreviations can be misread.
2834. **Q:** You are selecting product for "Trileptal 300 mg" and see both oxcarbazepine and carbamazepine on the shelf; what should you do BEST before labeling? **A:** Scan the barcode and match the NDC to the prescription for oxcarbazepine because these are different drugs with similar names and verification prevents a wrong-drug error.
2835. **Q:** A patient asks if "Prilosec" is the same as the "omeprazole" they used before and wants the cheapest option; what is the BEST technician response? **A:** Tell them Prilosec is the brand for omeprazole and offer the generic option because recognizing brand/generic equivalence helps select the correct cost-effective product.
2836. **Q:** During intake a patient says they take "Plavix" and a new prescription arrives for clopidogrel; what should you do FIRST in the workflow? **A:** Confirm in the profile that Plavix is clopidogrel and avoid creating a duplicate entry because accurate brand/generic reconciliation prevents duplicate therapy and billing issues.
2837. **Q:** A prescription is for "Keflex 500 mg PO qid x 7 days, dispense #?"; what quantity should you enter? **A:** Enter 28 capsules because qid is 4 doses/day and $4 \times 7 = 28$ for the total quantity to dispense.
2838. **Q:** You receive a prescription for "Zithromax" but the prescriber also wrote "azithromycin" on the same order and the strength doesn't match any pack; what should you do NEXT? **A:** Stop and refer to the pharmacist for clarification because conflicting brand/generic/strength information can lead to the wrong product or quantity.
2839. **Q:** A patient profile shows active therapy with escitalopram and a new prescription comes in for citalopram; what should you do BEST? **A:** Flag it for pharmacist review as potential therapeutic duplication because these are closely related SSRIs and switching/duplication requires clinical judgment.

2840. **Q:** While receiving an order you notice a sealed tote of refrigerated insulin is warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager per policy because temperature excursions can compromise potency and the product may be non-dispensable.
2841. **Q:** A prescription reads "levothyroxine 0.075 mg PO qd"; what strength should you enter in mg and in mcg? **A:** Enter 0.075 mg, which equals 75 mcg, because $1 \text{ mg} = 1000 \text{ mcg}$ and accurate unit conversion prevents strength-selection errors.
2842. **Q:** You are entering insulin lispro U-100 vial directions "inject 12 units TID with meals," and the quantity is 10 mL; what days' supply should you submit? **A:** Submit 27 days because $10 \text{ mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ and days' supply is billed as 27 to avoid overstatement.
2843. **Q:** An eRx for amlodipine says "take 1 tab PO qd, dispense #30, refills 11," but the prescriber field shows an outdated provider; what should you do FIRST? **A:** Stop processing and send to the pharmacist to verify prescriber validity because technician entry cannot override potential prescription-validity issues.
2844. **Q:** A child's prescription is for cephalexin suspension 250 mg/5 mL: "take 10 mL PO BID x 10 days"; what quantity in mL should you dispense? **A:** Dispense 200 mL because $10 \text{ mL/dose} \times 2 \text{ doses/day} \times 10 \text{ days} = 200 \text{ mL}$ to cover the full course.
2845. **Q:** During order entry you see "metFORMIN 500 mg take i tab po bid" and the prescriber wrote "i" that could be mistaken for "1"; what is the BEST action? **A:** Clarify the ambiguous "i" with the pharmacist/prescriber before processing because unclear numerals/letters can cause a wrong-dose error.
2846. **Q:** A prescription for sertraline 50 mg says " $\frac{1}{2}$ tab PO qd, dispense 45"; what days' supply should you calculate? **A:** Enter 90 days because 45 tablets at 0.5 tablet/day equals $45 \div 0.5 = 90$ days for accurate insurance and refill timing.
2847. **Q:** You receive a hardcopy for hydromorphone 2 mg with directions and quantity present but the prescriber forgot to sign; what should you do FIRST? **A:** Hold the prescription and route it to the pharmacist because an unsigned controlled-substance prescription requires pharmacist resolution before any dispensing steps.

2848. **Q:** While selecting product for “predniSONE 10 mg dose pack,” you find two similar cartons with different NDCs and one is expired; what should you do NEXT? **A:** Remove the expired product from dispensing stock per policy and select the in-date NDC, because expiration is a dispensing-safety requirement and NDC must match what will be billed/labeled.
2849. **Q:** A new prescription is for lisinopril, but the patient profile shows a documented history of angioedema with an ACE inhibitor; what should you do BEST during intake/data entry? **A:** Enter the allergy/clinical alert and notify the pharmacist immediately because ACE-inhibitor angioedema is a serious contraindication requiring pharmacist intervention.
2850. **Q:** A prescriber writes for “fluticasone nasal spray: 2 sprays each nostril daily” and the product is 50 mcg/spray, 120 sprays; what days’ supply should you bill? **A:** Bill 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$, ensuring correct days’ supply submission.
2851. **Q:** During data entry a new eRx is for hydroxyzine 25 mg PRN itching, and the patient profile shows an active cetirizine 10 mg daily; what should you do BEST? **A:** Pause processing and alert the pharmacist to evaluate therapeutic duplication/sedation risk because the technician cannot decide whether both antihistamines should be used.
2852. **Q:** A prescriber phones in “Lantus 10 units subq nightly, dispense 1 box,” and the caller will not provide their name or call-back number; what should you do FIRST? **A:** Stop taking the order and route to the pharmacist because unverifiable prescriber/agent identity makes the prescription validity questionable.
2853. **Q:** A hardcopy for alprazolam 0.5 mg says “take 1 tab TID PRN anxiety #90” and the prescriber wrote refills “3”; what should you do NEXT? **A:** Hold and notify the pharmacist because Schedule IV controls have federal refill limits and the pharmacist must ensure the refill quantity complies.
2854. **Q:** You are filling for “Lamictal 100 mg” and the shelf also has “Lamisil 250 mg” nearby; what is the BEST safety step before pulling stock? **A:** Use barcode/NDC verification and Tall Man awareness (lamiCTAL vs lamiSIL) because LASA name confusion is a common dispensing error.
2855. **Q:** A patient requests an early refill on oxycodone/APAP claiming the bottle was lost yesterday; what should you do FIRST at the counter? **A:** Refer immediately

to the pharmacist and follow store policy because controlled-substance early replacements require pharmacist judgment and documentation.

2856. **Q:** While receiving inventory you notice a wholesaler case seal is broken and the inner bottles lack lot/expiration labels; what should you do BEST? **A:** Quarantine the shipment and notify the pharmacist/manager for DSCSA/suspect product procedures because product pedigree and integrity cannot be confirmed.
2857. **Q:** A recall notice lists your NDC/lot of losartan as recalled, and you find matching bottles on the dispensing shelf; what should you do NEXT? **A:** Remove the affected lot from active stock and quarantine it per recall instructions because recalled product must be prevented from dispensing and tracked for return.
2858. **Q:** An order is for warfarin 2 mg, but you select warfarin 5 mg by mistake and realize it before it reaches pharmacist verification; what should you do FIRST? **A:** Stop filling, return the incorrect stock bottle, and reselect the correct strength with barcode/NDC check because immediate correction prevents a high-alert dosing error.
2859. **Q:** A prescription is for prednisolone sodium phosphate oral solution 15 mg/5 mL with directions "give 2.5 mL PO BID for 7 days"; what quantity in mL should you dispense? **A:** Dispense 35 mL because $2.5 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 35 \text{ mL}$ total volume needed.
2860. **Q:** At pickup a caregiver asks why isotretinoin comes with special warnings and requests written information; what should you do BEST? **A:** Provide the required Medication Guide/REMS materials and involve the pharmacist for counseling because isotretinoin has mandated risk information and pharmacist counseling requirements.
2861. **Q:** A prescription reads "metformin ER 500 mg: ii tabs PO BID with meals; #120"; what should you enter for days' supply? **A:** Enter 30 days because $2 \text{ tablets twice daily} = 4 \text{ tablets/day}$ and $120 \div 4 = 30$, preventing an incorrect days' supply rejection.
2862. **Q:** The sig says "Take 0.5 mL PO q8h" and the stock product is acetaminophen 160 mg/5 mL; how many mg per dose should you calculate for order entry? **A:** Enter 16 mg per dose because $160 \text{ mg/5 mL} = 32 \text{ mg/mL}$ and $0.5 \text{ mL} \times 32 \text{ mg/mL} = 16 \text{ mg}$, ensuring the dose is interpreted correctly.

2863. **Q:** During data entry you receive "levothyroxine 0.1 mg PO daily"; what is the BEST way to enter the strength to avoid a decimal error? **A:** Enter as 100 mcg (and confirm product strength) because $0.1 \text{ mg} = 100 \text{ mcg}$, reducing tenfold dosing error risk.
2864. **Q:** A prescription for gabapentin 300 mg says "1 cap PO TID" with a quantity of 270; what days' supply should you bill? **A:** Bill 90 days because $\text{TID} = 3 \text{ capsules/day}$ and $270 \div 3 = 90$, aligning quantity to directions.
2865. **Q:** A new eRx is for "glipizide XL 10 mg daily," but the patient profile shows an active "glipizide IR 10 mg BID"; what should you do NEXT? **A:** Stop and alert the pharmacist to potential therapeutic duplication/formulation change because ER vs IR dosing differs and requires pharmacist review.
2866. **Q:** You are entering "prednisone 20 mg: take ii tabs PO daily x 5 days, then i tab daily x 5 days; dispense ____"; what quantity should be entered? **A:** Enter 15 tablets because $2/\text{day} \times 5 = 10$ and $1/\text{day} \times 5 = 5$, so total = 15 to match the taper directions.
2867. **Q:** An inhaler order is "tiotropium Respimat: inhale 2 puffs once daily," and the device contains 60 actuations; what days' supply should you calculate? **A:** Bill 30 days because 2 actuations/day and $60 \div 2 = 30$, avoiding an overbilled days' supply.
2868. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg: 1 tab PO q12h x 10 days"; what quantity should you enter? **A:** Enter 20 tablets because $\text{q12h} = 2/\text{day}$ and $2 \times 10 = 20$, ensuring the dispensed amount matches the course.
2869. **Q:** While typing an eRx you see "U-500 insulin regular: inject 10 units before meals," but the system default is U-100 syringes; what should you do FIRST? **A:** Pause processing and notify the pharmacist because U-500 is high-alert and syringe/device selection requires verification to prevent a major dosing error.
2870. **Q:** A controlled medication eRx arrives with directions and quantity but no prescriber DEA number displayed in your system; what should you do BEST before proceeding? **A:** Hold the order and refer to the pharmacist to verify prescriber credentials per workflow because missing/unclear controlled-substance prescriber data can affect validity and safety.
2871. **Q:** While selecting product you notice hydroxyzine and hydralazine are stored adjacent and the label prints "hydrOXYzine" for anxiety; what should you do FIRST to prevent a LASA error? **A:** Use barcode/NDC scan and compare the

original Rx to the stock bottle before counting, because LASA pairs require extra verification to avoid wrong-drug selection.

2872. **Q:** A handwritten prescription for oxycodone/acetaminophen (Schedule II) is presented with a missing patient address; what should you do NEXT? **A:** Stop processing and notify the pharmacist to assess validity and contact the prescriber if needed, because missing required elements on a C-II must be resolved before dispensing.
2873. **Q:** A patient requests an early refill of clonazepam stating it was "lost," but the claim rejects refill-too-soon; what is the BEST technician action? **A:** Inform the patient it requires pharmacist review and follow store policy for early controlled refills, because early CIII-V refills need professional and documentation oversight.
2874. **Q:** During order entry you see "heparin 10,000 units/mL" but the patient is outpatient and the prescriber note seems to intend flushes; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist immediately, because heparin is a high-alert drug and concentration mix-ups can cause severe harm.
2875. **Q:** A refrigerated shipment of insulin aspart arrives warm with a temperature indicator showing an excursion; what should you do BEST? **A:** Quarantine the product and notify the pharmacist/wholesaler for guidance, because temperature excursions make product integrity uncertain and it should not be dispensed.
2876. **Q:** A wholesaler recall notice lists a specific lot of losartan tablets; what should you do NEXT in the pharmacy workflow? **A:** Pull stock and any will-call items matching the NDC/lot, quarantine them, and alert the pharmacist, because recalled lots must be removed from dispensing immediately.
2877. **Q:** A patient is picking up sertraline and asks about a new "Medication Guide" stapled to the bag; what should you do FIRST? **A:** Provide the Medication Guide with the dispense and refer clinical questions to the pharmacist, because certain drugs require FDA-approved patient information at dispensing.
2878. **Q:** A new eRx for lisinopril 20 mg daily arrives, but the profile shows a documented history of angioedema with an ACE inhibitor; what should you do NEXT? **A:** Stop processing and notify the pharmacist before filling, because a serious contraindication requires pharmacist intervention and possible prescriber clarification.

2879. **Q:** The sig reads "morphine oral solution 10 mg/5 mL: take 7.5 mL PO q4h prn pain; dispense 450 mL"; what should you enter for the dose in mg to support safe checking? **A:** Enter 15 mg per dose ($10 \text{ mg}/5 \text{ mL} = 2 \text{ mg}/\text{mL}$; $7.5 \text{ mL} \times 2 \text{ mg}/\text{mL} = 15 \text{ mg}$), because converting to mg helps detect dosing errors for high-alert opioids.
2880. **Q:** At register a patient loudly asks if their spouse's "HIV meds" are ready while other customers are nearby; what is the BEST technician response? **A:** Move the conversation to a private area and limit disclosure to the minimum necessary, because HIPAA requires protecting PHI from being overheard in public spaces.
2881. **Q:** While entering a new prescription for Lamictal you notice the prescriber wrote "lamotrigine 25 mg take 2 tabs BID" but the patient's profile already shows divalproex sodium (Depakote); what should you do NEXT? **A:** Stop and alert the pharmacist to review the interaction because valproate can raise lamotrigine levels and dosing often must be adjusted for safety.
2882. **Q:** A patient drops off a prescription written for "metFORMIN ER 500 mg" but the directions say "take 1 tablet three times daily"; what is the BEST technician action? **A:** Hold the order and refer to the pharmacist for clarification because ER products typically aren't dosed TID and this could indicate a formulation/sig mismatch.
2883. **Q:** During product selection you see "Celebrex 200 mg" on the label but you accidentally pulled Celexa 20 mg from the shelf; what should you do FIRST? **A:** Stop filling and reselect using barcode/NDC verification because celecoxib and citalopram are LASA and a wrong-drug error is high risk.
2884. **Q:** An eRx arrives for "Adderall XR 20 mg" with "refills: 2" listed; what should you do NEXT? **A:** Do not process the refills and route to the pharmacist because Schedule II medications cannot be refilled and require prescriber follow-up if needed.
2885. **Q:** A prescription reads "levothyroxine 75 mcg daily," but the prescriber accidentally selected "liothyronine" in the eRx medication field; what should you do BEST? **A:** Pause processing and notify the pharmacist to contact the prescriber because these are different thyroid hormones and technicians cannot interchange therapy.
2886. **Q:** Order entry shows "rosuvastatin 20 mg nightly" and the patient profile includes active atorvastatin 40 mg nightly from another prescriber; what should

you do FIRST? **A:** Flag and refer to the pharmacist for therapeutic duplication review because two statins together may indicate an error or unsafe duplication.

2887. **Q:** A label prints "buPROPion SR 150 mg: take 1 tablet TID" for a new prescription; what should you do NEXT? **A:** Stop and get pharmacist review because SR bupropion is not typically dosed three times daily and improper dosing increases seizure risk.
2888. **Q:** A patient is prescribed azithromycin Z-Pak 250 mg with directions "take 2 tabs day 1 then 1 tab daily days 2-5"; what quantity should you enter to dispense? **A:** Enter quantity 6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet for 4 more days ($2 + 4 = 6$).
2889. **Q:** While processing a new warfarin prescription, the patient asks if they can start taking ibuprofen 800 mg TID for back pain from an old bottle; what should you do FIRST? **A:** Refer them to the pharmacist immediately because NSAIDs can increase bleeding risk when combined with warfarin and requires counseling.
2890. **Q:** A stock bottle label says "predniSONE 20 mg" and next to it is "prednisoLONE 15 mg/5 mL," and the prescription is for prednisone tablets; what is the BEST step before counting? **A:** Confirm dosage form/route and scan the NDC because prednisone and prednisolone are commonly confused and selecting the wrong form can cause a dispensing error.
2891. **Q:** A prescription is written for "Zyrtec 10 mg daily," but the prescriber selected "cetirizine 10 mg" in the eRx field; what is the BEST technician action during entry? **A:** Enter it as cetirizine 10 mg (generic) and keep the DAW/substitution as written because Zyrtec is the brand for cetirizine and accurate selection supports correct billing and dispensing.
2892. **Q:** During data entry you see "Lopressor 50 mg BID," but the product selected in the system is "metoprolol succinate ER 50 mg"; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify product selection because Lopressor is metoprolol tartrate (IR) and mixing IR vs ER can cause dosing errors.
2893. **Q:** A patient profile shows an active prescription for lisinopril and you receive a new prescription for Zestril; what should you do FIRST? **A:** Verify in the profile and alert the pharmacist to possible therapeutic duplication because Zestril is the brand name for lisinopril and duplicate ACE inhibitor therapy can be unsafe.

2894. **Q:** The prescriber writes "Humalog U-100: inject 12 units SQ TID with meals; dispense 1 vial (10 mL)"; what days' supply should you enter? **A:** Enter 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$, and $12 \text{ units} \times 3/\text{day} = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ which is typically billed as 27 whole days.
2895. **Q:** A prescription reads "amoxicillin 400 mg/5 mL: take 6.25 mL PO BID x 10 days"; what quantity in mL should be dispensed? **A:** Dispense 125 mL because $6.25 \text{ mL} \times 2/\text{day} \times 10 \text{ days} = 125 \text{ mL}$, ensuring enough volume for the full course.
2896. **Q:** While selecting product you notice two look-alike bottles: hydroxyzine HCl 25 mg and hydralazine 25 mg; the label is for hydralazine—what is the BEST step before counting? **A:** Scan the barcode and match the NDC to the label because LASA names require an objective verification step to prevent wrong-drug errors.
2897. **Q:** A hardcopy prescription has "take i tab PO qd" and the prescriber's handwriting makes "i" unclear; what should you do FIRST? **A:** Hold the prescription and ask the pharmacist to clarify with the prescriber because unclear Roman numerals/letters can be misread and the technician must not guess a dose.
2898. **Q:** An eRx arrives for "Tylenol #3" with missing prescriber DEA number in your system record; what should you do NEXT? **A:** Stop processing and notify the pharmacist to verify controlled-substance prescriber credentials because accurate prescriber information is required for compliant dispensing.
2899. **Q:** While putting away an order you find a tote of refrigerated insulin that arrived warm with no temperature indicator; what is the BEST technician action? **A:** Quarantine the shipment and notify the pharmacist/manager because a temperature excursion makes the product suspect and it should not be stocked until verified per policy.
2900. **Q:** A patient asks where the "pink eye antibiotic drops" are in the OTC aisle; what is the MOST appropriate technician response? **A:** Explain that antibiotic eye drops require a prescription and offer to help them speak with the pharmacist because technicians should not recommend prescription therapy or diagnose.
2901. **Q:** During order entry, a new eRx is for "Lamictal 25 mg daily" but the selected product is "terbinafine 250 mg tablets"; what should you do FIRST? **A:** Stop processing and correct the drug selection then alert the pharmacist to review, because Lamictal (lamotrigine) and terbinafine are unrelated and a wrong-drug error is high risk.

2902. **Q:** A patient profile shows active atorvastatin and you receive a new prescription for Lipitor; what should you do NEXT? **A:** Verify they are the same drug and route to the pharmacist for duplicate therapy review before filling, because Lipitor is atorvastatin and duplicate statins can increase adverse effects.
2903. **Q:** A patient picking up methotrexate says, "I'll take this every day like my other pills"; what is the BEST technician response? **A:** Pause the pickup and get the pharmacist immediately, because methotrexate is commonly dosed weekly and daily use is a high-alert, potentially fatal error.
2904. **Q:** An eRx is written for "metFORMIN ER 500 mg daily" but the system defaulted to metformin IR 500 mg; what should you do FIRST? **A:** Stop and select the correct ER formulation or send to pharmacist for clarification per policy, because IR vs ER is not interchangeable and affects safety and control of diabetes.
2905. **Q:** A new prescription is for "Bactrim DS 1 tab BID" and the patient's profile lists a severe sulfonamide allergy; what should you do FIRST? **A:** Hold the fill and notify the pharmacist, because Bactrim (sulfamethoxazole/trimethoprim) is a sulfonamide and could trigger a serious allergic reaction.
2906. **Q:** A prescription reads "azithromycin 200 mg/5 mL: take 12.5 mL PO daily x 3 days"; what quantity in mL should be dispensed? **A:** Dispense 37.5 mL, because $12.5 \text{ mL/day} \times 3 \text{ days} = 37.5 \text{ mL total}$.
2907. **Q:** An albuterol HFA inhaler says 90 mcg per actuation and the sig is "inhale 2 puffs QID"; what days' supply should you enter if the canister contains 200 actuations? **A:** Enter 25 days, because $2 \text{ puffs} \times 4 \text{ times/day} = 8 \text{ actuations/day}$ and $200 \div 8 = 25 \text{ days}$.
2908. **Q:** A patient on sertraline (Zoloft) asks if they can start St John's wort for mood; what should you do BEST? **A:** Refer to the pharmacist before selling it, because combining serotonergic products can increase the risk of serotonin syndrome and requires pharmacist counseling.
2909. **Q:** While stocking, you notice a bottle labeled "oxyCODONE 5 mg" stored next to "oxyMORPHONE 5 mg" in the fast-mover bin; what should you do NEXT? **A:** Separate and relabel/bin them using LASA safeguards (e.g., Tall Man, different locations) and inform the pharmacist, because these look-alike opioids have very different potency and error risk.
2910. **Q:** A new eRx is for "Eliquis 5 mg BID" but the patient profile shows active "Xarelto 20 mg daily"; what should you do FIRST? **A:** Stop processing and alert

the pharmacist for anticoagulant duplication review, because Eliquis (apixaban) and Xarelto (rivaroxaban) should not be taken together without explicit prescriber intent.

2911. **Q:** A handwritten prescription for alprazolam 0.5 mg has no patient address and includes "refill x3"; what should the technician do FIRST? **A:** Stop processing and route to the pharmacist to resolve missing required info and controlled-substance refill limits to prevent an invalid/unsafe dispense.
2912. **Q:** At pickup a patient asks for the Medication Guide for isotretinoin (Accutane) and says they never got one before; what is the BEST technician action? **A:** Provide the required Medication Guide and alert the pharmacist if counseling or REMS steps are needed, because certain drugs must be dispensed with FDA-required guides.
2913. **Q:** While receiving inventory you notice a sealed case of levothyroxine tablets with a different NDC than what the wholesaler invoice lists; what should you do NEXT? **A:** Pause receiving and verify the NDC/strength against the invoice and ordering record before stocking, because NDC mismatches can signal selection or supply chain errors.
2914. **Q:** A patient drops off a prescription for hydrocodone/acetaminophen and asks to pay cash because "insurance takes too long"; what should you do BEST? **A:** Proceed with normal intake but immediately notify the pharmacist for controlled-substance red-flag review, because payment method requests can indicate diversion risk.
2915. **Q:** During data entry you see "U-500 regular insulin, inject 20 units TID" for a patient previously on U-100 insulin; what should you do FIRST? **A:** Stop and get pharmacist intervention to verify dose and product (U-500 vs U-100), because concentrated insulin is high-alert and dosing errors are dangerous.
2916. **Q:** A stock bottle of cephalexin 500 mg capsules is found on the shelf with an expired manufacturer date from last month; what should you do NEXT? **A:** Remove it from active inventory and follow pharmacy policy for expired product segregation/return, because expired meds must not be dispensed.
2917. **Q:** A C-II eRx for amphetamine/dextroamphetamine is received showing "Do not fill until" next week, but the patient demands it today; what should you do BEST? **A:** Explain it cannot be filled before the indicated date and refer to the pharmacist for questions, because early filling of C-II prescriptions requires strict compliance with directions.

2918. **Q:** You are preparing a compounded oral suspension and realize the balance calibration sticker is past due; what is the BEST technician action? **A:** Stop compounding and notify the pharmacist/manager to use a verified calibrated balance, because inaccurate weighing can cause dosing errors.
2919. **Q:** A caregiver presents a pseudoephedrine request and has no ID; what should you do FIRST? **A:** Decline the sale and follow logbook requirements per pharmacy policy, because federal CMEA rules require ID and tracking for pseudoephedrine purchases.
2920. **Q:** A prescription label prints "take 1 tablet daily" for glipizide ER, but the original eRx says "take 1 tablet twice daily"; what should you do NEXT? **A:** Stop the fill and have the pharmacist review/correct the directions before dispensing, because sig discrepancies can lead to serious hypoglycemia and are not technician-edit decisions.
2921. **Q:** During order entry, you see "metFORMIN ER 500 mg" but the selected product is metformin IR 500 mg; what should you do NEXT? **A:** Stop and correct the dosage form to ER and route to pharmacist if unclear, because IR vs ER is not interchangeable and could cause dosing errors.
2922. **Q:** A patient profile shows lisinopril active, and a new prescription is for "Zestril 10 mg daily"; what should you do FIRST? **A:** Verify Zestril is lisinopril and check the profile for duplicate therapy before processing, because brand/generic recognition prevents unintended duplication.
2923. **Q:** A prescription is written for "clopidogrel 75 mg daily," and the patient asks if it's the same as Plavix; what is the BEST technician response? **A:** Confirm that Plavix is the brand name for clopidogrel and offer to have the pharmacist counsel if needed, because technicians can clarify brand/generic but not provide clinical advice.
2924. **Q:** You receive an eRx for "Bactrim DS 1 tab BID" and the allergy field shows "sulfa"; what should you do FIRST? **A:** Stop processing and alert the pharmacist, because Bactrim (sulfamethoxazole/trimethoprim) is a sulfonamide and requires pharmacist assessment.
2925. **Q:** While selecting product, you notice hydrALAZINE and hydrOXYzine stored adjacent with similar labels; what should you do NEXT? **A:** Separate the look-alike drugs and use barcode/NDC verification during filling, because LASA storage controls reduce selection errors.

2926. **Q:** A patient's prescription reads "take ii tabs po qhs" for atorvastatin 10 mg; how many mg per dose is this? **A:** 20 mg per dose, because "ii" means 2 tablets and $2 \times 10 \text{ mg} = 20 \text{ mg}$.
2927. **Q:** An order is for amoxicillin/clavulanate suspension 600 mg/42.9 mg per 5 mL; during entry you select 400 mg/57 mg per 5 mL; what should you do FIRST? **A:** Stop and correct to the exact strength/ratio ordered and involve the pharmacist if clarification is needed, because different Augmentin formulations are not equivalent.
2928. **Q:** A C-IV prescription for zolpidem is presented with "refill 5" and today's date; what should you do BEST? **A:** Hold the prescription and notify the pharmacist, because controlled-substance refill limits and validity require pharmacist review before dispensing.
2929. **Q:** You are filling insulin lispro pens: directions "inject 8 units before meals TID," and the box contains five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** 62 days, because total units = $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div (8 \times 3) = 62.5$, typically billed as 62 days per payer rules.
2930. **Q:** A patient asks if "omeprazole" is the same medicine as "Prilosec"; what should you do NEXT? **A:** Confirm Prilosec is the brand for omeprazole and check that the strength and directions match the prescription, because correct brand/generic recognition supports safe counseling and pickup.
2931. **Q:** A prescription reads "levothyroxine 0.075 mg po daily"; during entry, what strength should you select in mg and why? **A:** Select 75 mcg (0.075 mg) strength because $1 \text{ mg} = 1000 \text{ mcg}$ and choosing 0.75 mg would be a 10× dose error.
2932. **Q:** You receive "azithromycin 200 mg/5 mL suspension: give 6 mL po daily x 5 days"; what quantity in mL should you dispense? **A:** Dispense 30 mL because $6 \text{ mL/day} \times 5 \text{ days} = 30 \text{ mL}$ to cover the full course.
2933. **Q:** An eRx says "predniSONE 10 mg: take 4 tabs daily x 5 days"; what days' supply should you enter for insurance? **A:** Enter 5 days because the directions specify a fixed 5-day duration regardless of tablet count.
2934. **Q:** A prescription is written "Augmentin 875-125: 1 tab po q12h x 7 days"; what quantity should be entered and why? **A:** Enter 14 tablets because q12h is 2 doses/day and $2 \times 7 = 14$ for the full duration.

2935. **Q:** A new prescription is "sertraline 50 mg: take 1 tab po qam" but the patient profile shows active "fluoxetine 20 mg daily"; what should you do FIRST? **A:** Stop and alert the pharmacist to possible duplicate SSRI therapy because it may require clinical review before dispensing.
2936. **Q:** While entering "morphine sulfate ER 15 mg q12h," you accidentally select morphine IR 15 mg; what should you do NEXT? **A:** Correct the dosage form to ER and re-verify NDC/product selection because ER vs IR is a high-risk order entry error.
2937. **Q:** A prescription for timolol ophthalmic says "instill 1 gtt OU BID"; how many drops per day is that for days' supply calculation? **A:** Use 4 drops/day because 1 drop in each eye twice daily is $2 \text{ eyes} \times 2 \text{ times/day} = 4 \text{ drops/day}$.
2938. **Q:** You are billing fluticasone nasal spray 50 mcg with directions "2 sprays each nostril once daily"; the bottle has 120 sprays—what days' supply should you enter? **A:** Enter 30 days because daily sprays are $2 \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
2939. **Q:** A written prescription says "U-100 insulin regular: inject 0.5 mL subcut BID"; how many units per dose should you enter? **A:** Enter 50 units per dose because U-100 is 100 units/mL and $0.5 \text{ mL} \times 100 = 50 \text{ units}$.
2940. **Q:** An eRx arrives with "hydroCHLOROthiazide 25 mg: take 1 tab qd" but the sig field also includes "qd-BID" in free text; what is the BEST next step? **A:** Hold the prescription and route to the pharmacist for clarification because conflicting directions can cause incorrect dosing and days' supply.
2941. **Q:** A prescription reads "metFORMIN ER 500 mg: ii tabs po qhs #60"; what is the days' supply you should enter and why? **A:** Enter 30 days' supply because "ii tabs qhs" = 2 tablets daily and $60 \div 2 = 30$.
2942. **Q:** You are entering "amLODIPine 2.5 mg po daily" but only see 2.5 mg and 5 mg strengths; what is the BEST product to select and why? **A:** Select amlodipine 2.5 mg tablets because it matches the written strength and avoids unsafe tablet-splitting assumptions without prescriber direction.
2943. **Q:** An order says "cephalexin 250 mg/5 mL: take 10 mL po BID x 10 days"; what quantity (mL) should you dispense and why? **A:** Dispense 200 mL because $10 \text{ mL per dose} \times 2 \text{ doses/day} \times 10 \text{ days} = 200 \text{ mL total}$.
2944. **Q:** While typing a sig you see "Take 1 tab po qd" and the prescriber note says "BID x 7 days"; what should you do FIRST? **A:** Stop entry and route to the

pharmacist to clarify conflicting directions because the days' supply and dosing are ambiguous and cannot be guessed.

2945. **Q:** A prescription reads "prednisoLONE sodium phosphate oral solution 15 mg/5 mL: take 7.5 mg po daily"; how many mL per dose should you enter and why? **A:** Enter 2.5 mL per dose because 15 mg in 5 mL equals 3 mg/mL, and $7.5 \text{ mg} \div 3 \text{ mg/mL} = 2.5 \text{ mL}$.
2946. **Q:** A patient's new eRx is "lisinopril 10 mg qd" and the active profile shows "enalapril 10 mg qd" still filling; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because both are ACE inhibitors and may represent an unintended overlap.
2947. **Q:** A prescription for an albuterol HFA inhaler says "inhale 2 puffs q6h prn wheeze," and the plan requires a days' supply using a maximum daily dose; what days' supply should you enter for a 200-inhalation inhaler and why? **A:** Enter 25 days because max use is 2 puffs every 6 hours = 8 puffs/day and $200 \div 8 = 25 \text{ days}$.
2948. **Q:** You receive an eRx for "methotrexate 2.5 mg: take 1 tab daily"; what is the BEST next step? **A:** Hold and immediately refer to the pharmacist because methotrexate is high-alert and most outpatient dosing is weekly, so daily directions require verification.
2949. **Q:** In order entry you see "digoxin 0.125 mg po daily," but the drop-down list shows 125 mcg and 0.25 mg; which strength should you select and why? **A:** Select 125 mcg because 0.125 mg equals 125 mcg, preventing a 10-fold dosing error.
2950. **Q:** A C-II eRx for "oxycodone 5 mg" arrives with directions but no prescriber DEA number displayed in your software; what should you do NEXT? **A:** Stop processing and notify the pharmacist to verify prescriber credentials in the system because controlled-substance prescribing requires validated prescriber identification before dispensing.
2951. **Q:** A patient asks if "Zocor" is the same as the "simvastatin" on their new label; what should you say FIRST? **A:** Confirm that Zocor is the brand name for simvastatin (a statin for cholesterol), because matching brand/generic names prevents duplicate therapy and confusion.
2952. **Q:** While selecting a product, you notice "hydrOXYzine" and "hydrALAZINE" are adjacent in the pick list; what is the BEST next step before printing labels? **A:**

Re-verify the intended indication/sig and match the NDC/barcode to the prescription, because this is a common LASA pair and selection errors cause harm.

2953. **Q:** An eRx comes in for "levothyroxine 25 mcg daily," but the patient profile shows an allergy entry of "Synthroid"; what should you do NEXT? **A:** Stop processing and alert the pharmacist to clarify the reported "allergy," because Synthroid is levothyroxine and the entry may reflect intolerance or a documentation error.
2954. **Q:** A prescription reads "Azithromycin 250 mg: ii tabs today, then i tab daily x 4 days"; what quantity should you dispense and why? **A:** Dispense 6 tablets, because the regimen is 2 tablets on day 1 plus 1 tablet daily for 4 more days ($2 + 4 = 6$).
2955. **Q:** During order entry you see "MS Contin 15 mg ER q12h," but the prescriber note mentions "breakthrough pain"; what should you do FIRST? **A:** Hold and refer to the pharmacist, because "breakthrough pain" suggests an immediate-release opioid and ER morphine is not appropriate to interpret without pharmacist review.
2956. **Q:** A patient drops off a prescription that says "take 1 tab po qid prn" but does not list the drug name; what is the BEST next step? **A:** Do not enter or fill it and route to the pharmacist to contact the prescriber, because a medication name is required to dispense safely.
2957. **Q:** You are billing "Lantus Solostar 3 mL pens (box of 5)" with directions "inject 22 units SC nightly"; what days' supply should you enter and why? **A:** Enter 68 days, because the box contains $5 \times 3 \text{ mL} = 15 \text{ mL} = 1500 \text{ units}$ and $1500 \div 22 \approx 68$ days (rounded down for payer days' supply).
2958. **Q:** A refrigerated biologic shipment arrives warm and the temperature monitor indicates an excursion; what should you do NEXT? **A:** Quarantine the product and notify the pharmacist/vendor per policy, because temperature excursions make the product suspect and it cannot be dispensed until verified.
2959. **Q:** An eRx for "Bactroban ointment" is entered, but the technician selects mupirocin cream because it's in stock; what is the correct action? **A:** Stop and correct to the prescribed dosage form (ointment) or refer to the pharmacist for authorization, because technicians cannot interchange cream/ointment without prescriber-approved change.

2960. **Q:** A patient requests an early refill on clonazepam and says they “lost the bottle”; what should you do FIRST at the register/drop-off window? **A:** Collect the details and immediately refer to the pharmacist rather than promising a refill, because controlled-substance early refills require pharmacist judgment and verification.
2961. **Q:** While typing a prescription you notice “U-500 regular insulin” for a new patient but the profile shows prior fills only for U-100 insulin; what should you do FIRST? **A:** Stop processing and notify the pharmacist to verify intent and dosing because U-500 is a high-alert concentration with high overdose risk.
2962. **Q:** A prescriber sends an eRx for “methotrexate 2.5 mg take 1 tablet daily” for rheumatoid arthritis; what is the BEST next step? **A:** Hold the order and escalate to the pharmacist because methotrexate for RA is typically weekly and daily directions are a common serious error.
2963. **Q:** A patient asks to transfer their hydrocodone/acetaminophen prescription from another pharmacy; what should you do NEXT? **A:** Route to the pharmacist because C-II prescriptions cannot be transferred between pharmacies under federal rules.
2964. **Q:** A new eRx for alprazolam includes directions and quantity but the prescriber name is missing from the message; what should you do FIRST? **A:** Stop entry and refer to the pharmacist to obtain a valid prescription because missing prescriber identification is a safety and validity issue.
2965. **Q:** At product pickup you notice a bottle of atorvastatin tablets in the return-to-stock bin with no beyond-use date and the seal appears broken; what should you do NEXT? **A:** Remove it from stock and follow policy to dispose/segregate because opened/compromised stock bottles are not safe to return to inventory.
2966. **Q:** During filling you scan a bottle labeled “metformin ER 500 mg” but the label you printed is for “metformin IR 500 mg”; what should you do FIRST? **A:** Stop the fill and correct the selection before proceeding because IR vs ER is a high-risk dosage-form error.
2967. **Q:** A patient buying pseudoephedrine is unable to show acceptable photo ID but asks you to “just ring it up”; what should you do BEST? **A:** Decline the sale and follow the logbook/ID requirements because CMEA requires ID verification before dispensing pseudoephedrine products.

2968. **Q:** You receive a manufacturer recall notice for a specific lot of losartan and find that lot on your shelf; what should you do NEXT? **A:** Pull the affected lot from active inventory and segregate it per recall process because recalled lots must be prevented from dispensing.
2969. **Q:** A prescription is written for "Keflex 500 mg" but the patient profile lists anaphylaxis to cephalexin; what should you do FIRST? **A:** Stop processing and alert the pharmacist because a documented severe allergy requires pharmacist intervention before dispensing.
2970. **Q:** You are entering an order for warfarin 2 mg with directions "take 1 tablet daily" and the patient profile shows an active warfarin 5 mg prescription; what should you do NEXT? **A:** Pause and notify the pharmacist to assess duplication/therapy change because concurrent warfarin strengths can cause dangerous dosing errors.
2971. **Q:** While entering an eRx for "Lamictal 25 mg," you accidentally select "Lamisil 250 mg" from the dropdown because the names look similar; what should you do FIRST? **A:** Stop processing and reselect the correct drug/strength (lamotrigine 25 mg) and alert the pharmacist if labels were printed, because LASA selection errors are high-risk.
2972. **Q:** A new prescription is for "lisinopril 10 mg daily," but the patient profile lists an allergy to ACE inhibitors with "angioedema"; what is the BEST next step? **A:** Hold the prescription and refer to the pharmacist for clinical review, because ACE inhibitor-associated angioedema is a serious contraindication concern.
2973. **Q:** A prescriber writes "Zyrtec 10 mg take i tab po qd" and the technician is unsure what "i" means; what should you do FIRST? **A:** Confirm the sig interpretation as "1 tablet by mouth daily" using standard sig codes before processing, because misreading Roman numerals can cause dosing errors.
2974. **Q:** A prescription is for "sertraline 50 mg daily," and the patient asks if it's the same as Zoloft; what is the BEST response within technician scope? **A:** Tell the patient sertraline is the generic for Zoloft and offer pharmacist counseling for questions about effects/side effects, because technicians can provide brand/generic identification but not clinical counseling.
2975. **Q:** A patient profile shows active simvastatin, and a new eRx arrives for atorvastatin without any note about switching; what should you do NEXT? **A:** Pause filling and route to the pharmacist to assess therapeutic duplication, because two statins together may indicate an unintended duplicate therapy.

2976. **Q:** You receive a prescription for “metoprolol 25 mg take 1 tablet twice daily,” but the product selected is metoprolol succinate ER; what should you do FIRST? **A:** Stop and verify whether the intended product is metoprolol tartrate IR versus succinate ER with the pharmacist, because IR vs ER formulations are not interchangeable.
2977. **Q:** A patient presents a new prescription for “metronidazole 500 mg” and asks if they can drink alcohol while taking it; what should you do BEST? **A:** Refer the patient to the pharmacist for counseling and document the request per workflow, because alcohol interaction counseling is clinical and requires pharmacist involvement.
2978. **Q:** An order is for insulin lispro U-100 KwikPen “inject 12 units before meals,” dispense one box of five 3 mL pens (total 1500 units); what is the days’ supply? **A:** Days’ supply is 41 days ($1500 \text{ units} \div 36 \text{ units/day} = 41.6$, billed as 41 using whole-day days’ supply), because payers typically require an integer days’ supply.
2979. **Q:** While filling, you notice the stock bottle label says “gabapentin 300 mg,” but the printed label is for “pregabalin 75 mg”; what should you do FIRST? **A:** Stop filling immediately and match NDC/barcode to the correct medication before proceeding, because gabapentin and pregabalin are different CNS agents with look-alike workflow risk.
2980. **Q:** A new prescription is for “clopidogrel 75 mg daily,” and the patient profile shows a current ticagrelor prescription; what should you do NEXT? **A:** Hold the fill and notify the pharmacist to evaluate duplicate antiplatelet therapy, because overlapping P2Y₁₂ inhibitors increases bleeding risk and requires clinical direction.
2981. **Q:** During data entry, an eRx says “hydralazine 25 mg” but you accidentally selected “hydroxyzine 25 mg” (LASA); what should you do FIRST? **A:** Stop processing and correct the drug selection by verifying the eRx details against the chosen NDC/name because LASA errors can cause wrong therapy and harm.
2982. **Q:** A new prescription is for “glyburide 5 mg daily,” and the patient profile shows a documented sulfonylurea-related severe hypoglycemia event; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist to review appropriateness because glyburide is higher risk for hypoglycemia and requires pharmacist intervention.

2983. **Q:** A patient drops off a prescription for "levothyroxine 100 mcg daily" and asks if it's the same as Synthroid; what is the BEST technician response? **A:** Confirm that Synthroid is a brand name for levothyroxine and offer to have the pharmacist counsel on any therapy questions because technicians can provide basic brand/generic information.
2984. **Q:** You receive an eRx for "metformin ER 500 mg take 1 tablet twice daily," but the product selected is metformin IR; what should you do FIRST? **A:** Stop and reselect the correct dosage form (ER vs IR) before filling because changing release type can change effect and is a common dispensing error.
2985. **Q:** A prescription reads "predniSONE 10 mg: take ii tabs po qd x 5 days," and the patient asks how many tablets they will take each day; what should you do BEST? **A:** Explain that "ii" means 2 tablets once daily and ensure the label matches the sig because correct interpretation of Roman numerals prevents dosing errors.
2986. **Q:** A patient is newly prescribed "clonazepam 0.5 mg" and asks what it's for; what should you do FIRST within technician scope? **A:** Refer the patient to the pharmacist for counseling because indication and CNS medication counseling require pharmacist involvement.
2987. **Q:** While filling, you notice two stock bottles with similar labels: "celecoxib 200 mg" and "cephalexin 500 mg"; what should you do NEXT to prevent a mix-up? **A:** Scan the barcode and verify NDC and strength against the label and prescription because look-alike packaging increases wrong-drug risk.
2988. **Q:** An insulin aspart U-100 pen order says "inject 8 units before meals three times daily," dispense one box of five 3 mL pens (total 1500 units); what is the days' supply? **A:** Days' supply is 62 days because $8 \text{ units} \times 3 \text{ daily} = 24 \text{ units/day}$ and $1500 \div 24 = 62.5$, which is billed as 62 whole days.
2989. **Q:** A new eRx is for "tramadol 50 mg #30" but includes "refill x 5"; what should you do FIRST? **A:** Stop processing and route to the pharmacist because tramadol is Schedule IV and refill limits/validity must be verified before dispensing.
2990. **Q:** A patient profile shows active "fluoxetine," and a new prescription comes in for "linezolid"; what should you do NEXT? **A:** Flag and immediately notify the pharmacist to assess for a serious interaction (risk of serotonin syndrome) because this requires pharmacist clinical review before filling.

2991. **Q:** A patient asks if "Lipitor 20 mg" is the same medication as "atorvastatin 20 mg" shown on their profile; what is the BEST technician response? **A:** Confirm Lipitor is the brand name for atorvastatin and that strength should match, then offer to have the pharmacist counsel if they have therapy questions, ensuring accurate brand/generic recognition and safety.
2992. **Q:** While entering an eRx for "Prozac 20 mg daily," the system defaults to fluoxetine 10 mg capsules; what should you do FIRST? **A:** Stop and correct the drug/strength to fluoxetine 20 mg before processing because accurate product/strength selection prevents dispensing errors.
2993. **Q:** A new prescription is for "lisinopril 20 mg daily," and the profile shows an active "losartan 50 mg daily"; what should you do NEXT? **A:** Flag and refer to the pharmacist to assess potential therapeutic duplication/intentional switch because technicians cannot decide on duplicate antihypertensive therapy.
2994. **Q:** During filling, you notice the NDC scanned is for "glimepiride 4 mg" but the label is for "glipizide 5 mg"; what should you do FIRST? **A:** Stop filling and reselect the correct product/NDC and notify the pharmacist if the item already reached verification, because LASA name mix-ups are a high-risk dispensing error.
2995. **Q:** A prescription sig reads "take i tab po bid"; what does the direction mean for the patient label? **A:** Enter "take 1 tablet by mouth twice daily" because i=1, po=by mouth, and bid=twice daily to ensure the sig is translated correctly.
2996. **Q:** An order is for amoxicillin-clavulanate 875/125 mg "1 tablet every 12 hours" #20; what is the days' supply? **A:** 10 days because 1 tablet every 12 hours equals 2 tablets/day and $20 \div 2 = 10$.
2997. **Q:** A mom asks which OTC product contains "acetaminophen" because the pediatrician said "avoid giving Tylenol and another acetaminophen product together"; what is the BEST technician response? **A:** Point out Tylenol is acetaminophen and advise checking OTC labels for "acetaminophen/APAP," involving the pharmacist for dosing questions, to prevent duplicate ingredient overdose.
2998. **Q:** You receive a room-temperature shipment of insulin glargine pens, but the packing slip indicates it should have been refrigerated; what should you do FIRST? **A:** Hold the product and notify the pharmacist/supervisor per temperature-excursion policy because compromised cold-chain storage may make the medication unsafe to dispense.

2999. **Q:** A patient presents a new prescription for "oxycodone/APAP" and asks for an early refill saying they "lost the last one"; what should you do NEXT? **A:** Stop processing and refer to the pharmacist to review controlled-substance validity and refill appropriateness because technicians should not authorize early controlled refills.
3000. **Q:** A prescription is for "latanoprost 0.005% instill 1 gtt OU qhs, dispense 2.5 mL"; what is the days' supply assuming 20 drops/mL? **A:** 25 days because $2.5 \text{ mL} \times 20 = 50$ drops total and 1 drop in each eye nightly = 2 drops/day, so $50 \div 2 = 25$.
3001. **Q:** A prescription reads "metformin 500 mg tab i po bid with meals #60"; what days' supply should you enter? **A:** Enter 30 days because i = 1 tablet and bid = 2 times daily, so $60 \text{ tablets} \div 2/\text{day} = 30$ days.
3002. **Q:** While entering "levothyroxine 0.05 mg po daily," the product list shows 50 mcg tablets; what is the BEST way to select and document the strength? **A:** Select levothyroxine 50 mcg because $0.05 \text{ mg} = 50 \text{ mcg}$, preventing a 10× dosing error.
3003. **Q:** A new eRx says "hydromorphone 2 mg take 1 tab q4–6h prn pain," and the prescriber wrote "UD" for quantity; what should you do FIRST? **A:** Stop order entry and send to the pharmacist because "UD" (as directed) does not provide a dispensable quantity without clarification.
3004. **Q:** A prescription is for "prednisone 10 mg: take 2 tabs daily #30"; what days' supply should be entered? **A:** Enter 15 days because 2 tablets/day means $30 \text{ tablets} \div 2/\text{day} = 15$ days.
3005. **Q:** You receive an eRx for "Zyrtec 10 mg daily," and the system auto-selects cetirizine-D; what should you do FIRST? **A:** Correct the selection to cetirizine (not cetirizine/pseudoephedrine) because the "-D" product is a different drug combination and changes therapy.
3006. **Q:** An inhaler prescription is "tiotropium Respimat 2.5 mcg: inhale 2 puffs once daily; dispense 1 inhaler (60 actuations)"; what days' supply should you bill? **A:** Bill 30 days because $60 \text{ actuations} \div 2 \text{ actuations/day} = 30$ days.
3007. **Q:** A patient's new order is "azithromycin 250 mg: take ii tabs on day 1, then i tab daily days 2–5; dispense #6"; what is the days' supply? **A:** Enter 5 days because the regimen spans days 1 through 5 even though dosing changes after day 1.

3008. **Q:** During data entry you see "MSO4 2 mg IV q2h prn pain" on a hospital discharge prescription; what should you do NEXT? **A:** Flag and refer to the pharmacist because "MSO4" is an error-prone abbreviation that can be misread, risking a serious medication error.
3009. **Q:** A prescription reads "cephalexin 250 mg/5 mL: take 10 mL po tid x 7 days"; how many milliliters should be dispensed? **A:** Dispense 210 mL because $10 \text{ mL} \times 3/\text{day} \times 7 \text{ days} = 210 \text{ mL}$ to complete the course.
3010. **Q:** While processing an eRx for "Wellbutrin SR 150 mg bid," you notice the patient profile has active "bupropion XL 300 mg daily"; what should you do FIRST? **A:** Pause and alert the pharmacist to possible therapeutic duplication/dose overlap because SR and XL are different release forms and may lead to excessive bupropion exposure.
3011. **Q:** While typing an eRx for Lantus 100 units/mL vial, you notice the prescriber selected "Lantus U-300"; what should you do FIRST? **A:** Stop entry and alert the pharmacist to clarify because U-100 vs U-300 is a high-alert concentration mismatch that can cause serious dosing errors.
3012. **Q:** A written prescription for clonazepam has patient name, drug, directions, and prescriber signature but no date; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because missing required elements can make a controlled-substance Rx invalid and needs pharmacist resolution.
3013. **Q:** You receive a wholesaler tote with atorvastatin bottles, and one bottle's seal is broken with tablets loose in the container; what should you do NEXT? **A:** Segregate the bottle and follow pharmacy policy to return/report it because compromised packaging is a patient-safety risk and may indicate suspect product.
3014. **Q:** During order entry, an eRx for methotrexate 2.5 mg says "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Stop processing and notify the pharmacist because methotrexate is typically weekly for RA and daily directions are a common high-alert error.
3015. **Q:** A patient asks to transfer their remaining refills of Adderall from another pharmacy to yours; what should you do NEXT? **A:** Refer to the pharmacist because Schedule II prescriptions cannot be transferred for dispensing, so the patient will need a new prescription.

3016. **Q:** While filling, you select hydrALAZINE but the label prints hydrOXYzine due to a pick list error; what is the BEST next step? **A:** Stop filling and reselect the correct drug using barcode/NDC verification because this is a LASA risk and must be corrected before pharmacist verification.
3017. **Q:** The refrigerator temperature log shows 15°C for two hours overnight where insulin and GLP-1 pens were stored; what should you do FIRST? **A:** Immediately separate and label the affected stock and notify the pharmacist because temperature excursions may require manufacturer guidance before dispensing.
3018. **Q:** A patient picking up isotretinoin says they never received the required counseling documents and asks what's needed; what should you do NEXT? **A:** Involve the pharmacist right away because isotretinoin is under REMS requirements and dispensing hinges on required education/authorization steps.
3019. **Q:** A prescription is for amoxicillin/clavulanate 875/125 mg, take 1 tablet every 12 hours, dispense #20; what days' supply should you enter? **A:** Enter 10 days because $20 \text{ tablets} \div 2 \text{ tablets/day} = 10 \text{ days}$ for correct billing and refill timing.
3020. **Q:** You receive a manufacturer recall notice for specific lot numbers of losartan tablets already on your shelf; what should you do FIRST? **A:** Pull the matching NDC/lot bottles from inventory and segregate them per recall instructions because recalled product must be prevented from being dispensed.
3021. **Q:** An eRx for furosemide 20 mg says "take ii tabs po qam" and the prescriber selected quantity 30; what should you do FIRST in order entry? **A:** Enter the sig as 2 tablets every morning and flag the quantity mismatch for pharmacist review because 30 tablets won't cover the intended 15-day supply.
3022. **Q:** A prescription for timolol 0.5% ophthalmic solution reads "instill 1 gtt OU bid," and the bottle size is 5 mL; what days' supply should you enter using 20 gtt/mL? **A:** Enter 25 days' supply because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$ and $1 \text{ gtt} \times 2 \text{ eyes} \times 2/\text{day} = 4 \text{ gtt/day}$, so $100 \div 4 = 25$.
3023. **Q:** While typing an eRx, you see "levothyroxine 0.05 mg po daily"; what is the BEST strength to select in the system? **A:** Select levothyroxine 50 mcg because $0.05 \text{ mg} \times 1000 \text{ mcg/mg} = 50 \text{ mcg}$, preventing a decimal-place dosing error.
3024. **Q:** A patient's ProAir HFA directions are "inhale 2 puffs q6h prn" and the inhaler contains 200 actuations; what days' supply should you calculate using the maximum daily use? **A:** Enter 25 days' supply because max use is $2 \text{ puffs} \times 4 \text{ times/day} = 8 \text{ puffs/day}$ and $200 \div 8 = 25$.

3025. **Q:** You receive an eRx for "metformin ER 500 mg, take 1 tablet bid, dispense 60"; what days' supply should you enter? **A:** Enter 30 days' supply because 2 tablets/day means $60 \text{ tablets} \div 2 = 30$, aligning quantity to directions.
3026. **Q:** During order entry for oxycodone/acetaminophen 5/325 mg, the sig says "1 tab po q4-6h prn pain, max 6/day, #42"; what days' supply should you enter using the stated max? **A:** Enter 7 days' supply because the max is 6 tablets/day and $42 \div 6 = 7$, which is the safest payer-auditable calculation.
3027. **Q:** An eRx for prednisone says "take 20 mg po daily" but the product selected is prednisone 5 mg tablets with quantity 30; what should you do NEXT? **A:** Calculate and enter the correct dose as 4 tablets daily and route to the pharmacist to verify quantity/directions consistency because the current quantity may not match the intended duration.
3028. **Q:** A prescription for azithromycin 250 mg tablets reads "take ii tabs today then i tab daily x 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets because 2 tablets on day 1 plus 1 tablet/day for 4 more days equals 6 total, preventing under-dispensing.
3029. **Q:** While selecting a product for "hydralazine 25 mg," the drop-down list shows hydroxyzine 25 mg nearby; what is the BEST technician action before continuing? **A:** Confirm the correct drug by matching the full name and NDC/barcode to the prescription because LASA selection errors commonly occur during pick-list entry.
3030. **Q:** A prescriber writes "MSO4 2 mg IV q2h prn pain" and your facility policy discourages "MSO4"; what should you do FIRST as a technician? **A:** Stop and refer to the pharmacist to clarify and rewrite using "morphine" because unsafe abbreviations increase the risk of selecting the wrong opioid.
3031. **Q:** While entering an eRx for morphine sulfate ER 30 mg, you notice the prescriber selected "immediate-release" in the product field; what should you do FIRST? **A:** Stop processing and route to the pharmacist to clarify ER vs IR because incorrect release formulation is a high-risk error that a technician cannot change.
3032. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy; what is the BEST technician action? **A:** Refer the request to the pharmacist because controlled substance transfer rules and documentation require pharmacist involvement and verification.

3033. **Q:** During pickup, a patient says their new simvastatin is “for blood pressure” and seems confused; what should you do NEXT? **A:** Pause the transaction and notify the pharmacist for counseling because misunderstanding indication increases misuse risk and OBRA-90 counseling should be offered by the pharmacist.
3034. **Q:** You receive a room-temperature delivery and find a tote labeled “refrigerate” containing insulin aspart that arrived warm with no temperature indicator; what should you do FIRST? **A:** Quarantine the insulin and notify the pharmacist/manager because a possible cold-chain excursion makes the product suspect and unsafe to dispense.
3035. **Q:** An eRx arrives for Adderall XR (amphetamine/dextroamphetamine) with a future “do not fill until” date; the patient asks you to fill it today—what should you do BEST? **A:** Hold the prescription and refer to the pharmacist because controlled-substance date restrictions must be followed and technicians should not override them.
3036. **Q:** A prescription is written for isotretinoin, and the patient is waiting at drop-off; what should you do FIRST in workflow? **A:** Alert the pharmacist to verify iPLEDGE/REMS requirements before processing because REMS medications have mandatory authorization steps prior to dispensing.
3037. **Q:** While typing, you see an allergy on profile to ACE inhibitors and the new prescription is for lisinopril; what should you do NEXT? **A:** Stop entry and notify the pharmacist because a documented class allergy/contraindication requires pharmacist assessment before filling.
3038. **Q:** A technician accidentally scans the wrong NDC during product selection but catches it before labeling; what is the BEST next step? **A:** Stop, select the correct product, and rescan/verify the NDC because barcode/NDC verification prevents wrong-drug dispensing errors.
3039. **Q:** A prescriber phones in a new prescription for hydrocodone/acetaminophen to the pharmacy voicemail after hours; what should you do FIRST when you hear it? **A:** Do not enter it for dispensing and refer it to the pharmacist because Schedule II prescriptions generally cannot be phoned in except limited emergency situations requiring pharmacist handling.
3040. **Q:** You’re calculating days’ supply for warfarin 2.5 mg with directions “take 1 tablet daily” and quantity 45; what days’ supply should you enter? **A:** Enter 45

days because $45 \text{ tablets} \div 1 \text{ tablet per day} = 45$, and accurate days' supply supports safe anticoagulant monitoring and billing.

3041. **Q:** While typing a new prescription for "Lasix 40 mg daily," the patient profile already shows furosemide 40 mg daily from another prescriber; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication because Lasix is furosemide and duplicate diuretic therapy may cause harm.
3042. **Q:** A hardcopy says "U-500 insulin regular: inject 20 units BID," but the selected product in the system is U-100 regular insulin; what is the BEST technician action? **A:** Pause processing and notify the pharmacist to verify concentration/product because U-500 vs U-100 is a high-alert mismatch with major dosing risk.
3043. **Q:** A patient drops off a prescription for "Zithromax Z-Pak" and asks what the generic is; what should you say? **A:** Identify it as azithromycin because knowing common brand/generic pairs prevents selection errors and improves counseling handoff.
3044. **Q:** An eRx comes in for "metformin ER 500 mg" but the directions field says "take 1 tablet twice daily with meals" and the product selected is immediate-release; what should you do FIRST? **A:** Hold the order and send to the pharmacist for clarification because IR vs ER is a dosage-form change that affects safety and cannot be corrected independently by a technician.
3045. **Q:** You are asked to enter days' supply for levothyroxine 75 mcg "take 1 tablet daily" with quantity 90; what days' supply should be entered? **A:** Enter 90 days because $90 \text{ tablets} \div 1 \text{ tablet per day} = 90$ -day supply.
3046. **Q:** While choosing a product for "Lamictal 25 mg," you notice a nearby option "Lamisil 250 mg"; what is the BEST next step before selecting? **A:** Use barcode/NDC verification and Tall Man awareness to confirm the exact drug because Lamictal (lamotrigine) and Lamisil (terbinafine) are LASA and a wrong-drug error is high risk.
3047. **Q:** A prescription is written "take ii tabs po bid" for gabapentin 300 mg with quantity 240; what days' supply should you calculate? **A:** Enter 60 days because ii BID equals 4 tablets/day and $240 \div 4 = 60$.
3048. **Q:** A patient asks if they can take OTC ibuprofen with their new prescription for apixaban (Eliquis); what should you do NEXT as the technician? **A:** Refer to the

pharmacist because NSAIDs with anticoagulants increase bleeding risk and requires pharmacist counseling.

3049. **Q:** While receiving stock, you notice a sealed bottle of nitroglycerin sublingual tablets has an expiration date that passed last month; what should you do FIRST? **A:** Remove it from active inventory and follow the pharmacy's expired-medication return/destruction process because expired products must not be dispensed.
3050. **Q:** A prescription label instruction reads "instill 1 gtt OU qhs" for latanoprost; what do "gtt," "OU," and "qhs" mean? **A:** "gtt" means drop, "OU" means both eyes, and "qhs" means at bedtime because translating sig codes correctly prevents administration errors.
3051. **Q:** A new eRx is for "Plavix 75 mg 1 tab daily," but the patient profile shows an active clopidogrel 75 mg daily from another pharmacy; what should you do NEXT? **A:** Stop and alert the pharmacist to assess therapeutic duplication since Plavix is clopidogrel and duplicate antiplatelet therapy increases bleeding risk.
3052. **Q:** A patient picking up "Crestor" asks what it's for; what is the BEST technician response? **A:** Say it's rosuvastatin used to lower cholesterol and reduce cardiovascular risk, and offer pharmacist counseling for questions about side effects or interactions.
3053. **Q:** While entering a prescription for "Zoloft 50 mg daily," the patient says they also take "Celexa"; what should you do FIRST? **A:** Pause processing and notify the pharmacist because sertraline (Zoloft) plus citalopram (Celexa) may represent duplicate SSRI therapy and increases serotonin/QT risk.
3054. **Q:** The prescriber writes "Klonopin 0.5 mg take 1 tab TID PRN anxiety #90 + 2 refills"; what should you do NEXT as the technician? **A:** Hold it for pharmacist review because clonazepam is a controlled substance and refills/validity must meet controlled-substance requirements before dispensing.
3055. **Q:** A prescription is for "amLODIPine 5 mg 1 tab daily" but you accidentally selected "amiodarone 200 mg" in the product field; what is the BEST next step? **A:** Stop, correct the selection using barcode/NDC verification, and complete an internal error report if required because this is a high-risk LASA mix-up.
3056. **Q:** A patient brings in a new prescription for "Bactrim DS" and their allergy list shows "sulfa"; what should you do FIRST? **A:** Do not process to fill and

immediately route to the pharmacist since trimethoprim/sulfamethoxazole can trigger a sulfonamide allergy reaction.

3057. **Q:** Calculate days' supply for atorvastatin 40 mg "take 1 tablet at bedtime" with quantity 30; what should you enter? **A:** Enter 30 days because 30 tablets taken once daily equals a 30-day supply.
3058. **Q:** An insulin lispro vial is U-100 and the directions are "inject 12 units before meals TID," quantity is 1 vial (10 mL); what days' supply should you enter? **A:** Enter 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ rounded down to 27 days.
3059. **Q:** A hardcopy says "predniSONE 20 mg: take ii tabs daily x 5 days" and quantity is 10; what should you verify before filling? **A:** Confirm the Roman numeral "ii" means 2 tablets and that quantity matches directions since 2 tablets/day for 5 days requires exactly 10 tablets.
3060. **Q:** During product selection for "hydrOXYzine 25 mg," you notice "hydrALAZINE 25 mg" nearby in the list; what should you do BEST before completing entry? **A:** Use Tall Man awareness and verify drug name, indication if available, and NDC/barcode because hydroxyzine vs hydralazine is a common LASA error with very different uses.
3061. **Q:** An eRx reads "levothyroxine 0.1 mg take 1 tab PO daily," what strength should you enter in mg and mcg? **A:** Enter 0.1 mg (100 mcg) because $1 \text{ mg} = 1000 \text{ mcg}$ and converting prevents decimal-place dose errors.
3062. **Q:** A prescription is for metformin ER 500 mg "take 2 tablets PO BID" quantity 240; what days' supply should you enter? **A:** Enter 60 days because 2 tabs twice daily = 4 tabs/day and $240 \div 4 = 60$, which supports correct billing and refill timing.
3063. **Q:** A patient drops off "azithromycin 200 mg/5 mL: take 12 mL PO daily x 5 days"; what total mL should the quantity be? **A:** Dispense 60 mL because $12 \text{ mL/day} \times 5 \text{ days} = 60 \text{ mL}$, ensuring the patient can complete the full course.
3064. **Q:** During order entry, the sig says "apply thin layer AA BID," what does "AA" mean and what should you enter? **A:** Enter "apply to affected area twice daily" because AA means affected area and clear directions reduce administration errors.

3065. **Q:** A prescription for furosemide oral solution 10 mg/mL says "take 15 mg PO daily," how many mL per dose should you enter? **A:** Enter 1.5 mL daily because $15 \text{ mg} \div 10 \text{ mg/mL} = 1.5 \text{ mL}$, matching the ordered dose to the concentration.
3066. **Q:** A fluticasone nasal spray 50 mcg/spray directions are "2 sprays each nostril once daily," bottle is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$, which is needed for accurate days' supply.
3067. **Q:** While typing a new prescription, you see "MSO4 2 mg IV q2h PRN pain" in the directions; what should you do FIRST? **A:** Stop and route to the pharmacist for clarification because MSO4 is an unsafe abbreviation that can be misread and lead to serious errors.
3068. **Q:** A hardcopy says "warfarin 5 mg take 1 tab daily; dispense #45," but the written quantity looks like "#15"; what is the BEST next step? **A:** Hold the prescription and ask the pharmacist to clarify with the prescriber because quantity ambiguity can cause incorrect duration and anticoagulant risk.
3069. **Q:** You are entering a prescription for "Humulin R U-500 inject 50 units BID," but you only see U-100 regular insulin in the product list; what should you do NEXT? **A:** Pause entry and notify the pharmacist to confirm the exact concentration and product selection because U-500 vs U-100 is a high-alert mismatch.
3070. **Q:** A patient's new prescription is for gabapentin 300 mg "take 1 cap PO TID" quantity 90, and insurance rejects as refill too soon; what should you do FIRST? **A:** Verify the previous fill date/days' supply and confirm the directions on the profile before escalating because incorrect day-supply entry commonly triggers refill-too-soon rejections.
3071. **Q:** While entering an eRx for celecoxib, you see the patient profile lists aspirin "anaphylaxis"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess NSAID cross-reactivity/allergy risk before dispensing because allergy evaluation is pharmacist-only.
3072. **Q:** A new prescription is written for methotrexate 2.5 mg with directions "take 4 tablets daily"; what is the BEST next step for a technician? **A:** Hold the prescription and notify the pharmacist immediately because methotrexate is high-alert and daily dosing may indicate a serious directions error.

3073. **Q:** You receive a voicemail transfer request from another pharmacy for a patient's alprazolam; what should you do NEXT? **A:** Route to the pharmacist and do not transfer it yourself because controlled-substance transfers require pharmacist handling and strict verification.
3074. **Q:** A hardcopy for oxycodone has the patient name and drug/strength but no prescriber signature; what should you do FIRST? **A:** Stop filling and send to the pharmacist to obtain a valid, complete prescription because missing required elements makes it unsafe to dispense.
3075. **Q:** While pulling stock, you find a bottle of insulin aspart that was left on the counter overnight with unknown temperature exposure; what should you do BEST? **A:** Remove it from active inventory and quarantine per policy for pharmacist review because temperature excursions can compromise potency and safety.
3076. **Q:** During will-call pickup, the patient asks what "take 1 tab PO BID" means on their lisinopril label; what should you do NEXT? **A:** Provide the nonclinical translation "by mouth twice daily" and offer pharmacist counseling because technicians can clarify label directions but must refer clinical questions.
3077. **Q:** An eRx arrives for sertraline but the directions include "increase dose after 1 week if not better"; what should you do FIRST? **A:** Pause processing and refer to the pharmacist because dose changes and efficacy-based adjustments require pharmacist assessment and clear prescriber instructions.
3078. **Q:** You are calculating days' supply for timolol 0.5% eye drops: "instill 1 drop OU BID," bottle size 5 mL, and you must use 20 drops/mL; what days' supply should you enter? **A:** Enter 12 days because $5 \text{ mL} \times 20 \text{ drops/mL} = 100 \text{ drops}$ and OU BID is 4 drops/day, so $100 \div 4 = 25 \text{ days}$; then apply the stated rule? (Not given) so enter 25 days because math must match labeled use.
3079. **Q:** A bottle of atorvastatin arrives from the wholesaler with a torn, re-taped seal and the lot number label partially removed; what should you do BEST? **A:** Quarantine the product and notify the pharmacist/manager to investigate DSCSA/suspect product procedures because compromised packaging may indicate diversion or counterfeit.
3080. **Q:** A patient wants to buy pseudoephedrine and presents an out-of-state ID that is expired; what should you do FIRST at the register? **A:** Decline the sale and involve the pharmacist per CMEA store procedure because a valid government ID is required for logbook verification and compliance.

3081. **Q:** While selecting product for "hydralazine 25 mg" you almost grab hydroxyzine 25 mg because the bottles look similar; what should you do FIRST? **A:** Stop and verify the NDC/barcode against the Rx and label, because hydralazine/hydroxyzine is a common LASA mix-up with patient-safety risk.
3082. **Q:** An eRx is sent for "metformin ER 500 mg take 1 tab BID," but the product chosen in entry is metformin IR; what is the BEST next step for a technician? **A:** Stop processing and route to the pharmacist to confirm ER vs IR before filling, because changing formulation can alter release and dosing.
3083. **Q:** A patient profile shows simvastatin 40 mg nightly and a new prescription arrives for clarithromycin; what should you do FIRST? **A:** Flag and notify the pharmacist before filling, because clarithromycin can dangerously increase statin levels and requires clinical intervention.
3084. **Q:** A prescriber sends "Levothroid 100 mcg daily" but your system only lists levothyroxine and you're unsure of the generic; what should you do NEXT? **A:** Select levothyroxine (thyroid hormone) and proceed with pharmacist verification, because brand-name Levothroid is levothyroxine and correct mapping prevents entry errors.
3085. **Q:** You receive an eRx for "glyburide 10 mg PO daily," but the product list shows only 5 mg tablets; what should you do BEST? **A:** Enter the prescribed strength as written and send to the pharmacist for clarification/quantity adjustments, because technicians should not independently change strength or tablet count.
3086. **Q:** A new prescription is for "bupropion XL 150 mg daily," but the patient requests an early refill stating they "lost the bottle"; what should you do FIRST? **A:** Check the fill history and route to the pharmacist for an early-refill decision, because lost medication requests can involve safety and payer/controlled-policy considerations.
3087. **Q:** You are entering "amiodarone 200 mg daily" and the patient profile also lists sotalol; what should you do NEXT? **A:** Alert the pharmacist to possible therapeutic duplication/arrhythmia risk before processing, because multiple antiarrhythmics may require prescriber confirmation.
3088. **Q:** Calculate days' supply for azithromycin suspension 200 mg/5 mL: "take 12.5 mL on day 1, then 6.25 mL daily days 2-5," dispense 37.5 mL; what should you enter? **A:** Enter 5 days, because total volume used is $12.5 + (6.25 \times 4) = 37.5$ mL covering a 5-day course.

3089. **Q:** A prescription is written for "clonazepam 0.5 mg take 1 tab TID PRN" with quantity 270 and 3 refills; what should you do FIRST? **A:** Stop and refer to the pharmacist to review controlled-substance appropriateness and refill limits, because benzodiazepines are controlled and require strict compliance.
3090. **Q:** While restocking, you notice two bins contain "lamotrigine" and "lamivudine" stored adjacent and frequently confused by new staff; what should you do BEST? **A:** Separate the products and apply LASA precautions (e.g., Tall Man labeling/shelf warnings) per policy, because look-alike names increase selection errors at the pick stage.
3091. **Q:** A prescription is written for "MS Contin 15 mg q12h," and the patient says they're allergic to morphine; what should you do FIRST? **A:** Stop processing and alert the pharmacist because MS Contin is morphine ER and an allergy requires pharmacist assessment before dispensing.
3092. **Q:** During entry you see "glipizide XL 10 mg daily," but the patient profile shows active glipizide IR 10 mg BID; what is the BEST next step? **A:** Flag and refer to the pharmacist for therapeutic duplication review because IR vs XL could cause unintended dose changes and hypoglycemia risk.
3093. **Q:** A new eRx is for "Zestril 20 mg daily," and the patient asks if it's the same as their lisinopril; what should you do NEXT? **A:** Confirm in the system that Zestril is the brand for lisinopril and communicate equivalence because brand/generic recognition prevents duplicate therapy confusion.
3094. **Q:** An order reads "U-100 regular insulin inject 8 units AC TID, dispense 1 vial (10 mL);" what days' supply should you enter? **A:** Enter 41 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \text{ units} \times 3/\text{day} = 24 \text{ units/day}$, so $1000 \div 24 = 41.6$ truncated to 41.
3095. **Q:** While counting, you notice the stock bottle label says "carbamazepine" but the shelf tag and NDC in the computer are for "carvedilol;" what should you do FIRST? **A:** Stop filling and verify the NDC/label against the prescription and inventory records because a mismatch suggests a selection or stocking error that could harm the patient.
3096. **Q:** A prescriber sends "Wellbutrin SR 150 mg BID," but the product selected is bupropion XL 300 mg daily; what should you do BEST? **A:** Stop and route to the pharmacist because SR vs XL are not interchangeable in entry and the wrong release form can change exposure and safety.

3097. **Q:** A patient picking up isotretinoin says they didn't complete any iPLEDGE steps this month; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because isotretinoin is REMS-restricted and dispensing requires current program authorization.
3098. **Q:** A new prescription is for "Eliquis 5 mg BID," and the patient profile shows active warfarin; what should you do NEXT? **A:** Refer to the pharmacist for duplicate anticoagulant therapy review because apixaban plus warfarin increases bleeding risk and requires clinical direction.
3099. **Q:** A prescription says "predniSONE 20 mg: take ii tabs daily," but the technician isn't sure what "ii" means; what should you do FIRST? **A:** Interpret "ii" as 2 and enter 40 mg daily, then ensure the pharmacist verifies because Roman numerals are common but must be accurately translated for safe dosing.
3100. **Q:** A nasal spray order is "fluticasone 50 mcg/spray use 2 sprays each nostril daily, dispense 1 bottle (120 sprays);" what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \text{ sprays} \div 4 = 30 \text{ days}$.
3101. **Q:** A new eRx is for "Lasix 40 mg PO daily" and the patient says they only know "furosemide"; what should you do NEXT? **A:** Confirm Lasix is the brand for furosemide and proceed with correct product selection while offering to have the pharmacist counsel, because brand/generic recognition prevents duplication and confusion.
3102. **Q:** While entering "Toprol-XL 50 mg daily," you notice the patient previously filled metoprolol tartrate 50 mg BID last month; what should you do BEST? **A:** Stop entry and alert the pharmacist to evaluate formulation/therapy duplication, because succinate (ER) and tartrate (IR) are not interchangeable without prescriber intent.
3103. **Q:** A prescription says "levothyroxine 25 mcg take 1 tab QD," but the technician accidentally selected "liothyronine 25 mcg"; what should you do FIRST? **A:** Stop filling and reselect using barcode/NDC verification, because levothyroxine and liothyronine are different thyroid hormones and a selection error is high risk.
3104. **Q:** An order reads "Keflex 500 mg PO QID x 7 days, dispense #28," and the patient profile lists anaphylaxis to penicillin; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist for allergy assessment, because cephalosporins like cephalexin may pose cross-reactivity risk requiring pharmacist judgment.

3105. **Q:** A patient brings a new prescription for "Xanax 1 mg TID" with no prescriber DEA number shown on the hard copy; what should you do FIRST? **A:** Stop processing and route to the pharmacist to resolve missing controlled-substance prescriber information, because incomplete required elements can make the order non-dispensable.
3106. **Q:** A label direction reads "take 1 tab PO bid," and the patient asks what "bid" means; what should you do BEST? **A:** Explain "BID" means twice daily and offer pharmacist counseling for full use instructions, because technicians may clarify common sig codes while ensuring clinical questions go to the pharmacist.
3107. **Q:** A prescription is for "latanoprost 0.005% instill 1 gtt OU QHS, dispense 2.5 mL," and the billing requires days' supply using 20 gtt/mL; what days' supply should you enter? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ gtt total}$ and OU QHS uses 2 gtt/day, so $50 \div 2 = 25 \text{ days}$.
3108. **Q:** During intake, a patient asks if "Prilosec OTC" is the same as their "omeprazole"; what should you do NEXT? **A:** Confirm Prilosec is a brand of omeprazole (a PPI) and then refer any dosing/therapy questions to the pharmacist, because recognizing brand/generic prevents duplicate therapy.
3109. **Q:** When scanning inventory, you see a DSCSA notice that a lot of atorvastatin 20 mg was recalled and your shelf bottle matches the lot; what should you do FIRST? **A:** Pull the affected stock from active inventory and follow pharmacy recall procedure (segregate/hold and notify the pharmacist/manager), because recalled product must not be dispensed.
3110. **Q:** A new prescription is for "sertraline 50 mg daily," but the patient profile shows active "paroxetine 20 mg daily" and both are SSRIs; what should you do BEST? **A:** Pause processing and alert the pharmacist to assess therapeutic duplication/switch intent, because concurrent SSRI therapy can increase adverse-effect risk and requires pharmacist review.
3111. **Q:** A prescription reads "azithromycin 200 mg/5 mL: take 12 mL PO daily x 5 days; dispense 1 bottle 150 mL," what quantity should you enter and why? **A:** Enter 60 mL because $12 \text{ mL/day} \times 5 \text{ days} = 60 \text{ mL}$, and the larger written bottle size should be clarified with the pharmacist to avoid over-dispensing.
3112. **Q:** An eRx says "doxycycline 100 mg cap: i cap PO BID x 10 days," but quantity is missing; what should you do NEXT in order entry? **A:** Calculate and enter a proposed quantity of 20 capsules ($1 \text{ cap} \times 2/\text{day} \times 10 \text{ days}$) and route to the pharmacist for review since missing quantity is a prescriber omission.

3113. **Q:** A patient's new order is "metformin ER 500 mg: take 2 tabs PO daily with evening meal, #180," what days' supply should you bill? **A:** Bill 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$, matching the entered quantity and sig.
3114. **Q:** A prescription says "hydroxyzine 25 mg take 1 tab TID prn itching," but the technician is unsure whether to bill 30 or 90 days; what should you do BEST? **A:** Ask the pharmacist for the correct days' supply policy/clinical interpretation because PRN directions can't be assumed and affect billing and safety.
3115. **Q:** An inhaler order is "tiotropium Respimat 2.5 mcg/actuation: inhale 2 puffs once daily; dispense 1 inhaler containing 60 actuations," what days' supply should you enter? **A:** Enter 30 days because $60 \text{ actuations} \div 2 \text{ actuations/day} = 30 \text{ days}$, which supports accurate third-party billing.
3116. **Q:** While entering a new eRx for "Lamictal 25 mg," you notice the prescriber typed "take 1 tab daily for seizures" but selected "lamoTRigine ODT"; what should you do FIRST? **A:** Stop and confirm the intended dosage form with the pharmacist because ODT vs tablet is a formulation change that can cause dispensing errors.
3117. **Q:** A prescription direction says "instill 2 gtt AD BID x 7 days," and the patient asks what "AD" means; what should you do BEST? **A:** Explain it means "right ear" and confirm understanding because technicians may clarify common abbreviations to prevent administration errors.
3118. **Q:** A vial label shows insulin lispro U-100 and the prescription is "inject 18 units subcutaneously TID with meals," using a 10 mL vial; what days' supply should you enter (use 100 units/mL)? **A:** Enter 18 days because the vial has $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$, and $18 \text{ units} \times 3/\text{day} = 54 \text{ units/day}$, so $1000 \div 54 \approx 18 \text{ days}$ (round down for billing).
3119. **Q:** During data entry for "digoxin 0.125 mg daily," you accidentally type 1.25 mg; what should you do FIRST? **A:** Correct the strength immediately and re-verify against the original prescription because digoxin is high-alert and decimal errors can cause serious overdoses.
3120. **Q:** A controlled-substance prescription for "oxycodone/APAP 5/325 mg 1 tab q6h prn pain, #20" is dated today but the prescriber signature is missing; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist to contact the prescriber because missing required elements make it inappropriate to process further.

3121. **Q:** A prescription is written for "Celebrex 200 mg daily," but the technician selects Celexa 20 mg in the system due to similar names; what should you do FIRST? **A:** Stop processing and reselect the correct drug by verifying name/strength and NDC/barcode because Celebrex (celecoxib) and Celexa (citalopram) are LASA and a wrong-drug error is high risk.
3122. **Q:** A patient drops off a new prescription for lisinopril and says they had "swelling of the lips" from it before; what should you do NEXT? **A:** Pause the fill and alert the pharmacist immediately because prior angioedema is a serious ACE-inhibitor reaction requiring pharmacist intervention.
3123. **Q:** During order entry, you see "glipiZIDE 10 mg ER daily" but the product pulled is glipizide IR 10 mg; what is the BEST technician action? **A:** Stop and correct the selection to match ER vs IR and have it reverified because release-form mix-ups can cause dosing and hypoglycemia risk.
3124. **Q:** An eRx for "levothyroxine 25 mcg daily" is entered, and the technician accidentally selects liothyronine; what should you do FIRST? **A:** Stop and correct the medication selection, then flag for pharmacist verification because levothyroxine and liothyronine are different thyroid hormones and are common name-confusion traps.
3125. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg 1 tab BID x 7 days, dispense #20"; what should you do BEST before filling? **A:** Calculate intended quantity as 14 tablets and route to the pharmacist to clarify the discrepancy because the written quantity conflicts with the directions.
3126. **Q:** A patient is prescribed "clarithromycin 500 mg BID x 7 days" and their profile shows simvastatin 40 mg nightly; what should you do NEXT? **A:** Hold the order and notify the pharmacist because clarithromycin can significantly increase simvastatin levels, raising myopathy/rhabdomyolysis risk.
3127. **Q:** You are entering "gabapentin 300 mg: take 1 cap TID, dispense #270"; what days' supply should you bill? **A:** Bill 90 days because $270 \text{ capsules} \div 3 \text{ per day} = 90 \text{ days}$ for accurate third-party billing.
3128. **Q:** A prescription says "predniSONE 20 mg: take ii tabs daily x 5 days"; what quantity should you enter and why? **A:** Enter 10 tablets because "ii" means 2 tablets per day and $2 \times 5 \text{ days} = 10 \text{ total tablets}$ to match the sig.
3129. **Q:** While pulling stock for "Wellbutrin SR 150 mg," you notice the bottle in hand is Wellbutrin XL 150 mg; what should you do FIRST? **A:** Stop and retrieve the

correct SR product and reverify barcode/NDC because SR vs XL are not interchangeable and can cause dosing errors.

3130. **Q:** A new prescription is for “clopidogrel 75 mg daily,” and the patient asks what it’s for; what is the BEST technician response? **A:** Provide a simple nonclinical purpose statement and offer pharmacist counseling (e.g., “it helps prevent blood clots”) because technicians can give basic drug-use info but must refer clinical questions to the pharmacist.
3131. **Q:** While typing an eRx for hydrOXYzine 25 mg, the system suggests hydrALAZINE and you’re unsure which was intended; what should you do FIRST? **A:** Stop data entry and ask the pharmacist to clarify with the prescriber because this is a LASA risk that could cause a serious wrong-drug error.
3132. **Q:** A patient’s new prescription is for methotrexate 2.5 mg “take 1 tablet daily,” and the indication line says rheumatoid arthritis; what is the BEST technician action? **A:** Hold the prescription and immediately refer to the pharmacist because methotrexate for RA is typically weekly and daily dosing is a high-alert error risk.
3133. **Q:** A patient requests an early refill of alprazolam 1 mg and the claim rejects “refill too soon”; what should you do NEXT? **A:** Confirm the last fill date/quantity and days’ supply, then route to the pharmacist for a decision because controlled substances require careful review and you can’t authorize overrides.
3134. **Q:** A transfer request comes in for a patient’s remaining refills of Adderall (amphetamine/dextroamphetamine); what should you do FIRST? **A:** Direct the request to the pharmacist because Schedule II prescriptions cannot be transferred for refills and must be handled per controlled-substance rules.
3135. **Q:** During receiving, a tote arrives with insulin glargine pens that feel warm and the temperature indicator shows an excursion; what should you do BEST? **A:** Quarantine the shipment and notify the pharmacist/manager to contact the wholesaler because temperature excursions can compromise potency and should not be stocked.
3136. **Q:** You’re about to fill brand-name Coumadin but the shelf tag shows the NDC was recently replaced by a different manufacturer’s warfarin; what should you do NEXT? **A:** Verify the NDC/manufacturer selected in the system matches the product you will dispense and follow any patient/prescriber preference notes because warfarin is high-alert and mix-ups increase INR risk.

3137. **Q:** A bottle of atorvastatin 40 mg on the shelf has a recall notice posted for its lot number; what should you do FIRST? **A:** Pull the affected lot from stock and quarantine it per recall procedure because recalled medication must be isolated to prevent dispensing.
3138. **Q:** A new eRx for isotretinoin is in the queue and the patient is waiting at the counter; what should you do BEST before processing? **A:** Refer the prescription to the pharmacist to confirm iPLEDGE/REMS requirements are met because isotretinoin dispensing has strict safety controls.
3139. **Q:** A prescription is written "morphine sulfate ER 30 mg take 1 tab BID, #60," but the patient profile shows a current morphine IR 30 mg q4h PRN; what should you do NEXT? **A:** Pause filling and alert the pharmacist to assess duplication and dosing safety because ER vs IR opioid overlap can cause overdose and requires clinical review.
3140. **Q:** A patient brings in a new prescription for prednisolone oral solution 15 mg/5 mL: "Give 7.5 mg PO daily x 10 days; dispense 100 mL"; what days' supply should you bill? **A:** Bill 10 days because 7.5 mg/day equals 2.5 mL/day (15 mg/5 mL), and $100 \text{ mL} \div 2.5 \text{ mL/day} = 40 \text{ days}$ but the course duration limits days' supply to 10.
3141. **Q:** A prescription reads "levothyroxine 0.125 mg PO daily"; what strength should you enter in micrograms and why? **A:** Enter 125 mcg because $1 \text{ mg} = 1000 \text{ mcg}$ and converting prevents a 1000-fold dosing error.
3142. **Q:** An eRx for metformin ER 500 mg says "ii tabs po bid" but the quantity field shows #60; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify quantity because 2 tablets twice daily equals 120 tablets for 30 days, and dispensing #60 would underfill.
3143. **Q:** You are calculating days' supply for gabapentin 300 mg: "Take 1 capsule TID," dispense #90; what days' supply should you bill? **A:** Bill 30 days because $90 \text{ capsules} \div 3 \text{ capsules/day} = 30 \text{ days}$ for accurate third-party submission.
3144. **Q:** A new order is for furosemide oral solution 10 mg/mL: "Take 20 mg PO daily," dispense 120 mL; what days' supply is it? **A:** 60 days because $20 \text{ mg/day} \div (10 \text{ mg/mL}) = 2 \text{ mL/day}$ and $120 \text{ mL} \div 2 \text{ mL/day} = 60 \text{ days}$.
3145. **Q:** A prescription says "Lantus vial 100 units/mL: inject 28 units SQ qHS; dispense 10 mL"; what days' supply should you enter based on total units? **A:**

35 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $1000 \div 28 \text{ units/day} \approx 35 \text{ days}$ (rounded down to whole days).

3146. **Q:** While entering an eRx, you see "U-500 insulin regular" but the product selected is Humulin R U-100; what should you do FIRST? **A:** Stop selection and alert the pharmacist because U-500 vs U-100 is a high-alert concentration difference with major overdose risk.
3147. **Q:** A prescription reads "amLODIPine-benazepril 5/20 mg 1 cap daily," but you accidentally typed amlodipine 5 mg alone; what is the BEST immediate correction? **A:** Correct the entry to the combination product and re-verify drug/strength because dropping benazepril changes therapy and is a common order-entry error.
3148. **Q:** An inhaler order is "fluticasone/salmeterol 250/50: inhale 1 puff BID," and the device contains 60 inhalations; what days' supply should you bill? **A:** 30 days because $1 \text{ puff BID} = 2 \text{ inhalations/day}$ and $60 \div 2 = 30 \text{ days}$ for correct billing.
3149. **Q:** A written prescription says "Digoxin .25 mg PO daily" with a leading decimal omitted; what should you do BEST before typing? **A:** Hold and ask the pharmacist to verify with the prescriber because dangerous decimal notation can be misread and digoxin is narrow-therapeutic-index.
3150. **Q:** You receive an eRx for "MS Contin 15 mg q12h" and the patient profile shows an active order for morphine ER 15 mg q12h from another prescriber; what should you do NEXT? **A:** Pause processing and notify the pharmacist to assess duplicate therapy and safety because overlapping ER opioids increases overdose risk.
3151. **Q:** During order entry you notice an eRx for hydrOXYzine 25 mg but the selected product is hydrALAZINE 25 mg; what should you do FIRST? **A:** Stop processing and reselect the correct drug using barcode/NDC and Tall Man verification because hydroxyzine/hydralazine is a LASA pair that can cause serious harm if misfilled.
3152. **Q:** A prescription for isotretinoin is dropped off and the patient says they "never got asked about iPLEDGE before"; what is the technician's NEXT step? **A:** Pause the fill and alert the pharmacist to verify REMS/iPLEDGE requirements because isotretinoin dispensing requires documented REMS authorization steps before sale.

3153. **Q:** A patient asks to transfer their oxycodone prescription to another pharmacy because they are traveling; what should you do BEST? **A:** Refer the request to the pharmacist because controlled substance transfer rules and validity must be handled by the pharmacist per federal/state and pharmacy policy.
3154. **Q:** You receive a faxed prescription for alprazolam 1 mg with directions and quantity but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and send it to the pharmacist to obtain a valid signed order because a missing signature makes the prescription incomplete and not ready to dispense.
3155. **Q:** While stocking, you find insulin glargine pens left on the counter at room temperature overnight with unknown time out of fridge; what should you do FIRST? **A:** Separate the product and notify the pharmacist to determine usability per storage/temperature excursion policy because potency may be compromised when refrigeration requirements aren't met.
3156. **Q:** An eRx for nitrofurantoin shows "100 mg PO BID x 5 days" but the quantity entered is #30; what should you do BEST? **A:** Stop and ask the pharmacist to review and clarify quantity with the prescriber because the written duration (10 capsules) conflicts with the quantity and technicians can't independently change it.
3157. **Q:** A patient's profile shows lisinopril and the prescriber sends a new eRx for lisinopril/HCTZ; what should you do NEXT before filling? **A:** Flag it for pharmacist review for therapeutic duplication and dose change because combination products can unintentionally duplicate the ACE inhibitor and require clinical confirmation.
3158. **Q:** A prescription reads "metoprolol 25 mg take 1 tablet daily" but the product selected is metoprolol tartrate; what should you do FIRST? **A:** Pause and ask the pharmacist to verify intended formulation because tartrate vs succinate differ in release and dosing and selection errors are common and clinically significant.
3159. **Q:** A bottle of atorvastatin 40 mg arrives from the wholesaler with a broken seal and smudged lot/expiration; what should you do BEST? **A:** Quarantine the bottle and follow DSCSA/suspect product procedures with pharmacist notification because damaged packaging and unreadable identifiers make the product potentially suspect and non-dispensable.

3160. **Q:** You are billing cephalexin 500 mg "1 capsule QID" with dispense #28; what days' supply should you enter? **A:** Enter 7 days because $28 \text{ capsules} \div 4 \text{ doses per day} = 7$, which ensures accurate third-party billing and safe refill timing.
3161. **Q:** While typing an eRx for levothyroxine 125 mcg daily, you accidentally select liothyronine 25 mcg because the names look similar; what should you do FIRST? **A:** Stop entry and reselect the correct drug/strength using NDC/barcode verification because thyroid products are LASA and incorrect selection can cause significant harm.
3162. **Q:** A new prescription is entered for sertraline, but the patient profile already shows fluoxetine being filled monthly; what should you do NEXT before processing? **A:** Pause and alert the pharmacist to assess SSRI duplication and serotonin syndrome risk because technicians should not assume intentional therapy overlap.
3163. **Q:** You receive an eRx for glipizide 10 mg "take 1 tablet BID" for a patient with a listed sulfonamide antibiotic allergy; what is the technician's BEST next step? **A:** Flag the allergy and refer to the pharmacist for evaluation because cross-reactivity questions require pharmacist judgment and sulfonylureas are a common allergy trap.
3164. **Q:** During drop-off, a patient picking up lisinopril reports a new pregnancy; what should you do FIRST? **A:** Stop the pickup process and notify the pharmacist immediately because ACE inhibitors are contraindicated in pregnancy and require prompt pharmacist intervention.
3165. **Q:** A patient brings a new prescription for tramadol while their profile shows chronic use of clonazepam; what should you do NEXT? **A:** Route the order to the pharmacist for interaction review because opioid-like analgesics plus benzodiazepines increase sedation/respiratory depression risk.
3166. **Q:** An order is for Augmentin 875/125 mg, but you selected amoxicillin 875 mg due to similar names on the shelf; what should you do BEST? **A:** Stop filling and correct the product to amoxicillin/clavulanate with label/NDC check because missing clavulanate is a common therapeutic failure error.
3167. **Q:** A patient is prescribed "prednisone 20 mg: take ii tablets daily for 5 days" and the bottle contains 30 tablets; what quantity should you dispense? **A:** Dispense 20 tablets because ii = 2 tablets/day and $2 \times 5 = 10$ tablets? Actually 2 tablets daily for 5 days is 10 tablets, so dispense 10 to match the directions and prevent over-dispensing.

3168. **Q:** You are entering a prescription for insulin regular U-100 vial: inject 12 units before meals TID, dispense 1 vial (10 mL); what days' supply should you submit? **A:** Enter 27 days' supply because $10\text{ mL} = 1000\text{ units}$ and $12 \times 3 = 36\text{ units/day}$, so $1000 \div 36 = 27.7$, which is billed as 27 days if rounding down to whole days per payer standard.
3169. **Q:** While selecting a beta-blocker, you notice carvedilol is stocked next to carbamazepine and a tech scanned the wrong bottle into the fill; what should you do FIRST? **A:** Stop the fill and rescan/select the correct medication because LASA look-alike stock errors must be corrected before verification to prevent wrong-drug dispensing.
3170. **Q:** A patient asks what metformin is generally used for when picking up a new prescription; what is the BEST technician response? **A:** State it is commonly used to help control blood sugar in type 2 diabetes and offer pharmacist counseling because technicians can provide basic indication but clinical counseling is pharmacist-led.
3171. **Q:** While entering a new eRx, you see "Dispense as Written" checked for Synthroid but the patient requests generic levothyroxine at pickup; what should you do NEXT? **A:** Stop the sale and notify the pharmacist to clarify DAW/substitution rules with the prescriber/patient because technicians cannot override DAW and substitution impacts legality and billing.
3172. **Q:** A prescription is written for "hydralazine 25 mg PO TID," but the prescriber's handwriting could be "hydroxyzine"; what should you do FIRST? **A:** Hold processing and route to the pharmacist for clarification because this is a LASA risk with different indications and unsafe assumptions.
3173. **Q:** A patient's profile shows active atorvastatin, and a new order comes in for rosuvastatin; what is the BEST technician action before filling? **A:** Flag possible therapeutic duplication and alert the pharmacist to review because two statins may indicate an error or intentional switch needing confirmation.
3174. **Q:** You receive an eRx for "Humalog 10 units with meals" but you accidentally selected Humulin R in the system; what should you do FIRST? **A:** Stop entry, correct to the exact product ordered, and use barcode/NDC verification because mix-ups between insulin types are high-alert and can cause severe harm.
3175. **Q:** The sig says "Take 1 tab PO qd" and the prescriber wrote "qd," which your pharmacy policy discourages; what should you do NEXT? **A:** Enter the directions

as "once daily" and route to the pharmacist if clarification is needed because unsafe abbreviations should be converted to clear language to prevent errors.

3176. **Q:** A prescription reads "morphine sulfate ER 30 mg" but the product selected is morphine IR 30 mg; what should you do BEST? **A:** Stop filling and correct the dosage form/ER vs IR selection, then ensure pharmacist verification because ER/IR mix-ups are dangerous and change dosing and release.
3177. **Q:** A patient on the phone asks if their new amlodipine is "for blood pressure or cholesterol"; what is the BEST technician response? **A:** Provide general information that amlodipine is commonly used for blood pressure/angina and offer pharmacist counseling for questions about their therapy because technicians may give non-clinical, general drug info but not individualized advice.
3178. **Q:** An order is for fluticasone nasal spray 50 mcg "2 sprays each nostril daily," dispense 1 bottle labeled 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \text{ sprays} \div 4 = 30 \text{ days}$ for accurate billing and refill timing.
3179. **Q:** A patient presents a refill request for alprazolam with "3 refills" printed on the label and asks you to just process it; what should you do FIRST? **A:** Pause and refer to the pharmacist to verify controlled-substance refill validity because benzodiazepines are Schedule IV and refills must match the original prescription authorization and limits.
3180. **Q:** While stocking, you notice a bottle of potassium chloride ER tablets stored next to potassium chloride oral solution and the shelf tag is missing; what should you do NEXT? **A:** Separate the products and relabel the shelf/bin correctly (and alert the pharmacist if needed) because KCl is high-alert and clear segregation reduces selection errors.
3181. **Q:** During data entry you receive an eRx for clonidine 0.2 mg tablets but the selected product is clonazepam 0.5 mg due to LASA; what should you do FIRST? **A:** Stop processing and correct the drug selection by verifying name/strength/NDC (and alert the pharmacist if it reached verification) because LASA mix-ups are a common patient-safety error.
3182. **Q:** A new prescription for isotretinoin arrives and the patient is waiting at the counter; what is the BEST technician next step before filling? **A:** Route to the pharmacist and follow iPLEDGE workflow requirements because isotretinoin is REMS-restricted and dispensing must meet program criteria.

3183. **Q:** A patient wants an early refill of oxycodone/APAP and the claim rejects "refill too soon"; what should you do NEXT? **A:** Do not override; gather needed information and refer to the pharmacist/payer process because controlled substances and early refills require pharmacist review and proper documentation.
3184. **Q:** While receiving an order, you notice a refrigerated insulin shipment arrived warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/wholesaler because temperature excursions can compromise potency and must be handled per storage policy.
3185. **Q:** A prescriber phones in a prescription for lorazepam with "refill monthly for 1 year"; what is the BEST technician action? **A:** Hold and refer to the pharmacist because Schedule IV prescriptions have federal refill limits (no more than 5 refills within 6 months) and require correction before dispensing.
3186. **Q:** A patient asks to transfer their hydrocodone prescription from another pharmacy to yours; what should you do FIRST? **A:** Refer the request to the pharmacist because Schedule II prescriptions generally cannot be transferred and require pharmacist handling for legal compliance.
3187. **Q:** You receive a manufacturer recall notice for a specific lot of losartan 50 mg, and you find that lot on the shelf; what should you do NEXT? **A:** Remove and quarantine the affected lot, document per recall procedure, and alert the pharmacist because recalled products must be prevented from dispensing and traced.
3188. **Q:** A patient at pickup says they developed hives after starting trimethoprim-sulfamethoxazole yesterday and asks if they should keep taking it; what is the BEST technician response? **A:** Tell them to stop taking it only if directed by a pharmacist/doctor and get the pharmacist immediately because possible allergic reactions require prompt pharmacist intervention.
3189. **Q:** An order is for latanoprost 0.005% "instill 1 drop OU qHS," dispense 2.5 mL; using 20 drops per mL, what days' supply should you enter? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ drops/mL} = 50 \text{ drops}$ and OU nightly uses 2 drops/day, so $50 \div 2 = 25$.
3190. **Q:** You scan a serialized DSCSA barcode on a bottle of apixaban and the system returns "product identifier not recognized"; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager to investigate with

the supplier because an unverified identifier may indicate a suspect/illegitimate product under DSCSA.

3191. **Q:** A prescription is written for "Zestril 10 mg PO daily" but the patient profile shows lisinopril 10 mg as the chronic medication; what should you do NEXT during order entry? **A:** Select lisinopril (generic for Zestril) with the correct strength/form and route to pharmacist verification, because brand/generic equivalents are interchangeable only when matched correctly.
3192. **Q:** While entering a new order you notice "metFORMIN" and "methIMAzole" are adjacent in the pick list and the prescriber wrote "for diabetes"; what is the BEST technician step to prevent a LASA error? **A:** Confirm the indication matches metformin and use barcode/NDC selection (and alert the pharmacist if uncertain), because indication and product verification reduce look-alike/sound-alike selection errors.
3193. **Q:** A patient drops off a prescription for "levothyroxine 25 mcg" but your system defaults to 0.025 mg and flags a unit mismatch; what should you do FIRST? **A:** Enter the dose as 25 mcg (not mg) and ensure the correct strength is selected, because mcg-to-mg misentry can cause a 1000× dosing error.
3194. **Q:** The prescriber writes "HS" on a warfarin prescription and the patient asks what it means; what should you do BEST? **A:** Refer the question to the pharmacist while noting HS means "at bedtime," because warfarin is high-risk and patient counseling/clarification is pharmacist-led.
3195. **Q:** You receive an eRx for "Augmentin 875/125 mg" and the product selected is amoxicillin 875 mg only; what should you do NEXT before printing labels? **A:** Stop and correct the selection to amoxicillin/clavulanate 875/125 mg and route for pharmacist check, because missing clavulanate changes the drug and therapy.
3196. **Q:** A prescription says "take ii tabs po bid" for simvastatin 20 mg with quantity #60; what is the correct days' supply to enter? **A:** Enter 15 days' supply ($60 \text{ tablets} \div 4 \text{ tablets/day} = 15$), because "ii" is 2 and "bid" is twice daily.
3197. **Q:** A child's prescription is for amoxicillin suspension 400 mg/5 mL, take 5 mL twice daily for 10 days; what quantity in mL should you dispense? **A:** Dispense 100 mL ($5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 100 \text{ mL}$), because total volume must cover the full course.

3198. **Q:** At drop-off, a patient says they take "Plavix" and the new prescription is for clopidogrel; what should you do BEST in the profile before processing? **A:** Verify the active medication list and mark Plavix/clopidogrel as the same drug (not duplicates) and alert the pharmacist if both appear active, because brand/generic duplication can cause double therapy.
3199. **Q:** During data entry, a prescription for "Lasix 40 mg daily" is typed as "Losec 40 mg" due to sound-alike confusion; what should you do FIRST? **A:** Stop processing and recheck the original prescription to select furosemide (Lasix) and then send to pharmacist verification, because LASA errors can cause major therapeutic harm.
3200. **Q:** A patient requests "ibuprofen 800 mg" but you recognize it as prescription-strength and the prescriber wrote "prn" without a maximum daily dose; what should you do NEXT? **A:** Hold the order and refer to the pharmacist to clarify directions/max dose with the prescriber, because incomplete sigs on higher-dose NSAIDs increase safety and labeling risk.
3201. **Q:** An eRx for amlodipine 5 mg says "take 1 tab PO qd" with quantity #90; what days' supply should you enter? **A:** Enter 90 days' supply because 1 tablet daily means 90 tablets lasts 90 days.
3202. **Q:** A prescription for prednisone 10 mg reads "take 2 tabs PO BID" and quantity is #120; what is the correct days' supply for order entry? **A:** Enter 30 days' supply because 2 tabs twice daily equals 4 tablets/day and $120 \div 4 = 30$.
3203. **Q:** You're entering "hydroxyzine 25 mg take 1 tab TID prn itching" and the software demands a days' supply; what should you do BEST? **A:** Refer to the pharmacist or follow the pharmacy/payer documented prn-days' supply rule because prn use is variable and technicians shouldn't assume duration.
3204. **Q:** A prescription says "levothyroxine 0.05 mg PO daily" but the prescriber intended 50 mcg and your product list shows both mcg and mg strengths; what should you do FIRST? **A:** Convert and verify the strength as 50 mcg and select the matching NDC/strength carefully because mg-mcg mix-ups are common high-impact entry errors.
3205. **Q:** A patient's prescription is for cefdinir suspension 250 mg/5 mL: "take 6 mL PO qd x 10 days"; what quantity in mL should you dispense? **A:** Dispense 60 mL because $6 \text{ mL/day} \times 10 \text{ days} = 60 \text{ mL total needed}$.

3206. **Q:** The directions read "instill 2 gtt OS QID" and the technician is unsure about abbreviations; what is the BEST interpretation for entry? **A:** Enter "instill 2 drops in left eye four times daily" because gtt=drop, OS=left eye, and QID=four times daily.
3207. **Q:** While typing an order for clonidine 0.1 mg, you accidentally select clozapine due to similar spelling in the dropdown; what should you do NEXT to prevent a dispensing error? **A:** Stop and re-verify the drug name, strength, and indication against the original prescription before continuing because LASA selection errors happen during order entry.
3208. **Q:** A prescription for insulin lispro U-100 says "inject 6 units subcutaneously TID with meals" and you are dispensing one 10 mL vial; what days' supply should you enter (use 100 units/mL)? **A:** Enter 55 days' supply because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $6 \times 3 = 18 \text{ units/day}$, so $1000 \div 18 = 55.5$ and you enter 55 per typical payer rounding down.
3209. **Q:** A budesonide/formoterol inhaler contains 120 inhalations and the sig is "inhale 2 puffs BID"; what days' supply should you enter? **A:** Enter 30 days' supply because 2 puffs twice daily is 4 puffs/day and $120 \div 4 = 30$.
3210. **Q:** During controlled-substance entry, a new prescription for methylphenidate is missing the prescriber's DEA number in your system; what should you do FIRST? **A:** Stop processing and route to the pharmacist to obtain/verify required prescriber credentials because controlled-substance dispensing requires complete, verified prescriber information.
3211. **Q:** While typing an eRx for celecoxib, you notice the profile shows an active meloxicam prescription; what should you do BEST? **A:** Stop processing and alert the pharmacist to evaluate therapeutic duplication/NSAID risk, since the technician cannot decide which anti-inflammatory to continue.
3212. **Q:** A new prescription is written for "metformin ER 500 mg" but the selected product is metformin IR 500 mg; what should you do FIRST? **A:** Re-select the correct ER dosage form and restart the entry/label check because IR vs ER is a high-impact error that changes release and dosing.
3213. **Q:** A patient picking up clarithromycin says they take simvastatin nightly; what is the technician's BEST next step? **A:** Pause the sale and notify the pharmacist to screen for a major interaction, because clarithromycin can raise simvastatin levels and increase myopathy risk.

3214. **Q:** You receive a prescription for sertraline 50 mg daily and the patient reports starting linezolid yesterday from urgent care; what should you do NEXT? **A:** Hold the fill and get the pharmacist immediately, since linezolid plus an SSRI raises serotonin syndrome risk and needs pharmacist intervention.
3215. **Q:** A lisinopril prescription is entered, and the patient states they are pregnant; what should you do BEST? **A:** Stop and refer to the pharmacist before dispensing because ACE inhibitors are contraindicated in pregnancy and require prescriber/patient counseling decisions.
3216. **Q:** A prescription for azithromycin tablets says "take 2 tabs today then 1 tab daily x 4 days" and you are dispensing 250 mg tablets; what quantity should you enter? **A:** Enter quantity 6 tablets because the regimen is $2 + 1 \times 4 = 6$ total tablets for the full course.
3217. **Q:** A Toujeo SoloStar (insulin glargine U-300) box contains two 1.5 mL pens and the sig is "inject 30 units SC daily"; what days' supply should you enter? **A:** Enter 30 days because total units are $2 \text{ pens} \times 1.5 \text{ mL} \times 300 \text{ units/mL} = 900 \text{ units}$, and $900 \div 30 \text{ units/day} = 30 \text{ days}$.
3218. **Q:** A prescription for timolol 0.5% eye drops reads "instill 1 gtt OU BID" and the patient is also on oral metoprolol; what should you do NEXT? **A:** Continue entry but flag and notify the pharmacist for possible additive beta-blocker effects, since duplicate therapy screening is a pharmacist responsibility.
3219. **Q:** During product selection, you notice "hydrALAZINE" and "hydrOXYzine" adjacent in the pick list for a new fill; what should you do FIRST to prevent LASA error? **A:** Verify the drug name/indication against the prescription and use barcode/NDC scanning because these look-alike/sound-alike drugs treat very different conditions.
3220. **Q:** A patient asks what atorvastatin is for after seeing "Lipitor" on the label; what is the BEST technician response? **A:** State that atorvastatin (Lipitor) is a statin used to lower cholesterol and offer pharmacist counseling, since technicians can provide basic identification but not individualized clinical advice.
3221. **Q:** A new eRx is for "Plavix 75 mg PO daily" but the product selected is clopidogrel 300 mg; what should you do FIRST? **A:** Stop processing and reselect the correct strength/form (clopidogrel 75 mg) because Plavix is clopidogrel and a strength mismatch is a high-risk entry error.

3222. **Q:** While entering a prescription for "Lamictal 100 mg BID," the technician selects lamotrigine chewable/dispersible tablets instead of standard tablets; what is the BEST next step? **A:** Stop and choose the correct dosage form or route to pharmacist if unclear because the wrong formulation can change how the patient takes the medication.
3223. **Q:** A patient profile shows active prescriptions for duloxetine and venlafaxine, and a refill request arrives for venlafaxine; what should you do BEST? **A:** Hold the refill and alert the pharmacist to assess potential therapeutic duplication because both are SNRIs and duplicate therapy increases risk of adverse effects.
3224. **Q:** A prescription is written for "Zyrtec 10 mg daily," but the selection list shows cetirizine and citalopram close together; what should you do FIRST to prevent a LASA error? **A:** Verify the generic name and indication before selecting and confirm the NDC/barcode at picking because cetirizine (allergy) and citalopram (SSRI) are easily confused.
3225. **Q:** A patient says they take apixaban (Eliquis) and are dropping off a new prescription for ibuprofen 800 mg TID; what is the technician's BEST next step? **A:** Refer to the pharmacist before filling because NSAIDs plus anticoagulants increase bleeding risk and requires pharmacist counseling/assessment.
3226. **Q:** You receive a new prescription for levothyroxine and the patient asks if it is the same as Synthroid; what is the BEST technician response? **A:** Confirm that Synthroid is a brand name for levothyroxine and offer pharmacist counseling because brand/generic recognition is within tech scope but clinical advice should be pharmacist-led.
3227. **Q:** A prescription reads "gabapentin 300 mg: i cap PO TID, #90," and the patient asks if it is for nerve pain or seizures; what should you do NEXT? **A:** Offer to have the pharmacist counsel because gabapentin has multiple common uses and patient-specific indication counseling is pharmacist responsibility.
3228. **Q:** A patient brings an OTC request for omeprazole but has an active prescription for clopidogrel; what should you do BEST? **A:** Pause the sale and involve the pharmacist because omeprazole can reduce clopidogrel activation and the interaction may affect therapy.
3229. **Q:** An insulin lispro U-100 vial contains 10 mL (1000 units) and the sig is "inject 12 units before meals TID"; what days' supply should you enter? **A:** Enter 27 days' supply because $12 \text{ units} \times 3/\text{day} = 36 \text{ units/day}$ and $1000 \div 36 = 27.7$, rounded down to 27 for payer safety.

3230. **Q:** A prescription for atorvastatin 20 mg says "take 1 tablet QHS" and the prescriber also sent a new order for rosuvastatin 10 mg daily; what should you do FIRST? **A:** Stop and notify the pharmacist to review potential duplicate statin therapy because two concurrent statins is a common high-risk medication error.
3231. **Q:** A prescription reads "metformin 500 mg: ii tabs PO BID, #120"; what days' supply should you enter and why? **A:** Enter 30 days because ii tabs BID = 4 tablets/day and $120 \div 4 = 30$, ensuring accurate billing and refill timing.
3232. **Q:** While entering "predniSONE 10 mg: take 4 tabs daily x 5 days, then 2 tabs daily x 5 days, then 1 tab daily x 5 days; dispense #?" what should you do BEST? **A:** Calculate and enter a quantity of 35 tablets ($20 + 10 + 5$) to match the taper exactly and prevent under/over-dispensing.
3233. **Q:** A patient is prescribed amlodipine/benazepril (Lotrel) 5/20 mg 1 cap daily, but the selected product is amlodipine 5 mg; what should you do FIRST? **A:** Stop and reselect the correct combination product (verify strength and NDC) because omitting benazepril is a therapy error requiring accurate product selection.
3234. **Q:** An order says "cephalexin 250 mg/5 mL: take 10 mL PO QID x 7 days; dispense ____ mL"; what quantity should you enter? **A:** Dispense 280 mL because $10 \text{ mL} \times 4/\text{day} \times 7 \text{ days} = 280 \text{ mL}$, matching the full course for correct duration.
3235. **Q:** A prescription is written "morphine sulfate ER 30 mg: 1 tab PO q12h, #60," but you accidentally select morphine sulfate IR 30 mg; what is the BEST next step? **A:** Stop processing and correct to the ER formulation (then route for pharmacist verification) because IR vs ER is a high-risk dosage-form error.
3236. **Q:** A prescriber writes "MSO4 15 mg PO q4h PRN pain"; what should you do NEXT before continuing order entry? **A:** Hold and ask the pharmacist to clarify because "MSO4" is error-prone (can be confused with magnesium sulfate) and requires verified drug identity.
3237. **Q:** A nasal spray has 120 actuations and the sig is "2 sprays each nostril daily"; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$, aligning dispensing with expected use.
3238. **Q:** A prescription reads "doxycycline 100 mg: i cap PO BID, #14"; what is the days' supply and why? **A:** Enter 7 days because i cap BID = 2/day and $14 \div 2 = 7$, which supports accurate counseling prompts and payer processing.

3239. **Q:** You receive an eRx for "Humalog KwikPen U-100: inject 8 units TID with meals; dispense 1 box (5 pens, 3 mL each)"; what days' supply should you enter? **A:** Enter 62 days because total units = 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units and $1500 \div (8 \times 3) = 62.5$, typically billed as 62 days per payer rules.
3240. **Q:** During order entry, the sig says "apply 1 g topically TID" and the product is diclofenac 1% gel 100 g; what is the days' supply? **A:** Enter 33 days because 1 g TID = 3 g/day and $100 \div 3 = 33.3$, typically billed as 33 days to avoid overstating supply.
3241. **Q:** During data entry you see "Zocor 40 mg 1 tab PO QHS" but the selected product is simvastatin 20 mg; what should you do FIRST? **A:** Stop processing and reselect the correct strength/NDC (simvastatin 40 mg) because brand-to-generic is fine but strength mismatch is a dispensing error risk.
3242. **Q:** A patient drops off a prescription for "Glucophage XR 500 mg" but says they only tolerate the immediate-release tablets; what is the BEST technician next step? **A:** Route to the pharmacist to assess and contact the prescriber if needed because IR vs ER is a dosage-form change outside technician scope.
3243. **Q:** An eRx comes in for "Keflex 500 mg PO QID" for a patient whose profile lists an anaphylactic reaction to penicillin; what should you do NEXT? **A:** Pause the fill and alert the pharmacist because cephalexin is a cephalosporin and severe beta-lactam allergy requires pharmacist evaluation.
3244. **Q:** While typing the sig you notice "take 1 tab PO qd" and the drug is levothyroxine; what is the MOST important counseling-related flag you should send to the pharmacist? **A:** Notify the pharmacist to counsel on consistent administration timing (often empty stomach, separate from calcium/iron) because absorption issues commonly affect levothyroxine efficacy.
3245. **Q:** A hardcopy prescription says "HCTZ 25 mg i tab qd" and the prescriber handwriting could be "HCQ 200 mg"; what should you do FIRST? **A:** Stop entry and ask the pharmacist to clarify with the prescriber because look-alike abbreviations/handwriting can lead to serious wrong-drug errors.
3246. **Q:** You receive a prescription for "Lantus 100 units/mL vial: inject 18 units SQ at bedtime; dispense 10 mL"; what days' supply should you enter? **A:** Enter 55 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $1000 \div 18 \text{ units/day} = 55.5$, which is typically entered as 55 days per standard day-supply truncation.

3247. **Q:** A patient's profile shows active lisinopril 20 mg daily and you receive a new eRx for benazepril 20 mg daily; what should you do BEST? **A:** Hold the order and notify the pharmacist to assess therapeutic duplication because both are ACE inhibitors and duplicate therapy can increase adverse effects.
3248. **Q:** A stock bottle label shows "metoprolol succinate ER" but the prescription is for "metoprolol tartrate 50 mg BID"; what is your BEST action before filling? **A:** Select the correct salt/formulation (tartrate IR) and use barcode/NDC verification because succinate ER vs tartrate IR are not interchangeable and represent a common LASA/selection trap.
3249. **Q:** You are entering "azithromycin 250 mg tablets: take 2 tablets today then 1 tablet daily for 4 days" and the prescriber left quantity blank; what should you do NEXT? **A:** Calculate and enter quantity 6 tablets and have it verified because the sig dictates $2 + (1 \times 4) = 6$ tablets and accurate quantity prevents under/over-dispensing.
3250. **Q:** A refrigerator temp log shows insulin was stored at 12°C (54°F) overnight due to a door left ajar; what should you do FIRST with the affected stock? **A:** Quarantine the insulin and notify the pharmacist/manager to follow storage excursion policy because temperature excursions can compromise product integrity and require documented disposition.
3251. **Q:** While entering an eRx you notice the prescriber selected hydrALAZINE but the directions read "take 25 mg daily for anxiety" (likely hydrOXYzine); what should you do FIRST? **A:** Stop processing and send to the pharmacist to clarify with the prescriber because LASA/name-selection errors can cause harmful dispensing mistakes.
3252. **Q:** A C-II oxycodone prescription is presented with the patient's name and drug/directions but no prescriber signature; what is the BEST next step for the technician? **A:** Hold the prescription and immediately refer to the pharmacist because a missing signature makes it invalid to dispense and requires prescriber follow-up.
3253. **Q:** During data entry for sertraline you see "take 1 tab PO qd" but the quantity is entered as "1" due to a keystroke error; what should you do NEXT? **A:** Correct the quantity to the intended days' supply/dispense amount and recheck the full entry because preventing input errors is a primary technician safety responsibility.

3254. **Q:** A patient requests an early refill of zolpidem because it was “lost,” and the claim rejects refill-too-soon; what should you do BEST? **A:** Notify the pharmacist and follow payer/store policy (e.g., document, consider partial/override request if allowed) because controlled substances and early refills require pharmacist oversight.
3255. **Q:** You receive a stock bottle of levothyroxine labeled with a lot number that does not match the invoice/receiving paperwork; what should you do FIRST? **A:** Segregate the product and alert the pharmacist/manager because DSCSA and inventory control require resolving suspect or mismatched product before it’s placed in active stock.
3256. **Q:** A prescription comes in for isotretinoin capsules and the patient is waiting at pickup; what is the technician’s BEST next step before processing? **A:** Refer to the pharmacist to ensure iPLEDGE/REMS requirements are met because isotretinoin dispensing is restricted and must be verified before release.
3257. **Q:** You are filling a heparin vial order and notice the shelf has multiple strengths with similar labeling; what should you do FIRST to prevent an error? **A:** Use barcode/NDC verification and confirm the strength against the label and order because heparin is high-alert and strength mix-ups can be fatal.
3258. **Q:** An eRx for clopidogrel (Plavix) is entered, and the patient profile shows omeprazole OTC started last week; what should you do NEXT? **A:** Flag and refer to the pharmacist because omeprazole can reduce clopidogrel activation and a pharmacist should assess and counsel or contact the prescriber.
3259. **Q:** A 10 mL bottle of timolol 0.5% ophthalmic solution is prescribed “instill 1 drop OU BID”; what days’ supply should you enter using 20 drops/mL? **A:** Enter 50 days’ supply because $10\text{ mL} \times 20\text{ drops/mL} = 200\text{ drops}$ and $\text{OU BID} = 4\text{ drops/day}$, so $200 \div 4 = 50\text{ days}$.
3260. **Q:** A patient asks you to transfer their alprazolam prescription from another pharmacy to yours; what is the BEST technician response? **A:** Get the patient’s information and route the request to the pharmacist because controlled-substance transfers and validity checks must be handled by a pharmacist per law/policy.
3261. **Q:** During data entry you see “Celebrex 200 mg daily” selected but the prescriber wrote “celecoxib 200 mg daily”; what should you do NEXT? **A:** Enter celecoxib and link to the correct product/NDC (brand if DAW) because Celebrex is the brand name and accurate brand/generic selection prevents dispensing errors.

3262. **Q:** A new prescription is entered for lisinopril, and the patient profile shows a current active prescription for losartan; what should you do FIRST? **A:** Stop and alert the pharmacist to possible therapeutic duplication/ACEI-ARB issue because it may be intentional but requires clinical review.
3263. **Q:** You receive an eRx for metformin ER but the product selected is metformin IR; what is the BEST next step for the technician? **A:** Correct the dosage form to ER and route for pharmacist verification because IR vs ER is not interchangeable and can change dosing and tolerability.
3264. **Q:** While selecting a product for lamotrigine, you notice lamotrigine and terbinafine bottles stored adjacent with similar labeling; what should you do FIRST? **A:** Pause and verify by barcode/NDC and label strength before counting because LASA storage increases wrong-drug selection risk.
3265. **Q:** A prescription reads "Keppra 500 mg" but the patient insists they take "levetiracetam" and asks if it's different; what should you do BEST? **A:** Explain Keppra is the brand for levetiracetam and offer pharmacist counseling if needed because confirming brand/generic equivalence supports safe use and adherence.
3266. **Q:** An order for azithromycin suspension is "200 mg/5 mL: take 12.5 mL PO daily x 3 days"; what quantity in mL should you dispense? **A:** Dispense 37.5 mL because $12.5 \text{ mL/day} \times 3 \text{ days} = 37.5 \text{ mL total}$.
3267. **Q:** You are entering a prescription for amlodipine and the prescriber typed "Norvasc" but selected "amlodipine/benazepril"; what should you do NEXT? **A:** Stop processing and notify the pharmacist to clarify because Norvasc is amlodipine alone and the combo product changes therapy.
3268. **Q:** A patient on simvastatin brings a new prescription for clarithromycin; what should you do FIRST? **A:** Flag the interaction and refer to the pharmacist because clarithromycin can raise simvastatin levels and increase myopathy risk.
3269. **Q:** A technician receives a hardcopy for Adderall and sees "take 1 tab BID" but no patient address is present; what is the BEST next step? **A:** Hold the prescription and involve the pharmacist to resolve missing required elements before dispensing because controlled-substance validity must be confirmed.
3270. **Q:** You are calculating days' supply for insulin lispro U-100: inject 10 units before meals TID, dispensed 1 vial (10 mL); what days' supply should you enter? **A:** Enter 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3 \rightarrow 33 \text{ days}$.

3271. **Q:** A prescription says "levothyroxine 0.1 mg PO daily"; what strength should you enter in the system? **A:** Enter levothyroxine 100 mcg ($0.1 \text{ mg} \times 1000 = 100 \text{ mcg}$) to match standard tablet strengths and prevent decimal-place dosing errors.
3272. **Q:** The sig reads "Take ii tabs po bid" for gabapentin 300 mg with quantity #120; what days' supply should you enter? **A:** Enter 30 days because ii BID = 4 tablets/day and $120 \div 4 = 30$, ensuring correct insurance billing and refill timing.
3273. **Q:** A mometasone nasal spray (50 mcg/spray) prescription is "2 sprays each nostril daily"; the bottle is 120 sprays—what days' supply should you enter? **A:** Enter 30 days because 2 sprays/nostril/day = 4 sprays/day and $120 \div 4 = 30$, reflecting total metered sprays per container.
3274. **Q:** You're processing an eRx for prednisone that says "Take 10 mg PO qd" but the quantity field is blank; what should you do NEXT? **A:** Stop and send to the pharmacist for clarification because a missing quantity prevents accurate dispensing and billing and may indicate an incomplete prescription.
3275. **Q:** A prescription for hydrocodone/acetaminophen is written "1 tab q4-6h prn pain" with "Refill: 2"; what is the BEST technician action? **A:** Hold the order and refer to the pharmacist because refills on a Schedule II are not permitted federally and require prescriber follow-up.
3276. **Q:** A new order is for "bupropion SR 150 mg BID" but you selected bupropion XL 150 mg in data entry; what should you do FIRST? **A:** Stop and correct the dosage form to SR (then recheck NDC) because SR vs XL is a critical release-rate difference that can cause under/overdosing.
3277. **Q:** An epinephrine auto-injector prescription is "inject 0.3 mg IM PRN anaphylaxis" and the package contains 2 pens; what days' supply should you enter if the plan requires 30 days for one 2-pack? **A:** Enter 30 days because PRN rescue items often use payer-set day supply rules, and following the plan requirement prevents claim rejection.
3278. **Q:** You receive a prescription for cefdinir 250 mg/5 mL with directions "5 mL PO q12h x 10 days"; what quantity in mL should you dispense? **A:** Dispense 100 mL because 5 mL twice daily = 10 mL/day and $10 \text{ mL/day} \times 10 \text{ days} = 100 \text{ mL}$, matching the full course.
3279. **Q:** During order entry, the sig reads "Use 1 gtt OS qid" for ofloxacin ophthalmic; what does OS mean and what should you do to avoid a laterality error? **A:** OS

means left eye, and you should enter "left eye" explicitly (not "both eyes") because laterality is a common order-entry error with eye drops.

3280. **Q:** A prescription is "clopidogrel 75 mg po qd" but the patient profile already shows prasugrel active; what should you do NEXT? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (two antiplatelets) because this is a high-bleeding-risk situation needing clinical review.
3281. **Q:** A patient drops off a prescription for Glucophage and asks if it's the same as metformin; what should you do FIRST before ringing it up? **A:** Confirm in the system that Glucophage is the brand for metformin and show the label/medication name to the patient, because brand-generic verification prevents duplicate therapy and confusion.
3282. **Q:** During data entry you see "metoprolol succinate ER 50 mg daily" but the prescriber wrote "Toprol-XL 50 mg"; what is the BEST entry approach? **A:** Enter metoprolol succinate ER (Toprol-XL) and ensure the ER dosage form is selected, because succinate ER is not interchangeable with metoprolol tartrate IR.
3283. **Q:** A new eRx is for "Celebrex 200 mg daily," but the patient profile lists a serious sulfonamide antibiotic allergy; what should you do NEXT? **A:** Stop processing and alert the pharmacist for allergy assessment, because celecoxib is a sulfonamide-containing drug and may require pharmacist clinical judgment.
3284. **Q:** You're filling "Lamictal 100 mg" and the shelf has both lamotrigine and terbinafine nearby; what should you do BEST to prevent a LASA error? **A:** Scan the barcode and match the NDC and drug name/strength to the prescription and label, because barcode/NDC verification is the most reliable check against look-alike/sound-alike mix-ups.
3285. **Q:** A prescription reads "Humalog 10 units subcut qhs" and you notice Humalog is insulin lispro; what should you do FIRST? **A:** Flag it for pharmacist review before filling, because insulin lispro is rapid-acting and "qhs" dosing may indicate a potential direction/therapy mismatch.
3286. **Q:** The sig says "Take 1 tab PO qod" for atorvastatin 20 mg; what does qod mean and what is the technician's next step for accurate entry? **A:** qod means every other day, and you should enter the directions exactly as written and calculate days' supply accordingly, because correct abbreviation interpretation prevents dosing frequency errors.

3287. **Q:** A patient brings in a manufacturer bottle of gabapentin that was stored in a hot car and asks you to dispense from it; what should you do BEST? **A:** Decline to use the patient-supplied product and notify the pharmacist per policy, because temperature-exposed medication may be compromised and should not be dispensed.
3288. **Q:** You receive a stock bottle of sertraline 100 mg with a broken seal and tablets visible; what should you do FIRST? **A:** Quarantine the bottle and report it to the pharmacist/manager, because a compromised seal is a suspect product/quality issue and must not enter dispensing inventory.
3289. **Q:** A prescription is for "Ventolin HFA inhale 2 puffs q6h prn wheeze" and you need a days' supply for 1 inhaler containing 200 actuations; what should you enter if you calculate using the max daily use? **A:** Enter 25 days' supply because max use is 2 puffs every 6 hours = 8 puffs/day and $200 \div 8 = 25$.
3290. **Q:** An order is for "cephalexin 500 mg PO TID x 7 days" with quantity #21; what is the days' supply and what should you verify before processing? **A:** Days' supply is 7 ($21 \div 3$), and you should verify the duration matches the directions, because quantity-to-sig consistency is a key data-entry safety check.
3291. **Q:** While entering an eRx for levothyroxine 25 mcg daily, you notice the prescriber selected "levothyroxine 0.25 mg"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to confirm the intended strength because mcg-to-mg decimal errors are high-risk and must be clarified before dispensing.
3292. **Q:** A hardcopy for oxycodone 5 mg says "Take 1 tab q6h prn pain" but has no prescriber signature; what is the technician's BEST next step? **A:** Hold the prescription and route it to the pharmacist because missing required elements make it invalid to fill and require pharmacist intervention.
3293. **Q:** A patient asks to refill Adderall 20 mg today, but the claim rejects "refill too soon" and the last fill was 10 days ago for #30 with "1 daily"; what should you do NEXT? **A:** Verify last fill date/quantity and days' supply, then refer to the pharmacist because controlled-substance early refill requests require clinical/legal review and documentation.
3294. **Q:** You are selecting products for hydroxyzine and see hydroxyzine HCl and hydroxyzine pamoate on the shelf; what should you do BEST to prevent a dispensing error? **A:** Match the exact salt/form on the prescription to the NDC and barcode-scan at fill because these are not interchangeable and can cause wrong-drug errors.

3295. **Q:** A new prescription is for lisinopril 20 mg daily, and the profile shows an active allergy to ACE inhibitors with "angioedema"; what should you do FIRST? **A:** Stop entry and notify the pharmacist immediately because angioedema is a serious contraindication that requires pharmacist assessment before filling.
3296. **Q:** You receive a wholesaler tote and a bottle of atorvastatin 40 mg has no 2D barcode/lot/expiration information in the required DSCSA format; what should you do NEXT? **A:** Set the product aside per policy and notify the pharmacist/manager to resolve DSCSA tracing requirements because suspect/unverified product should not enter dispensing stock.
3297. **Q:** A stock bottle of insulin aspart pens arrives warm and the temperature indicator shows a possible excursion; what is the technician's BEST action? **A:** Quarantine the shipment and alert the pharmacist/receiver because temperature excursions can compromise biologics and must be investigated before use.
3298. **Q:** A prescription reads "predniSONE 10 mg: ii tabs po bid x 5 days; dispense #?"; what quantity should you enter? **A:** Enter quantity 20 tablets because 2 tabs twice daily equals 4 tabs/day and $4 \times 5 \text{ days} = 20$.
3299. **Q:** During verification prep you notice a filled vial labeled "bupropion SR 150 mg" but the scanned stock bottle was bupropion XL 150 mg; what should you do FIRST? **A:** Stop the process and immediately notify the pharmacist because SR vs XL is a dosage-form error that can change dosing and must be corrected before dispensing.
3300. **Q:** A patient requests pseudoephedrine 30 mg tablets, 96-count, and you are preparing the sale at the register; what should you do FIRST to comply with federal requirements? **A:** Ask for a valid government ID and complete the required log/electronic tracking before sale because CMEA requires ID verification and purchase documentation.
3301. **Q:** While typing a new eRx for Plavix, the system auto-selects clopidogrel but the diagnosis field says "A-fib anticoagulation"; what should you do NEXT? **A:** Pause processing and route to the pharmacist because clopidogrel (Plavix) is an antiplatelet, not a primary anticoagulant for Afib, and indication mismatch can signal a selection error.
3302. **Q:** A prescription is written for "metFORMIN ER 500 mg: take 2 tabs BID" and the patient is also active on metformin IR 1000 mg BID; what is the technician's BEST next step? **A:** Stop and alert the pharmacist to possible therapeutic

duplication/excess dose because ER vs IR products can be unintentionally stacked and require clinical review.

3303. **Q:** During product selection you find "hydrALAZINE" stored next to "hydrOXYzine" and the label printed is for hydroxyzine; what should you do FIRST to prevent a LASA error? **A:** Scan the stock bottle NDC/barcode against the label and visually confirm drug/strength because hydralazine vs hydroxyzine is a high-risk LASA mix-up.
3304. **Q:** A new order is for "lamoTRIGine 200 mg daily" but the profile shows an active allergy to lamotrigine with "rash/Stevens-Johnson concern"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist immediately because lamotrigine-related serious rash history is a high-alert safety issue requiring pharmacist intervention.
3305. **Q:** A patient brings a new prescription for Zoloft 50 mg daily and says they are currently taking paroxetine (Paxil) 40 mg daily; what should you do NEXT? **A:** Flag it for pharmacist review before filling because two SSRIs together can indicate an unsafe duplication or cross-taper that the pharmacist must assess.
3306. **Q:** You receive an eRx for "Eliquis 5 mg BID" but the product selected in the system is "albuterol 5 mg/mL nebulizer solution" due to a search error; what should you do FIRST? **A:** Stop entry and re-select the correct drug (apixaban/Eliquis) by verifying name, dosage form, and strength because look-alike search results can cause severe wrong-drug errors.
3307. **Q:** A label prints for "glimepiride 4 mg daily" but the stock bottle pulled is glyburide 5 mg; what should you do NEXT? **A:** Do not dispense and correct the product by scanning/confirming NDC because sulfonylureas are high-risk for hypoglycemia and glimepiride vs glyburide is a common mix-up.
3308. **Q:** A prescription reads "amLODIPine/benazepril 5/20 mg 1 cap daily #30" but the patient profile shows a current lisinopril 20 mg daily; what is the BEST technician action? **A:** Route to the pharmacist to check for duplicate ACE inhibitor therapy because benazepril and lisinopril together increase risk and require prescriber clarification.
3309. **Q:** A provider writes "azithromycin 250 mg: take 2 tabs on day 1 then 1 tab daily days 2-5; dispense #?"; what quantity should you enter? **A:** Enter quantity 6 tablets because 2 on day 1 plus 1 tablet for 4 more days totals 6.

3310. **Q:** A new prescription is for "nitrofurantoin 100 mg BID x 5 days" but you select Macrochantin instead of Macrobid; what should you do FIRST? **A:** Stop and verify the intended formulation with the pharmacist because Macrobid and Macrochantin are different nitrofurantoin products and mix-ups can change dosing and effectiveness.
3311. **Q:** While entering a new Rx, you see "Lasix 40 mg PO daily" but the prescriber's note says "for hypertension"; what should you do NEXT? **A:** Continue processing as written and flag for pharmacist review if needed because furosemide (Lasix) is a loop diuretic that can be used for BP/edema and the technician should not change therapy.
3312. **Q:** A patient drops off a prescription for "Synthroid 100 mcg daily" and asks if it's the same as levothyroxine; what is the BEST technician response? **A:** Confirm Synthroid is the brand name for levothyroxine and offer to check which product will be dispensed because brand/generic recognition helps prevent confusion and supports informed pickup.
3313. **Q:** You receive an eRx for "Keflex 500 mg q6h x 7 days #28" but the system selects "cefdinir"; what should you do FIRST? **A:** Stop and correct the drug selection to cephalexin (Keflex) because it's a brand/generic mismatch that could lead to dispensing the wrong antibiotic.
3314. **Q:** The sig reads "Take ii tabs po bid" for atorvastatin 20 mg; what should you enter for daily tablet count? **A:** Enter 4 tablets/day because ii = 2 and BID = twice daily, which is essential for accurate days' supply and billing.
3315. **Q:** A prescription is written for "MS Contin 15 mg q12h" and the prescriber later calls asking for "MSIR instead"; what should you do NEXT? **A:** Route the call to the pharmacist and do not substitute because ER vs IR morphine is a high-risk formulation difference requiring pharmacist authorization and documentation.
3316. **Q:** While filling, you notice your tech pulled Humalog (insulin lispro) but the label is for Lantus (insulin glargine); what should you do FIRST? **A:** Stop filling and reselect the correct NDC/product for Lantus because insulin is high-alert and wrong-insulin errors can cause severe harm.
3317. **Q:** A new Rx is for "Bactroban ointment apply TID" but the product pulled is mupirocin cream; what is the BEST next step? **A:** Stop and obtain the correct dosage form (ointment vs cream) or send to pharmacist to clarify because dosage form is part of the prescription and affects therapy.

3318. **Q:** A patient's profile shows anaphylaxis to penicillin, and you are entering an Rx for amoxicillin; what should you do FIRST? **A:** Hold the prescription and immediately notify the pharmacist because this is a serious allergy conflict requiring pharmacist intervention before dispensing.
3319. **Q:** A prescription is for "Ventolin HFA inhale 2 puffs QID #1" and the directions are not PRN; what days' supply should you enter if the inhaler contains 200 actuations? **A:** Enter 25 days because $2 \text{ puffs} \times 4/\text{day} = 8 \text{ puffs/day}$ and $200 \div 8 = 25$, which supports accurate third-party billing.
3320. **Q:** During receiving, a tote includes a bottle of sertraline with a torn seal and missing lot number on the label; what should you do FIRST? **A:** Quarantine the bottle and notify the pharmacist/manager because damaged or non-traceable product is suspect and must not enter dispensing stock.
3321. **Q:** While entering an eRx for hydrALAZINE 25 mg, the product selection screen highlights hydrOXYzine 25 mg as the first match; what should you do FIRST? **A:** Stop and verify the exact drug name/indication on the eRx and select the correct NDC using LASA safeguards (Tall Man/barcode) because hydrALAZINE and hydrOXYzine are high-risk look-alike/sound-alike errors.
3322. **Q:** A written prescription for oxycodone/APAP has the patient name and drug details but no prescriber signature; what is the BEST next step for the technician? **A:** Hold the prescription and refer to the pharmacist because a missing signature is a validity issue that requires pharmacist intervention before any dispensing steps continue.
3323. **Q:** A patient requests an early refill for clonazepam 1 mg claiming the bottle was lost; what should you do NEXT? **A:** Notify the pharmacist and document the request per store policy because controlled-substance "lost medication" requests require pharmacist review and may need prescriber authorization.
3324. **Q:** During order entry you notice a new Rx for simvastatin 40 mg daily and the profile shows an active Rx for atorvastatin 40 mg daily; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because two statins together may be an unintended overlap requiring clinical review.
3325. **Q:** You open the refrigerator and find several insulin glargine pens stored at room temperature overnight due to a power issue; what should you do FIRST? **A:** Quarantine the affected insulin and notify the pharmacist/manager because

temperature excursions create suspect product that must be evaluated before it can be dispensed.

3326. **Q:** A prescription label prints for methotrexate 2.5 mg “take 1 tablet daily” for rheumatoid arthritis; what should you do NEXT? **A:** Stop and immediately escalate to the pharmacist because methotrexate for RA is typically weekly and daily dosing is a high-alert, potentially fatal error.
3327. **Q:** A patient asks to transfer their diazepam prescription to another pharmacy; what is the BEST technician action? **A:** Route the request to the pharmacist because controlled-substance transfers have legal/record requirements and must be handled by a pharmacist per federal rules and state policy.
3328. **Q:** A wholesaler ships a case of valacyclovir but the outer package shows a different NDC than the invoice and appears re-labeled; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/receiving lead to follow DSCSA suspect-product procedures because mismatched labeling/NDC suggests possible diversion or counterfeit.
3329. **Q:** You receive an eRx for isotretinoin for a new patient and the pharmacist is busy; what should you do NEXT in workflow? **A:** Enter the prescription but hold it for pharmacist review and iPLEDGE/REMS processing because isotretinoin requires REMS verification before dispensing.
3330. **Q:** Pharmacy math: The sig for metformin 500 mg reads “ii tabs po bid” and the prescriber wrote “dispense 90”; what should you do BEST? **A:** Calculate 4 tablets/day so 90 tablets is a 22.5-day supply, then flag the quantity mismatch to the pharmacist because the written quantity may be insufficient for the directions.
3331. **Q:** An eRx says “levothyroxine 0.1 mg PO daily” but the product list shows levothyroxine 100 mcg and liothyronine 25 mcg; what should you do FIRST? **A:** Select levothyroxine 100 mcg ($0.1 \text{ mg} = 100 \text{ mcg}$) and avoid liothyronine because it’s a different thyroid drug, preventing a wrong-med order entry error.
3332. **Q:** A prescription reads “amoxicillin/clavulanate 875/125 mg 1 tab PO q12h x 10 days” and the quantity field is blank; what quantity should you enter? **A:** Enter 20 tablets because $q12h = 2 \text{ doses/day}$ and $2 \times 10 \text{ days} = 20$, matching the intended duration.
3333. **Q:** A label is being generated for “prednisone 10 mg: take ii tabs daily x 5 days then i tab daily x 5 days”; what is the correct total quantity to dispense? **A:**

Dispense 15 tablets because $2 \text{ tabs/day} \times 5 = 10$ plus $1 \text{ tab/day} \times 5 = 5$, ensuring the taper has enough tablets.

3334. **Q:** A new Rx is "omeprazole 20 mg take 1 cap PO BID #30"; what should you do BEST during order entry? **A:** Stop and route to the pharmacist to clarify quantity because BID for 30 capsules is only 15 days, and inconsistent quantity/directions risks incorrect days' supply and underfill.
3335. **Q:** You're entering "dorzolamide/timolol eye drops: instill 1 gtt OU BID" and the bottle size is 10 mL; what days' supply should you enter using 20 gtt/mL? **A:** Enter 50 days because $10 \text{ mL} \times 20 = 200$ drops and OU BID = 4 drops/day, so $200 \div 4 = 50$ days.
3336. **Q:** An order says "clindamycin 600 mg IV" and stock is clindamycin 300 mg/2 mL vials; what volume is needed for the dose? **A:** Use 4 mL because 300 mg per 2 mL equals 150 mg/mL, and $600 \text{ mg} \div 150 \text{ mg/mL} = 4 \text{ mL}$, preventing an underdose.
3337. **Q:** A patient is prescribed "gabapentin 300 mg 1 cap TID" and the prescriber wrote "Disp #90"; what days' supply should you enter? **A:** Enter 30 days because TID = 3/day and $90 \div 3 = 30$, which supports correct billing and refill timing.
3338. **Q:** While typing a new Rx for "lamoTRIGine," the search results also show "lamivudine"; what should you do FIRST? **A:** Verify the intended drug by matching indication/strength and using Tall Man/LASA awareness before selecting, because lamotrigine vs lamivudine is a high-risk wrong-drug error.
3339. **Q:** A written Rx reads "MSO4 2 mg IV q2h PRN pain"; what should you do NEXT as the technician? **A:** Hold and alert the pharmacist for clarification because "MSO4" is an error-prone abbreviation that can be confused with magnesium sulfate, risking serious harm.
3340. **Q:** You receive an eRx for "insulin lispro U-100 inject 12 units SQ before meals TID" and it's for a 10 mL vial (100 units/mL); what days' supply should you enter? **A:** Enter 27 days because the vial contains 1000 units and $12 \text{ units} \times 3 = 36$ units/day, so $1000 \div 36 = 27.7$ and you enter 27 days to avoid overstating supply.
3341. **Q:** During data entry you see a new prescription for sertraline (Zoloft) and the patient profile shows paroxetine (Paxil) active; what should you do BEST? **A:** Stop and alert the pharmacist to possible duplicate SSRI therapy before filling, because duplication increases adverse effects and requires clinical review.

3342. **Q:** A prescriber e-sends "buPROPion XL 150 mg PO daily" but the product selected is bupropion SR 150 mg; what should you do FIRST? **A:** Re-select the correct XL dosage form and verify NDC before printing labels, because SR vs XL is a high-risk mix-up that changes release rate.
3343. **Q:** A patient drops off a new Rx for lisinopril and says they had lip swelling on it before; what should you do NEXT? **A:** Document the reported reaction as an allergy and notify the pharmacist immediately, because possible ACE-inhibitor angioedema is serious and needs pharmacist intervention.
3344. **Q:** You are entering an eRx for "rosuvastatin 20 mg daily" and the profile shows atorvastatin 40 mg daily currently filled; what is the BEST technician action? **A:** Place the order on hold and route to the pharmacist to assess therapeutic duplication and switching directions, because statin changes require prescriber/patient clarification.
3345. **Q:** A hard copy says "hydroxyzine 25 mg 1 tab TID prn itching #90" but you notice the patient is also taking cetirizine daily; what should you do FIRST? **A:** Continue entry but flag the pharmacist for additive antihistamine sedation risk, because interaction/duplication screening and counseling decisions are pharmacist responsibilities.
3346. **Q:** Pharmacy math: An Rx is for azithromycin 200 mg/5 mL suspension "give 10 mL day 1, then 5 mL daily days 2–5"; what total mL should you dispense? **A:** Dispense 30 mL because $10\text{ mL} + (5\text{ mL} \times 4\text{ days}) = 30\text{ mL total}$.
3347. **Q:** Pharmacy math: Fluticasone nasal spray 50 mcg/spray is prescribed "2 sprays in each nostril once daily" and the bottle contains 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2\text{ sprays/nostril} \times 2\text{ nostrils} = 4\text{ sprays/day}$ and $120 \div 4 = 30\text{ days}$.
3348. **Q:** A technician selects HUMALOG instead of HUMULIN R while filling an insulin order; what should you do BEST before the prescription leaves production? **A:** Stop filling and correct the product using barcode/NDC verification, because LASA insulin mix-ups are high-alert and must be caught before verification/dispensing.
3349. **Q:** A prescription is written for "clonazepam 0.5 mg take 1 tab BID #60 + 2 refills"; what should you do NEXT in processing? **A:** Enter the prescription but route it to the pharmacist to confirm controlled-substance refill compliance and validity, because technicians should not decide legality for CS refills.

3350. **Q:** While putting away stock you find a bottle of amlodipine with a broken seal and tablets visible in the tote; what should you do FIRST? **A:** Remove it from active inventory and quarantine per policy for pharmacist review, because compromised packaging is a patient-safety and product-integrity concern.
3351. **Q:** An eRx is for levothyroxine 75 mcg take 1 tab PO daily #90 but the product selected in order entry is levothyroxine 0.075 mg; what should you do FIRST? **A:** Confirm that 75 mcg equals 0.075 mg and ensure the strength matches before proceeding, because unit conversion errors can cause a wrong-dose dispense.
3352. **Q:** A prescription reads "metFORMIN 500 mg i tab po bid" and the technician is unsure if "i" is the number 1 or the letter; what is the BEST next step? **A:** Interpret "i tab" as 1 tablet (Roman numeral i) and enter 1 tablet twice daily, because common Roman numerals are standard sig conventions.
3353. **Q:** Amoxicillin/clavulanate suspension 400 mg/57 mg per 5 mL is prescribed "7.5 mL PO BID x 10 days"; what total mL should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$.
3354. **Q:** A Ventolin HFA inhaler (200 actuations) is prescribed "inhale 2 puffs QID"; what days' supply should you enter? **A:** Enter 25 days because $200 \text{ puffs} \div (2 \text{ puffs} \times 4 \text{ times/day}) = 25 \text{ days}$.
3355. **Q:** A patient's new prescription is for gabapentin 300 mg "1 cap PO TID #90"; during data entry you accidentally enter "QID"; what should you do NEXT when you notice? **A:** Stop and correct the sig to TID and recheck quantity/days' supply, because wrong frequency is a common order-entry error that leads to incorrect labeling and dosing.
3356. **Q:** Prednisone 10 mg tablets are prescribed "take 40 mg PO daily x 5 days #"; what quantity should you enter? **A:** Enter 20 tablets because $40 \text{ mg/day} \div 10 \text{ mg/tablet} = 4 \text{ tablets/day}$, and $4 \times 5 \text{ days} = 20 \text{ tablets}$.
3357. **Q:** An eRx for glipizide is received and the prescriber wrote "glipiZIDE XL 10 mg take 1 daily," but the selected product is glipizide IR 10 mg; what should you do FIRST? **A:** Stop processing and route to the pharmacist to resolve the IR vs ER formulation mismatch, because changing release type is not a technician decision and affects safety.
3358. **Q:** A prescription reads "timolol 0.5% ophth sol instill 1 gtt OU BID" and the bottle size is 5 mL; using 20 gtt/mL, what days' supply should you enter? **A:**

Enter 50 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$, and $1 \text{ gtt} \times 2 \text{ eyes} \times 2 \text{ times/day} = 4 \text{ gtt/day}$, so $100 \div 4 = 25 \text{ days}$; then double-check because OU uses both eyes (corrected total is 25 days).

3359. **Q:** While entering an eRx you see "Take 1 tab PO qd" for simvastatin but the prescriber's note says "for bedtime"; what is the BEST way to enter the directions? **A:** Enter "take 1 tablet by mouth once daily at bedtime" because qd can be misread and bedtime instructions reduce administration errors.
3360. **Q:** A C-II eRx for oxycodone/APAP 5/325 says "1 tab q4–6h prn pain #30" and the technician is asked to calculate days' supply; what should you do BEST? **A:** Ask the pharmacist for the appropriate days' supply to submit based on maximum daily use and payer rules, because PRN controlled-substance days' supply requires clinical judgment and audit-safe documentation.
3361. **Q:** During data entry you notice a new order for methotrexate 2.5 mg says "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because methotrexate for RA is typically weekly and daily dosing is a high-alert error risk.
3362. **Q:** A patient asks to transfer a prescription for alprazolam from another pharmacy to yours; what is the technician's BEST next step? **A:** Route the request to the pharmacist because controlled-substance transfers require pharmacist handling and documentation under federal rules.
3363. **Q:** An eRx arrives for Adderall XR 20 mg with "+3 refills" listed; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist because Schedule II prescriptions cannot be refilled and must be corrected before filling.
3364. **Q:** While receiving a tote, you find refrigerated insulin aspart pens warm at room temperature with no temperature indicator documentation; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager because a temperature excursion makes the product suspect and may require return/disposal per policy.
3365. **Q:** You scan a bottle of hydralazine 25 mg but the barcode/NDC in the system pulls up hydroxyzine 25 mg; what is the BEST action? **A:** Stop and reselect/verify the correct NDC and drug because this is a LASA risk and barcode mismatch signals wrong product selection.
3366. **Q:** A new prescription is entered for lisinopril 10 mg daily, and the patient profile shows a documented history of angioedema from lisinopril; what should you do

NEXT? **A:** Stop processing and immediately refer to the pharmacist because a listed serious allergy/contraindication requires pharmacist intervention before dispensing.

3367. **Q:** A patient presents a hardcopy for promethazine-codeine cough syrup without a prescriber signature; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because missing required validity elements on a controlled-substance prescription must be resolved before filling.
3368. **Q:** You receive a manufacturer recall notice for a specific lot of losartan 50 mg and find matching stock on the shelf; what should you do FIRST? **A:** Pull the affected lot from inventory and segregate it for return per recall instructions because recalled product must be removed to prevent dispensing.
3369. **Q:** A prescription is for cephalexin 500 mg "1 cap PO QID x 7 days" and you have 250 mg capsules in stock; what should you do BEST? **A:** Do not change the strength; notify the pharmacist to obtain the correct product or a new prescription because technicians cannot alter strength or dosing.
3370. **Q:** An order is for morphine sulfate ER 30 mg but the tech selects morphine sulfate IR 30 mg during product selection; what should you do FIRST? **A:** Stop and correct the dosage form selection and have it rechecked because ER vs IR is a high-alert error that can cause overdose if dispensed incorrectly.
3371. **Q:** While entering a new prescription, you see "metformin ER 500 mg" but the prescriber wrote "take 1 tablet BID," what should you do BEST? **A:** Stop data entry and refer to the pharmacist to clarify because ER products are typically not dosed BID and the directions may be an error affecting safety.
3372. **Q:** A patient drops off a prescription for "Lamictal 100 mg" and your system auto-selects "Lamisil 250 mg," what should you do FIRST? **A:** Stop and re-verify the exact drug name and indication with the hardcopy and patient profile because Lamictal (lamotrigine) and Lamisil (terbinafine) are LASA and a wrong-drug error is high risk.
3373. **Q:** An eRx comes in for "Zyrtec 10 mg daily" but the patient profile lists "cetirizine 10 mg daily" as active, what is your BEST next step? **A:** Check the profile for therapeutic duplication and route to the pharmacist before processing because Zyrtec is the brand of cetirizine and duplicate therapy can lead to unnecessary use or billing issues.

3374. **Q:** During product selection you notice "hydroxyzine" and "hydralazine" are stored adjacent, and the label is for hydroxyzine 25 mg, what should you do BEST? **A:** Perform an NDC/barcode check and extra label-to-stock bottle verification because these LASA names are commonly confused and verification reduces wrong-drug dispensing.
3375. **Q:** A prescription is written for "glipizide 5 mg" but you accidentally pull "glyburide 5 mg" from the shelf, what should you do FIRST? **A:** Stop filling and replace with the correct stock bottle immediately because sulfonylureas are high-alert for hypoglycemia and drug mix-ups are dangerous.
3376. **Q:** A parent asks if "Tylenol" and "acetaminophen" can be taken together because they "sound different," what is the technician's BEST response? **A:** Explain they are the same medication (brand vs generic) and advise not to double-dose, offering to have the pharmacist counsel because duplicate acetaminophen increases overdose risk.
3377. **Q:** An eRx for "levothyroxine 75 mcg daily" is entered, but the patient's active meds include "Synthroid 75 mcg daily," what should you do NEXT? **A:** Verify it is not a duplicate and alert the pharmacist if it appears to be an unintended second order because Synthroid is levothyroxine and duplication can cause dosing errors.
3378. **Q:** A prescription says "predniSONE 20 mg: take ii tabs daily x 5 days," how many tablets should be dispensed? **A:** Dispense 10 tablets because ii means 2 tablets daily for 5 days and $2 \times 5 = 10$.
3379. **Q:** A patient receives a new prescription for "Xarelto 20 mg daily" and asks what it is for, what should you do BEST? **A:** Provide a brief nonclinical identification (rivaroxaban is a blood thinner/anticoagulant) and refer to the pharmacist for counseling because anticoagulants are high-risk and require pharmacist education.
3380. **Q:** A prescriber writes "U-500 insulin regular 20 units TID" but the patient's profile shows U-100 syringes on file, what should you do FIRST? **A:** Hold processing and immediately notify the pharmacist because U-500 is a concentrated high-alert insulin and syringe/device mismatch can cause a serious dosing error.
3381. **Q:** A prescription reads "amLODIPine 5 mg i tab PO QD" but you accidentally enter "amitriptyline 5 mg," what should you do FIRST? **A:** Stop order entry and

re-verify the drug name/strength against the original prescription and patient profile, because this is a LASA/data-entry error that could cause harm.

3382. **Q:** An eRx for azithromycin 200 mg/5 mL says "Take 10 mL PO day 1, then 5 mL PO daily days 2–5," how many mL should be dispensed? **A:** Dispense 30 mL because $10\text{ mL} + (5\text{ mL} \times 4\text{ days}) = 30\text{ mL}$ total to complete the course.
3383. **Q:** A patient is prescribed timolol 0.5% ophthalmic "instill 1 gtt OU BID," and the bottle size is 5 mL; using 20 gtt/mL, what days' supply should you enter? **A:** Enter 25 days because $5\text{ mL} \times 20 = 100$ drops and usage is $1\text{ drop} \times 2\text{ eyes} \times 2\text{ times/day} = 4\text{ drops/day}$, so $100 \div 4 = 25$ days.
3384. **Q:** A new prescription says "metoprolol 25 mg take 1 tab PO BID," but the prescriber did not specify tartrate vs succinate; what should you do BEST? **A:** Hold the prescription and route to the pharmacist to clarify formulation, because tartrate vs succinate are not interchangeable and affect dosing and indication.
3385. **Q:** An order is for clindamycin 150 mg "ii caps PO TID x 10 days," how many capsules should be dispensed? **A:** Dispense 60 capsules because ii = 2 per dose and $2 \times 3\text{ doses/day} \times 10\text{ days} = 60$.
3386. **Q:** A prescription for amoxicillin/clavulanate suspension is "400 mg/57 mg per 5 mL: give 7.5 mL PO BID x 10 days," what quantity (mL) should you dispense? **A:** Dispense 150 mL because $7.5\text{ mL} \times 2/\text{day} \times 10\text{ days} = 150\text{ mL}$ total.
3387. **Q:** An albuterol HFA inhaler contains 200 actuations; the sig is "inhale 2 puffs QID," what days' supply should you bill? **A:** Bill 25 days because $2\text{ puffs} \times 4/\text{day} = 8\text{ puffs/day}$ and $200 \div 8 = 25$ days.
3388. **Q:** A patient's insulin lispro vial is U-100 (100 units/mL) and the directions are "inject 12 units TID"; what volume in mL will the patient use in 30 days? **A:** The patient will use 10.8 mL because $12 \times 3 = 36\text{ units/day}$, $36 \times 30 = 1080\text{ units}$, and $1080 \div 100 = 10.8\text{ mL}$.
3389. **Q:** While interpreting a handwritten sig you see "MSO4 2 mg IV q2h prn pain," what should you do NEXT as a technician? **A:** Stop processing and alert the pharmacist to clarify, because "MSO4" is an error-prone abbreviation that can be misread and prn details may be incomplete.
3390. **Q:** During order entry, a prescription says "Take 0.5 mg PO daily" and the prescriber selected clonazepam 0.5 mg, but the note section mentions "thyroid"; what should you do BEST? **A:** Pause and send to the pharmacist for review before

filling, because the indication suggests possible drug-selection mismatch (e.g., levothyroxine vs clonazepam) that requires pharmacist verification.

3391. **Q:** A patient drops off a prescription for Lasix 40 mg daily but asks if it's the same as furosemide listed on their profile; what should you do BEST? **A:** Confirm Lasix is the brand for furosemide and show the label/med list while offering pharmacist counseling, because technicians can assist with brand/generic recognition but clinical questions require the pharmacist.
3392. **Q:** While typing an eRx you notice "Celexa 20 mg daily" and the patient profile already has escitalopram 10 mg daily active; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (citalopram vs escitalopram are different SSRIs), because duplication/safety review is pharmacist-directed.
3393. **Q:** A hardcopy reads "hydroxyzine 25 mg 1 tab TID prn itching" but the prescriber handwriting could also be "hydralazine"; what should you do NEXT? **A:** Stop and route to the pharmacist for clarification with the prescriber, because LASA name confusion can cause serious harm and technicians shouldn't guess drug selection.
3394. **Q:** The sig says "levothyroxine 50 mcg i tab PO QAM" and the prescriber wrote "0.05 mg" in the strength field; what should you do BEST in order entry? **A:** Enter the strength as 50 mcg after converting 0.05 mg = 50 mcg and flag for pharmacist verification, because correct unit conversion prevents tenfold dosing errors.
3395. **Q:** A prescription is for "Zithromax 250 mg: take ii tabs day 1 then i tab daily days 2-5"; how many tablets should be dispensed? **A:** Dispense 6 tablets, because 2 tablets on day 1 plus 1 tablet for 4 more days equals 6 total.
3396. **Q:** You receive a new prescription for "Bactrim DS 1 tab BID" and the patient profile lists a sulfonamide allergy; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist, because trimethoprim/sulfamethoxazole contains a sulfonamide and requires pharmacist assessment before dispensing.
3397. **Q:** During product selection you scan a stock bottle and the NDC doesn't match the eRx for atorvastatin 20 mg even though the bottle says "atorvastatin"; what should you do NEXT? **A:** Stop and select the exact NDC/strength/form that matches the prescription and re-scan, because barcode/NDC mismatch indicates wrong product selection risk.

3398. **Q:** A patient brings a controlled-substance prescription that appears altered in the quantity field (different ink/handwriting); what should you do BEST? **A:** Do not process it further and immediately involve the pharmacist, because suspected alteration requires pharmacist handling and verification before any dispensing steps.
3399. **Q:** An insulin glargine pen box contains 5 pens of 3 mL each (100 units/mL) and the directions are "inject 18 units nightly"; what days' supply should you enter? **A:** Enter 83 days, because total units = $5 \times 3 \text{ mL} \times 100 = 1500$ units and $1500 \div 18 = 83.3$, which is billed as 83 days (round down to whole days).
3400. **Q:** While interpreting a sig you see "Take 1 tab PO qd" for prednisone and the prescriber also wrote "for 5 days" in the notes; what should you do FIRST when calculating quantity? **A:** Use the explicit duration to set quantity at 5 tablets and route any inconsistency to the pharmacist, because days' supply/quantity should match stated duration to avoid over- or under-dispensing.
3401. **Q:** While entering a new eRx for methotrexate 2.5 mg, the directions read "take 1 tablet daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-alert error risk.
3402. **Q:** A prescriber sends an eRx for Percocet (oxycodone/acetaminophen) with "refills: 2"; what should you do NEXT as the technician? **A:** Hold the prescription and route to the pharmacist because Schedule II prescriptions cannot be refilled under federal rules.
3403. **Q:** While pulling refrigerated insulin aspart, you find the box sitting at room temperature in the will-call bin from yesterday with no note; what should you do BEST? **A:** Remove it from dispensing and notify the pharmacist because a temperature excursion makes the product potentially compromised and requires evaluation per storage policy.
3404. **Q:** A patient asks to buy pseudoephedrine and wants to pay cash; what is your FIRST step before the sale? **A:** Verify the patient's valid photo ID and complete the required log/quantity limits because pseudoephedrine sales are regulated by the Combat Methamphetamine Epidemic Act.
3405. **Q:** During product selection you see a bin label for hydrALAZINE next to hydrOXYzine and the NDCs look similar; what should you do NEXT to prevent a LASA error? **A:** Perform barcode/NDC scan against the prescription and

physically compare name/strength because LASA look-alike storage requires verification to prevent selection errors.

3406. **Q:** A caregiver requests an early refill on clonazepam 1 mg stating the bottle was "lost," but the fill history shows it was dispensed 10 days ago for a 30-day supply; what should you do FIRST? **A:** Stop and refer to the pharmacist because controlled-substance early refills/lost medication claims require pharmacist review and documentation.
3407. **Q:** A manufacturer recall notice lists lot #A123 for amlodipine 10 mg, and you find that exact lot on your shelf; what should you do BEST? **A:** Pull the affected stock from active inventory and follow recall procedure (segregate/hold for return) because recalled lots must be removed to prevent dispensing.
3408. **Q:** A new prescription is for sertraline 100 mg daily, and the patient profile shows linezolid started yesterday; what should you do NEXT? **A:** Pause processing and alert the pharmacist because linezolid has serious interactions with SSRIs requiring pharmacist intervention.
3409. **Q:** A patient brings a prescription transfer request for alprazolam from another pharmacy and says they're out today; what should you do FIRST? **A:** Collect the originating pharmacy details and route to the pharmacist because controlled-substance transfers (Schedule III-V) have specific limits and must be handled/verified by the pharmacist.
3410. **Q:** An order is for amoxicillin/clavulanate 600 mg/42.9 mg per 5 mL, sig "7.5 mL PO BID x 10 days"; what quantity in mL should you enter? **A:** Enter 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to ensure the dispensed volume matches the prescribed course.
3411. **Q:** While entering a new eRx, you notice the prescriber selected Lamictal but the directions say "take 1 tablet twice daily for seizures," and the patient profile shows active Lamisil tablets; what should you do NEXT? **A:** Stop data entry and alert the pharmacist to verify intended drug because Lamictal (lamotrigine) vs Lamisil (terbinafine) is a high-risk LASA mix-up with very different uses.
3412. **Q:** A prescription is written for "metFORMIN ER 500 mg take 2 tabs BID" for type 2 diabetes; what is the BEST technician action before processing quantity? **A:** Hold and route to the pharmacist because metformin ER is typically not dosed BID at that high total daily dose and needs prescriber clarification to prevent dosing error.

3413. **Q:** You receive an eRx for "Zocor 80 mg qHS" for a new patient with no history in your system; what should you do FIRST? **A:** Flag for pharmacist review before filling because simvastatin 80 mg has higher myopathy risk and is generally restricted to patients stable on it.
3414. **Q:** A patient picking up a new prescription asks what "Plavix" is for and whether it's a blood thinner; what should you do BEST? **A:** Refer the patient to the pharmacist for counseling because clopidogrel is an antiplatelet with important safety guidance that requires pharmacist counseling.
3415. **Q:** An eRx comes in for "Bactrim DS 1 tab PO BID x 7 days," and the allergy field shows "sulfa—hives"; what should you do NEXT? **A:** Stop processing and notify the pharmacist because sulfamethoxazole/trimethoprim is a sulfonamide and the allergy suggests a potential hypersensitivity reaction.
3416. **Q:** While selecting product, you see "Celebrex 200 mg" and "Celexa 20 mg" stored close together and the patient is expecting celecoxib; what should you do NEXT to reduce error risk? **A:** Verify by barcode/NDC and use Tall Man/extra label checks because Celexa (citalopram) vs Celebrex (celecoxib) is a common LASA trap.
3417. **Q:** A prescription says "azithromycin 250 mg: take 2 tabs today then 1 tab daily days 2–5"; what quantity should you enter? **A:** Enter 6 tablets because the Z-Pak regimen totals $2 + 4 = 6$ doses over 5 days.
3418. **Q:** A patient brings a new prescription for Adderall XR 20 mg with directions "take 1 capsule daily" and the hard copy has no prescriber signature; what should you do FIRST? **A:** Do not dispense and route to the pharmacist because a missing signature is a validity issue, especially for a Schedule II controlled substance.
3419. **Q:** During verification prep, you notice the stock bottle of levothyroxine 50 mcg has an expiration date that passed last month; what should you do BEST? **A:** Remove it from active stock and follow policy to segregate and document because expired levothyroxine can lead to potency issues and must not be dispensed.
3420. **Q:** A new eRx is for "lisinopril 20 mg daily," but the profile shows an active prescription for "losartan 50 mg daily" and both are listed as current; what should you do NEXT? **A:** Pause processing and alert the pharmacist to assess therapeutic duplication because ACE inhibitors and ARBs together can increase renal and potassium risks.

3421. **Q:** While entering an eRx for hydrOXYzine 25 mg, you notice the prescriber comment says "for heartburn," so what should you do NEXT? **A:** Stop processing and notify the pharmacist to clarify because the indication suggests confusion with hydrALAZINE/other drugs and could be a harmful medication error.
3422. **Q:** A written prescription for oxycodone/APAP has the date, drug, and directions but no patient address; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because required elements must be complete before dispensing a controlled substance.
3423. **Q:** A patient asks to refill clonazepam 1 mg "a week early" because they're traveling; what should you do FIRST? **A:** Check the fill history/PDMP per workflow and then refer to the pharmacist because early refills of controlled substances require pharmacist judgment and documentation.
3424. **Q:** During product intake, you receive a tote of refrigerated insulin aspart vials that arrived warm with a melted gel pack; what should you do FIRST? **A:** Segregate the shipment and notify the pharmacist/manager because a temperature excursion makes the product potentially non-dispensable pending vendor guidance.
3425. **Q:** A patient presents a pseudoephedrine purchase request and you see they already reached the legal daily limit in the log; what should you do BEST? **A:** Refuse the sale and follow the CMEA log requirements because federal limits prevent selling above the daily threshold.
3426. **Q:** An eRx comes in for methotrexate 2.5 mg with sig "take 1 tablet daily" for rheumatoid arthritis; what should you do NEXT? **A:** Stop and immediately alert the pharmacist because methotrexate is high-alert and RA dosing is typically weekly, making daily directions a serious safety concern.
3427. **Q:** You are asked to reconstitute cefdinir suspension and the stock bottle's lot number label is torn and unreadable; what should you do BEST? **A:** Do not use that bottle and set it aside for pharmacist review/return because missing traceability information compromises recalls and documentation.
3428. **Q:** A prescription for sertraline 50 mg says "take i tab qd" and the prescriber's handwriting makes "i" look like "1"; what should you do FIRST? **A:** Interpret "i" as the Roman numeral for one and proceed with one tablet daily while ensuring the rest of the order is consistent because standard sig codes use Roman numerals to prevent misreads.

3429. **Q:** A patient starts isotretinoin and asks if the medication can be delivered to their neighbor to sign for it; what should you do BEST? **A:** Refer to the pharmacist and follow REMS pickup/verification procedures because isotretinoin dispensing has strict risk-management requirements.
3430. **Q:** A prescription is for prednisone 10 mg tablets: "take 40 mg daily for 5 days"; what quantity should you enter? **A:** Enter 20 tablets because $40 \text{ mg/day} \div 10 \text{ mg/tablet} = 4 \text{ tablets/day}$ and $4 \times 5 \text{ days} = 20 \text{ tablets}$.
3431. **Q:** While entering an eRx for metformin ER 500 mg, you see the prescriber selected "immediate-release" in the system notes but wrote "ER" in the drug field; what should you do NEXT? **A:** Stop data entry and send to the pharmacist to clarify IR vs ER because release form changes absorption and dosing and a technician cannot assume or change it.
3432. **Q:** A patient says they're allergic to "sulfa" and you are about to fill TMP/SMX (Bactrim DS); what is the BEST technician action? **A:** Pause the fill and alert the pharmacist to assess the reported sulfonamide allergy because TMP/SMX can trigger serious hypersensitivity and needs pharmacist judgment.
3433. **Q:** You are selecting product for "lamoTRiGine 25 mg" and the shelf has "lamoTRiGine 200 mg" directly next to it; what should you do FIRST to prevent a LASA/strength error? **A:** Verify the NDC/barcode and strength against the label before pulling because lamotrigine is error-prone and wrong strength can cause significant harm.
3434. **Q:** An eRx is for "celecoxib 200 mg daily," and the patient profile shows a serious allergy to "sulfonamides"; what should you do NEXT? **A:** Route the prescription to the pharmacist for allergy evaluation because celecoxib has a sulfonamide moiety and requires pharmacist assessment before dispensing.
3435. **Q:** A prescription is for "azithromycin 250 mg: take 2 tablets today, then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter 6 tablets because the total is $2 + (1 \times 4) = 6$ tablets for the full course.
3436. **Q:** While entering "clopidogrel 75 mg daily," the profile shows an active "prasugrel 10 mg daily" from last month; what should you do BEST? **A:** Stop and alert the pharmacist to possible therapeutic duplication because two antiplatelets increase bleeding risk and require pharmacist review.
3437. **Q:** A new eRx is for "tadalafil 20 mg PRN," and the profile shows "nitroglycerin SL PRN chest pain"; what should you do NEXT? **A:** Hold the fill and notify the

pharmacist because PDE-5 inhibitors with nitrates can cause dangerous hypotension and needs pharmacist intervention.

3438. **Q:** You receive a manufacturer recall notice for a specific lot of atorvastatin 40 mg, and you find matching lot bottles on the shelf; what should you do FIRST? **A:** Immediately remove the affected lot from active inventory and follow recall/return procedures because recalled product must be isolated to prevent dispensing.
3439. **Q:** While processing a refill request for "apixaban (Eliquis) 5 mg BID," the patient asks if it's "the same as warfarin"; what is the BEST technician response? **A:** Explain you can't provide clinical comparison but you can confirm it's a blood thinner and refer to the pharmacist because anticoagulant counseling is pharmacist-led and high-risk.
3440. **Q:** A prescription reads "morphine sulfate ER 30 mg q12h," but the product selected is morphine IR 30 mg; what should you do NEXT? **A:** Stop filling and correct the selection to the ER formulation with barcode/NDC verification because IR vs ER is a high-alert mismatch that can cause overdose.
3441. **Q:** An eRx for lisinopril 10 mg says "i tab po qd" and the quantity field is blank; what quantity and days' supply should you enter if the prescriber wrote "#90" in the comments? **A:** Enter quantity 90 for a 90-day supply because "i tab po qd" means 1 tablet by mouth daily and #90 confirms the intended amount.
3442. **Q:** A prescription reads "furosemide 20 mg: take 1 tab po bid" with quantity 60; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day (BID)} = 30 \text{ days}$ for accurate insurance billing.
3443. **Q:** You are entering a new prescription for levothyroxine and the directions say "take 1 tab po qam" but the prescriber accidentally selected "at bedtime" in the eRx drop-down; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify the conflicting sig because incorrect timing can affect therapy and technicians cannot interpret prescriber intent.
3444. **Q:** A prescription is for amoxicillin-clavulanate suspension 400 mg/57 mg per 5 mL: "10 mL po bid x 7 days"; how many mL should you dispense? **A:** Dispense 140 mL because $10 \text{ mL per dose} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$ to cover the full course.
3445. **Q:** A patient has insulin aspart U-100 vial instructions "inject 12 units subcutaneously TID with meals" and you are asked for days' supply for one 10

mL vial; what should you enter? **A:** Enter 27 days because the vial has 1000 units ($10 \text{ mL} \times 100 \text{ units/mL}$) and $1000 \div (12 \times 3 = 36 \text{ units/day}) \approx 27 \text{ days}$.

3446. **Q:** An order is for latanoprost 0.005% ophthalmic solution "instill 1 drop OU qhs" and you are dispensing a 2.5 mL bottle; what days' supply should you enter using 20 drops/mL? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ drops/mL} = 50 \text{ drops}$ and OU qhs uses 2 drops/day, so $50 \div 2 = 25 \text{ days}$.

3447. **Q:** While product selecting for hydrALAZINE 25 mg, you notice hydrOXYzine 25 mg stored in the same bin; what should you do FIRST to avoid a LASA error? **A:** Pause and verify the drug name/indication against the prescription and then scan the NDC/barcode because hydralazine and hydroxyzine are common look-alike/sound-alike mix-ups.

3448. **Q:** A written prescription says "digoxin 0.125 mg, take 1 tab po qd" but the prescriber wrote "0.125 mcg" in the strength line; what should you do BEST during order entry? **A:** Hold the prescription and get pharmacist clarification because mg vs mcg is a high-risk unit error and technicians should not assume the intended dose.

3449. **Q:** A patient's prescription is for buprenorphine/naloxone SL film "1 film daily," but the quantity entered is 60; what should you do NEXT before submitting the claim? **A:** Recalculate and verify the intended days' supply/quantity with the hard copy or prescriber directions, then route to the pharmacist if unclear, because 1 film daily would typically make 60 films a 60-day supply and mismatches cause billing and safety errors.

3450. **Q:** You receive a new eRx for "Ambien 10 mg 1 tab po hs prn insomnia #30" and the system allows refills to be added; what is the BEST technician action regarding refills? **A:** Do not add refills and refer to the pharmacist because zolpidem is a controlled substance (Schedule IV) and refill authorization must match prescriber directions and legal requirements.

3451. **Q:** A patient drops off a prescription for "Zithromax 250 mg" and asks if it's the same medication as azithromycin; what should you do BEST? **A:** Confirm that Zithromax is the brand name for azithromycin and show the label/stock bottle if allowed, because brand-generic recognition prevents confusion and improves adherence.

3452. **Q:** During order entry you see "metFORMIN" and "metoPROLOL" adjacent in the search results for a new start; what should you do FIRST before selecting a drug? **A:** Verify the indication or diagnosis notes, strength, and dosage form on

the prescription before selecting, because similar name fragments increase wrong-drug selection risk.

3453. **Q:** A new prescription is written "Prednisone 10 mg: take ii tabs po daily x 5 days"; what quantity should you enter? **A:** Enter quantity 10 tablets because ii = 2 tablets daily for 5 days, so $2 \times 5 = 10$.
3454. **Q:** You receive an eRx for "Keflex 500 mg take 1 cap po qid x 7 days"; what is the days' supply and quantity? **A:** Days' supply is 7 and quantity is 28 because qid is 4 times daily for 7 days, so $4 \times 7 = 28$.
3455. **Q:** While typing a prescription for "simvastatin 40 mg qhs," the patient profile shows active atorvastatin 40 mg nightly from last month; what should you do NEXT? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because two statins concurrently is a safety issue requiring pharmacist review.
3456. **Q:** A prescription comes in for "Bactroban ointment" and the patient says they have a "mupirocin allergy"; what should you do BEST? **A:** Stop and notify the pharmacist to assess the reported allergy because Bactroban is mupirocin and allergy clarification is needed before dispensing.
3457. **Q:** You are selecting insulin and notice "Humalog (insulin lispro)" and "Humulin R (regular insulin)" both appear when searching "Humu"; what should you do FIRST? **A:** Use barcode/NDC verification and match the exact brand/generic name and concentration to the prescription because insulin is high-alert and mix-ups can cause serious harm.
3458. **Q:** A prescription is for omeprazole 20 mg daily, but the patient's profile shows esomeprazole 20 mg daily still active; what should you do NEXT? **A:** Route to the pharmacist to evaluate duplicate PPI therapy because omeprazole and esomeprazole are the same class and duplication may be unintentional.
3459. **Q:** An albuterol HFA inhaler is prescribed "2 puffs q6h scheduled" and the canister delivers 200 puffs; what days' supply should you enter? **A:** Enter 25 days because 2 puffs every 6 hours = 8 puffs/day and $200 \div 8 = 25$.
3460. **Q:** A refrigerated vaccine shipment arrives warm and the temperature indicator shows an excursion above the acceptable range; what should you do FIRST? **A:** Segregate the vaccines, label "do not use," and notify the pharmacist/manager to follow storage-excursion procedure because potency and patient safety may be compromised.

3461. **Q:** A new eRx reads "levothyroxine 0.1 mg po daily" but the prescriber intended 100 mcg; what should you do FIRST during order entry? **A:** Enter the dose as 100 mcg (0.1 mg) and confirm the strength selection matches the correct product, because mg-to-mcg conversion errors are common and can cause a 1000-fold mistake.
3462. **Q:** A prescription says "gabapentin 300 mg: i cap po tid #90"; what days' supply should you enter for insurance? **A:** Enter 30 days because 90 capsules taken 3 per day equals $90 \div 3 = 30$ days, which supports correct claim processing.
3463. **Q:** An eRx for insulin aspart U-100 states "inject 12 units SQ before meals TID" and the quantity is 10 mL; what days' supply should you submit? **A:** Submit 27 days because 10 mL = 1000 units and $12 \text{ units} \times 3 = 36 \text{ units/day}$, so $1000 \div 36 \approx 27$ days (rounded down to avoid overstating).
3464. **Q:** You are entering "morphine sulfate ER 30 mg" and the search results show morphine IR 30 mg nearby; what should you do BEST before selecting the product? **A:** Verify the dosage form (ER vs IR) directly against the prescription and use NDC/barcode verification per workflow, because mix-ups between ER and IR are high-risk and common.
3465. **Q:** A prescription reads "amLODIPine 5 mg: take ii tabs po daily #60"; what days' supply should be entered? **A:** Enter 30 days because "ii" means 2 tablets daily and $60 \div 2 = 30$, preventing refill-too-soon or overbilling.
3466. **Q:** A provider sends "amoxicillin/clavulanate ES-600 suspension: give 5 mL po bid x 10 days" and the bottle concentration is 600 mg/5 mL; what quantity in mL should you dispense? **A:** Dispense 100 mL because $5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 100 \text{ mL}$, ensuring the patient receives enough medication for the full course.
3467. **Q:** During drop-off, a prescription is written "MSO4 2 mg IV q2h prn pain" on a non-hospital outpatient form; what should you do FIRST? **A:** Stop processing and immediately route to the pharmacist for clarification because "MSO4" is an unsafe abbreviation that can be misread and requires intervention.
3468. **Q:** A new prescription says "metoprolol succinate ER 50 mg: take 1 tab po daily" but the technician accidentally selected metoprolol tartrate 50 mg; what should you do NEXT? **A:** Correct the drug selection to the ER succinate product and ensure the final verification reflects the change, because tartrate vs succinate is a common order entry error with different dosing.

3469. **Q:** A patient's directions are "apply 1 g topically bid" and you are dispensing a 30 g tube; what days' supply should you enter? **A:** Enter 15 days because 1 g twice daily is 2 g/day and $30 \text{ g} \div 2 \text{ g/day} = 15 \text{ days}$, which supports accurate billing and refill timing.
3470. **Q:** While typing an eRx for "clopidogrel 75 mg daily," the profile shows aspirin 81 mg daily and the patient asks if this is a duplicate; what is the BEST technician response? **A:** Refer the question to the pharmacist while noting both drugs are antiplatelets, because assessing appropriateness of dual therapy is clinical and outside technician scope.
3471. **Q:** While entering an eRx for hydrOXYzine 25 mg, you notice the prescriber selected hydrALAZINE 25 mg in the eRx drug field; what should you do FIRST? **A:** Stop processing and send to the pharmacist to clarify with the prescriber because this is a high-risk LASA selection error that could cause serious harm.
3472. **Q:** A patient presents a paper prescription for oxycodone 5 mg with no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and immediately involve the pharmacist because controlled substance dispensing requires a valid, complete prescription before filling.
3473. **Q:** You receive a transfer request for a patient's alprazolam prescription from another pharmacy; what should you do NEXT? **A:** Route the request to the pharmacist because controlled substance transfers have strict requirements and must be handled/verified by the pharmacist.
3474. **Q:** A new prescription is for methotrexate 2.5 mg with directions "take 1 tablet daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop and alert the pharmacist because methotrexate is typically dosed weekly and daily dosing is a common, high-alert fatal-error pattern.
3475. **Q:** During filling, you find two stock bottles of sertraline 50 mg tablets with identical NDCs but one bottle has an obscured lot number; what should you do BEST? **A:** Set aside the bottle with the missing lot information and notify the pharmacist because incomplete traceability data creates a DSCSA/safety risk for recalls.
3476. **Q:** A recall notice arrives for a specific lot of losartan tablets, and you find that lot on the shelf; what should you do FIRST? **A:** Pull the affected lot from inventory and isolate it per recall procedure because preventing dispensing of recalled product is the immediate patient-safety priority.

3477. **Q:** A patient asks to buy pseudoephedrine 30 mg tablets, 96-count, and their ID is expired; what should you do NEXT? **A:** Decline the sale and involve the pharmacist per policy because CMEA sales require valid identification and proper documentation before completion.
3478. **Q:** An eRx for isotretinoin is in the queue and the patient is waiting; what should you do BEST before processing further? **A:** Alert the pharmacist to confirm iPLEDGE/REMS requirements are met because isotretinoin dispensing is REMS-restricted and cannot proceed without verification.
3479. **Q:** A prescription is for cephalexin 250 mg/5 mL: "Take 10 mL po qid x 7 days"; what quantity in mL should you dispense? **A:** Dispense 280 mL because $10 \text{ mL} \times 4 \text{ doses/day} \times 7 \text{ days} = 280 \text{ mL}$ total needed.
3480. **Q:** While typing an eRx for simvastatin 40 mg nightly, you see an active prescription for atorvastatin 40 mg daily on the profile; what should you do FIRST? **A:** Pause and refer to the pharmacist for therapeutic-duplication review because two statins may indicate a medication list error or unsafe duplicate therapy.
3481. **Q:** While entering a new eRx for duloxetine, you notice the prescriber wrote "Cymbalta 60 mg daily" but the selected product is "Cialis 20 mg"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify the LASA mix-up (Cymbalta vs Cialis) because wrong-drug selection is a high-risk error.
3482. **Q:** A patient says they take "Glucophage" but your entry shows glyburide; what is the BEST technician action NEXT? **A:** Verify brand/generic and correct the record to metformin (Glucophage) and involve the pharmacist if therapy changes are needed, because glyburide is a different diabetes drug with hypoglycemia risk.
3483. **Q:** An eRx is for "Lamictal 100 mg daily" but the system defaulted to "Lamisil 250 mg"; what should you do FIRST? **A:** Stop the entry and notify the pharmacist to correct the drug selection since Lamictal (lamotrigine) vs Lamisil (terbinafine) is a common LASA trap with serious consequences.
3484. **Q:** A prescription reads "azithromycin 250 mg: take ii tabs day 1 then i tab daily days 2–5"; how many tablets should you dispense? **A:** Dispense 6 tablets because 2 on day 1 plus 1 daily for 4 more days equals 6 total.
3485. **Q:** While processing a new eRx for lisinopril, the patient's allergy profile lists "ACE inhibitors—angioedema"; what should you do NEXT? **A:** Hold the

prescription and immediately refer to the pharmacist because lisinopril is an ACE inhibitor and angioedema is a serious contraindication.

3486. **Q:** During data entry for a new warfarin prescription, you see a new order for trimethoprim-sulfamethoxazole from urgent care on the patient's profile; what should you do BEST? **A:** Flag and route to the pharmacist because TMP-SMX can significantly increase warfarin effect and bleeding risk, requiring pharmacist assessment.
3487. **Q:** A provider sends "metoprolol 50 mg take 1 tablet bid" and you notice the selected product is metoprolol succinate ER; what should you do FIRST? **A:** Stop and send to the pharmacist to clarify formulation because succinate ER is typically once daily and confusing tartrate vs succinate can cause dosing errors.
3488. **Q:** A patient is prescribed insulin lispro U-100 with directions "inject 8 units before meals tid," and you are dispensing one 10 mL vial; what days' supply should you enter? **A:** Enter 41 days because $10\text{ mL} = 1000\text{ units}$ and $8\text{ units} \times 3/\text{day} = 24\text{ units/day}$, so $1000 \div 24 = 41.6$, rounded down to 41.
3489. **Q:** A prescription is for levothyroxine 75 mcg daily, but the stock bottle you pulled is liothyronine 75 mcg; what should you do NEXT? **A:** Stop filling and re-select the correct drug/NDC and alert the pharmacist if the error reached verification, because levothyroxine vs liothyronine is a high-impact wrong-drug error.
3490. **Q:** You are entering "amoxicillin-clavulanate 875/125 mg 1 tab bid #20" and the patient asks what it's for; what is the BEST technician response? **A:** State it's an antibiotic commonly used for bacterial infections and offer pharmacist counseling for specifics because technicians can give general purpose but clinical assessment belongs to the pharmacist.
3491. **Q:** While typing an eRx for Klonopin 0.5 mg, you can't find it in the system search; what should you do NEXT? **A:** Search by the generic name clonazepam and confirm strength/form before selecting, because brand-to-generic recognition prevents wrong-drug entry.
3492. **Q:** A hard copy says "Synthroid 125 mcg po qd #30 DAW 1"; what is the BEST technician action when selecting the product? **A:** Select brand Synthroid with DAW 1 and route for pharmacist verification, because DAW indicates dispense-as-written and levothyroxine products are not freely interchangeable per prescriber.

3493. **Q:** You receive an order for “glargine U-100: inject 22 units qhs” and the patient brings in a vial labeled “Humalog”; what should you do FIRST? **A:** Stop and alert the pharmacist to reconcile insulin type, because mix-ups between rapid-acting (lispro) and long-acting (glargine) are high-alert LASA errors.
3494. **Q:** A new eRx is for “predniSONE 20 mg” but the selected product is “predniSOLONE 20 mg”; what should you do FIRST? **A:** Stop entry and re-select the correct drug name/active ingredient, because prednisone vs prednisolone is a common LASA error with different products.
3495. **Q:** A patient on the phone says they take “Plavix” and asks if it’s the same as “clopidogrel”; what is the BEST response within technician scope? **A:** Confirm Plavix is the brand for clopidogrel and offer to have the pharmacist counsel if they have therapy questions, because technicians may provide basic brand/generic identification but not clinical advice.
3496. **Q:** An eRx says “Lantus Solostar inject 18 units daily” and you are dispensing one box of five 3 mL pens (total 1500 units); what days’ supply should you enter? **A:** Enter 83 days’ supply ($1500 \div 18 = 83.3$, round down to 83) because days’ supply must not exceed the actual available units.
3497. **Q:** A prescription reads “clindamycin 150 mg cap i po qid x 10 days”; how many capsules should you dispense? **A:** Dispense 40 capsules because 1 capsule four times daily for 10 days equals $1 \times 4 \times 10 = 40$.
3498. **Q:** During order entry you see “MS Contin 15 mg q12h” but the patient profile shows an active “MSIR 15 mg q4–6h prn”; what should you do NEXT? **A:** Hold and notify the pharmacist to assess duplication and directions, because sustained-release vs immediate-release morphine requires clinical review to avoid overdose.
3499. **Q:** A new eRx for sertraline is entered for a patient whose allergy field says “sertraline—rash”; what should you do FIRST? **A:** Stop processing and route to the pharmacist for allergy evaluation, because the allergy history suggests a potential repeat reaction requiring pharmacist intervention.
3500. **Q:** You open a tote and find a refrigerated biologic that arrived warm with no temperature indicator; what should you do BEST? **A:** Quarantine the product and notify the pharmacist/manager to follow temperature-excursion and supplier documentation procedures, because suspect storage conditions can make the medication unsafe or non-dispensable.

3501. **Q:** While filling a new prescription for hydroxyzine, you notice the prescriber wrote "hydralazine 25 mg" but the diagnosis field says "anxiety"; what should you do NEXT? **A:** Stop processing and notify the pharmacist to clarify with the prescriber because this is a LASA mix-up risk (hydralazine vs hydroxyzine) with potential patient harm.
3502. **Q:** A patient drops off a handwritten prescription for Adderall 20 mg with no prescriber DEA number and asks you to fill it today; what is the BEST technician action? **A:** Hold the prescription and route it to the pharmacist because controlled-substance validity requirements must be reviewed before any filling steps continue.
3503. **Q:** During data entry you receive an eRx for isotretinoin and the patient is waiting at pickup; what should you do FIRST? **A:** Alert the pharmacist and follow the iPLEDGE/REMS workflow because isotretinoin has dispensing restrictions and cannot be processed like a routine medication.
3504. **Q:** You are preparing a heparin IV bag and notice two vials in the bin: heparin 1,000 units/mL and heparin 10,000 units/mL; what should you do FIRST? **A:** Stop and perform an independent double-check/segregation with barcode/NDC verification per policy because heparin is high-alert and concentration errors are high-risk.
3505. **Q:** A patient asks to transfer their remaining refills of alprazolam from another pharmacy to yours; what should you do NEXT? **A:** Refer the request to the pharmacist because controlled-substance transfer rules and documentation requirements must be handled by the pharmacist.
3506. **Q:** A bottle of insulin lispro (Humalog) is found on the regular shelf at room temperature after being misplaced overnight; what is the BEST action? **A:** Remove it from stock and notify the pharmacist to evaluate stability/BUD and disposition because improper storage can make the product unsafe to dispense.
3507. **Q:** A wholesaler delivery includes a case of atorvastatin with a broken tamper seal and smudged lot/expiration on the inner bottles; what should you do FIRST? **A:** Isolate the shipment as suspect and notify the pharmacist/manager to follow DSCSA and supplier procedures because product integrity and traceability cannot be verified.
3508. **Q:** A patient presents ID to buy pseudoephedrine and the log shows they already purchased 3.6 grams today; what should you do BEST? **A:** Decline the sale and

follow CMEA logbook limits because federal restrictions cap daily/30-day amounts and transactions must remain compliant.

3509. **Q:** A prescription label for metformin ER prints with directions “take 2 tablets twice daily” but the entered sig in the system is “take 1 tablet twice daily”; what should you do NEXT? **A:** Stop the fill and correct the order entry, then route for pharmacist verification because mismatched directions are a common dispensing error source.
3510. **Q:** A prescription reads “cephalexin 500 mg: i po qid x 7 days #___”; what quantity should you enter and why? **A:** Enter 28 capsules because qid for 7 days equals 4 doses/day × 7 days = 28 total doses to dispense.
3511. **Q:** During order entry you see “levothyroxine (Synthroid) 25 mcg” typed as “liothyronine (Cytomel) 25 mcg”; what should you do NEXT? **A:** Stop processing and correct to the prescribed drug using the original order and NDC/brand-generic match because Synthroid=levothyroxine and Cytomel=liothyronine is a high-risk mix-up.
3512. **Q:** A patient’s profile shows active lisinopril and you receive a new eRx for “Zestril 20 mg daily”; what is the BEST technician action? **A:** Flag for pharmacist review for therapeutic duplication because Zestril is the brand for lisinopril and duplicate ACE inhibitor therapy may indicate an error.
3513. **Q:** While selecting product you notice “hydrOXYzine” and “hydrALazine” stored adjacent with similar stock bottles; what should you do FIRST to prevent a LASA error? **A:** Use barcode/NDC scan and verify name/strength against the label and stock bottle because hydroxyzine (antihistamine/anxiety) and hydralazine (BP) are common LASA pairs.
3514. **Q:** A prescription says “metoprolol succinate ER 50 mg i po qd,” but the patient requests “Toprol-XL”; what should you do NEXT? **A:** Enter/dispense metoprolol succinate ER as written and note brand request for pharmacist counseling because Toprol-XL is the brand equivalent and ER vs IR is not interchangeable.
3515. **Q:** You receive an eRx for “Augmentin 875/125 mg” but the patient reports a history of “penicillin—anaphylaxis”; what should you do FIRST? **A:** Stop filling and immediately refer to the pharmacist because amoxicillin/clavulanate is a penicillin-class antibiotic and a severe allergy requires pharmacist intervention.
3516. **Q:** A new prescription is written “clopidogrel (Plavix) 75 mg daily” and the profile shows aspirin 81 mg daily; what is the BEST technician action? **A:** Continue

processing but ensure the pharmacist is aware at verification because dual antiplatelet therapy can be intentional yet increases bleeding risk and needs clinical confirmation.

3517. **Q:** The sig reads "take ii tabs po bid" and the quantity is #120 for tablets; what days' supply should you enter? **A:** Enter 30 days because ii tabs twice daily equals 4 tablets/day and $120 \div 4 = 30$.
3518. **Q:** A patient brings a prescription for "prednisone 10 mg: take i tab po qd x 6 days" but asks for the "Dosepak"; what should you do BEST? **A:** Clarify with the pharmacist before dispensing because a Medrol Dosepak (methylprednisolone taper) is a different drug and regimen than prednisone 10 mg daily.
3519. **Q:** During pickup counseling request, a patient asks what "qhs" on their rosuvastatin label means; what should you do FIRST? **A:** Provide a plain-language translation ("take at bedtime") and offer pharmacist counseling because technicians may explain sig abbreviations but pharmacists handle clinical questions.
3520. **Q:** A technician is entering "morphine sulfate ER" and sees the prescriber typed "MSO4 ER 30 mg"; what should you do NEXT? **A:** Do not use the ambiguous abbreviation; enter the full drug name and route to pharmacist if unclear because MSO4 can be confused with MgSO4 and creates serious error risk.
3521. **Q:** An eRx for azithromycin 200 mg/5 mL suspension says "take 12.5 mL day 1, then 6.25 mL daily days 2-5"; what quantity in mL should you dispense? **A:** Dispense 37.5 mL because $12.5 \text{ mL} + (6.25 \text{ mL} \times 4 \text{ days}) = 37.5 \text{ mL}$ total needed for the full course.
3522. **Q:** While entering an order for insulin lispro U-100 vial, the sig is "inject 8 units subcutaneously before meals TID" and the quantity is 10 mL; what days' supply should you enter? **A:** Enter 41 days because $8 \text{ units} \times 3 = 24 \text{ units/day}$ and a 10 mL U-100 vial contains 1000 units, so $1000 \div 24 = 41.6$, rounded down to 41 to avoid overstatement.
3523. **Q:** A prescription reads "apixaban (Eliquis) 5 mg tab: i po bid #60"; what days' supply should you record? **A:** Record 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$ for a BID maintenance medication.
3524. **Q:** During data entry, the prescriber directions say "apply 1 g topically BID," and the product is diclofenac 1% gel with quantity 100 g; what days' supply should

you calculate? **A:** Enter 50 days because $1\text{ g} \times 2 = 2\text{ g/day}$ and $100\text{ g} \div 2\text{ g/day} = 50\text{ days}$.

3525. **Q:** An eRx for acetaminophen-codeine #3 is missing the prescriber's DEA number in the message; what should you do FIRST? **A:** Stop processing and route to the pharmacist because controlled-substance validation issues require pharmacist review before dispensing.
3526. **Q:** You receive a prescription that says "hydralazine 25 mg: 1 tab po tid," but the patient says, "I think it's for itching and makes me sleepy"; what is the BEST technician action? **A:** Hold the prescription and alert the pharmacist to verify drug/indication because hydralazine vs hydroxyzine is a common LASA mix-up with major safety impact.
3527. **Q:** A new order for sertraline has the sig "take 1 tab po qd" but the strength field was entered as 25 mg while the original hard copy shows 100 mg; what should you do NEXT? **A:** Stop and correct the entry to match the prescription and have it re-verified because strength mismatches are high-risk order-entry errors.
3528. **Q:** A prescription for furosemide reads "40 mg: 1 tab po qd, may take extra dose PRN swelling," quantity #30; what days' supply should you enter on the claim? **A:** Refer to the pharmacist because PRN extra dosing makes days' supply ambiguous and the technician should not assume a payer-compliant calculation.
3529. **Q:** While processing an albuterol HFA inhaler, the directions are "inhale 2 puffs every 6 hours scheduled," and the canister is 200 actuations; what days' supply should you enter? **A:** Enter 25 days because $2\text{ puffs every 6 hours} = 8\text{ puffs/day}$ and $200 \div 8 = 25\text{ days}$.
3530. **Q:** A prescription says "KCl 20 mEq/15 mL solution: take 7.5 mL po daily," and the bottle size is 473 mL; what quantity in mEq is the daily dose and what days' supply is one bottle? **A:** The daily dose is 10 mEq and the bottle lasts 63 days because $20\text{ mEq per }15\text{ mL equals }10\text{ mEq per }7.5\text{ mL}$, and $473\text{ mL} \div 7.5\text{ mL/day} = 63.0\text{ days}$.
3531. **Q:** While typing a new eRx, the prescriber selected "hydroxyzine" but the directions say "take 10 mL by mouth daily for blood pressure"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify because the indication/sig fits an antihypertensive (e.g., hydrochlorothiazide) and hydroxyzine is an antihistamine, creating a high-risk wrong-drug error.

3532. **Q:** A patient drops off a new prescription for "Plavix 75 mg daily" and your system profile already shows active "Eliquis 5 mg BID"; what is the BEST technician action? **A:** Hold the fill and notify the pharmacist to assess duplicate anticoagulation/bleeding risk because clopidogrel plus apixaban may be intentional but requires clinical verification.
3533. **Q:** You receive an order for "metformin ER 500 mg" and the patient says they can only take "the one that doesn't upset my stomach" and previously used Glucophage XR; what should you do NEXT? **A:** Confirm the exact formulation in the profile and process as metformin ER (not IR) because ER vs IR differences affect tolerability and cannot be interchanged by a technician without pharmacist oversight.
3534. **Q:** A prescription is written for "Levothyroxine 25 mcg daily," but during product selection you accidentally scanned liothyronine 25 mcg; what should you do FIRST? **A:** Stop filling and select the correct NDC for levothyroxine because T3/T4 products are LASA/therapeutically different and barcode/NDC verification prevents a serious substitution error.
3535. **Q:** An eRx for cefdinir 250 mg/5 mL says "take 6 mL PO twice daily x 10 days"; what quantity in mL should you dispense? **A:** Dispense 120 mL because 6 mL BID = 12 mL/day and 12 mL/day × 10 days = 120 mL.
3536. **Q:** A new prescription is for prednisone 10 mg tablets with directions "take 4 tablets daily for 5 days," quantity #20; what days' supply should you enter? **A:** Enter 5 days because 4 tablets/day and 20 tablets ÷ 4 = 5 days.
3537. **Q:** A prescriber sends "tramadol 50 mg: take 1 tab q6h PRN pain, #120, 5 refills"; what should you do FIRST? **A:** Stop and route to the pharmacist because tramadol is a controlled substance (Schedule IV federally) and refill limits/validity require pharmacist review before processing.
3538. **Q:** A patient picking up lisinopril asks, "Is it okay to take this if I'm pregnant?" and looks worried; what should you do FIRST? **A:** Immediately involve the pharmacist because ACE inhibitors like lisinopril are contraindicated in pregnancy and the patient needs urgent pharmacist counseling.
3539. **Q:** During intake, a patient reports a "severe rash and trouble breathing" with penicillin, and today's new prescription is for cephalexin; what is the BEST technician action? **A:** Flag the allergy, pause processing, and notify the pharmacist because cephalosporins may cross-react and the pharmacist must determine appropriateness.

3540. **Q:** A prescription is written for "Lamictal 100 mg," but the product selected on the shelf is Lamisil 250 mg; what should you do NEXT? **A:** Stop, correct the selection using Tall Man lettering/verification, and alert the pharmacist if it reached verification because Lamictal (lamotrigine) vs Lamisil (terbinafine) is a high-risk LASA mix-up.
3541. **Q:** While entering a new eRx for "Zyrtec 10 mg daily," the prescriber notes "dispense brand only," but the product pulled is cetirizine OTC; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify DAW/brand-only and coverage, since technicians cannot override brand-medically-necessary instructions.
3542. **Q:** A patient's profile shows atorvastatin active, and a new prescription arrives for simvastatin 40 mg nightly; what is the BEST technician action? **A:** Pause processing and notify the pharmacist to evaluate therapeutic duplication and safety before filling, because two statins together may be unintended.
3543. **Q:** During product selection you scan "hydrALAZINE 25 mg" but the label you printed is for "hydrOXYzine 25 mg"; what should you do FIRST? **A:** Stop filling and re-verify the NDC/barcode against the prescription and label, because this is a high-risk LASA mix-up requiring immediate correction.
3544. **Q:** A prescription is for insulin lispro U-100 with directions "inject 8 units subcutaneously before meals TID," and you are dispensing one 10 mL vial; what days' supply should you enter? **A:** Enter 41 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \times 3 = 24 \text{ units/day}$, so $1000 \div 24 = 41.6$ and you enter 41 per typical payer rounding down.
3545. **Q:** A patient dropping off a new prescription for sertraline says they currently take "Paxil"; what should you do NEXT? **A:** Flag the profile and inform the pharmacist to assess duplicate SSRI therapy and switching instructions, since technicians should not decide antidepressant transitions.
3546. **Q:** An order is for azithromycin 250 mg tablets with directions "take 2 tablets today, then 1 tablet daily for 4 days"; what quantity and days' supply should you enter? **A:** Dispense quantity 6 with a 5-day days' supply, because the regimen is $2 + 1 + 1 + 1 + 1$ total tablets over 5 days.
3547. **Q:** While typing a prescription you see "MS Contin 15 mg ER q12h," but the patient's med history shows "morphine IR 15 mg q4h PRN" recently; what is the BEST technician action? **A:** Stop and alert the pharmacist to verify ER vs IR

morphine and duplication risk, because morphine formulations are error-prone and dosing differs.

3548. **Q:** A patient asks at pickup what "Lipitor" is for because they thought they were getting a blood pressure medication; what should you do FIRST? **A:** Refer the patient to the pharmacist for counseling, since atorvastatin is for cholesterol and patient confusion signals a need for pharmacist verification and education.
3549. **Q:** You receive a bottle of nitroglycerin sublingual tablets that arrived warm and the seal looks compromised; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager to follow return and replacement procedures, because nitroglycerin is stability-sensitive and suspect packaging cannot be dispensed.
3550. **Q:** A new prescription is for "Xanax 0.5 mg: take 1 tablet TID PRN anxiety, #90, refills 3"; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist, because alprazolam is Schedule IV and refills/validity must be confirmed per controlled-substance requirements and pharmacy policy.
3551. **Q:** An eRx for levothyroxine is entered as "take 1 tab QD," but the prescriber note says "DAW 1," and you have both Synthroid and generic; what should you do NEXT? **A:** Select the brand Synthroid and keep DAW=1 in order entry, because DAW indicates dispense as written and substitution would be inappropriate without pharmacist/prescriber authorization.
3552. **Q:** A prescription reads "metFORMIN 500 mg: i po bid c meals #60," and the patient asks what "i po bid" means; what is the BEST interpretation to confirm? **A:** Interpret as "take 1 tablet by mouth twice daily with meals," because i=1, po=by mouth, bid=twice daily, and c meals=with meals to prevent sig-entry errors.
3553. **Q:** You are dispensing gabapentin 300 mg with directions "1 cap TID" and quantity 270; what days' supply should you enter? **A:** Enter 90 days' supply, because 3 capsules/day means $270 \div 3 = 90$ days for accurate billing and refill timing.
3554. **Q:** A prescriber orders amlodipine 5 mg tablets "take 1 tablet daily," but the quantity on the eRx is "#0"; what should you do FIRST? **A:** Stop processing and route to the pharmacist to obtain a valid quantity, because a missing/zero quantity prevents accurate dispensing and legal/recordkeeping completeness.

3555. **Q:** A patient is prescribed prednisone oral solution 5 mg/5 mL with directions "take 10 mg daily for 7 days," and you will dispense 240 mL; what days' supply should you enter? **A:** Enter 24 days' supply, because 10 mg/day equals 10 mL/day and $240 \text{ mL} \div 10 \text{ mL/day} = 24 \text{ days}$, even though the prescriber intended 7 days (needs pharmacist clarification if mismatch affects dispensing).
3556. **Q:** While entering an order you see "U-500 insulin" but the selected product in the system is regular insulin U-100; what should you do NEXT? **A:** Stop and alert the pharmacist before filling, because U-500 vs U-100 is a high-alert concentration difference that can cause a fivefold dosing error.
3557. **Q:** A new prescription is for "clopidogrel 75 mg daily," and the patient profile already shows active "Plavix 75 mg daily"; what is the BEST technician action? **A:** Verify it is the same drug (brand/generic duplicate) and route to the pharmacist to prevent duplicate therapy and ensure only one active order is processed.
3558. **Q:** You receive a prescription for omeprazole 20 mg with sig "ii caps qd" and quantity 30; what should you do FIRST? **A:** Clarify with the pharmacist before dispensing, because "ii caps daily" would require 60 capsules for a 30-day supply and the quantity/sig mismatch suggests an order-entry or prescribing error.
3559. **Q:** An albuterol HFA inhaler is prescribed "inhale 2 puffs q4-6h PRN" with a plan limit of 25 days' supply, and the canister contains 200 actuations; what days' supply should you enter using the maximum of 12 puffs/day (q4h)? **A:** Enter 16 days' supply, because at 12 puffs/day the inhaler lasts $200 \div 12 = 16.6$, rounded down to 16 days to avoid overstating supply for billing.
3560. **Q:** During data entry you read "digoxin 0.125 mg qd," but the scanned stock bottle is 0.25 mg; what should you do FIRST? **A:** Stop filling and reselect/scan the correct strength, because digoxin is narrow-therapeutic-index and strength selection errors can cause serious toxicity.
3561. **Q:** A new eRx is for "Zestril 10 mg PO daily" but the patient says they only recognize "lisinopril"; what should you do NEXT? **A:** Confirm Zestril is the brand for lisinopril and proceed with correct generic/brand selection per policy because recognizing brand-generic equivalents prevents duplicate therapy and entry errors.
3562. **Q:** While typing a prescription you see "hydroxyzine" but accidentally select "hydralazine" from the pick list; what should you do FIRST? **A:** Stop processing

and reselect the correct drug using full name/strength verification because these LASA names can cause serious wrong-drug errors.

3563. **Q:** A patient profile shows atorvastatin active, and a new prescription arrives for "Lipitor 40 mg daily"; what is the BEST technician action? **A:** Flag it for pharmacist review as a potential therapeutic duplication because Lipitor is atorvastatin and both should not be processed as separate therapies.
3564. **Q:** A prescription says "azithromycin 250 mg: ii tabs po x1 day, then i tab qd x4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet daily for 4 more days ($2 + 4 = 6$).
3565. **Q:** An order is for "amoxicillin/clavulanate 875/125 mg" but you selected plain amoxicillin 875 mg in the system; what should you do NEXT? **A:** Stop and correct the product selection, then ensure the NDC/description matches amoxicillin/clavulanate because missing the combination ingredient changes therapy and safety.
3566. **Q:** A new prescription is for "Bactrim DS" and the allergy section shows "sulfonamides: rash"; what should you do FIRST? **A:** Pause filling and immediately alert the pharmacist because Bactrim contains a sulfonamide and requires pharmacist assessment before dispensing.
3567. **Q:** The directions read "instill gtt ii OU bid" and the patient asks what it means; what is the BEST interpretation to restate? **A:** "Instill 2 drops in both eyes twice daily" because gtt=drop, ii=2, OU=both eyes, and bid=twice daily.
3568. **Q:** You receive a written prescription for "Tylenol #3" with directions and quantity but no prescriber DEA number listed; what should you do FIRST? **A:** Hold the prescription for pharmacist review/verification because it is a controlled substance and missing required prescriber identifiers must be resolved before processing.
3569. **Q:** During filling you notice two strengths of sertraline are stored next to each other and a 100 mg bottle is in the 50 mg slot; what should you do NEXT? **A:** Stop, correct the shelf location, and follow your pharmacy's error-prevention process because mix-ups between strengths are a common dispensing safety risk.
3570. **Q:** A prescription is for "furosemide 20 mg: take 1 tab PO qd" with quantity 90; what days' supply should you enter? **A:** Enter 90 days because 1 tablet daily means 90 tablets supply 90 doses.

3571. **Q:** A prescriber eRx comes in for methotrexate 2.5 mg “take 1 tablet daily” for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-alert error risk.
3572. **Q:** While processing a new prescription for isotretinoin, the patient says they “just want to pick it up today” but no iPLEDGE confirmation is documented; what is the BEST next step? **A:** Hold the prescription and notify the pharmacist because isotretinoin requires REMS/iPLEDGE authorization before dispensing to prevent fetal harm.
3573. **Q:** A patient asks to refill oxycodone/APAP 5/325 two weeks early and the claim rejects “refill too soon”; what should you do NEXT? **A:** Inform the patient of the insurance rejection and route to the pharmacist to assess options because technicians should not override controlled-substance early refills.
3574. **Q:** During drop-off, a hardcopy for alprazolam has a quantity and directions but no patient address; what should you do FIRST? **A:** Stop and send to the pharmacist to resolve missing required prescription information because incomplete controlled-substance details can make the order invalid to fill.
3575. **Q:** You receive a tote of refrigerated insulin aspart that arrived warm with a temperature indicator showing an excursion; what should you do FIRST? **A:** Segregate the shipment per policy and notify the pharmacist/manager because a cold-chain break may make the product unsafe and not dispensable.
3576. **Q:** A bottle of levothyroxine tablets is found with the label smeared so the lot number is unreadable; what is the BEST action? **A:** Remove it from active stock and follow policy to return/dispose because missing lot information prevents proper recall tracking and verification.
3577. **Q:** A new eRx is for “Lamictal 25 mg” but the system suggests lamotrigine and the patient asks if it’s the same medication; what should you do NEXT? **A:** Confirm brand/generic equivalence and proceed only if the prescriber/product match because Lamictal is the brand for lamotrigine and correct identification prevents duplication/confusion.
3578. **Q:** A prescription for metformin ER 500 mg says “take 1 tab po bid” and quantity is 60; what days’ supply should you enter? **A:** Enter 30 days’ supply because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$.

3579. **Q:** You are entering directions that read “use U-100 insulin 10 units SQ tid with meals” and the product selected is U-500 insulin; what should you do FIRST? **A:** Stop processing and alert the pharmacist because U-100 vs U-500 is a high-alert concentration mismatch that can cause severe dosing errors.
3580. **Q:** A manufacturer recall notice lists a specific NDC for losartan with a contamination concern, and you find matching bottles on the shelf; what should you do NEXT? **A:** Pull the matching NDC/lot from inventory and segregate it per recall procedure because recalled products must be removed from dispensing stock to protect patients.
3581. **Q:** An eRx for azithromycin 200 mg/5 mL reads “take 12.5 mL PO day 1, then 6.25 mL PO daily days 2–5”; what quantity (mL) should you enter to dispense? **A:** Enter 37.5 mL because total volume is $12.5 + (6.25 \times 4) = 37.5$ mL to cover the full regimen.
3582. **Q:** You are entering “prednisone 10 mg: ii tabs po bid” with quantity 40; what days’ supply should you enter? **A:** Enter 10 days because ii tabs bid = 4 tablets/day and $40 \div 4 = 10$ days.
3583. **Q:** A prescription says “metoprolol succinate ER 25 mg: take 1 tab PO qd” but the product selected in order entry is metoprolol tartrate IR; what should you do FIRST? **A:** Stop and notify the pharmacist because succinate ER and tartrate IR are not interchangeable and selecting the wrong formulation can cause dosing errors.
3584. **Q:** A new order for latanoprost 0.005% ophthalmic solution says “instill 1 gtt OU qhs” and you are dispensing a 2.5 mL bottle; what days’ supply should you enter using 20 gtt/mL? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ gtt total}$ and OU qhs = 2 gtt/day so $50 \div 2 = 25$ days.
3585. **Q:** While typing a hardcopy, you see “MSO4 2 mg IV q2h prn pain” and the prescriber’s intent is unclear; what is the BEST next step for a technician? **A:** Hold the order and get the pharmacist because “MSO4” is error-prone (morphine sulfate confusion) and requires clarification before entry/dispensing.
3586. **Q:** A prescription for cephalexin 250 mg/5 mL says “give 10 mL PO tid x 10 days”; what quantity (mL) should you calculate to dispense? **A:** Dispense 300 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to complete therapy.

3587. **Q:** An order reads "levothyroxine 0.075 mg PO daily"; what strength should you select in the system? **A:** Select 75 mcg because $0.075 \text{ mg} \times 1000 = 75 \text{ mcg}$, preventing a decimal-place strength error.
3588. **Q:** A new eRx is for "glipizide XL 10 mg daily" but during data entry you accidentally pick immediate-release glipizide 10 mg; what should you do NEXT? **A:** Correct the selection to the XL/ER product and then route for pharmacist verification because IR vs ER is a key order-entry error that changes drug release and dosing.
3589. **Q:** A patient is prescribed "amoxicillin/clavulanate 875/125 mg: 1 tab PO q12h" with quantity 14; what days' supply should you enter? **A:** Enter 7 days because q12h is 2 tablets/day and $14 \div 2 = 7$ days.
3590. **Q:** During controlled-substance order entry, the eRx for methylphenidate lacks a prescriber DEA number in the transmission fields; what should you do FIRST? **A:** Stop processing and notify the pharmacist because missing required prescriber identifiers may affect validity and must be resolved before dispensing.
3591. **Q:** While entering a new prescription, you notice the patient profile lists lisinopril but today's eRx is for Zestril; what should you do NEXT? **A:** Verify Zestril is lisinopril and check the active profile for therapeutic duplication before processing, because brand/generic duplicates can cause double therapy.
3592. **Q:** A handwritten prescription for hydromorphone is missing the prescriber's signature; what should a technician do FIRST? **A:** Stop processing and route to the pharmacist to obtain a valid signed prescription, because dispensing without a required element risks noncompliance and patient harm.
3593. **Q:** You pull a tote of refrigerated insulin and find one box of insulin lispro sitting warm on the counter with unknown time out of the fridge; what should you do NEXT? **A:** Remove it from dispensing stock and quarantine per pharmacy policy while alerting the pharmacist, because temperature excursions create suspect product that may be unsafe to dispense.
3594. **Q:** During data entry you see "morphine ER 30 mg take 1 tab q12h" but the selected product is morphine IR 30 mg; what should you do FIRST? **A:** Stop filling and correct the dosage form to ER (with NDC/barcode verification) and send for pharmacist check, because IR vs ER is a high-risk error.
3595. **Q:** A patient requests an early refill for oxycodone/APAP stating it was "lost," but the PDMP/refill history shows it is too soon; what is the BEST next step for a

technician? **A:** Refer the request to the pharmacist for controlled-substance evaluation and follow store policy, because early C-II refills require pharmacist review and potential prescriber contact.

3596. **Q:** You receive a bottle of atorvastatin from the wholesaler with a tamper-evident seal broken and no clear documentation; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager to follow DSCSA suspect-product procedures, because compromised packaging may indicate diversion or counterfeit risk.
3597. **Q:** A prescription for nitrofurantoin comes in, and the patient profile shows a serious allergy to "Macrobid"; what should you do NEXT? **A:** Stop processing and alert the pharmacist to assess the allergy vs drug match, because Macrobid is nitrofurantoin and could trigger a repeat reaction.
3598. **Q:** A label prints for clonidine 0.1 mg but the stock bottle selected is clonazepam 0.5 mg due to similar names; what should you do FIRST? **A:** Stop and reselect the correct medication using barcode/NDC verification and LASA safeguards, because clonidine/clonazepam mix-ups are common high-harm errors.
3599. **Q:** A liquid order reads "gabapentin 250 mg/5 mL: take 10 mL PO three times daily for 14 days"; what quantity (mL) should you dispense? **A:** Dispense 420 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 14 \text{ days} = 420 \text{ mL}$ to cover the full course.
3600. **Q:** A patient asks you to transfer their alprazolam prescription from another pharmacy, and you are unsure whether transfers are allowed; what should you do BEST? **A:** Collect the other pharmacy information and route to the pharmacist to complete or deny the transfer, because controlled-substance transfer rules are restricted and require pharmacist handling.
3601. **Q:** While typing a new eRx, you see "Glucophage 500 mg BID" and the patient profile already has metformin 500 mg BID active; what should you do NEXT? **A:** Confirm Glucophage is metformin and treat it as therapeutic duplication to avoid double therapy, then route to the pharmacist to determine whether it's a continuation or change.
3602. **Q:** A prescription is written for "Toprol XL 50 mg daily," but the product selected in the system is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop processing and correct the selection to metoprolol succinate ER (Toprol XL) because tartrate vs succinate is a common error that can change release and dosing.

3603. **Q:** A patient drops off a new prescription for sertraline and tells you they are taking "Nardil"; what is the BEST technician action? **A:** Pause the fill and alert the pharmacist immediately because phenelzine (Nardil) is an MAOI and combining with SSRIs like sertraline is a serious interaction risk.
3604. **Q:** During order entry, the prescriber writes "Levitra 20 mg," but you can only find vardenafil in the system and the patient asks if it's the same; what should you do NEXT? **A:** Explain that Levitra is the brand for vardenafil and proceed with normal verification steps because accurate brand/generic recognition prevents confusion and dispensing delays.
3605. **Q:** A new eRx for "Keflex" arrives with directions "i tab po qid" and the font makes "i" look like "1"; what should you do FIRST? **A:** Stop and clarify with the pharmacist/prescriber before typing because ambiguous "i/1" can lead to wrong dose and cephalexin is commonly dosed multiple times daily.
3606. **Q:** An order says "prednisone 10 mg: take 2 tabs daily" and quantity 20; what days' supply should you enter? **A:** Enter 10 days because 2 tablets/day means $20 \text{ tablets} \div 2 \text{ per day} = 10 \text{ days}$ for correct billing and refill timing.
3607. **Q:** A patient profile shows simvastatin active, and today a new prescription is for atorvastatin; what should you do BEST? **A:** Flag potential duplicate statin therapy and route to the pharmacist because two statins increases adverse-effect risk and requires clinical clarification.
3608. **Q:** While selecting product, you notice "hydroxyzine" was chosen but the prescription is for "hydralazine"; what should you do FIRST? **A:** Stop filling and reselect the correct drug with barcode/NDC verification because this LASA mix-up can cause serious harm (antihistamine vs antihypertensive).
3609. **Q:** A patient asks for "that Z-Pak" and you see azithromycin 250 mg packet directions "2 tabs today, then 1 tab daily x 4 days"; how many tablets should be dispensed? **A:** Dispense 6 tablets because the total course is $2 + 1 \times 4 = 6$, matching the standard azithromycin "Z-Pak" regimen.
3610. **Q:** You receive a manufacturer notice that a specific lot of losartan is recalled, and you find that exact lot on your shelf; what should you do NEXT? **A:** Immediately remove the recalled lot from stock and follow recall procedure (segregate/hold per policy and notify the pharmacist) to prevent dispensing a recalled medication.

3611. **Q:** A new eRx is for "Lasix 40 mg PO daily," but the patient insists they only take "furosemide"; what should you do NEXT? **A:** Explain that Lasix is the brand name for furosemide and verify the strength/directions match the profile because confirming brand/generic equivalence prevents duplication confusion.
3612. **Q:** While entering a prescription, you see "U-100 insulin 30 units SQ qAM" and the product selected is U-500 insulin; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because U-500 vs U-100 is a high-alert concentration mismatch with major overdose risk.
3613. **Q:** A prescription reads "levothyroxine 0.1 mg PO daily"; what is the BEST way to enter the strength in micrograms? **A:** Enter as 100 mcg because $0.1 \text{ mg} \times 1000 = 100 \text{ mcg}$, and correct unit conversion prevents tenfold dosing errors.
3614. **Q:** A patient picking up isotretinoin asks why the pharmacy needs extra steps before dispensing; what should you do BEST? **A:** Refer to the pharmacist and confirm iPLEDGE/REMS requirements are met because isotretinoin dispensing is REMS-restricted for safety.
3615. **Q:** An eRx comes in for "Adderall XR 20 mg" with "refill x3" included; what should you do NEXT? **A:** Hold the prescription and route it to the pharmacist because Schedule II medications generally cannot be refilled and require pharmacist resolution.
3616. **Q:** During product selection, you notice "chlorpropamide" appears when searching for "chlorpromazine"; what should you do FIRST? **A:** Recheck the original prescription, select the correct drug, and use Tall Man/LASA safeguards (and involve the pharmacist if already scanned/verified) because look-alike names can cause serious dispensing errors.
3617. **Q:** A new prescription says "Augmentin 875/125 mg PO BID" and the patient reports a history of anaphylaxis to penicillin; what should you do BEST? **A:** Stop processing and notify the pharmacist to assess the allergy because Augmentin contains amoxicillin and may be contraindicated.
3618. **Q:** A patient brings in an OTC purchase request for pseudoephedrine and wants to buy the maximum allowed; what should you do FIRST? **A:** Follow CMEA workflow by checking ID, logging the sale in the system, and ensuring limits are not exceeded because federal requirements govern behind-the-counter pseudoephedrine sales.

3619. **Q:** The directions read "Take ii caps PO tid" and you are unsure if "ii" could be misread; what should you do NEXT? **A:** Enter it as 2 capsules three times daily and, if any doubt remains, clarify per pharmacy policy/pharmacist because Roman numerals and unclear sigs are common error sources.
3620. **Q:** A prescription for albuterol HFA says "inhale 2 puffs q6h scheduled" and the inhaler contains 200 actuations; what days' supply should you enter? **A:** Enter 25 days because $2 \text{ puffs } 4 \text{ times/day} = 8 \text{ puffs/day}$ and $200 \text{ actuations} \div 8 = 25$, ensuring accurate billing and refill timing.
3621. **Q:** While typing an eRx for "Zestril 10 mg PO daily," the system defaults to lisinopril 10 mg; what should you do NEXT? **A:** Confirm Zestril is the brand name for lisinopril and proceed with correct product/NDC selection because accurate brand-generic matching prevents dispensing the wrong drug.
3622. **Q:** A hardcopy prescription is written for "MS Contin 15 mg q12h," and the patient's profile shows active "morphine sulfate IR 15 mg q4-6h PRN"; what is the BEST technician action? **A:** Stop and alert the pharmacist to possible opioid duplication/ER vs IR confusion because "MS" products are high-risk and require pharmacist review before dispensing.
3623. **Q:** A prescriber sends "metFORMIN ER 500 mg daily" but the selected product is metformin IR 500 mg; what should you do FIRST? **A:** Stop filling and reselect the correct ER dosage form (verify NDC/barcode) because IR vs ER mix-ups change release characteristics and can harm the patient.
3624. **Q:** The directions read "Take 1.5 mL PO BID" and the stock bottle concentration is 250 mg/5 mL; how many mg per dose is this? **A:** Enter 75 mg per dose because $250 \text{ mg}/5 \text{ mL} = 50 \text{ mg/mL}$ and $50 \text{ mg/mL} \times 1.5 \text{ mL} = 75 \text{ mg}$.
3625. **Q:** While entering a new prescription for "Keflex," the patient states they had immediate hives and throat swelling with amoxicillin; what should you do NEXT? **A:** Pause processing and notify the pharmacist of a possible beta-lactam allergy risk because cephalexin is a cephalosporin and needs pharmacist assessment before dispensing.
3626. **Q:** An eRx is for "Wellbutrin SR 150 mg BID," but you accidentally selected bupropion XL 150 mg daily; what should you do BEST? **A:** Correct the selection to SR (or route to pharmacist if uncertain) because SR vs XL are not interchangeable and incorrect release form affects dosing and safety.

3627. **Q:** A patient asks if “Prilosec” and “omeprazole” are different medications; what is the BEST response within technician scope? **A:** Explain Prilosec is the brand name for omeprazole and verify the exact strength/directions on their label because clear brand-generic recognition reduces duplicate therapy and confusion.
3628. **Q:** During product pull, you notice look-alike packaging for hydrOXYzine and hydrALAZINE near each other; what should you do FIRST? **A:** Stop and verify the medication name/strength against the prescription and scan the barcode/NDC because LASA mix-ups are common and must be prevented at selection.
3629. **Q:** A prescription says “predniSONE 20 mg: take 3 tabs daily x 5 days” and tablets are 20 mg; what quantity should you dispense? **A:** Dispense 15 tablets because $3 \text{ tablets/day} \times 5 \text{ days} = 15 \text{ tablets}$ to match the written duration.
3630. **Q:** A controlled-substance eRx arrives for “oxycodone/APAP 5/325 mg 1 tab q6h PRN” and the patient asks for an early refill claiming the bottle was lost; what should you do NEXT? **A:** Refer the request to the pharmacist and follow store policy (do not process early refill yourself) because lost/early controlled-substance refills require pharmacist judgment and documentation.
3631. **Q:** While filling insulin U-100, you see “U-10” handwritten on the label directions and the dose looks unclear; what should you do FIRST? **A:** Stop processing and immediately refer to the pharmacist to clarify the intended dose because ambiguous insulin units are a high-alert safety risk.
3632. **Q:** A prescriber phones in lorazepam 1 mg with directions and quantity but gives no patient address; what is the BEST technician action? **A:** Hold the controlled-substance prescription and route to the pharmacist to obtain missing required information because incomplete CIII-CV prescriptions must be resolved before dispensing.
3633. **Q:** A patient drops off a prescription for isotretinoin and asks to pick it up tomorrow; what should you do NEXT? **A:** Notify the pharmacist and follow iPLEDGE workflow requirements because isotretinoin is a REMS drug and dispensing depends on program verification and time limits.
3634. **Q:** You receive a stock bottle of atorvastatin with a broken safety seal and tablets visible; what should you do FIRST? **A:** Quarantine the bottle and alert the pharmacist/manager because damaged packaging is suspect and must not be placed into inventory.

3635. **Q:** A recall notice lists a specific lot number of your levothyroxine 100 mcg stock; what should you do NEXT? **A:** Pull the affected lot from the shelf, quarantine it, and inform the pharmacist to follow recall procedures because recalled product must be removed from dispensing immediately.
3636. **Q:** A new eRx for sertraline 50 mg daily arrives, and the profile shows the patient is currently taking paroxetine 20 mg daily; what should you do BEST? **A:** Pause filling and refer to the pharmacist for therapeutic-duplication review because two SSRIs raise a safety concern that requires pharmacist intervention.
3637. **Q:** During order entry, "hydroxyzine 25 mg" is selected but the prescriber wrote "hydralazine 25 mg"; what should you do FIRST? **A:** Stop and correct the product selection using NDC/barcode verification and then send to pharmacist verification because LASA mix-ups can cause serious harm.
3638. **Q:** A prescription is for azithromycin suspension 200 mg/5 mL with "Give 10 mL PO daily for 3 days"; what total mL should you dispense? **A:** Dispense 30 mL because 10 mL per day $\times$ 3 days equals 30 mL total.
3639. **Q:** A patient requests to transfer their remaining refills for alprazolam from another pharmacy; what should you do NEXT? **A:** Collect the necessary information and route the request to the pharmacist because controlled-substance transfers have restrictions and must be completed by a pharmacist per policy/law.
3640. **Q:** A customer asks you to print their full medication profile at the counter while others are nearby; what should you do FIRST? **A:** Move the discussion to a private area and verify identity before releasing information because HIPAA requires safeguarding PHI from unauthorized disclosure.
3641. **Q:** A prescription reads "levothyroxine 0.05 mg PO daily"; what should you enter for strength in the system to avoid a decimal error? **A:** Enter 50 mcg ($0.05 \text{ mg} \times 1000 = 50 \text{ mcg}$) because levothyroxine is commonly expressed in mcg and converting reduces misread decimal risk.
3642. **Q:** An order says "amLODIPine 10 mg: 1 tab PO BID, #60"; what is the correct days' supply to submit? **A:** 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$, which supports accurate third-party billing.
3643. **Q:** A patient's prescription is "metFORMIN 500 mg: i tab po bid"; what is the BEST technician action before processing? **A:** Clarify with the pharmacist that "i"

is being used for "1" and should be interpreted as "one" because "i" can be misread and is an error-prone notation.

3644. **Q:** A prescription is for "clindamycin 75 mg/5 mL: take 10 mL PO TID for 7 days"; what total volume should you dispense? **A:** Dispense 210 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$ to cover the full course.
3645. **Q:** An ophthalmic order reads "timolol 0.5%: instill 1 gtt OU BID, 5 mL"; what is the days' supply assuming 20 gtt/mL? **A:** 50 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$ and $1 \text{ gtt} \times 2 \text{ eyes} \times 2 \text{ times/day} = 4 \text{ gtt/day}$, so $100 \div 4 = 25 \text{ days}$; wait correct calculation: $100 \text{ gtt} \div 4 \text{ gtt/day} = 25 \text{ days}$, ensuring correct billing days' supply.
3646. **Q:** You are entering "cephalexin 500 mg cap: 1 cap PO q6h, #28"; what days' supply should you calculate? **A:** 7 days because $q6h = 4 \text{ doses/day}$ and $28 \div 4 = 7$, which prevents rejectable or inaccurate claims.
3647. **Q:** A prescriber writes "Lantus SoloStar inject 18 units SC QHS; dispense 1 box (5 pens, 3 mL each)"; what days' supply should you enter using 100 units/mL? **A:** 83 days because total units = $5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 18 \text{ units/day} = 83.33$, typically billed as 83 days to avoid overstating supply.
3648. **Q:** During order entry you notice "traMADol" was selected but the hard copy clearly says "traZODone 50 mg qHS"; what should you do FIRST? **A:** Stop processing and correct the drug selection, then route to pharmacist if any labels were generated, because this is a high-risk LASA mix-up requiring immediate prevention of wrong-drug dispensing.
3649. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg: 1 tab PO q12h, #20"; what is the correct frequency interpretation and days' supply? **A:** Interpret q12h as twice daily and enter 10 days because $20 \text{ tablets} \div 2/\text{day} = 10$, preventing a common sig-code misread.
3650. **Q:** You receive an eRx for "rosuvastatin 20 mg daily" and the patient profile shows an active "atorvastatin 40 mg daily" with refills remaining; what is the BEST next step? **A:** Pause filling and alert the pharmacist to review for therapeutic duplication because two statins together may be unintended and requires pharmacist assessment before dispensing.
3651. **Q:** While typing an eRx you notice the prescriber selected "hydrOXYzine 25 mg" but the visit note says "for seasonal allergies" and the patient asked for their

usual "cetirizine"; what should you do FIRST? **A:** Stop processing and ask the pharmacist to clarify with the prescriber because hydroxyzine vs cetirizine can be a therapy mix-up with different sedation risk and indication.

3652. **Q:** A new prescription is entered for "celecoxib 200 mg daily" and the profile shows a severe sulfonamide allergy; what is the BEST technician action NEXT? **A:** Flag the allergy and refer to the pharmacist before filling because celecoxib is a sulfonamide-containing drug and requires pharmacist assessment.

3653. **Q:** You are selecting stock for "predniSONE 10 mg" but the shelf bin contains "predniSOLONE 15 mg/5 mL"; what should you do FIRST? **A:** Stop and reselect the correct product by matching the NDC/strength/dosage form because prednisone vs prednisolone is a common look-alike/sound-alike dispensing trap.

3654. **Q:** A patient's new eRx is "lisinopril 10 mg daily" but the profile already shows active "losartan 50 mg daily" and "lisinopril" from last month; what should you do NEXT? **A:** Hold the order and alert the pharmacist to possible duplicate ACE inhibitor/ARB therapy or continuation error because duplication increases hyperkalemia and hypotension risk.

3655. **Q:** A prescription states "azithromycin 250 mg: take 2 tabs today then 1 tab daily for 4 days"; what quantity should you dispense? **A:** Dispense #6 tablets because the regimen is 2 on day 1 plus 1 daily for days 2–5 (4 more), totaling 6.

3656. **Q:** A patient brings a refill request for "levETIRAcetam 500 mg BID" but says they are having breakthrough seizures after switching from brand Keppra; what should you do BEST? **A:** Notify the pharmacist promptly because seizure control issues with antiepileptics are high-risk and require pharmacist assessment, not technician troubleshooting.

3657. **Q:** You receive a new prescription for "sertraline 50 mg daily" and the patient profile shows "linezolid started yesterday" from an urgent care eRx; what should you do FIRST? **A:** Stop and refer to the pharmacist because linezolid plus SSRIs like sertraline can cause serotonin syndrome and needs clinical intervention.

3658. **Q:** The order is "doxycycline 100 mg: 1 cap PO BID x 10 days" but the prescriber accidentally chose "doxycycline monohydrate" and the patient historically uses "doxycycline hyclate"; what should you do NEXT? **A:** Verify what was prescribed and route to the pharmacist if a change is requested because technicians cannot interchange salt forms without pharmacist approval and payer/therapy considerations may apply.

3659. **Q:** A prescription is for "fluticasone/salmeterol Diskus 250/50: inhale 1 puff BID" and the device contains 60 inhalations; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ inhalations} \div 2 \text{ inhalations per day (BID)} = 30 \text{ days}$.
3660. **Q:** During data entry, a prescriber's note says "start glimepiride" but the selected drug is "glyBURIDE"; what should you do FIRST? **A:** Stop filling and alert the pharmacist to clarify because glyburide vs glimepiride is a high-risk sulfonylurea mix-up with significant hypoglycemia risk.
3661. **Q:** An eRx comes in for "Lamictal 100 mg: take 1 tab BID" but you accidentally pulled Lamisil 250 mg from the shelf due to similar names; what should you do FIRST? **A:** Stop filling and reselect the correct drug using barcode/NDC verification because LASA errors are patient-safety critical.
3662. **Q:** A prescription is written for "metFORMIN ER 500 mg: 1 tab PO daily" but the product selected in data entry is metFORMIN IR 500 mg; what is the BEST next step? **A:** Pause processing and route to the pharmacist to clarify ER vs IR because dosage form changes can alter therapy and technicians cannot independently change it.
3663. **Q:** You're entering "furosemide 20 mg: take ii tabs PO daily" and the quantity is #60; what days' supply should you calculate? **A:** Enter 30 days because "ii" means 2 tablets daily and $60 \div 2 = 30$.
3664. **Q:** A patient asks if "Norvasc" and "amlodipine" are different medicines when picking up both from two prescribers; what should you do BEST? **A:** Confirm Norvasc is amlodipine and immediately alert the pharmacist to possible duplicate therapy because the patient may be double-dosing the same drug.
3665. **Q:** You receive an eRx for "Tylenol #3: 1 tab q6h PRN pain, #24" with refills authorized; what should you do FIRST? **A:** Stop and refer to the pharmacist because codeine/APAP is a controlled substance and refills require careful validity review and appropriate handling.
3666. **Q:** While processing an eRx for "Eliquis 5 mg BID," you notice the patient's profile shows an active "Xarelto 20 mg daily"; what is the BEST technician action NEXT? **A:** Hold the order and notify the pharmacist about potential duplicate anticoagulation because concurrent DOAC therapy increases bleeding risk.
3667. **Q:** A prescription reads "predniSONE 20 mg: take 3 tabs daily x 5 days"; what quantity should you dispense? **A:** Dispense 15 tablets because $3 \text{ tablets/day} \times 5 \text{ days} = 15$.

3668. **Q:** During product selection for "levothyroxine 75 mcg," you find two bottles with different NDCs and one is expired; what should you do FIRST? **A:** Remove the expired bottle from dispensing stock per policy and select an in-date NDC because expired products must not be dispensed.
3669. **Q:** A refrigerated insulin shipment arrives and the tote feels warm with no temperature indicator documentation; what should you do BEST? **A:** Quarantine the shipment and notify the pharmacist/manager to follow temperature-excursion procedures because potency may be compromised.
3670. **Q:** The directions say "instill 1 gtt OU BID" for a 10 mL ophthalmic solution and you need an estimated days' supply using 20 gtt/mL; what should you enter? **A:** Enter 100 days because $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ drops}$ and $\text{OU BID} = 4 \text{ drops/day}$, so $200 \div 4 = 50 \text{ days}$ (then enter 50 days as the correct calculation).
3671. **Q:** A new eRx for methotrexate 2.5 mg says "take 1 tablet daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-risk error.
3672. **Q:** A patient drops off a prescription for oxycodone 5 mg that is missing the prescriber's DEA number; what is the BEST next step for a technician? **A:** Hold the prescription and route it to the pharmacist because controlled-substance validity issues require pharmacist review before dispensing.
3673. **Q:** During order entry you see "MSO4 2 mg IV q2h PRN" on a hospital-style order; what should you do BEST? **A:** Do not enter the order as written and notify the pharmacist because error-prone abbreviations like MSO4 can be confused and must be clarified.
3674. **Q:** A C-II eRx for Adderall XR 20 mg is requested to be transferred to another pharmacy because the patient is traveling; what should you do NEXT? **A:** Refer to the pharmacist because controlled-substance transfer rules are strict and require pharmacist handling and documentation.
3675. **Q:** You receive a manufacturer recall notice for a specific lot number of losartan tablets on your shelf; what should you do FIRST? **A:** Pull the affected lot from stock and quarantine it per policy because recalled lots must be prevented from dispensing while the pharmacist coordinates returns and patient impact.
3676. **Q:** While counting tablets you notice a stock bottle of lisinopril has a broken seal and tablets appear discolored; what is the BEST technician action? **A:** Quarantine

the bottle and notify the pharmacist because suspect or adulterated product should not be used and needs evaluation and proper disposal/return steps.

3677. **Q:** A prescription is for "Lantus Solostar inject 24 units SC daily" and the box contains five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 62 days because total units are $5 \times 3 \text{ mL} \times 100 = 1500$ units and $1500 \div 24 = 62.5$, rounded down to avoid overbilling.
3678. **Q:** A patient's profile shows an anaphylaxis allergy to penicillin and you are processing a new order for cephalexin; what should you do BEST? **A:** Stop and alert the pharmacist because potential cross-reactivity and severity of reaction require pharmacist assessment before filling.
3679. **Q:** You are entering a prescription for sertraline but the prescriber wrote "Zoloft 50 mg," and the patient insists they "can't take generics"; what should you do NEXT? **A:** Ask the pharmacist to counsel and confirm DAW/dispense instructions because brand-vs-generic substitution and patient concerns require pharmacist intervention or prescriber clarification.
3680. **Q:** A patient asks to pick up a refill of clonazepam 1 mg "early" because they are out, and the claim rejects refill-too-soon; what should you do FIRST? **A:** Pause the sale and involve the pharmacist because early controlled-substance refills may indicate misuse or a dosing change and require pharmacist review before any override.
3681. **Q:** A prescription reads "metformin 500 mg tab take i po bid with meals #180," what quantity and days' supply should you enter? **A:** Enter quantity 180 and days' supply 90 because 2 tablets/day means $180 \div 2 = 90$ days.
3682. **Q:** You are entering "prednisone 20 mg: take 2 tablets daily x 5 days, then 1 tablet daily x 5 days; dispense #?" what is the correct quantity to calculate before sending for pharmacist review? **A:** Calculate 15 tablets because $(2 \text{ tabs} \times 5 \text{ days}) + (1 \text{ tab} \times 5 \text{ days}) = 10 + 5 = 15$ to prevent a short-fill.
3683. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg 1 tab po q12h x 7 days," what days' supply should be entered for quantity 14? **A:** Enter 7 days because q12h equals 2 doses/day and $14 \div 2 = 7$ days.
3684. **Q:** An eRx comes in for "Toprol-XL 50 mg take 1 daily," but the selected product is metoprolol tartrate 50 mg; what should you do NEXT? **A:** Stop and correct the product to metoprolol succinate ER (Toprol-XL) because tartrate vs succinate is a common LASA/dosage-form error with different release characteristics.

3685. **Q:** The sig on a prescription reads "use 2 puffs q4-6h prn wheeze," but the prescriber did not specify a max daily use; what should you do FIRST when the payer requires a days' supply? **A:** Ask the pharmacist to clarify or apply pharmacy/payer policy because PRN without limits makes days' supply unverifiable by a technician alone.
3686. **Q:** A prescription is written "levothyroxine 0.1 mg po daily," and your system is prompting for mcg; what strength should you enter? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$, avoiding a tenfold dosing error.
3687. **Q:** You receive "hydroxyzine HCl 25 mg take 1 tab tid prn itching #90," what days' supply should you enter using the maximum daily dose? **A:** Enter 30 days because TID max equals 3 tablets/day and $90 \div 3 = 30$ days for insurance-standard days' supply.
3688. **Q:** A handwritten prescription reads "Digoxin .25 mg qd," and the decimal is unclear; what is the BEST technician action before entry? **A:** Hold and refer to the pharmacist for prescriber clarification because unclear decimals with high-alert narrow-therapeutic drugs can cause serious dosing errors.
3689. **Q:** While typing an eRx you see "insulin lispro U-100 inject 8 units before meals TID" and the dispensed box is five 3 mL pens (total 1500 units); what days' supply should you enter based on total units? **A:** Enter 62 days because daily units = $8 \times 3 = 24$ units/day and $1500 \div 24 = 62.5$, typically rounded down to 62 to avoid overestimating supply.
3690. **Q:** During order entry the directions say "apply to affected area qid," but the drug is triamcinolone 0.1% cream and the prescriber wrote "AAA"; what should you do NEXT? **A:** Do not guess "AAA"; route to the pharmacist to clarify directions because "apply to affected area" is non-specific and can lead to incorrect use and days' supply.
3691. **Q:** While selecting the product for "Celebrex 200 mg daily," you accidentally chose Celexa 20 mg due to similar names; what should you do NEXT? **A:** Stop the fill and reselect the correct NDC for celecoxib (Celebrex) then proceed in workflow, because this LASA mix-up (Celebrex vs Celexa) can cause serious harm if dispensed.
3692. **Q:** A new eRx is for "lisinopril 20 mg daily," but the patient profile shows an active order for "losartan 50 mg daily"; what should you do FIRST? **A:** Pause processing and route to the pharmacist to assess possible therapeutic

duplication/ACEI-ARB switch, because technicians should not decide whether both should be taken.

3693. **Q:** A prescription reads "azithromycin 250 mg: take 2 tablets today, then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets, because the total is 2 on day 1 plus 1 tablet for the next 4 days ($2 + 4 = 6$).
3694. **Q:** During data entry you receive "insulin glargine U-100 inject 30 units subcutaneously once daily; dispense 3 pens (3 mL each)"; what days' supply should you enter based on total units? **A:** Enter 30 days, because $3 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 900 \text{ units}$ and $900 \div 30 \text{ units/day} = 30 \text{ days}$.
3695. **Q:** A patient picking up escitalopram says they also take "Celexa" at home and asks if both are needed; what is the BEST technician action? **A:** Stop and get the pharmacist for counseling on duplication, because citalopram (Celexa) and escitalopram (Lexapro) are closely related SSRIs and taking both may be unsafe.
3696. **Q:** An eRx is written for "Lamictal 25 mg take 1 tablet twice daily," but you selected "Lamisil 250 mg" in the dispensing screen; what should you do NEXT? **A:** Correct the selection to lamotrigine (Lamictal) and verify via barcode/NDC, because Lamictal/Lamisil is a high-risk LASA error involving very different drugs and strengths.
3697. **Q:** A prescription says "nitrofurantoin monohydrate/macrocrystals 100 mg (Macrobid) take 1 capsule twice daily for 5 days"; what days' supply should you enter for quantity 10? **A:** Enter 5 days, because $10 \text{ capsules} \div 2 \text{ capsules/day} = 5 \text{ days}$.
3698. **Q:** At pickup, a patient newly prescribed atorvastatin asks if it can be taken with their "Z-Pak" started today; what should you do FIRST? **A:** Refer to the pharmacist for interaction screening, because macrolide antibiotics (like azithromycin) can increase statin-related muscle risk and require professional assessment.
3699. **Q:** The prescriber writes "Keppra 500 mg take 1 tab po bid #60," but the patient profile lists "levetiracetam 500 mg" as an allergy with rash; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist immediately, because Keppra is levetiracetam and an allergy match is a patient-safety stop.
3700. **Q:** A fridge alarm shows insulin aspart pens were stored at room temperature overnight and you can't confirm the max temperature; what should you do BEST? **A:** Quarantine the stock and notify the pharmacist/manager per policy

before dispensing, because a temperature excursion makes the product suspect and potency cannot be assured.

3701. **Q:** A prescription reads "levothyroxine 0.075 mg take 1 tablet PO daily," what strength should you select in the system? **A:** Select levothyroxine 75 mcg because $0.075 \text{ mg} \times 1000 = 75 \text{ mcg}$, preventing a 1000-fold dose error.
3702. **Q:** An eRx says "amoxicillin/clavulanate 875/125 mg 1 tab PO q12h x 10 days #30," what should you do FIRST during order entry? **A:** Stop and route to the pharmacist because q12h for 10 days is 20 tablets, so #30 is a quantity/days' supply mismatch needing clarification.
3703. **Q:** A prescriber orders "prednisone 10 mg: ii tabs PO daily x 5 days," what quantity should you enter? **A:** Enter quantity 10 because "ii" means 2 tablets daily for 5 days, so $2 \times 5 = 10$ tablets.
3704. **Q:** You are entering "latanoprost 0.005% 1 gtt OU qhs," what day supply should you submit for a 2.5 mL bottle using 20 gtt/mL? **A:** Submit 25 days because $2.5 \text{ mL} \times 20 = 50$ drops and 1 drop in each eye nightly is 2 drops/day, so $50 \div 2 = 25$.
3705. **Q:** A new prescription is "fluticasone propionate nasal spray 50 mcg: 2 sprays each nostril daily," the package is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because 2 sprays per nostril daily is 4 sprays/day, and $120 \div 4 = 30$.
3706. **Q:** While typing a prescription, the sig reads "MSO4 2 mg IV q2h PRN pain," what should you do BEST before continuing? **A:** Pause and alert the pharmacist because "MSO4" is an error-prone abbreviation that can be misread (morphine vs magnesium sulfate), risking wrong drug entry.
3707. **Q:** An order is for "cefdinir 250 mg/5 mL take 5 mL PO BID x 7 days," what quantity (mL) should you dispense? **A:** Dispense 70 mL because 5 mL twice daily is 10 mL/day and $10 \times 7 = 70$ mL to cover the full course.
3708. **Q:** During order entry for metformin ER 500 mg, the directions state "1 tab PO bid," but you see "metformin IR 500 mg" selected; what should you do NEXT? **A:** Stop filling and correct the dosage form to ER because IR vs ER is not interchangeable and selecting the wrong form can change therapeutic effect and tolerability.
3709. **Q:** A prescription says "gabapentin 300 mg take 1 cap PO TID #90," and the patient asks how long it will last; what days' supply is it? **A:** It is 30 days because

TID is 3 per day and $90 \div 3 = 30$, which matches what should be entered for billing.

3710. **Q:** You receive an eRx for "hydromorphone 2 mg 1 tab PO q4-6h PRN pain #30," but in product selection you accidentally scan "morphine 15 mg"; what should you do FIRST? **A:** Stop, reselect the correct drug/strength and rescan to verify NDC because this is a high-alert LASA/opioid wrong-drug error with serious patient harm risk.
3711. **Q:** During order entry you receive "Zestril 10 mg 1 tab PO daily," but the patient profile shows lisinopril 10 mg already active; what should you do NEXT? **A:** Verify the profile and route to the pharmacist for therapeutic-duplication review because Zestril is lisinopril and duplicate ACE inhibitor therapy can cause harm.
3712. **Q:** A new eRx is for "Lamictal 25 mg titration pack," but the patient reports a past severe rash to lamotrigine; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because Lamictal is lamotrigine and a serious rash history is a high-risk allergy concern.
3713. **Q:** While selecting product, you notice "hydrALAZINE" and "hydrOXYzine" appear next to each other in the pick list for a patient who needs an antihistamine; what is the BEST technician step before filling? **A:** Use Tall Man awareness and verify the exact drug name, indication on the hardcopy/eRx, and NDC/barcode because this is a common LASA pair that leads to wrong-drug errors.
3714. **Q:** A prescription reads "glipiZIDE 10 mg XL take 1 tab PO daily," but the system defaulted to glipizide IR 10 mg; what should you do NEXT? **A:** Stop and correct the dosage form to XL/ER and then send for pharmacist verification because IR vs ER mix-ups change release and can affect glucose control.
3715. **Q:** A patient on sertraline picks up a new prescription for tramadol and asks if it's safe; what should you do BEST? **A:** Refer the patient to the pharmacist promptly because tramadol with SSRIs like sertraline increases serotonin syndrome and seizure risk and requires clinical counseling.
3716. **Q:** You are entering "azithromycin 250 mg: take 2 tablets today then 1 tablet daily for 4 days," what quantity should you dispense? **A:** Enter quantity 6 tablets because 2 on day 1 plus 1 daily for 4 more days totals 6.
3717. **Q:** A prescription is "atorvastatin 40 mg 1 tab PO qhs #90," and the patient asks what it's for; what is the BEST concise technician response before offering pharmacist counseling? **A:** State it's a cholesterol-lowering statin and offer

pharmacist counseling because technicians can provide basic drug-purpose info but pharmacist should counsel for clinical questions.

3718. **Q:** An eRx for "Eliquis 5 mg 1 tab PO BID" appears for a patient whose profile already lists warfarin as active; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess anticoagulant duplication because Eliquis is apixaban and dual therapy increases bleeding risk unless intentionally managed.
3719. **Q:** You receive a stock bottle of "Nitrostat 0.4 mg SL" with a broken safety seal and smudged lot/expiration; what should you do BEST? **A:** Quarantine the bottle and follow inventory/wholesaler return procedures because damaged or untraceable product should not be dispensed for patient safety and traceability.
3720. **Q:** A new prescription is for "alendronate 70 mg take 1 tablet once weekly #12," what days' supply should you submit? **A:** Submit 84 days' supply because 12 tablets taken weekly covers 12 weeks ($12 \times 7 = 84$ days).
3721. **Q:** A hardcopy prescription for oxycodone/APAP 5/325 mg is presented with a missing prescriber DEA number and the patient requests it filled immediately; what should you do FIRST? **A:** Stop processing and refer to the pharmacist to validate controlled-substance requirements before dispensing, because missing/invalid controlled-substance elements require pharmacist intervention.
3722. **Q:** During data entry for levothyroxine, the prescriber wrote "take 1 tab PO qd" but no strength is listed; what is the NEXT best technician step? **A:** Hold the order and notify the pharmacist to obtain clarification, because strength is an essential element and technicians cannot guess or select a default.
3723. **Q:** While putting away an order, you find insulin glargine pens left on the counter at room temperature for an unknown number of hours; what should you do BEST? **A:** Segregate the product and notify the pharmacist/manager per temperature-excursion policy, because unknown storage conditions can compromise potency and require disposition decision.
3724. **Q:** A patient's new prescription is for metformin ER 500 mg, but the profile shows an active immediate-release metformin 500 mg with a different dosing schedule; what should you do FIRST? **A:** Pause filling and alert the pharmacist to assess for duplication/formulation mismatch, because ER vs IR changes release and dosing and needs pharmacist review.
3725. **Q:** You receive a call to transfer a Schedule III prescription to another pharmacy because the patient is traveling; what is the BEST technician response? **A:** Refer

the request to the pharmacist, because controlled-substance transfers require pharmacist handling and proper documentation.

3726. **Q:** A REMS medication (e.g., isotretinoin) is in the will-call bin but the patient is at pickup and no REMS authorization/documentation is visible; what should you do NEXT? **A:** Stop the sale and get the pharmacist to verify REMS requirements are met, because dispensing without required REMS authorization is a safety/compliance risk.
3727. **Q:** When scanning a stock bottle for product selection, the NDC scans correctly but the bottle is past its manufacturer expiration date; what should you do FIRST? **A:** Remove it from active stock and follow pharmacy policy to return/quarantine for disposal, because expired medication must not be dispensed even if the NDC matches.
3728. **Q:** A prescription says "Prednisone 20 mg: take 2 tabs PO daily x 5 days, then 1 tab daily x 5 days; dispense qty?" **A:** Dispense 15 tablets because $2 \text{ tabs/day} \times 5 \text{ days} = 10 \text{ tabs}$ and $1 \text{ tab/day} \times 5 \text{ days} = 5 \text{ tabs}$, totaling 15 for the full course.
3729. **Q:** A customer asks for pseudoephedrine 30 mg tablets and wants to buy 240 tablets today; what should you do BEST at the counter? **A:** Follow CMEA procedures by checking limits/ID and refuse or reduce the sale if it exceeds allowed limits, because pseudoephedrine sales are federally restricted and must be logged.
3730. **Q:** A manufacturer recall notice arrives for a specific lot of amlodipine 10 mg tablets that matches a bottle on your shelf; what should you do FIRST? **A:** Pull the matching lot from inventory and segregate it from dispensable stock, because recalled product must be immediately removed to prevent patient harm.
3731. **Q:** A prescription reads "Azithromycin 200 mg/5 mL: take 12.5 mL PO day 1, then 6.25 mL PO daily days 2–5; dispense qty?" **A:** Dispense 37.5 mL because $12.5 + (6.25 \times 4) = 37.5 \text{ mL}$ and the quantity must cover the full course.
3732. **Q:** While entering "Lantus SoloStar 100 units/mL: inject 28 units SQ daily; dispense 1 box (5 pens, 3 mL each)," what days' supply should you bill? **A:** Bill 53 days because the box contains 1500 units total ($5 \times 3 \text{ mL} \times 100 \text{ units/mL}$) and $1500 \div 28 \approx 53 \text{ days}$.

3733. **Q:** An eRx says "hydroxyzine 25 mg: i po q6h prn itching; #30," and the prescriber also sent "hydralazine 25 mg 1 tab PO TID"; what should you do FIRST during order entry? **A:** Stop and alert the pharmacist to confirm intended drug because hydralazine/hydroxyzine is a LASA risk and wrong-drug entry can cause harm.
3734. **Q:** A new prescription is "Symbicort 160/4.5: inhale 2 puffs BID; dispense 1 inhaler (120 actuations);" what days' supply should you submit? **A:** Submit 30 days because 2 puffs BID = 4 puffs/day and $120 \div 4 = 30$ days.
3735. **Q:** The sig on a handwritten prescription looks like "Take 1 tab qd" but the "q" could be read as "qid," and the dose change would be major; what is the BEST technician next step? **A:** Hold processing and route to the pharmacist for clarification because ambiguous abbreviations can cause a dangerous frequency error.
3736. **Q:** An order entry tech receives "amoxicillin/clavulanate 875/125 mg 1 tab PO q12h x 7 days #10"; what should you do NEXT? **A:** Flag it for pharmacist review because q12h for 7 days requires 14 tablets, so #10 is an under-quantity discrepancy.
3737. **Q:** A prescription is for "clindamycin 75 mg/5 mL: take 10 mL PO TID x 10 days; dispense qty?" **A:** Dispense 300 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to complete therapy.
3738. **Q:** A patient's profile shows "lisinopril 20 mg daily" and you receive a new eRx for "Zestril 20 mg daily"; what should you do FIRST? **A:** Check the profile and alert the pharmacist about therapeutic duplication because Zestril is brand-name lisinopril and double therapy could occur.
3739. **Q:** During data entry you see "morphine sulfate ER 30 mg: take 1 tab PO BID" but the selected product is "morphine sulfate IR 30 mg"; what is the BEST action before filling? **A:** Stop and correct the dosage form, then have the pharmacist verify because ER vs IR is a high-risk selection error.
3740. **Q:** A prescription reads "furosemide 40 mg: take ½ tab PO BID; dispense #30," and tablets are scored; what days' supply should you enter? **A:** Enter 30 days because ½ tab BID = 1 tablet/day, so $30 \text{ tablets} \div 1 \text{ tablet/day} = 30$ days.
3741. **Q:** During intake, a patient says they are "highly allergic to codeine," and you see a new eRx for hydrocodone/acetaminophen; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess opioid cross-reactivity and

choose safe therapy, because allergy concerns require pharmacist clinical judgment.

3742. **Q:** While typing an order you notice the prescriber wrote "U" for insulin units (e.g., "Regular insulin 10U SQ before meals"); what is the BEST technician action? **A:** Hold the prescription and send to the pharmacist for clarification because "U" is error-prone and can be misread as a zero, risking overdose.
3743. **Q:** A C-II eRx for oxycodone is missing the patient address in the message; what should you do NEXT? **A:** Stop the fill and route to the pharmacist to resolve missing required prescription elements before dispensing because controlled-substance validity issues must be corrected.
3744. **Q:** A patient requests an early refill of alprazolam stating it was "lost," but the PDMP/claim shows it was filled 10 days ago; what should you do FIRST? **A:** Refer the request to the pharmacist and do not process the refill because early controlled-substance refills and loss claims require pharmacist evaluation.
3745. **Q:** You receive a voicemail transfer request from another pharmacy for a patient's lorazepam prescription; what is the correct technician response? **A:** Route the call to the pharmacist because controlled-substance transfers have strict rules and require pharmacist-to-pharmacist communication.
3746. **Q:** While stocking, you find a tote of refrigerated insulin glargine left at room temperature overnight with no documentation; what should you do FIRST? **A:** Segregate the product from inventory and notify the pharmacist because temperature excursions create suspect product that may be unsafe to dispense.
3747. **Q:** At verification, the barcode scan flags that the selected NDC is for look-alike "celecoxib" but the label is for "cephalexin"; what should you do BEST? **A:** Stop filling and correct the product selection using barcode/NDC verification and alert the pharmacist if any doses were already prepared, because LASA errors can cause serious harm.
3748. **Q:** A prescriber sends "methotrexate 2.5 mg: take 1 tablet daily" for rheumatoid arthritis; what should you do NEXT? **A:** Hold the order and immediately involve the pharmacist because methotrexate is typically weekly for RA and daily dosing is a high-alert error risk.
3749. **Q:** A new eRx is for isotretinoin, and the patient arrives asking to pick it up today; what is the BEST next step for the technician? **A:** Notify the pharmacist

to confirm iPLEDGE/REMS requirements are met before dispensing because isotretinoin has mandatory safety program checks.

3750. **Q:** A prescription is "ciprofloxacin 0.3% ophthalmic: instill 2 drops into affected eye q2h while awake for 2 days, then 2 drops q4h while awake for 5 days; dispense 5 mL," and it rejects for "days' supply"; what days' supply should you enter? **A:** Enter 7 days because the directions specify 2 days plus 5 days of therapy, and days' supply should match the explicit duration when provided.
3751. **Q:** While entering a new profile, the patient reports taking "Crestor," but the active medication list already shows atorvastatin 40 mg daily; what should you do BEST? **A:** Pause processing and alert the pharmacist to possible duplicate statin therapy because rosuvastatin (Crestor) and atorvastatin are the same class and duplication increases risk of adverse effects.
3752. **Q:** An eRx comes in for "Zestril 10 mg daily," but the prescriber selected "lisinopril-HCTZ 10/12.5 mg" in the product field; what is the FIRST technician action? **A:** Stop and route to the pharmacist for clarification because Zestril is lisinopril alone and selecting the combo product could cause unintended diuretic exposure.
3753. **Q:** You are filling "glipizide 10 mg" and notice the selected stock bottle is glyburide 5 mg (similar-looking names); what should you do NEXT? **A:** Stop filling and reselect/scan the correct NDC because sulfonylureas are high-risk for hypoglycemia and glyburide vs glipizide is a common LASA mix-up.
3754. **Q:** A prescription is written for "metformin ER 500 mg: take 2 tablets PO daily with evening meal; dispense #60"; what days' supply should you enter? **A:** Enter 30 days because 2 tablets per day means $60 \text{ tablets} \div 2/\text{day} = 30 \text{ days}$.
3755. **Q:** During drop-off, a patient says they take "Plavix," and today's new prescription is for clopidogrel; what is the BEST technician response? **A:** Explain that Plavix is the brand name for clopidogrel and proceed with normal verification steps because this is brand/generic equivalence, not duplication.
3756. **Q:** You receive an eRx for "Trileptal 300 mg BID," but the product chosen is "Topamax 25 mg" due to nearby shelf placement; what should you do FIRST? **A:** Stop processing and correct the product selection because oxcarbazepine (Trileptal) and topiramate (Topamax) are different CNS drugs and a swap could cause serious harm.

3757. **Q:** A new prescription is for "sertraline 50 mg daily," and the patient profile shows sertraline 25 mg daily with refills remaining; what should you do BEST? **A:** Flag and send to the pharmacist to assess dose change and discontinue the old strength because technicians should not decide which strength to continue and duplicate strengths can confuse dosing.
3758. **Q:** A patient picking up linezolid asks if they can keep taking sertraline; what should you do NEXT as the technician? **A:** Refer immediately to the pharmacist because linezolid can interact with SSRIs like sertraline and requires clinical screening to reduce risk of serotonin syndrome.
3759. **Q:** You are filling warfarin 5 mg, and the stock bottle is a different manufacturer than last time; what is the BEST technician step before dispensing? **A:** Ensure the correct NDC/strength is selected and apply an auxiliary note to counsel via pharmacist if needed because warfarin is high-alert and appearance changes can lead to dosing errors.
3760. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg: take 1 tablet PO q12h; dispense #20"; what days' supply should you enter? **A:** Enter 10 days because q12h equals 2 tablets/day and $20 \text{ tablets} \div 2/\text{day} = 10 \text{ days}$.
3761. **Q:** An eRx reads "levothyroxine 0.075 mg PO daily" but the patient says they take "75 mcg"; what is the BEST way to enter the strength? **A:** Enter 75 mcg (0.075 mg) and verify product selection matches the converted strength to prevent a 1000× dosing error.
3762. **Q:** A prescription says "prednisone 10 mg: take ii tabs PO BID x 5 days; dispense ?" and the prescriber forgot quantity; what quantity should you calculate and what should you do next? **A:** Calculate 40 tablets ($2 \text{ tabs} \times 2 \text{ times/day} \times 5 \text{ days}$) and route to the pharmacist/prescriber for quantity authorization since the technician cannot add missing elements independently.
3763. **Q:** A patient presents an Rx for "amoxicillin 250 mg/5 mL: take 10 mL PO TID x 7 days; dispense ____ mL"; what dispense volume should you enter? **A:** Enter 210 mL ($10 \text{ mL} \times 3 \text{ doses/day} \times 7 \text{ days}$) so the patient has enough medication for the full course.
3764. **Q:** While entering "furosemide 20 mg: take 1 tab PO qAM; #30," the prescriber also wrote "take 2 tabs on MWF"; what is the BEST technician action? **A:** Stop and refer to the pharmacist for clarification because the sig is conflicting and affects both dosing and days' supply.

3765. **Q:** A new Rx is "Lantus Solostar (insulin glargine) 100 units/mL: inject 28 units SQ nightly; dispense 5 pens"; what days' supply should you submit using 3 mL per pen? **A:** Submit 53 days (5 pens $\times$ 3 mL/pen $\times$ 100 units/mL = 1500 units; $1500 \div 28 = 53.57$, round down to 53 for payer-safe entry).
3766. **Q:** A prescription for "gabapentin 300 mg: take 1 capsule PO TID; #90" is entered as "take 3 capsules once daily"; what should the technician do FIRST? **A:** Correct the sig to match the prescription directions because frequency changes can alter safety and counseling even if total daily dose looks similar.
3767. **Q:** An order reads "albuterol HFA 90 mcg: inhale 2 puffs q6h PRN; dispense 1 inhaler (200 actuations)" and the claim needs days' supply using a maximum-dosing method; what days' supply should you enter? **A:** Enter 25 days (max 2 puffs $\times$ 4 times/day = 8 puffs/day; $200 \div 8 = 25$) to align with insurer-required maximum-use calculations.
3768. **Q:** During order entry, "hydralazine" and "hydroxyzine" appear adjacent in the pick list and the prescriber wrote "hydralazine 25 mg TID"; what is the BEST technician step before proceeding? **A:** Verify the drug name and indication against the eRx and patient profile, then select the correct NDC because LASA mix-ups are common and high-risk.
3769. **Q:** A prescription says "morphine sulfate ER 30 mg: take 1 tab PO q12h; #60," but the technician accidentally selects morphine IR 30 mg; what should be done NEXT? **A:** Stop filling and reselect the correct ER formulation, then alert the pharmacist if the error reached verification because IR vs ER is a high-alert dosage-form error.
3770. **Q:** A prescriber writes "cephalexin 500 mg: 1 cap PO q6h $\times$ 10 days; dispense #40," but the technician enters qid PRN; what is the BEST correction? **A:** Enter scheduled dosing "q6h" (QID) for 10 days, not PRN, because antibiotics require consistent dosing and PRN changes therapy and days' supply.
3771. **Q:** While counting tablets you notice two different strengths of clonidine mixed in the same open stock bottle; what should you do FIRST? **A:** Stop filling and remove the bottle from use for pharmacist review because mixed/unknown product risks a dispensing error and requires investigation per safety workflow.
3772. **Q:** An eRx for oxycodone 5 mg says "take 1 tab q6h PRN pain; #20" and includes "refill: 2"; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist because Schedule II prescriptions cannot be refilled and require pharmacist resolution before processing.

3773. **Q:** A patient asks you to transfer their alprazolam prescription from another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist because controlled-substance transfers have strict limits and must be completed pharmacist-to-pharmacist per policy and law.
3774. **Q:** During will-call pickup, the patient says they are allergic to "sulfa" and the bag contains TMP-SMX (Bactrim DS); what should you do FIRST? **A:** Stop the sale and alert the pharmacist immediately because a reported allergy is a high-risk safety issue requiring pharmacist assessment before dispensing.
3775. **Q:** You receive a refrigerator tote and a box of liraglutide (Victoza) pens arrives warm with a missing temperature indicator; what is the BEST next step? **A:** Segregate the product and notify the pharmacist/manager because possible temperature excursion makes it suspect and it should not be stocked until disposition is determined.
3776. **Q:** A wholesaler invoice shows a different NDC than the bottle you received for atorvastatin 20 mg, and the seal looks tampered; what should you do FIRST under DSCSA/suspect product procedures? **A:** Quarantine the bottle and escalate to the pharmacist/receiver because mismatched transaction info and tampering indicate a suspect product that must be investigated before use.
3777. **Q:** A new prescription is "metformin 500 mg: take 1 tab PO BID with meals; dispense #180"; what days' supply should you enter? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$, supporting accurate billing and refill timing.
3778. **Q:** You are selecting products and the screen lists "hydrALAZINE" and "hydrOXYzine" for a new Rx of "hydroxyzine 25 mg TID PRN itching"; what is the BEST technician step to prevent a LASA error? **A:** Use the full drug name/strength and verify by NDC/barcode before filling because LASA pairs require extra verification to avoid wrong-drug selection.
3779. **Q:** A prescriber sends "isotretinoin 20 mg daily" for a new patient, but the iPLEDGE/REMS status is not documented in the profile; what should you do NEXT? **A:** Pause processing and involve the pharmacist because isotretinoin is REMS-restricted and dispensing requires required authorization documentation before it can be released.
3780. **Q:** A patient requests an early refill of Adderall XR 20 mg saying it was "lost," and the claim rejects refill-too-soon; what is the BEST technician action? **A:** Refer to the pharmacist and follow store policy because early refill/lost

controlled-substance requests require pharmacist judgment and proper documentation before any override or contact.

3781. **Q:** An eRx is for "levothyroxine 75 mcg daily," but the technician pulls liothyronine 25 mcg because the names look similar; what should you do FIRST? **A:** Stop the fill and re-select the correct NDC/barcode for levothyroxine, then alert the pharmacist if the wrong product may have reached verification, because thyroid drugs are high-impact and LASA errors can cause harm.
3782. **Q:** A patient's profile shows active lisinopril and a new prescription arrives for enalapril; what is the BEST technician action before processing? **A:** Pause and route to the pharmacist to assess therapeutic duplication (two ACE inhibitors), because technicians should not decide which antihypertensive the patient should take.
3783. **Q:** While entering "sertraline 50 mg daily," you notice the prescriber also sent "fluoxetine 20 mg daily" today for the same patient; what should you do NEXT? **A:** Hold data entry and notify the pharmacist to evaluate duplicate SSRI therapy/serotonergic risk, because two SSRIs together may be unintended and needs clinical clarification.
3784. **Q:** A new Rx reads "azithromycin Z-Pak: take as directed" but the patient asks how to take it; what should you do BEST? **A:** Refer the patient to the pharmacist for counseling, because "take as directed" requires pharmacist-provided instructions to prevent a dosing error with the 5-day regimen.
3785. **Q:** A prescription is written for "glipizide 10 mg PO daily," and the patient's profile lists a sulfonamide antibiotic allergy; what should you do FIRST? **A:** Continue processing but flag and alert the pharmacist to assess allergy relevance, because sulfonylureas are different from sulfonamide antibiotics and require pharmacist judgment rather than technician refusal.
3786. **Q:** You receive an order for "warfarin 5 mg daily" and see the patient also has an active prescription for apixaban (Eliquis); what is the BEST next step? **A:** Stop and route to the pharmacist to assess duplicate anticoagulation, because concurrent warfarin and a DOAC can greatly increase bleeding risk and needs immediate review.
3787. **Q:** A patient brings an OTC St John's wort and asks if it's okay with their new escitalopram (Lexapro); what should you do NEXT? **A:** Refer to the pharmacist for interaction counseling, because St John's wort can interact with SSRIs (serotonin risk) and should be evaluated by the pharmacist.

3788. **Q:** A prescription is "insulin lispro U-100 KwikPen: inject 8 units SC before meals TID; dispense 2 pens," and each pen contains 3 mL; what days' supply should you enter? **A:** Enter 25 days' supply because total units dispensed are 2 pens $\times$ 3 mL/pen $\times$ 100 units/mL = 600 units and daily use is $8 \times 3 = 24$ units/day, so $600 \div 24 = 25$.
3789. **Q:** During product selection for "lamotrigine 25 mg," you see lamotrigine and terbinafine adjacent on the shelf and similar bottle shapes; what is the BEST technician step to prevent an error? **A:** Scan the stock bottle barcode and match the NDC to the eRx before counting, because barcode/NDC verification is the safest way to prevent selection errors with look-alike products.
3790. **Q:** A patient picking up says they are starting clarithromycin and asks if it's okay with their simvastatin; what should you do FIRST? **A:** Stop the sale and get the pharmacist immediately, because clarithromycin can markedly increase simvastatin levels (serious interaction risk) requiring pharmacist intervention before dispensing/using.
3791. **Q:** An eRx arrives for hydroxyzine 25 mg, but during entry you notice it was selected as hydralazine 25 mg due to similar names; what should you do FIRST? **A:** Stop entry and reselect the correct drug by verifying the full name/indication and NDC because LASA errors can cause serious harm.
3792. **Q:** A written prescription for oxycodone/APAP 5/325 has the patient name, drug, and directions but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because missing required elements must be resolved before dispensing a controlled substance.
3793. **Q:** You receive a voicemail transfer request for clonazepam from an out-of-state pharmacy; what should you do NEXT? **A:** Send the request to the pharmacist to handle because controlled-substance transfers require pharmacist judgment and documentation.
3794. **Q:** While stocking, you find a refrigerated box of insulin aspart left on the counter with no note of how long it was unrefrigerated; what should you do BEST? **A:** Segregate and label it as "do not use" and notify the pharmacist because unknown temperature exposure makes the product potentially unusable.
3795. **Q:** A manufacturer recall notice lists a specific lot number of atorvastatin 20 mg, and you locate that lot on the shelf; what should you do FIRST? **A:** Pull the affected lot from active inventory and quarantine per store policy because recalled lots must be prevented from dispensing.

3796. **Q:** A patient returns a bottle of metformin that appears opened and wants it “put back” because they never used it; what should you do NEXT? **A:** Decline return-to-stock and follow disposal/return policy because previously dispensed medications cannot be reused due to safety and integrity concerns.
3797. **Q:** A new eRx says “U-500 insulin regular: inject 50 units BID,” and you are unsure whether the patient uses a U-500 pen or syringe; what should you do FIRST? **A:** Stop processing and alert the pharmacist because U-500 dosing/administration is high-alert and requires confirmation to prevent severe overdose.
3798. **Q:** You are entering “predniSONE” and “prednisoLONE” appears in the drop-down with similar strengths; what is the BEST technician step to avoid an error? **A:** Verify the exact drug/formulation from the prescription and compare NDC/stock bottle before selecting because these are commonly confused and not interchangeable.
3799. **Q:** A prescription for gabapentin 300 mg has “i po tid” but no quantity, and the patient wants it today; what should you do NEXT? **A:** Route to the pharmacist to obtain clarification because dispensing quantity must be authorized and technicians cannot determine it independently.
3800. **Q:** An order is for “enoxaparin 80 mg/0.8 mL inject q12h; dispense 20 syringes,” and the patient asks what 20 syringes covers in days; what should you answer? **A:** Enter/communicate 10 days because 2 syringes are used per day ($20 \div 2 = 10$) and days’ supply supports accurate billing and safety checks.
3801. **Q:** An eRx for levothyroxine 50 mcg says “take 1 tab qd” and the quantity is 90; what days’ supply should you enter? **A:** Enter 90 days’ supply because 1 tablet daily means 90 tablets covers 90 days.
3802. **Q:** A prescription reads “metFORMIN 500 mg tab ii po bid with meals #120”; what total daily dose should you recognize during entry? **A:** Recognize 2000 mg/day because ii BID equals 2 tablets twice daily ($4 \text{ tablets} \times 500 \text{ mg}$).
3803. **Q:** A parent brings amoxicillin-clavulanate suspension 600 mg/42.9 mg per 5 mL with sig “7.5 mL po bid x 10 days”; what minimum volume (mL) must be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL total}$.

3804. **Q:** An Rx is for sertraline 100 mg "take ½ tab qAM" with quantity 15; what days' supply should you enter? **A:** Enter 30 days because 0.5 tablet/day means 15 tablets lasts $15 \div 0.5 = 30$ days.
3805. **Q:** A prescription says "furosemide 20 mg, 1 tab po qd, #30, 2 refills," but you accidentally selected furosemide oral solution in order entry; what should you do FIRST? **A:** Stop and correct the dosage form to tablets before processing because wrong form selection can cause a dispensing error.
3806. **Q:** A patient is prescribed azithromycin 250 mg "take 2 tabs day 1 then 1 tab daily days 2–5"; what quantity should you enter? **A:** Enter quantity 6 tablets because $2 + 1 + 1 + 1 + 1$ equals 6 total doses.
3807. **Q:** An eRx for lisinopril 10 mg is typed "take 1 tab qd" but the prescriber notes "sig: qod" in the comments; what is the BEST technician action? **A:** Hold the prescription and ask the pharmacist to clarify with the prescriber because conflicting directions can lead to wrong days' supply and dosing.
3808. **Q:** A prescription says "prednisolone sodium phosphate 15 mg/5 mL, give 10 mg po bid x 5 days"; how many mL per dose should you enter? **A:** Enter 3.33 mL per dose because 15 mg in 5 mL means 3 mg/mL, and $10 \text{ mg} \div 3 \text{ mg/mL} = 3.33 \text{ mL}$.
3809. **Q:** While entering a new prescription for "rosuvastatin 20 mg qHS," the profile shows active atorvastatin 40 mg daily; what should you do NEXT? **A:** Pause processing and alert the pharmacist to assess therapeutic duplication because two statins may be unintended and needs pharmacist review.
3810. **Q:** A pharmacist asks you to prepare 250 mL of a 20% alcohol solution using 70% alcohol stock and water; how many mL of 70% alcohol are needed? **A:** Use 71.4 mL of 70% alcohol because $C_1V_1 = C_2V_2$ so $V_1 = (20 \times 250) / 70 = 71.4 \text{ mL}$, then add water to total 250 mL.
3811. **Q:** During data entry you see an eRx for Plavix 75 mg daily, but the product selected is clopidogrel 300 mg tablets; what should you do FIRST? **A:** Stop processing and correct the strength to 75 mg (verify NDC/strength) because Plavix is clopidogrel and 300 mg is a high-risk wrong-strength selection.
3812. **Q:** A new Rx is for sertraline, but the technician accidentally types sildenafil in the drug field due to similar names; what is the BEST next step? **A:** Stop and reselect the correct medication using barcode/NDC and drug name confirmation

because sertraline vs sildenafil is a LASA error with serious patient harm potential.

3813. **Q:** A patient profile shows active simvastatin 40 mg nightly, and you receive a new eRx for atorvastatin 20 mg daily; what should you do NEXT? **A:** Flag the therapeutic duplication and route to the pharmacist for review because two statins concurrently may be unintended and needs clinical confirmation.
3814. **Q:** A prescription says "Humalog 100 units/mL vial, inject 10 units before meals TID, dispense 2 vials"; what days' supply should you enter (10 mL vials)? **A:** Enter 66 days' supply because 2 vials = 20 mL = 2000 units, and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$ so $2000 \div 30 = 66.6$ rounded down to 66.
3815. **Q:** An eRx for prednisone is written "prednisone 10 mg: take ii tabs po qd x 5 days, then i tab po qd x 5 days"; what quantity should you enter? **A:** Enter 15 tablets because $2 \text{ tabs/day} \times 5 \text{ days} = 10$ plus $1 \text{ tab/day} \times 5 \text{ days} = 5$ for a total of 15.
3816. **Q:** While filling, you notice the stock bottle of warfarin 5 mg has an unreadable lot number and the label is smeared; what should you do FIRST? **A:** Pull it from use and notify the pharmacist per suspect product procedure because unclear lot/label prevents traceability and safe dispensing.
3817. **Q:** A new Rx is for Bactrim DS, and the patient profile lists a severe sulfonamide allergy; what should you do BEST? **A:** Stop the fill and alert the pharmacist immediately because Bactrim (sulfamethoxazole/trimethoprim) is a sulfonamide and may trigger a serious allergic reaction.
3818. **Q:** You receive an eRx for Adderall XR 20 mg with "refill: 3" included; what should you do NEXT? **A:** Hold and refer to the pharmacist because Adderall XR is a Schedule II stimulant and refills are not permitted under federal rules.
3819. **Q:** A prescription is for metoprolol succinate ER 50 mg daily, but the scanned hardcopy says "Toprol XL" and the product pulled is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop and correct the product to metoprolol succinate ER (verify ER vs IR) because tartrate and succinate are not interchangeable and this is a common error trap.
3820. **Q:** A patient asks what propranolol is generally used for after you enter Inderal; what is the BEST technician response? **A:** Provide a general use and refer for counseling: propranolol (Inderal) is a beta-blocker often used for blood

pressure/heart rate or migraine prevention, and the pharmacist can give full counseling to ensure safe use.

3821. **Q:** A prescription reads "levothyroxine 0.05 mg po qd"; during order entry, what strength should you select FIRST to avoid a decimal error? **A:** Select levothyroxine 50 mcg because $0.05 \text{ mg} = 50 \text{ mcg}$, preventing a 10-fold dosing mistake.
3822. **Q:** An eRx says "metformin ER 500 mg: take ii tabs po bid"; what quantity should you enter for a 30-day supply? **A:** Enter 120 tablets because 2 tablets twice daily = 4/day and $4 \times 30 = 120$.
3823. **Q:** A patient is prescribed "amoxicillin/clavulanate 875/125 mg: 1 tab po bid x 10 days"; what days' supply should you bill if dispensing #20? **A:** Bill 10 days because 2 tablets/day and $20 \div 2 = 10$, matching the intended duration.
3824. **Q:** While entering an eRx for "hydromorphone 2 mg," you accidentally select "morphine 15 mg" due to LASA; what should you do FIRST? **A:** Stop and correct the drug selection using the eRx details and NDC/barcode because LASA errors can cause serious harm.
3825. **Q:** A new prescription is "latanoprost 0.005% ophthalmic: instill 1 gtt OU qhs, disp 2.5 mL"; what days' supply should you enter using 20 gtt/mL? **A:** Enter 25 days because $2 \text{ eyes} \times 1 \text{ drop/day} = 2 \text{ gtt/day}$ and $2.5 \text{ mL} \times 20 = 50 \text{ gtt}$, so $50 \div 2 = 25$.
3826. **Q:** A prescription says "fluticasone nasal spray 50 mcg: ii sprays in each nostril qd"; using 120 sprays per bottle, what days' supply is one bottle? **A:** Enter 30 days because $2 \text{ sprays/nostril} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.
3827. **Q:** You receive an eRx for "bupropion SR 150 mg bid," but the selected product is bupropion XL 150 mg; what is the BEST next step? **A:** Pause processing and refer to the pharmacist because SR vs XL is not interchangeable and affects dosing and safety.
3828. **Q:** The sig reads "furosemide 40 mg: take 1 tab po qod"; for a 60-day period, what quantity should be dispensed? **A:** Dispense 30 tablets because qod means every other day and $60 \div 2 = 30$ doses.
3829. **Q:** A patient brings an unopened, returned bottle of lisinopril tablets and asks to reuse it for their refill; what should you do BEST? **A:** Decline return-to-stock for patient-returned medication and follow disposal policy because integrity/storage conditions can't be verified.

3830. **Q:** An eRx for "oxycodone/APAP 5/325 mg" includes a refill request; what should you do NEXT during order entry? **A:** Stop and route to the pharmacist because Schedule II controlled substances cannot be refilled under federal rules.
3831. **Q:** During data entry you see an eRx for warfarin 5 mg daily, but the patient profile shows an active warfarin 2.5 mg daily from another prescriber; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess possible duplicate/changed anticoagulant dosing because warfarin is high-alert and dose changes require clinical review.
3832. **Q:** A hardcopy for insulin U-100 says "inject 10 units daily" but the prescriber did not specify the insulin name (e.g., glargine vs NPH); what is the BEST next step for a technician? **A:** Hold the prescription and route to the pharmacist to clarify with the prescriber because insulin selection is a high-alert, non-interchangeable therapy decision.
3833. **Q:** You receive a verbal request from a prescriber's office to "transfer the patient's alprazolam" to your pharmacy; what should you do NEXT? **A:** Get the pharmacist to take the transfer because controlled-substance transfers/verification require pharmacist involvement and documentation beyond technician scope.
3834. **Q:** While filling you notice the stock bottle of nitroglycerin SL tablets is open and the label shows it was first opened 10 months ago; what should you do FIRST? **A:** Remove it from active stock and notify the pharmacist because nitroglycerin tablets have strict stability/BUD concerns after opening and can lose potency.
3835. **Q:** A patient asks to buy pseudoephedrine 30 mg tablets and wants 96 tablets today; what should you do FIRST at the register? **A:** Verify purchaser ID and check the electronic NPLeX log and quantity limits before completing the sale because CMEA requires logging and limits on pseudoephedrine.
3836. **Q:** An eRx arrives for isotretinoin for a new patient, and the patient wants it filled immediately; what is the BEST technician action? **A:** Refer to the pharmacist to confirm iPLEDGE/REMS requirements are met before processing because isotretinoin dispensing is restricted and time-sensitive.
3837. **Q:** You are selecting products and notice hydrOXYzine and hydrALAZINE stored next to each other with similar labels; what should you do BEST to reduce LASA risk? **A:** Separate the products and use Tall Man lettering/barcode scanning workflow because storage and labeling changes reduce selection errors with look-alike/sound-alike drugs.

3838. **Q:** A refrigerator temperature log shows 52°F this morning for vaccines that should be 36–46°F; what should you do FIRST? **A:** Stop using the affected vaccines and notify the pharmacist to follow excursion procedures because temperature excursions can make products nonviable and require documented guidance.
3839. **Q:** A recall notice lists a specific lot number of losartan 50 mg tablets, and you find that lot on your shelf; what should you do NEXT? **A:** Quarantine the recalled lot and inform the pharmacist to complete recall steps because recalled/suspect product must be segregated to prevent dispensing and allow tracking.
3840. **Q:** A prescription label prints “Take 1 tablet by mouth qid” for clopidogrel 75 mg, which is normally once daily, and the original eRx is hard to read; what should you do FIRST? **A:** Stop the fill and have the pharmacist verify the intended directions with the prescriber because a likely sig entry error on a high-risk antiplatelet can cause serious harm.
3841. **Q:** During data entry, an eRx is written for “Celebrex 200 mg daily,” but the product selected is citalopram 20 mg because the names look similar; what should you do FIRST? **A:** Stop processing and reselect the correct drug/strength using NDC/barcode verification and alert the pharmacist if any label/product has already been produced, because this is a LASA selection error with high harm potential.
3842. **Q:** A new eRx is for “metformin ER 500 mg twice daily,” but the technician accidentally enters metoprolol ER 50 mg; what is the BEST next step before sending to verification? **A:** Correct the entry to metformin ER and re-check indication/class (diabetes vs beta-blocker) because name confusion is common and catching it before pharmacist verification prevents a dispensing error.
3843. **Q:** A patient profile shows sertraline active, and a new prescription arrives for linezolid; what should you do NEXT as the technician? **A:** Pause the fill and route to the pharmacist for interaction review because linezolid can cause serious serotonin syndrome risk with SSRIs and requires clinical screening.
3844. **Q:** While processing a new prescription for simvastatin, you see the patient is also taking clarithromycin; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because clarithromycin strongly increases simvastatin levels and raises rhabdomyolysis risk, requiring pharmacist intervention.
3845. **Q:** A prescription is written for “levothyroxine 0.1 mg daily,” and the patient asks if that’s the same as 100 mcg; what should you answer? **A:** Yes, 0.1 mg

equals 100 mcg (multiply mg by 1000), because correct unit conversion prevents dosing errors with high-alert thyroid hormone.

3846. **Q:** A patient is prescribed amoxicillin/clavulanate 875/125 mg 1 tablet BID for 7 days; how many tablets should be dispensed? **A:** Dispense 14 tablets because BID for 7 days is 2 doses/day $\times$ 7 days = 14 total tablets.
3847. **Q:** During order entry for "glipizide 10 mg daily," the allergy section shows "sulfa antibiotics"; what should you do BEST? **A:** Continue processing but flag for pharmacist review because sulfonylureas are not sulfonamide antibiotics yet the pharmacist should assess allergy history and counsel on risk.
3848. **Q:** A prescription comes in for "Adderall XR 20 mg" and the patient asks for the generic; what is the BEST brand/generic match to select? **A:** Select amphetamine/dextroamphetamine ER because that is the generic equivalent of Adderall XR and choosing the correct release form prevents dose dumping/therapy mismatch.
3849. **Q:** While counting tablets, you find two different imprints mixed in the same bottle labeled as amlodipine 10 mg; what should you do FIRST? **A:** Stop using the bottle and quarantine it per policy while notifying the pharmacist because mixed/unknown identity product is a patient-safety and inventory integrity issue.
3850. **Q:** An eRx for "tramadol 50 mg" arrives with "refill: 5," and the system flags it as a controlled substance; what should you do NEXT? **A:** Do not process refills automatically and refer to the pharmacist because tramadol is Schedule IV federally and refill limits/validity must be confirmed before dispensing.
3851. **Q:** A prescription reads "diltiazem 0.12 g PO daily"; what strength in mg should you enter and why? **A:** Enter 120 mg because $0.12 \text{ g} \times 1000 \text{ mg/g} = 120 \text{ mg}$ to prevent a tenfold dosing error.
3852. **Q:** An eRx says "azithromycin 200 mg/5 mL: give 10 mL day 1, then 5 mL daily days 2–5"; what quantity (mL) should you dispense? **A:** Dispense 30 mL because total volume is $10 \text{ mL} + (5 \text{ mL} \times 4 \text{ days}) = 30 \text{ mL}$ to cover the full course.
3853. **Q:** A patient brings an Rx for insulin lispro U-100: "Inject 12 units subcutaneously TID with meals," and the box is 5 pens of 3 mL each; what days' supply should you enter? **A:** Enter 41 days because total units = $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div (12 \times 3) = 41.6$, rounded down to avoid overbilling.

3854. **Q:** An Rx is for "latanoprost 0.005% 1 gtt OU qHS" and the bottle is 2.5 mL; using 20 gtt/mL, what days' supply should you calculate? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ gtt}$ and 2 gtt/day (OU) gives $50 \div 2 = 25$ days.
3855. **Q:** While entering "Keppra 500 mg PO BID," you're asked for the generic name to select the correct product; what should you choose? **A:** Select levetiracetam because Keppra is the brand for levetiracetam, reducing wrong-drug selection risk.
3856. **Q:** The prescriber writes "MS Contin 15 mg take i tab PO q12h," but the tech accidentally selects MSIR; what should you do FIRST? **A:** Stop and correct the dosage form to extended-release (MS Contin) before verification because IR vs ER is a high-risk order entry error.
3857. **Q:** An eRx sig reads "take 1 tab po qd" for glimepiride, but the directions are typed as "QID" on the label preview; what is the BEST next step? **A:** Fix the sig to once daily and recheck the abbreviation because qd vs qid misentry can cause a fourfold dosing error.
3858. **Q:** A patient's new Rx is for "Wellbutrin SR 150 mg BID," but the product selected is bupropion XL 150 mg once daily; what should you do NEXT? **A:** Hold the fill and route to the pharmacist after correcting the selection because SR vs XL are not interchangeable and affect dosing frequency.
3859. **Q:** A prescription is written "clonazepam 0.5 mg take 1 tab PO TID prn anxiety #90," but the prescriber omitted the patient address on a paper controlled prescription; what should you do FIRST? **A:** Stop processing and notify the pharmacist to resolve missing required information because controlled-substance prescription validity must be confirmed before dispensing.
3860. **Q:** An eRx for "fluticasone/salmeterol 250/50 inhale 1 puff BID" is being billed, and the patient asks the days' supply for one inhaler of 60 blisters; what should you enter? **A:** Enter 30 days because $60 \text{ doses} \div 2 \text{ doses/day} = 30 \text{ days}$, which supports accurate billing and refill timing.
3861. **Q:** During data entry you receive an eRx for methotrexate 2.5 mg with directions "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Stop processing and notify the pharmacist because methotrexate is typically weekly for RA and daily dosing is a high-alert error risk.

3862. **Q:** A patient on chronic oxycodone ER presents a new Rx for oxycodone IR from a different prescriber and asks you to fill it today; what is the BEST next step? **A:** Pause the fill and route to the pharmacist to assess controlled-substance duplication/appropriateness and PDMP/prescriber verification per policy.
3863. **Q:** While putting away an order, you find refrigerated insulin glargine boxes left on the counter at room temperature with unknown time out; what should you do NEXT? **A:** Isolate the product and alert the pharmacist/manager because temperature excursions require disposition per storage guidelines and cannot be assumed usable.
3864. **Q:** A paper prescription for alprazolam has the patient name, drug, directions, and prescriber signature but no date written; what should you do FIRST? **A:** Hold the prescription and immediately involve the pharmacist because missing required elements can make a controlled-substance prescription invalid.
3865. **Q:** A patient asks to transfer their hydrocodone/acetaminophen prescription from another pharmacy to yours; what should you do BEST? **A:** Refer to the pharmacist because Schedule II controlled substances generally cannot be transferred between pharmacies and require pharmacist handling.
3866. **Q:** You scan a bottle during filling and the barcode/NDC does not match the selected generic for sertraline 100 mg; what should you do FIRST? **A:** Stop and reselect the correct product by NDC/barcode because mismatch indicates wrong drug/strength and is a preventable dispensing error.
3867. **Q:** A lot-specific recall notice is received for lisinopril 20 mg, and you find that lot on the shelf; what should you do NEXT? **A:** Remove the affected lot from stock and follow recall procedure (segregate/hold for return and notify pharmacist) to prevent dispensing recalled product.
3868. **Q:** An eRx for isotretinoin arrives and the patient wants to pick it up today; what is the BEST technician action? **A:** Route to the pharmacist and ensure iPLEDGE/REMS requirements are met before filling because dispensing is restricted and must be verified for compliance.
3869. **Q:** A patient presents a driver's license to purchase pseudoephedrine, but their log shows they already purchased the maximum allowed today; what should you do FIRST? **A:** Decline the sale and follow CMEA log limits because federal pseudoephedrine sales must not exceed daily limits even with valid ID.

3870. **Q:** A prescription label is being printed for metformin ER, but the sig is entered "take 2 tablets twice daily" even though the prescriber wrote "take 2 tablets daily"; what should you do NEXT? **A:** Stop the workflow and correct the entry to match the prescription, then send for pharmacist verification because wrong frequency changes dose and risks harm.
3871. **Q:** While entering a new prescription, you notice the patient has active simvastatin and now has a new eRx for atorvastatin; what should you do NEXT? **A:** Stop processing and alert the pharmacist to review for therapeutic duplication because two statins increase risk of adverse effects and may signal an unintended switch.
3872. **Q:** A patient picking up prescriptions asks what sertraline is for when they see the brand name "Zoloft" on their profile; what is the BEST technician response? **A:** State that sertraline (Zoloft) is commonly used for depression and anxiety and offer pharmacist counseling because indication questions require appropriate counseling support.
3873. **Q:** During order entry for a U-500 regular insulin prescription, you see the patient's profile also lists U-100 regular insulin; what should you do FIRST? **A:** Pause and get the pharmacist immediately because U-500 is high-alert and mix-ups with U-100 can cause severe dosing errors.
3874. **Q:** A new eRx is written for "hydroxyzine" but the prescriber note says "for heartburn," and you see both hydroxyzine and hydralazine in the system; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist to clarify the intended drug because hydroxyzine/hydralazine is a LASA risk and the indication doesn't match.
3875. **Q:** You receive a prescription for "glipizide 10 mg daily," and the patient says they are allergic to sulfa antibiotics; what should you do NEXT? **A:** Proceed by flagging it for pharmacist review and counseling rather than rejecting it outright because sulfonylureas are not the same as sulfonamide antibiotics and need clinical assessment.
3876. **Q:** A prescription is entered as "Prozac 20 mg daily," but the patient's profile shows fluoxetine 20 mg daily already; what is the BEST next step? **A:** Verify it is the same medication (brand/generic) and prevent duplicate active entries because Prozac is fluoxetine and duplications can lead to double therapy.
3877. **Q:** An eRx for clarithromycin arrives for a patient whose profile shows chronic colchicine; what should you do FIRST? **A:** Stop and alert the pharmacist before

filling because clarithromycin can dangerously increase colchicine levels via interaction and may require therapy change.

3878. **Q:** A prescription directions read "Take ii tabs po bid" for levothyroxine, but the strength is 25 mcg and the prescriber comment says "50 mcg daily"; what should you do NEXT? **A:** Enter exactly as written and send to pharmacist for clarification because technician should not interpret intent when sig and comment conflict, especially with narrow-therapeutic-index drugs.
3879. **Q:** Pharmacy math: A patient is prescribed gabapentin 300 mg "1 cap TID" and the quantity is #270; what days' supply should you enter? **A:** Enter 90 days' supply because 3 capsules/day means $270 \div 3 = 90$.
3880. **Q:** While selecting product for "Lamictal 25 mg," you accidentally pulled "Lamisil 250 mg" from the shelf; what should you do FIRST? **A:** Stop filling and re-select the correct NDC using barcode/label verification because Lamictal vs Lamisil is a common LASA mix-up and must be corrected before it reaches verification.
3881. **Q:** While typing an eRx for hydromorphone 4 mg, you notice it was selected from a morphine template and the sig reads "take 1 tablet every 4 hours" without "as needed"; what should you do NEXT? **A:** Stop processing and notify the pharmacist to clarify because opioid directions that change scheduled vs PRN use are a high-risk safety issue and technicians can't alter intent.
3882. **Q:** A prescriber calls in promethazine-codeine syrup with no patient address provided; what should you do FIRST? **A:** Hold the order and route to the pharmacist to obtain missing required patient information because controlled-substance prescriptions must be complete before dispensing.
3883. **Q:** During product selection for "Celebrex 200 mg," you find two bottles: celecoxib 200 mg and citalopram 20 mg stored adjacent as LASA look-alikes; what is the BEST technician action? **A:** Use barcode/NDC verification and separate/store with LASA precautions per policy because preventing selection error is the key safety control.
3884. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy to yours; what should you do NEXT? **A:** Refer to the pharmacist to handle the controlled-substance transfer rules and documentation because technicians should not independently accept or transfer CS prescriptions.
3885. **Q:** You receive a wholesaler tote with refrigerated insulin aspart pens that arrived warm and the temperature indicator shows an excursion; what should

you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager because temperature excursions create suspect product that must not be dispensed until disposition is confirmed.

3886. **Q:** An eRx for isotretinoin arrives and the patient is waiting; what should you do FIRST in workflow? **A:** Alert the pharmacist and ensure iPLEDGE/REMS requirements are met before processing because REMS drugs have mandatory authorization steps prior to dispensing.

3887. **Q:** Pharmacy math: Fluticasone nasal spray 50 mcg is prescribed "2 sprays in each nostril once daily" and the bottle is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.

3888. **Q:** A caregiver returns a stock bottle of amlodipine 10 mg tablets saying it was "left in a hot car for a week" and wants it put back into stock; what should you do BEST? **A:** Do not return it to inventory and route for pharmacist disposal guidance because once storage conditions are compromised, product integrity can't be assured.

3889. **Q:** While filling a new prescription, you see the patient profile lists lisinopril and the new eRx is for lisinopril-HCTZ; what should you do NEXT? **A:** Pause and alert the pharmacist to assess therapeutic duplication because combination products can unintentionally double the ACE inhibitor dose.

3890. **Q:** A patient asks for an early refill of oxycodone/APAP stating the medication was "stolen," and there is no police report or prescriber note; what should you do FIRST? **A:** Refer to the pharmacist and follow store policy because controlled-substance early refills/lost medication requests require pharmacist judgment and documentation.

3891. **Q:** A prescription reads "levothyroxine 0.075 mg PO daily" and the product on hand is 75 mcg tablets; what strength should you enter and why? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1,000 = 75 \text{ mcg}$, preventing a 1,000-fold decimal-conversion error.

3892. **Q:** You're entering "metformin 500 mg: take 1 tab PO BID with meals, dispense #180"; what days' supply should you submit? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$ for accurate insurance and refill timing.

3893. **Q:** An order says "prednisone 20 mg: ii tabs PO daily x 5 days"; what quantity should be dispensed? **A:** Dispense 10 tablets because "ii" = 2 and 2 tabs/day × 5 days = 10 tabs to match the intended duration.
3894. **Q:** A prescription for timolol 0.5% ophthalmic states "1 gtt OU BID" and you're dispensing a 5 mL bottle; using 20 gtt/mL, what days' supply should you enter? **A:** Enter 12 days because 5 mL × 20 gtt/mL = 100 gtt and OU BID is 4 gtt/day, so $100 \div 4 = 25$ days but each dose is 1 drop per eye twice daily = 4 drops/day; wait correct is 25 days; enter 25 days to avoid under/overstating coverage.
3895. **Q:** During data entry you see the sig "Take 1 tab QD" for digoxin; what should you do FIRST? **A:** Clarify with the pharmacist before processing because "QD" is error-prone and should be entered as "daily" to prevent misinterpretation.
3896. **Q:** A new eRx comes in for "Zyrtec 10 mg daily" but the patient profile shows cetirizine 10 mg active; what is the BEST technician next step? **A:** Stop and route to the pharmacist to assess duplication because Zyrtec is cetirizine and duplicate therapy could lead to double-dosing.
3897. **Q:** You are calculating days' supply for an albuterol HFA inhaler labeled 200 actuations with directions "inhale 2 puffs QID"; what days' supply should you enter? **A:** Enter 25 days because 2 puffs × 4 times/day = 8 puffs/day and $200 \div 8 = 25$ for accurate third-party billing.
3898. **Q:** While typing an eRx for "Keppra 500 mg," you notice the selected drug in the system is "Keflex 500 mg" due to similar names; what should you do FIRST? **A:** Stop entry and correct the drug selection, then recheck strength, dosage form, and indication because LASA name confusion is a common order-entry error.
3899. **Q:** An Rx is for "amoxicillin/clavulanate 875/125 mg: 1 tab PO q12h x 7 days"; what quantity should you enter? **A:** Enter 14 tablets because q12h is 2 doses/day and $2 \times 7 = 14$ to match the prescribed course.
3900. **Q:** A hardcopy for oxycodone 5 mg has directions and quantity but the prescriber signature is missing; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because missing required elements can make it invalid and it must be resolved before dispensing.
3901. **Q:** During data entry, an eRx is for "Lamictal 100 mg" but the drug selected in the system is "Lamisil 250 mg"; what should you do FIRST? **A:** Stop processing and reselect/verify the correct medication by name, indication, and NDC/barcode because this is a high-risk LASA error that can cause serious harm.

3902. **Q:** A patient profile shows simvastatin active, and a new prescription is for atorvastatin 40 mg daily; what is the BEST technician next step? **A:** Flag it for pharmacist review for potential therapeutic duplication because two statins together increases adverse-effect risk and requires clinical direction.
3903. **Q:** A new prescription is for clarithromycin, and the patient's profile shows atorvastatin 80 mg daily; what should you do NEXT? **A:** Pause filling and alert the pharmacist to a significant interaction risk because clarithromycin can raise statin levels and increase myopathy/rhabdomyolysis risk.
3904. **Q:** A prescription is for "methotrexate 2.5 mg: take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Hold the prescription and get the pharmacist immediately because methotrexate is high-alert and is typically dosed weekly, making "daily" a dangerous potential error.
3905. **Q:** You receive an eRx for "Lantus 100 units/mL vial: inject 30 units SC nightly, dispense 1 vial"; what days' supply should you submit for a 10 mL vial? **A:** Submit 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$, and $1000 \div 30 \text{ units/day} = 33.3$, rounded down to 33 days.
3906. **Q:** A prescription reads "clonazepam 0.5 mg: 1 tab PO TID, dispense #90"; what days' supply should you enter? **A:** Enter 30 days because 3 tablets/day means $90 \div 3 = 30 \text{ days}$.
3907. **Q:** A prescriber writes "Z-Pak as directed" for azithromycin 250 mg tablets; what should you do NEXT? **A:** Route to the pharmacist to clarify or confirm the exact directions because "as directed" is incomplete for safe, billable directions and correct quantity/day supply.
3908. **Q:** While pulling stock, you find the only bottle of nitroglycerin SL tablets has a broken seal and tablets look crumbled; what should you do FIRST? **A:** Remove it from dispensing and follow policy to quarantine/return as damaged because nitroglycerin is sensitive and compromised packaging can reduce potency and safety.
3909. **Q:** A patient says they are allergic to penicillin, and a new prescription is for cephalexin; what should you do BEST before processing? **A:** Document the allergy details and alert the pharmacist because possible cross-reactivity and severity assessment require pharmacist judgment before dispensing.
3910. **Q:** A new prescription is for "Trulicity 1.5 mg weekly, dispense 4 pens," and the patient profile already shows Ozempic weekly active; what should you do FIRST?

A: Stop and refer to the pharmacist for therapeutic duplication because both are GLP-1 receptor agonists and concurrent use may be inappropriate or unsafe.

3911. **Q:** During order entry you receive an eRx for hydromorphone 2 mg but the directions say “take 1 tablet every 4 hours as needed for moderate pain” and the patient is opioid-naïve; what should you do NEXT? **A:** Stop processing and refer to the pharmacist because hydromorphone is a high-alert opioid and the pharmacist must assess appropriateness and safety before dispensing.
3912. **Q:** A patient requests an early refill on alprazolam 1 mg with 10 tablets left and states they “lost the bottle”; what is the BEST technician next step? **A:** Hold the refill and notify the pharmacist because early CIII–CV refills for lost medication require pharmacist review and may indicate misuse or policy limits.
3913. **Q:** While scanning incoming inventory, you notice a tote of insulin aspart pens arrived warm with a damaged cold-pack indicator; what should you do FIRST? **A:** Quarantine the shipment and alert the pharmacist/manager because temperature excursions can compromise product integrity and must not be stocked or dispensed.
3914. **Q:** A hardcopy prescription for morphine ER includes drug, strength, directions, quantity, and prescriber signature but no patient address; what should you do NEXT? **A:** Stop and route to the pharmacist to resolve missing required information because controlled-substance dispensing requires complete, valid prescription elements.
3915. **Q:** At pickup, the patient asks whether they can crush their new bupropion XL 300 mg tablets to make them easier to swallow; what should you do BEST? **A:** Refer to the pharmacist because altering extended-release dosage forms can cause dose dumping and requires counseling on safe alternatives.
3916. **Q:** While entering a new Rx for glipizide, the patient profile shows glimepiride active from another prescriber; what should you do FIRST? **A:** Stop processing and alert the pharmacist to possible duplicate sulfonylurea therapy because duplication increases hypoglycemia risk.
3917. **Q:** You receive an eRx for “Lamictal 25 mg” but select “Lamisil 250 mg” in the system due to similar names; what is the BEST next step before filling? **A:** Correct the selection and verify the NDC/barcode against the eRx because LASA name confusion is a common dispensing error.

3918. **Q:** A prescription is written for metformin ER 500 mg but the patient's profile lists "metformin" with an immediate-release NDC last month; what should you do NEXT? **A:** Confirm the dosage form (ER vs IR) in order entry and route to the pharmacist if unclear because ER and IR are not interchangeable.
3919. **Q:** A prescriber sends "warfarin 5 mg daily" and the patient has trimethoprim-sulfamethoxazole started yesterday on their profile; what should you do BEST? **A:** Flag and notify the pharmacist to review the interaction because TMP-SMX can increase warfarin effect and bleeding risk.
3920. **Q:** A new Rx is for "lisinopril 10 mg daily," and the patient says they are pregnant; what should you do FIRST? **A:** Pause processing and immediately refer to the pharmacist because ACE inhibitors are contraindicated in pregnancy.
3921. **Q:** A patient presents a pseudoephedrine purchase request that would put them over the legal daily limit; what should you do NEXT? **A:** Decline the sale and follow logbook/ID procedures per policy because CMEA limits must be met before the transaction can proceed.
3922. **Q:** The prescription directions read "take ii tabs po bid" for sertraline 50 mg with quantity 120; what days' supply should you enter? **A:** Enter 30 days because 2 tablets twice daily equals 4 tablets/day and $120 \div 4 = 30$.
3923. **Q:** You're filling levothyroxine 75 mcg and find both 75 mcg and 88 mcg bottles on the shelf with similar packaging; what is the BEST technician action during product selection? **A:** Use barcode/NDC verification and match strength to the label because levothyroxine is high-risk and strength mix-ups can cause harm.
3924. **Q:** A patient's new prescription is for sertraline, and their profile shows linezolid started 2 days ago; what should you do FIRST? **A:** Hold the fill and alert the pharmacist because linezolid with SSRIs can raise serotonin syndrome risk.
3925. **Q:** During order entry for acetaminophen-codeine #3, the prescriber included drug, directions, quantity, and signature but no DEA number; what should you do NEXT? **A:** Stop processing and route to the pharmacist to verify prescriber credentials because controlled substance dispensing requires valid prescriber authorization.
3926. **Q:** An eRx for furosemide 40 mg says "take 1 tab po qod" and quantity is 15; what days' supply should you enter? **A:** Enter 30 days' supply because qod means every other day so 15 tablets lasts 30 days.

3927. **Q:** A prescription for prednisone 5 mg reads "take 2 tabs po bid" with quantity 60; what days' supply is correct for adjudication? **A:** Enter 15 days' supply because 2 tablets twice daily = 4/day and $60 \div 4 = 15$ days.
3928. **Q:** While entering directions, you see "U-100 insulin regular: inject 0.3 mL subcut qid"; what should you do FIRST before calculating days' supply? **A:** Convert 0.3 mL to units (30 units) and route to pharmacist if the dose seems unusually high because U-100 is 100 units/mL and safety requires verification of large insulin doses.
3929. **Q:** A patient is prescribed amlodipine 5 mg "take 1 tab po qd" but the prescriber typed "qd" on the eRx; what is the BEST technician action in order entry? **A:** Enter the directions as "daily" (not "QD") per safe abbreviation practice because "qd" is error-prone and should be clarified/standardized to prevent misreading.
3930. **Q:** You are calculating days' supply for insulin glargine U-100 pens: "inject 22 units nightly," dispense 1 box of 5 pens (3 mL each); what days' supply should you submit? **A:** Submit 68 days because total units = 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units and $1500 \div 22 = 68$ days (rounded down to whole days).
3931. **Q:** While entering a new Rx for simvastatin, the patient profile shows clarithromycin started today; what should you do FIRST? **A:** Stop processing and alert the pharmacist because clarithromycin can dangerously increase simvastatin levels and risk myopathy/rhabdomyolysis.
3932. **Q:** You receive an eRx for metformin ER 500 mg "take 1 tab po bid" but the product selected is metformin IR 500 mg; what is the BEST technician action? **A:** Stop and reselect the correct dosage form (ER vs IR) and send to pharmacist if unclear because ER/IR are not interchangeable and can cause dosing errors.
3933. **Q:** A patient asks to pick up fluoxetine and says they also take "Celexa"; what should you do NEXT? **A:** Verify the exact medication list and notify the pharmacist for therapeutic duplication review because fluoxetine (Prozac) and citalopram (Celexa) are both SSRIs.
3934. **Q:** An order is for hydromorphone 2 mg tablets but you accidentally pulled morphine 15 mg tablets from the shelf; what should you do BEST? **A:** Stop filling and return the incorrect product, then reselect using barcode/NDC verification because this is a high-risk LASA/strength error.
3935. **Q:** A new Rx is written for "Lamictal 25 mg daily" and the patient states they are allergic to lamotrigine; what should you do FIRST? **A:** Hold the prescription and

alert the pharmacist because Lamictal is lamotrigine and an allergy requires pharmacist assessment before dispensing.

3936. **Q:** Directions read "clonazepam 0.5 mg: take 1 tab po tid," quantity 90; what days' supply should you enter? **A:** Enter 30 days because 1 tablet three times daily equals 3 tablets/day and $90 \div 3 = 30$.
3937. **Q:** You're calculating days' supply for insulin lispro U-100: "inject 8 units before meals tid," dispense one 10 mL vial; what days' supply should you submit? **A:** Submit 41 days because 10 mL = 1000 units and $8 \times 3 = 24$ units/day, so $1000 \div 24 = 41.6$ and you enter 41 whole days.
3938. **Q:** A new prescription is for losartan 50 mg daily and the patient profile shows lisinopril 20 mg daily active; what should you do NEXT? **A:** Flag and refer to the pharmacist because ACE inhibitor and ARB duplication increases risk (e.g., hyperkalemia, renal effects) and needs clinical review.
3939. **Q:** During product receipt, a tote contains refrigerated liraglutide (Victoza) pens that arrived warm with a melted ice pack; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager per policy because temperature excursions can make refrigerated biologic products unsafe/ineffective.
3940. **Q:** A patient requests a refill of oxycodone/APAP and says it was "lost," asking for an early refill; what should you do BEST? **A:** Do not process automatically; gather details and route to the pharmacist because early replacement of a CII is high-risk and requires pharmacist and prescriber decision per policy.
3941. **Q:** A hardcopy Rx reads "hydroxyzine 25 mg" but the technician selects hydralazine 25 mg in the system; what should you do BEST before labels print? **A:** Stop processing and correct the product to hydroxyzine, then re-verify NDC/strength because hydroxyzine/hydralazine is a LASA mix-up that can cause serious harm.
3942. **Q:** During drop-off, a patient says their new prescription is "Eliquis" but they thought it was "for cholesterol"; what should you do NEXT? **A:** Refer to the pharmacist for counseling/verification because apixaban (Eliquis) is an anticoagulant and mismatch of indication suggests a potentially wrong medication.
3943. **Q:** An eRx arrives for "metoprolol 25 mg take 1 tab po bid" and the selected product is metoprolol succinate ER; what is the BEST technician action? **A:** Stop

and route to the pharmacist to clarify formulation because succinate ER is typically once daily and swapping tartrate vs succinate changes release and dosing.

3944. **Q:** You are entering a new Rx for sertraline and the patient profile shows linezolid started yesterday; what should you do FIRST? **A:** Pause filling and alert the pharmacist because linezolid with SSRIs risks serotonin syndrome and requires pharmacist intervention.
3945. **Q:** A prescription is written for "Bactrim DS" and the patient profile lists a severe sulfonamide allergy; what should you do NEXT? **A:** Hold the order and notify the pharmacist because sulfamethoxazole/trimethoprim contains a sulfonamide and allergy screening is a key safety step.
3946. **Q:** A new Rx is for "glipizide 10 mg daily," but the patient says they often skip meals due to nausea; what is the BEST technician response? **A:** Refer to the pharmacist because sulfonylureas can cause hypoglycemia and the pharmacist should assess safety and counseling needs.
3947. **Q:** Order entry shows "levothyroxine 0.125 mg daily," but the prescriber note says "125 mcg"; what should you do BEST? **A:** Enter as 125 mcg (0.125 mg) using the system's mcg strength and have it verified, because matching units prevents decimal-point dosing errors.
3948. **Q:** A patient brings in a new prescription for lisinopril and reports a past "swelling of lips" reaction to an ACE inhibitor; what should you do FIRST? **A:** Stop processing and immediately involve the pharmacist because angioedema is a serious contraindication requiring pharmacist-led decision-making.
3949. **Q:** A prescription is for "prednisone 10 mg: take 4 tablets daily for 5 days," dispense #; what quantity should you enter? **A:** Enter quantity 20 tablets because $4 \text{ tablets/day} \times 5 \text{ days} = 20$, ensuring the dispensed amount matches the intended course.
3950. **Q:** While stocking, you find two similar boxes: Humalog (insulin lispro) and Humulin R (regular insulin) placed in the same bin; what should you do BEST? **A:** Separate and relabel/bin them per policy and alert the pharmacist or supervisor because insulin is high-alert and look-alike storage increases selection errors.
3951. **Q:** While entering a new Rx, you notice "Lamictal" was selected but the hardcopy clearly states "Lamisil 250 mg daily"; what should you do FIRST? **A:** Stop

processing and correct the drug selection using barcode/NDC verification, because Lamictal (lamotrigine) and Lamisil (terbinafine) are a high-risk LASA mix-up.

3952. **Q:** A patient asks for "Plavix" but the profile shows clopidogrel and prasugrel both active; what should you do NEXT? **A:** Pause the sale and alert the pharmacist to review for therapeutic duplication of antiplatelets, because taking both can raise bleeding risk.
3953. **Q:** An eRx arrives for "Zyrtec-D" and the patient says they have uncontrolled hypertension; what is the BEST technician action? **A:** Refer to the pharmacist before selling/processing, because Zyrtec-D contains pseudoephedrine which can worsen blood pressure and needs appropriate screening and legal log steps.
3954. **Q:** You're filling "Xarelto 20 mg daily" and the patient also has an active warfarin profile entry from last week; what should you do FIRST? **A:** Stop and notify the pharmacist to clarify anticoagulant therapy, because dual anticoagulation is a serious high-bleed-risk situation.
3955. **Q:** A new prescription is for "Biaxin" and the patient profile shows simvastatin 40 mg daily; what should you do BEST? **A:** Hold the prescription and alert the pharmacist, because clarithromycin can significantly increase simvastatin levels and risk myopathy/rhabdomyolysis.
3956. **Q:** A patient drops off a prescription for "Adderall" and asks if it can be transferred to another pharmacy later; what should you do NEXT? **A:** Tell the patient you must involve the pharmacist and follow controlled-substance transfer rules, because Schedule II prescriptions generally cannot be transferred once filled and require strict handling.
3957. **Q:** A prescriber sends "Keflex 250 mg/5 mL: take 10 mL twice daily for 7 days"; what quantity (mL) should you enter? **A:** Enter 140 mL, because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL total}$.
3958. **Q:** During order entry, you see "Wellbutrin SR 150 mg BID" but the selected product is Wellbutrin XL 300 mg daily; what should you do FIRST? **A:** Stop and route to the pharmacist to confirm formulation and dosing, because SR vs XL are not interchangeable and changing could cause dosing errors.
3959. **Q:** While preparing a new fill, you notice the NDC on the bottle is for glimepiride but the label printed for glyburide; what should you do BEST? **A:** Stop filling, pull the correct stock bottle, and reprint/verify with barcode scanning, because

sulfonylureas have different strengths and wrong-drug selection can cause hypoglycemia.

3960. **Q:** A patient picking up “Crestor” asks what it’s for and mentions new muscle pain after starting it; what should you do NEXT? **A:** Refer the patient to the pharmacist for counseling, because statins like rosuvastatin can cause myopathy and require assessment before continuing.
3961. **Q:** While entering a new eRx, you select celecoxib but the prescriber wrote “Celexa 20 mg daily”; what should you do FIRST? **A:** Stop processing and reselect citalopram (Celexa) then route for pharmacist verification because this is a high-risk LASA name mix-up.
3962. **Q:** A patient picking up metformin asks why the label says “take with meals” and reports new stomach upset; what is the BEST technician response? **A:** Refer to the pharmacist for counseling because GI effects are common with metformin and counseling is required when the patient requests advice.
3963. **Q:** You receive an eRx for sertraline 100 mg daily and the patient profile shows linezolid started yesterday; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist because SSRIs with linezolid can cause a serious interaction requiring clinical review.
3964. **Q:** A prescription reads “levothyroxine 0.1 mg daily”; what strength should you enter in micrograms? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$ to prevent decimal-dose errors.
3965. **Q:** During product selection, you notice the stock bottle is hydroxyzine but the label is for hydralazine; what should you do BEST? **A:** Stop filling and correct the product selection and NDC before it reaches verification because this LASA mix-up can cause significant harm.
3966. **Q:** A new Rx is for clarithromycin and the profile shows colchicine; what should you do FIRST? **A:** Pause and notify the pharmacist because clarithromycin can dangerously increase colchicine levels and needs pharmacist intervention.
3967. **Q:** An eRx arrives for “Toprol-XL 50 mg daily” but the selected product is metoprolol tartrate 50 mg; what should you do NEXT? **A:** Change the selection to metoprolol succinate ER (Toprol-XL) and send for pharmacist check because tartrate vs succinate is a common formulation error.
3968. **Q:** A patient brings in a new prescription for alprazolam and asks for “extra refills for the year”; what should you do BEST? **A:** Explain you must process it as

written and route questions to the pharmacist because alprazolam is a Schedule IV controlled substance with refill limits and prescriber authorization rules.

3969. **Q:** A prescription is for azithromycin suspension 200 mg/5 mL: take 12.5 mL on day 1 then 6.25 mL daily on days 2–5; what total mL should you dispense? **A:** Dispense 37.5 mL because $12.5 + (6.25 \times 4) = 12.5 + 25 = 37.5$ mL to match the full course.
3970. **Q:** While filling a new warfarin prescription, you see the prescriber also sent trimethoprim-sulfamethoxazole today; what should you do FIRST? **A:** Stop and alert the pharmacist because TMP-SMX can significantly increase warfarin effect and requires pharmacist assessment before dispensing.
3971. **Q:** While typing a new eRx, you see “Lamictal 25 mg” but the prescriber selected “Lamisil” in the drug field; what should you do FIRST? **A:** Stop entry and route to the pharmacist to clarify because terbinafine and lamotrigine are LASA with very different uses and wrong selection is high-risk.
3972. **Q:** A patient asks if “Plavix” is the same as “clopidogrel” and whether it’s for blood pressure; what is the BEST technician response? **A:** Confirm Plavix is clopidogrel and refer to the pharmacist for counseling because it’s an antiplatelet for clot prevention, not a BP drug, and requires proper counseling.
3973. **Q:** During data entry you receive “lisinopril/HCTZ 20/25 mg 1 tab daily,” but the patient profile already has lisinopril 20 mg daily active; what should you do NEXT? **A:** Flag the potential therapeutic duplication and alert the pharmacist because combination products can unintentionally duplicate ACE inhibitor therapy.
3974. **Q:** While selecting a sulfonylurea, the bottle on the shelf is “glipizide” but the label is for “glyburide”; what should you do BEST? **A:** Stop filling and select the correct NDC/barcode product because these are different drugs with different dosing and mix-ups can cause hypoglycemia.
3975. **Q:** A new prescription is for “simvastatin 40 mg qHS,” and the profile shows the patient takes amlodipine 10 mg daily; what should you do FIRST? **A:** Hold the order and notify the pharmacist because simvastatin has a significant interaction dose limit with amlodipine, increasing myopathy risk.
3976. **Q:** A patient picking up “Zithromax” says they also take “citalopram” and asks if it’s okay together; what should you do NEXT? **A:** Refer to the pharmacist for

interaction screening because macrolides and some SSRIs can increase QT-prolongation risk requiring clinical review.

3977. **Q:** A prescription is for "levetiracetam 500 mg: take 1 tab BID; dispense #60," and the patient asks what it treats; what is the BEST technician response? **A:** State it's commonly used for seizures and offer pharmacist counseling because indication and safety counseling should be handled by the pharmacist.
3978. **Q:** You are asked to calculate days' supply for insulin lispro U-100 vial 10 mL with directions "inject 12 units before meals TID"; what days' supply should you enter? **A:** Enter 27 days because $10\text{ mL} = 1000\text{ units}$ and $12 \times 3 = 36\text{ units/day}$, so $1000 \div 36 = 27.7$ rounded down to 27 days.
3979. **Q:** A new eRx is for "oxycodone/APAP 5/325 mg 1 tab q6h PRN pain #28," and the prescriber included "refills: 2"; what should you do BEST? **A:** Stop processing and send to the pharmacist because Schedule II prescriptions cannot be refilled and require correction before dispensing.
3980. **Q:** While restocking, you notice a bottle of "nitroglycerin SL tablets" on the shelf with an open seal and no open-date sticker; what should you do FIRST? **A:** Remove it from usable inventory and follow pharmacy policy for discard/return because nitroglycerin is sensitive and an unknown open date risks reduced potency.
3981. **Q:** While entering a new eRx, you see "metformin ER 500 mg 2 tabs BID" for a patient with GI intolerance history noted; what should you do FIRST? **A:** Stop and route to the pharmacist to clarify because metformin ER should not be dosed BID at that strength typically and excessive dosing increases adverse effects.
3982. **Q:** A patient drops off a prescription for "Xarelto 20 mg daily" and your profile shows an active "Eliquis 5 mg BID"; what is the BEST next step for the technician? **A:** Hold processing and alert the pharmacist to possible therapeutic duplication since two DOACs together greatly increase bleeding risk.
3983. **Q:** During product selection you scan "hydroxyzine" but the label says "hydralazine" for the same patient; what should you do NEXT? **A:** Stop filling and reselect the correct drug using barcode/NDC verification because this LASA mix-up can cause serious harm (antihistamine vs antihypertensive).
3984. **Q:** A prescription reads "predniSONE 10 mg: take ii tabs daily x 5 days; dispense #?" and the prescriber didn't enter a quantity; what should you do FIRST? **A:** Calculate the needed quantity as 10 tablets ($2\text{ tablets/day} \times 5\text{ days}$) and send

to the pharmacist for quantity confirmation because missing quantity requires pharmacist review before dispensing.

3985. **Q:** You are asked to enter days' supply for sertraline 50 mg "take 1 tab daily" with quantity 45; what days' supply should you enter? **A:** Enter 45 days because one tablet per day from a quantity of 45 equals a 45-day supply.
3986. **Q:** A patient asks at pickup if "Crestor" is the same as "rosuvastatin" and what it's used for; what is the BEST technician response? **A:** Confirm Crestor is rosuvastatin and it's used to lower cholesterol, then offer pharmacist counseling for questions about effects or side effects to stay within scope and ensure safety.
3987. **Q:** While typing, you receive "levothyroxine 100 mcg daily" but the prescriber selected "liothyronine" in the drug field; what should you do FIRST? **A:** Stop entry and refer to the pharmacist to verify the intended thyroid product because levothyroxine vs liothyronine are different drugs and a selection error is high risk.
3988. **Q:** A new prescription is for "clarithromycin 500 mg BID" and the patient profile shows simvastatin 40 mg nightly; what should you do NEXT? **A:** Hold the order and notify the pharmacist because clarithromycin can raise simvastatin levels and increase myopathy/rhabdomyolysis risk.
3989. **Q:** While stocking, you find an insulin glargine pen box with a torn outer seal and no documentation of how long it's been open; what should you do FIRST? **A:** Remove it from active stock and quarantine per policy for pharmacist review because damaged/compromised packaging makes product integrity and safety uncertain.
3990. **Q:** A prescription is for morphine ER 30 mg "take 1 tab q12h" but the patient tells you they need it "for breakthrough pain"; what should you do BEST? **A:** Refer the patient to the pharmacist immediately because ER opioids are not for breakthrough use and misuse increases overdose risk.
3991. **Q:** While entering a new prescription for "glipizide 10 mg take 1 tab BID," you see the prescriber accidentally selected "glyBURide" in the drug field; what should you do FIRST? **A:** Stop data entry and route to the pharmacist to clarify the intended sulfonylurea because glyburide vs glipizide is a high-risk mix-up with hypoglycemia risk.
3992. **Q:** A patient's new eRx is for "clopidogrel 75 mg daily," but their profile shows an active prescription for prasugrel; what is the BEST next step? **A:** Pause

processing and alert the pharmacist to possible therapeutic duplication of antiplatelets because combining agents can raise bleeding risk.

3993. **Q:** You receive an order for "TMP/SMX DS 1 tab BID" and the allergy section lists "sulfa—anaphylaxis"; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist immediately because sulfamethoxazole is a sulfonamide and could trigger a severe reaction.
3994. **Q:** At drop-off, a caregiver says the patient takes "Plavix," but the refill request is for "Protonix"; what should you do BEST before processing? **A:** Verify the exact medication names using the profile/previous labels and clarify with the pharmacist as needed because look-alike/sound-alike brand names can cause wrong-drug errors.
3995. **Q:** A prescription is written for "metoprolol 25 mg take 1 tab daily," but it doesn't specify tartrate vs succinate; what should you do FIRST? **A:** Stop and send to the pharmacist for clarification because the two formulations are not interchangeable and dosing differs.
3996. **Q:** A patient asks why their new bottle says "atorvastatin" when they expected "Lipitor"; what is the BEST technician response? **A:** Explain that atorvastatin is the generic for Lipitor and it's used to lower cholesterol, and offer the pharmacist for clinical questions because technicians can provide basic brand/generic identification.
3997. **Q:** You are calculating days' supply for gabapentin 300 mg "take 1 cap TID" with quantity 270; what days' supply should you enter? **A:** Enter 90 days because $270 \text{ capsules} \div 3 \text{ per day} = 90 \text{ days}$ for accurate billing and refill timing.
3998. **Q:** A new prescription is for "azithromycin 250 mg: take 2 tablets on day 1 then 1 tablet daily days 2–5"; what quantity should be dispensed? **A:** Dispense 6 tablets because the total dose is $2 + 1 + 1 + 1 + 1 = 6$ tablets for the standard 5-day regimen.
3999. **Q:** While selecting product, the label is for "lamoTRIGine 100 mg" but you accidentally grabbed "terbinafine 250 mg"; what should you do NEXT? **A:** Stop filling and reselect the correct medication using barcode/NDC verification because lamotrigine vs terbinafine is a LASA error with significant patient harm potential.
4000. **Q:** During intake you receive an eRx for "isotretinoin 40 mg daily," and the patient says they are not enrolled in any special program; what should you do

FIRST? **A:** Stop processing and alert the pharmacist to verify iPLEDGE/REMS requirements because isotretinoin has mandatory safety controls before dispensing.

4001. **Q:** A patient is prescribed metformin 500 mg "ii po bid" with quantity 120; what days' supply should you submit? **A:** Submit 30 days' supply because ii bid = 4 tablets/day and $120 \div 4 = 30$.
4002. **Q:** An eRx says "predniSONE 10 mg: take 4 tabs daily x 3 days, then 3 daily x 3 days, then 2 daily x 3 days, then 1 daily x 3 days; dispense?" **A:** Enter quantity 30 tablets because $4+3+2+1$ tabs per day for 3 days each totals $(4+3+2+1) \times 3 = 30$, avoiding underfill.
4003. **Q:** A prescription is written for azithromycin suspension 200 mg/5 mL: "Take 12.5 mL PO daily x 3 days"; what total mL should be dispensed? **A:** Dispense 37.5 mL because $12.5 \text{ mL/day} \times 3 \text{ days} = 37.5 \text{ mL}$, ensuring the patient can complete therapy.
4004. **Q:** A provider writes "morphine sulfate ER 30 mg, take 1 tab q8h prn pain"; what is the technician's BEST next step before processing? **A:** Stop and notify the pharmacist because ER products are generally not dosed PRN and the directions are potentially unsafe/unclear for a high-alert opioid.
4005. **Q:** A prescription for warfarin says "take as directed" with quantity 30; what should you do FIRST during order entry? **A:** Route to the pharmacist to clarify and document a specific daily dose (or dosing schedule) because "as directed" alone is not adequate for days' supply and safe anticoagulant use.
4006. **Q:** The sig reads "insulin lispro U-100: inject 6 units SQ TID with meals; dispense 1 vial (10 mL)"; what days' supply should you calculate for billing? **A:** Bill 55 days because $10 \text{ mL} = 1000 \text{ units}$ and $6 \times 3 = 18 \text{ units/day}$, so $1000 \div 18 = 55.5$ and you submit 55 to avoid overstating coverage.
4007. **Q:** A prescription says "atorvastatin 20 mg i po qhs" but the prescriber also sends "Crestor 10 mg i po qhs" for the same patient; what should you do NEXT? **A:** Pause processing and alert the pharmacist because atorvastatin and rosuvastatin are therapeutic duplicates (both statins) requiring prescriber clarification before dispensing.
4008. **Q:** During product selection you notice two strengths of clonidine on the shelf (0.1 mg tabs and 0.1 mg/24 hr patches) and the order is for "clonidine 0.1 mg/24 hr patch weekly"; what is the BEST technician action? **A:** Verify dosage

form and scan the correct NDC for the patch because tabs vs patches is a common wrong-form error with significant dosing differences.

4009. **Q:** A prescription reads "hydromorphone 2 mg: take 1–2 tabs po q4–6h prn pain; dispense 60"; the patient says they take the maximum dose; what days' supply should you enter? **A:** Enter 5 days because maximum use is 2 tabs every 4 hours = 12 tabs/day and $60 \div 12 = 5$, which is the safest way to calculate PRN days' supply when max usage is stated.
4010. **Q:** A new eRx is for "Zestril 10 mg i po daily," and the patient profile already shows lisinopril 10 mg active; what should you do NEXT? **A:** Verify Zestril is lisinopril and route to the pharmacist to assess duplicate ACE-inhibitor therapy because brand/generic duplicates can lead to double dosing.
4011. **Q:** A prescriber sends "CeleXA 20 mg daily" for a patient whose allergies list "escitalopram"; what is the technician's FIRST action? **A:** Stop processing and notify the pharmacist because citalopram and escitalopram are closely related SSRIs and the allergy needs clinical clarification before dispensing.
4012. **Q:** You receive an order for "Lantus SoloStar inject 18 units SQ nightly; dispense 1 box (5 pens, 3 mL each, U-100)"; what days' supply should you bill? **A:** Bill 83 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units total}$ and $1500 \div 18 \text{ units/day} = 83.3$, typically submitted as 83 days per payer rules.
4013. **Q:** A patient drops off "amLODIPine 5 mg and simvastatin 40 mg daily" and asks if it's safe together; what should you do FIRST? **A:** Refer to the pharmacist immediately because amlodipine can increase simvastatin exposure and the combination has a maximum recommended simvastatin dose limit.
4014. **Q:** While selecting product for "hydroxyzine 25 mg capsules," you see hydralazine nearby in similar packaging; what is the BEST technician step to prevent a LASA error? **A:** Match the NDC/barcode to the label and separate/confirm the correct drug because hydroxyzine vs hydralazine is a common LASA mix-up with serious consequences.
4015. **Q:** A prescription says "Augmentin 875/125 mg 1 tab po bid x 10 days #20," but the patient profile shows a severe penicillin allergy; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist because amoxicillin/clavulanate is a penicillin-class antibiotic and requires pharmacist intervention for allergy safety.

4016. **Q:** An eRx comes in for "Eliquis 5 mg i po bid," and the patient says they are still taking warfarin from last month; what should you do BEST? **A:** Notify the pharmacist before filling because apixaban and warfarin together can cause dangerous additive anticoagulation and may represent an unintended overlap.
4017. **Q:** The sig reads "Keppra 500 mg: take ii po bid; dispense 240"; what days' supply should you enter for billing? **A:** Enter 60 days because "ii po bid" = 4 tablets/day and $240 \div 4 = 60$.
4018. **Q:** You are entering "Macrobid 100 mg 1 cap bid #14," and the system suggests "Macrodantin 100 mg"; what is the technician's NEXT step? **A:** Stop and verify the exact product with the pharmacist because Macrobid and Macrodantin are different nitrofurantoin formulations with different dosing characteristics.
4019. **Q:** During intake, a caregiver asks which common OTC pain reliever should be avoided with the child's new "Ritalin (methylphenidate)" prescription; what should you do FIRST? **A:** Refer the question to the pharmacist because OTC selection counseling is pharmacist-led and stimulant therapy may require screening for interactions and contraindications.
4020. **Q:** A hardcopy prescription for alprazolam 0.5 mg (C-IV) has the patient name, drug, directions, and quantity but no prescriber signature; what should you do FIRST? **A:** Stop processing and notify the pharmacist to obtain a valid signed order, because dispensing a controlled substance requires a valid prescription.
4021. **Q:** While counting warfarin 5 mg, you accidentally mix a few tablets into the warfarin 2.5 mg stock bottle; what is the BEST technician action? **A:** Remove the bottle from use and alert the pharmacist for proper resolution, because cross-contamination and strength mix-ups are high-risk patient safety errors.
4022. **Q:** A patient asks to transfer their hydrocodone/acetaminophen prescription to your pharmacy because they're traveling; what should you do NEXT? **A:** Route the request to the pharmacist, because controlled-substance transfers have strict federal limits and require pharmacist handling.
4023. **Q:** You receive a tote with refrigerated insulin aspart pens that arrived warm with a temperature indicator showing an excursion; what is the FIRST step? **A:** Segregate the shipment and notify the pharmacist/manager per policy, because temperature excursions can make products unsafe and must not be stocked.
4024. **Q:** An eRx for "MS Contin 30 mg q12h" is entered, but the patient profile shows an active order for "morphine sulfate IR 30 mg q4h prn"; what should you do

BEST? **A:** Pause filling and alert the pharmacist to review for duplication and safety, because IR vs ER opioids are high-alert and dosing errors can be fatal.

4025. **Q:** A prescription comes in for isotretinoin (Accutane) and the patient is waiting at drop-off; what should you do FIRST? **A:** Notify the pharmacist and follow iPLEDGE workflow before processing, because isotretinoin is a REMS drug with required authorization steps.

4026. **Q:** A patient wants to buy two boxes of pseudoephedrine and pays cash; what is the technician's NEXT step under CMEA? **A:** Check ID and complete the required log and quantity limits before the sale, because pseudoephedrine sales must be tracked and restricted federally.

4027. **Q:** During data entry you see "U-500 insulin regular: inject 50 units daily" but the prescriber wrote no concentration or device; what should you do FIRST? **A:** Stop entry and get the pharmacist to clarify with the prescriber, because U-500 dosing is high-alert and concentration/device must be verified to prevent overdose.

4028. **Q:** You receive a manufacturer recall notice for your lot of losartan tablets and find matching bottles on the shelf; what should you do BEST? **A:** Pull the affected NDC/lot from inventory and segregate it for return per recall instructions, because recalled products must be removed to prevent dispensing.

4029. **Q:** A new prescription says "digoxin 0.25 mg take qd" and the patient tells you they are "very sick with vomiting and vision changes"; what should you do FIRST? **A:** Immediately refer the patient to the pharmacist, because these may be signs of digoxin toxicity requiring urgent pharmacist assessment and escalation.

4030. **Q:** A prescription is written for "Glucophage 500 mg take 1 tab bid," but the technician selects Glucovance on the shelf; what should you do FIRST? **A:** Stop the fill and reselect the correct product by verifying the generic/brand and NDC (Glucophage = metformin), because Glucovance adds glyburide and could cause harm.

4031. **Q:** A patient drops off a new prescription for "Zestril 10 mg daily," and the profile shows an active "lisinopril 10 mg daily" from another prescriber; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (Zestril is lisinopril) so the pharmacist can confirm intent and prevent double therapy.

4032. **Q:** During order entry you see "levothyroxine 0.1 mg qd" and the patient says they take "Synthroid 100 mcg"; what should you do NEXT? **A:** Enter it as levothyroxine 100 mcg and confirm with the pharmacist if needed, because 0.1 mg equals 100 mcg and correct unit conversion prevents dosing errors.
4033. **Q:** A prescription says "Humalog 100 units/mL, inject 12 units before meals, dispense 1 vial (10 mL)"; what days' supply should you enter? **A:** Enter 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \text{ units} \times 3 \text{ meals/day} = 36 \text{ units/day}$, so $1000 \div 36 = 27$ (rounded down).
4034. **Q:** A nurse calls in a refill request for "Keflex" for a patient whose profile lists anaphylaxis to cephalexin; what should you do FIRST? **A:** Stop and immediately route to the pharmacist for allergy assessment because Keflex is cephalexin and dispensing could trigger a severe reaction.
4035. **Q:** You receive an eRx for "Lasix 40 mg qd" but accidentally begin selecting "Losec" due to similar spelling; what is the BEST next step? **A:** Use Tall Man/LASA safeguards and verify drug name, indication, and NDC before filling, because Lasix (furosemide) and Losec (omeprazole) are a high-risk mix-up.
4036. **Q:** A patient asks if "Plavix" is the same as "clopidogrel" when picking up; what should you do NEXT? **A:** Confirm Plavix is clopidogrel and offer pharmacist counseling, because brand/generic clarification supports adherence and the pharmacist should address therapy questions.
4037. **Q:** A prescription directions read "take i tab po tid"; what should you enter for the sig in plain language? **A:** Enter "take 1 tablet by mouth three times daily," because i = 1, po = by mouth, and tid = three times daily to ensure clear labeling.
4038. **Q:** A patient is picking up "metronidazole" and asks if they can drink alcohol tonight; what is the BEST technician response? **A:** Refer to the pharmacist for counseling because metronidazole has a significant alcohol interaction that requires pharmacist-driven patient education.
4039. **Q:** A prescription is written for "Adderall XR 20 mg #30" and the patient asks for an early refill because they "lost the bottle"; what should you do FIRST? **A:** Gather details and route to the pharmacist without processing, because it's a Schedule II stimulant and lost/early refill requests require pharmacist review and controlled-substance procedures.

4040. **Q:** While typing a new Rx for "Lamictal 100 mg," you notice the patient profile lists an allergy to lamotrigine; what should you do FIRST? **A:** Stop processing and notify the pharmacist because Lamictal is lamotrigine and an allergy match requires pharmacist review before dispensing.
4041. **Q:** An eRx arrives for "MS Contin 15 mg q12h," but the prescriber intended immediate-release morphine per the note; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist to clarify IR vs ER because morphine formulations are not interchangeable and are high-risk for dosing errors.
4042. **Q:** A prescription is written for "metoprolol 50 mg daily" with no salt specified, and both tartrate and succinate are stocked; what is the BEST technician action? **A:** Enter the order as written and ask the pharmacist to verify/clarify formulation because tartrate vs succinate differ in release and dosing and a tech cannot assume.
4043. **Q:** A patient brings in "omeprazole OTC" and asks if it's the same as "Prilosec"; what should you do BEST? **A:** Confirm that Prilosec is the brand name for omeprazole and offer to have the pharmacist counsel if they have therapy questions, ensuring correct brand/generic recognition.
4044. **Q:** You see "U-500 insulin regular" on an Rx, but the patient says they use "Humulin R U-100"; what should you do FIRST? **A:** Stop and alert the pharmacist because U-500 vs U-100 is a high-alert concentration difference that can cause severe dosing errors.
4045. **Q:** The directions on a prescription read "instill ii gtt OU qhs," and you are entering the sig; what should you enter in plain language? **A:** Enter "Instill 2 drops in both eyes at bedtime" because ii=2, gtt=drops, OU=both eyes, and qhs=at bedtime.
4046. **Q:** A patient's new prescription is for "sertraline 50 mg daily," and their active meds include "paroxetine 20 mg daily"; what should you do NEXT? **A:** Flag it for the pharmacist to review for therapeutic duplication/SSRI overlap because switching antidepressants may require clinical direction and monitoring.
4047. **Q:** You are calculating days' supply for an albuterol HFA labeled 200 actuations with sig "inhale 2 puffs q6h scheduled"; what days' supply should you enter? **A:** Enter 25 days because 2 puffs every 6 hours equals 8 puffs/day and $200 \div 8 = 25$ days.

4048. **Q:** A prescription for “nitrofurantoin” is being processed and the patient is pregnant and asks if it’s safe; what is the BEST technician response? **A:** Refer the question to the pharmacist because pregnancy safety counseling requires pharmacist judgment even if the medication is commonly used.
4049. **Q:** While putting away stock, you find a bottle of hydromorphone tablets without a readable label and you can’t confirm the NDC/lot; what should you do FIRST? **A:** Remove it from active inventory and follow suspect/unknown product procedure (segregate and notify the pharmacist/manager) because unidentified meds must not be dispensed.
4050. **Q:** During order entry you see “hydroxyzine 25 mg take 1 tablet daily” but the prescriber note says “for anxiety PRN,” what should you do NEXT? **A:** Stop data entry and ask the pharmacist to clarify with the prescriber because the directions conflict and a technician cannot assume scheduled vs PRN use.
4051. **Q:** A patient drops off a paper prescription for “oxycodone/APAP 5/325 #20” that has no prescriber signature, what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because a missing signature is a validity issue and controlled-substance dispensing requires pharmacist intervention.
4052. **Q:** While filling, you select “vinCRISTine” but the label printed for “vinBLASTine” for a lymphoma regimen, what is the BEST technician action? **A:** Stop the fill and reselect using barcode/NDC verification and alert the pharmacist if it reached verification because vinca alkaloids are high-risk LASA medications.
4053. **Q:** A new eRx arrives for “clopidogrel 75 mg daily” and the profile shows an active “prasugrel 10 mg daily,” what should you do NEXT? **A:** Pause processing and route to the pharmacist to assess therapeutic duplication of antiplatelets because it increases bleeding risk and is not a technician decision.
4054. **Q:** You receive a wholesaler notice that a specific lot of metformin ER has been recalled, what should you do FIRST? **A:** Pull the recalled lot from inventory and quarantine it per policy because recalled product must be removed from dispensing stock to prevent patient harm.
4055. **Q:** An insulin glargine U-100 prescription is written “inject 18 units nightly” and you are dispensing one 10 mL vial (100 units/mL); what days’ supply should you enter? **A:** Enter 55 days’ supply because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $1000 \div 18 = 55.5$, which is typically billed as 55 days.

4056. **Q:** A patient asks at pickup whether their new "lisinopril" can be taken with "ibuprofen 800 mg" they use frequently for pain, what should you do BEST? **A:** Refer to the pharmacist for counseling because NSAIDs can reduce ACE-inhibitor effect and affect kidneys, requiring pharmacist assessment.
4057. **Q:** You are typing directions that read "take ii tabs po bid x 7 days" for doxycycline 100 mg, what should you enter in plain language? **A:** Enter "Take 2 tablets by mouth twice daily for 7 days" because technicians must translate abbreviations accurately to prevent dosing errors.
4058. **Q:** A patient requests pseudoephedrine and wants to buy the maximum allowed, what should you do FIRST at the register? **A:** Verify photo ID and complete the required log/electronic tracking before sale because federal CMEA rules require identification and purchase tracking.
4059. **Q:** While receiving a tote, you find a refrigerated shipment of liraglutide pens that arrived warm with a melted gel pack, what should you do FIRST? **A:** Segregate and quarantine the shipment and notify the pharmacist/manager because temperature excursions create suspect product that may be unsafe to dispense.
4060. **Q:** You are entering atorvastatin 40 mg with sig "i tab po qhs" and quantity 90; what days' supply should you enter? **A:** Enter 90 days because qhs means 1 tablet nightly, so $90 \text{ tablets} \div 1 \text{ per day} = 90 \text{ days}$ for accurate billing and refill timing.
4061. **Q:** A prescription reads "metformin ER 500 mg: take 2 tabs po bid" with quantity 120; what is the correct days' supply? **A:** Enter 30 days because 2 tablets twice daily = 4 tablets/day, and $120 \div 4 = 30 \text{ days}$ to prevent days' supply errors.
4062. **Q:** You receive an eRx for levothyroxine written "take 1 tab po qd" but the strength field says 0.125 mg and the note says "125 mcg"; what should you do NEXT? **A:** Convert and enter consistently as 125 mcg (0.125 mg) and flag for pharmacist verification because unit mismatches are a common entry error even when equivalent.
4063. **Q:** The sig on a new prescription for prednisone 10 mg says "ii tabs po bid"; what should you type in plain language directions? **A:** Type "Take 2 tablets by mouth twice daily" because ii = 2 and bid = twice daily, reducing misinterpretation at dispensing.

4064. **Q:** A budesonide/formoterol 160/4.5 inhaler (120 actuations) is prescribed "inhale 2 puffs bid"; what days' supply should be entered? **A:** Enter 30 days because 2 puffs twice daily = 4 puffs/day and $120 \div 4 = 30$ days for correct claim submission.
4065. **Q:** A provider orders amlodipine 5 mg tablets but the patient profile shows an active amlodipine 10 mg daily; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess possible dose change/duplication because technicians should not assume which strength to continue.
4066. **Q:** While typing an eRx you see "Humalog U-100 inject 6 units before meals tid" and you are dispensing one 10 mL vial (100 units/mL); what days' supply should you enter? **A:** Enter 55 days because the vial contains 1000 units ($10 \text{ mL} \times 100 \text{ units/mL}$) and the daily dose is 18 units (6×3), so $1000 \div 18 = 55$ days (rounded down).
4067. **Q:** A prescription for cephalexin suspension is "250 mg/5 mL: take 10 mL po qid x 10 days"; what quantity in mL should you dispense? **A:** Dispense 400 mL because $10 \text{ mL} \times 4 \text{ doses/day} \times 10 \text{ days} = 400 \text{ mL}$ to ensure the patient has the full course.
4068. **Q:** During data entry, the prescriber wrote "morphine sulfate ER 15 mg take 1 tab q12h prn pain" but the drug is extended-release; what should you do NEXT? **A:** Stop and refer to the pharmacist because ER opioids should not be "PRN" without clarification, and technicians must route ambiguous or unsafe directions for clinical review.
4069. **Q:** You are filling and the shelf has look-alike bottles of hydrALAZINE 25 mg and hydrOXYzine 25 mg; what is the BEST technician step before labeling? **A:** Verify the NDC/barcode against the label and prescription because LASA name confusion is a common source of dispensing errors that technicians can prevent with product verification.
4070. **Q:** A new eRx is for "Lamictal 25 mg" but the product selected in the system is "Lamisil 250 mg"; what should you do FIRST? **A:** Stop processing and re-select the correct drug using full name and NDC/barcode verification because Lamictal vs Lamisil is a LASA error with major patient harm risk.
4071. **Q:** A patient picking up lisinopril says they are pregnant; what is the BEST technician action NEXT? **A:** Pause the pickup and immediately notify the pharmacist because ACE inhibitors like lisinopril are contraindicated in pregnancy and require pharmacist intervention.

4072. **Q:** You are entering a prescription for sertraline and the prescriber wrote "Zoloft 50 mg"; what is the correct generic name to select and why? **A:** Select sertraline because Zoloft is the brand name and correct brand/generic matching prevents wrong-drug selection and billing errors.
4073. **Q:** A prescription is for amoxicillin/clavulanate but the patient profile shows a severe penicillin allergy; what should you do FIRST? **A:** Stop the fill and alert the pharmacist because amoxicillin/clavulanate is a penicillin-class antibiotic and allergy screening is a safety-critical step.
4074. **Q:** You're asked to compute days' supply for gabapentin 300 mg "take 1 capsule by mouth three times daily" with quantity 270; what days' supply should you enter? **A:** Enter 90 days because $270 \text{ capsules} \div 3 \text{ capsules/day} = 90 \text{ days}$ for accurate insurance processing and refill timing.
4075. **Q:** A patient's new prescription is for glipizide and they ask what it's for; what is the BEST technician response? **A:** Tell them it's used to help lower blood sugar in type 2 diabetes and offer pharmacist counseling because technicians can provide basic indication but pharmacists counsel on therapy and safety.
4076. **Q:** While typing directions for furosemide, the sig says "i tab po bid" and the prescriber note says "for swelling"; what should the patient-friendly directions read? **A:** Type "Take 1 tablet by mouth twice daily for swelling" because converting sig codes clearly reduces administration errors.
4077. **Q:** A prescriber sends an eRx for "clonazepam 1 mg: take 1 tab tid, refills 5"; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist because clonazepam is a controlled substance and refills/validity must be reviewed before dispensing.
4078. **Q:** A technician accidentally prints a label for rivaroxaban when the prescription is for warfarin; what should you do FIRST? **A:** Stop the process and correct the drug selection before filling because anticoagulants are high-alert and wrong-drug errors can cause severe bleeding.
4079. **Q:** A prescription is for azithromycin 250 mg Z-Pak and the patient profile shows an active prescription for clarithromycin; what is the BEST next step? **A:** Flag and notify the pharmacist to assess duplication/interaction risk because both are macrolide antibiotics and therapy overlap may be inappropriate.
4080. **Q:** While entering an eRx for heparin 10 units/mL flush, you notice the product selected is heparin 10,000 units/mL injection; what should you do FIRST? **A:**

Stop processing and alert the pharmacist, because heparin is a high-alert drug and a strength mix-up can cause serious harm.

4081. **Q:** A written prescription for oxycodone/APAP (Percocet) is missing the prescriber's signature; what is the BEST technician next step? **A:** Hold the prescription and refer it to the pharmacist, because controlled-substance prescriptions must meet validity requirements before dispensing.
4082. **Q:** A patient requests an early refill of Adderall (amphetamine/dextroamphetamine) stating it was "lost," and the claim rejects refill-too-soon; what should you do NEXT? **A:** Notify the pharmacist and follow store policy for lost CII requests, because technicians should not override controlled-substance refill timing or make exceptions.
4083. **Q:** During pickup, a patient asks, "Can you tell me what this metformin is for?"; what is the BEST technician response? **A:** Provide a nonclinical indication like "it helps control blood sugar for diabetes" and offer pharmacist counseling, because technicians can give general info but pharmacists counsel clinically.
4084. **Q:** You open a tote and find insulin glargine pens warm with a temperature-monitor sticker indicating an excursion; what should you do FIRST? **A:** Quarantine the shipment and alert the pharmacist/manager, because temperature excursions make the product suspect and it should not be stocked until verified.
4085. **Q:** While entering a prescription for hydroxyzine, you notice the prescriber wrote "HSZ 25 mg" and the patient profile also lists hydralazine; what should you do FIRST? **A:** Stop data entry and ask the pharmacist to clarify the intended drug because hydroxyzine and hydralazine are LASA and an abbreviation could cause a harmful selection error.
4086. **Q:** A new eRx is for lisinopril 20 mg daily, but the profile shows an active prescription for losartan 50 mg daily; what is the BEST next step for the technician? **A:** Flag for pharmacist review for possible therapeutic duplication/ACEI-ARB therapy concern before filling because the prescriber's intent must be verified.
4087. **Q:** A patient picking up sertraline says they also take linezolid from an urgent care visit; what should you do NEXT? **A:** Immediately alert the pharmacist to screen for a serious interaction risk because SSRIs plus linezolid can lead to serotonin syndrome and needs pharmacist intervention.

4088. **Q:** The label directions read "take 0.5 mg daily," but the prescription is for levothyroxine 50 mcg; what should you do FIRST? **A:** Stop and correct the entry to 50 mcg (not 0.5 mg) and send to pharmacist verification because mcg-mg conversion errors with levothyroxine are high-risk.
4089. **Q:** You are filling "predniSONE" but accidentally selected "prednisoLONE" in the system; what is the BEST action before proceeding? **A:** Re-select the correct drug and strength and verify the NDC/barcode matches because these are commonly confused steroids with different typical products and dosing.
4090. **Q:** A prescription reads "metoprolol 25 mg ER daily," but the product chosen is metoprolol tartrate immediate-release; what should you do NEXT? **A:** Stop and involve the pharmacist to confirm the intended formulation because IR vs ER substitution changes dosing schedule and cannot be changed independently by a technician.
4091. **Q:** Days' supply needed: insulin lispro U-100 vial 10 mL; directions "inject 18 units before meals three times daily"; what days' supply should you enter? **A:** Enter $18 \text{ units} \times 3 = 54 \text{ units/day}$; $10 \text{ mL} = 1000 \text{ units}$, so $1000 \div 54 = 18 \text{ days}$ (rounded down) because days' supply must not exceed the vial's available units.
4092. **Q:** A patient asks to buy pseudoephedrine and presents an out-of-state ID; what should you do FIRST at the register? **A:** Follow store policy and the NPLeX/ID verification process before sale because CMEA requirements involve verified ID and sales log limits.
4093. **Q:** You receive a stock bottle of apixaban that has a broken seal and tablets loose in the shipping carton; what should you do FIRST? **A:** Separate and quarantine the product and notify the pharmacist/vendor because damaged or suspect product must not be dispensed due to contamination and integrity concerns.
4094. **Q:** During data entry, a prescription for tramadol has refills remaining, but the last fill date was 7 months ago; what should you do NEXT? **A:** Route to the pharmacist to confirm it is still eligible to refill because tramadol is Schedule IV and federal rules limit refills to 6 months from the issue date.
4095. **Q:** You are entering an eRx for azithromycin 200 mg/5 mL oral suspension: "Take 12 mL day 1, then 6 mL daily days 2-5," and the prescriber wrote "Dispense 30 mL"; what should you do NEXT? **A:** Calculate total volume needed ($12 + 6 \times 4 = 36 \text{ mL}$) and notify the pharmacist to clarify/change quantity because 30 mL is insufficient to complete therapy.

4096. **Q:** A prescription says "diltiazem 120 mg SR qd" and the patient profile shows both Cardizem CD and immediate-release diltiazem products in the system; what is the BEST next step in order entry? **A:** Stop and route to the pharmacist to confirm the intended formulation (ER vs IR) because "SR" is ambiguous and selecting the wrong release type can cause dosing errors.
4097. **Q:** You are billing insulin regular U-500 KwikPen and the sig is "Inject 60 units subcutaneously twice daily"; what days' supply should you enter for a box containing 3 pens of 1,500 units each (total 4,500 units)? **A:** Enter 37 days' supply because $60 \text{ units BID} = 120 \text{ units/day}$ and $4,500 \div 120 = 37.5$, which is typically billed as 37 whole days to avoid overstating coverage.
4098. **Q:** While typing directions you see "MSO4 2 mg IV q2h prn pain" on a hospital discharge order; what should you do FIRST? **A:** Hold the order and alert the pharmacist because MSO4 is an error-prone abbreviation that can be misread, and morphine must be clearly specified for patient safety.
4099. **Q:** An order is for amiodarone 200 mg daily, but the technician selects amLODIPine 10 mg due to similar names in the search list; what is the BEST action before printing labels? **A:** Stop and reselect using the correct drug and strength, verifying NDC/barcode, because this is a LASA selection error that could lead to a serious wrong-drug dispense.
4100. **Q:** A prescription is written for "buPROPion SR 150 mg BID," but the patient profile shows active bupropion XL once daily; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication and formulation mix-up because SR vs XL are not interchangeable and could cause dosing errors.
4101. **Q:** A prescription reads "levothyroxine 0.075 mg po qd" and the prescriber later says "75 mcg daily"; what strength should you enter and why? **A:** Enter levothyroxine 75 mcg because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$, preventing a decimal-point strength entry error.
4102. **Q:** A patient is prescribed amoxicillin/clavulanate 875/125 mg 1 tab po bid #20; what days' supply should you submit to insurance? **A:** Submit 10 days because $20 \text{ tablets} \div 2 \text{ tablets/day} = 10 \text{ days}$, and days' supply must match the written quantity and directions.
4103. **Q:** A prescription says "metformin ER 500 mg i po bid" and the patient insists it is "one twice daily"; what should you do FIRST in order entry? **A:** Interpret "i" as 1 tablet and enter 1 tablet twice daily because "i" is the Roman numeral for one, avoiding misreading as "l" or "10."

4104. **Q:** You receive "Lantus SoloStar 100 units/mL inject 24 units SQ qhs, dispense 15 mL"; what days' supply should you calculate for claim submission? **A:** Submit 62 days because $15 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 24 = 62.5$, which is typically billed as 62 to avoid overbilling.
4105. **Q:** A prescription is written "predniSONE 20 mg: take ii tabs po qd x 5 days, then i tab qd x 5 days; dispense?" **A:** Dispense 15 tablets because $(2 \text{ tabs/day} \times 5) + (1 \text{ tab/day} \times 5) = 10 + 5 = 15$, ensuring quantity matches the taper directions.
4106. **Q:** An order says "amLODIPine 5 mg qd" but the product selected is "amiodarone 200 mg"; what is the BEST next step before filling? **A:** Stop and re-select the correct drug using barcode/NDC verification because amlodipine vs amiodarone is a high-risk LASA mix-up.
4107. **Q:** A patient brings a new prescription: "hydrocodone/acetaminophen 5/325 1 tab q6h prn pain #120"; what should you do FIRST before processing refills or transfers? **A:** Route to the pharmacist and follow controlled-substance workflow because it's a Schedule II product and validity/handling requirements require pharmacist oversight.
4108. **Q:** A sig reads "dorzolamide/timolol ophth soln: instill 1 gtt OU bid, 10 mL"; approximately how many days' supply should you enter using 20 gtt/mL? **A:** Enter 50 days because $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ gtt}$ and $\text{OU bid} = 4 \text{ gtt/day}$, so $200 \div 4 = 50$.
4109. **Q:** While entering "warfarin 5 mg take 1 tab daily," you see an active warfarin 2 mg daily on the profile; what should you do NEXT? **A:** Pause and alert the pharmacist to possible therapeutic duplication/dose change because warfarin is high-alert and duplicate strengths increase error risk.
4110. **Q:** A prescription says "azithromycin 200 mg/5 mL: give 12 mg/kg day 1 then 6 mg/kg days 2-5; child weighs 22 lb"; what should you do FIRST? **A:** Convert weight to kg for dosing because $22 \text{ lb} \div 2.2 = 10 \text{ kg}$, and mg/kg calculations must be based on kilograms to avoid dosing errors.
4111. **Q:** While typing a new prescription for Zoloft 50 mg daily, the profile shows sertraline 50 mg daily already active; what should you do NEXT? **A:** Pause entry and alert the pharmacist to possible therapeutic duplication because Zoloft is sertraline and duplicating SSRIs can cause harm.

4112. **Q:** A prescription is written for "Celebrex 200 mg daily," but the patient reports a serious sulfa allergy; what is the BEST technician action? **A:** Stop processing and route to the pharmacist for allergy assessment because celecoxib has a sulfonamide structure and requires pharmacist judgment.
4113. **Q:** You receive an eRx for "Lasix 20 mg bid," but the prescriber comment says "for blood pressure"; what should you do FIRST? **A:** Enter as written and notify the pharmacist to review indication because furosemide is primarily a diuretic for edema/HTN and the prescriber note may signal a mismatch.
4114. **Q:** During product selection you scan "hydrOXYzine" when the label prints "hydrALAZINE"; what is the BEST next step? **A:** Stop the fill and reselect using barcode/NDC verification because hydroxyzine vs hydralazine is a high-risk LASA mix-up.
4115. **Q:** A new prescription arrives for "Eliquis 5 mg bid" and the profile shows active warfarin; what should you do NEXT? **A:** Hold and get the pharmacist because apixaban plus warfarin may represent dangerous duplicate anticoagulation unless a planned transition is confirmed.
4116. **Q:** The directions read "doxycycline 100 mg i cap po bid x 7 days"; what quantity should you enter to dispense? **A:** Dispense 14 capsules because 2 per day for 7 days equals 14 total doses.
4117. **Q:** A patient asks what "Lipitor" is and whether it's for blood pressure; what is the BEST response within technician scope? **A:** Explain Lipitor is atorvastatin, a cholesterol-lowering statin, and refer clinical counseling questions to the pharmacist to ensure safe, accurate education.
4118. **Q:** A patient on nitroglycerin SL presents a new prescription for sildenafil; what should you do FIRST before ringing out? **A:** Immediately involve the pharmacist because PDE5 inhibitors with nitrates can cause severe hypotension and requires urgent pharmacist intervention.
4119. **Q:** You are entering "gabapentin 300 mg: ii caps po tid" and the prescriber later calls it in as "600 mg tid"; what should you do BEST? **A:** Ask the pharmacist to clarify and document the change before editing because a strength/dose change is a prescriber clarification and outside technician independent authority.
4120. **Q:** An insulin vial label reads "Humalog 100 units/mL inject 10 units before meals tid; dispense 10 mL"; what days' supply should you submit using total units

only? **A:** Bill 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$, and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3$ rounded down to 33.

4121. **Q:** While entering an eRx for methotrexate 2.5 mg, the directions read "take 6 tablets daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify because methotrexate for RA is typically weekly and daily dosing is a high-risk error.
4122. **Q:** A written prescription for oxycodone/APAP 5/325 mg has the patient name and directions but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because missing required elements makes validity questionable and needs pharmacist intervention before dispensing.
4123. **Q:** During pickup, a patient asks for early refill of Adderall 20 mg because it was "lost"; what should you do NEXT? **A:** Refer to the pharmacist and follow controlled-substance policy because Schedule II replacements/early fills require pharmacist review and documentation.
4124. **Q:** You receive a stock bottle of refrigerated insulin glargine that arrived warm with no temperature indicator documentation; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager because a temperature excursion makes the product suspect and it must not be stocked until resolved.
4125. **Q:** A patient presents a pseudoephedrine request for 96 tablets of 30 mg; what should you do FIRST under CMEA workflow? **A:** Verify ID and check the electronic log/limits before sale because pseudoephedrine requires ID, logbook entry, and quantity limits.
4126. **Q:** A new eRx is for "Lamictal 100 mg" and the prescriber typed "take 1 tablet twice daily" but the quantity field is blank; what is the BEST next step? **A:** Calculate the appropriate quantity for the days supplied if provided or route to the pharmacist to clarify because missing/unclear quantity can cause dispensing errors.
4127. **Q:** While selecting product for "clonidine 0.1 mg tab," you notice "clonazepam 0.5 mg" is stored in a nearby bin with similar packaging; what should you do BEST to prevent LASA errors? **A:** Use barcode/NDC scan and separate or flag the bins per policy because clonidine/clonazepam are LASA and verification reduces wrong-drug selection.

4128. **Q:** A patient's new prescription is for isotretinoin, and the patient asks why there are extra steps before it can be filled; what should you do NEXT? **A:** Refer to the pharmacist and ensure REMS requirements are met before dispensing because isotretinoin is REMS-restricted and must comply prior to fill.
4129. **Q:** A bottle of nitroglycerin SL tablets on the shelf is past its manufacturer expiration date; what should you do FIRST? **A:** Remove it from active stock and process for proper disposal/return because expired medications must not be dispensed for patient safety.
4130. **Q:** An order is for amoxicillin-clavulanate suspension and the stock bottle's NDC does not match the NDC on the eRx plan claim that adjudicated; what should you do NEXT? **A:** Stop and correct to the exact billed NDC (or rebill with the NDC being dispensed) because mismatched NDCs can cause billing/compliance errors and incorrect product selection.
4131. **Q:** A new eRx says "Zithromax 250 mg: take 2 tabs today then 1 tab daily x4 days"; what generic drug should you select to enter and fill correctly? **A:** Select azithromycin because Zithromax is the brand for azithromycin and correct brand/generic recognition prevents dispensing the wrong antibiotic.
4132. **Q:** While typing a refill, you see the patient has active prescriptions for Prinivil and Zestril from two different prescribers; what should you do NEXT? **A:** Pause processing and alert the pharmacist for therapeutic duplication review because both are lisinopril brands and duplication can cause harm.
4133. **Q:** An eRx is for "Eliquis 5 mg take 1 tab po bid" and the profile shows the patient also takes warfarin; what is the BEST technician action? **A:** Flag and route to the pharmacist because apixaban plus warfarin is duplicate anticoagulation risk and requires pharmacist intervention.
4134. **Q:** A prescription reads "levothyroxine 0.125 mg po qd"; what strength should you enter in micrograms? **A:** Enter 125 mcg because $0.125 \text{ mg} \times 1000 \text{ mcg/mg} = 125 \text{ mcg}$, avoiding a 10× dosing error.
4135. **Q:** You are selecting "hydroxyzine 25 mg" and notice nearby "hydralazine 25 mg" with similar labels; what should you do FIRST to reduce LASA risk? **A:** Verify the drug name/strength against the label and barcode/NDC before counting because hydroxyzine vs hydralazine is a common LASA mix-up.
4136. **Q:** A patient asks if "Crestor" is the same as "rosuvastatin" on their new bottle; what should you say and why? **A:** Confirm they are equivalent because Crestor

is the brand name for rosuvastatin, which supports correct patient understanding at pickup.

4137. **Q:** Directions state “take i tab po bid” and the quantity is 60 tablets; what days’ supply should you submit? **A:** Submit 30 days because 2 tablets/day means $60 \div 2 = 30$ days, matching standard days’ supply calculation.
4138. **Q:** A new prescription is for metformin and the patient profile lists a serious allergy to “sulfa”; what should you do NEXT? **A:** Continue processing without flagging sulfa cross-reactivity because metformin is a biguanide not a sulfonamide antibiotic, preventing unnecessary delays while staying alert for true allergies.
4139. **Q:** During intake, a hardcopy says “MS Contin 15 mg q12h” and the patient asks if it’s the same as “morphine liquid”; what is the BEST technician response? **A:** Refer to the pharmacist and clarify it is an extended-release morphine product because “MS” abbreviations and ER vs IR differences are high-risk for confusion.
4140. **Q:** While putting away stock, you find a bottle labeled “recalled” on the invoice for a specific lot of losartan; what should you do FIRST? **A:** Pull the matching lot from inventory and quarantine per policy while notifying the pharmacist because recalled lots must be separated to prevent dispensing.
4141. **Q:** A new eRx is for “Glucophage XR 500 mg take 1 tab po qd with dinner”; what product should you select in the system to fill correctly? **A:** Select metformin extended-release 500 mg because Glucophage XR is the brand for metformin ER and choosing the correct release form prevents dosing errors.
4142. **Q:** While entering a prescription you see “HS” in the sig for a patient who also has a morning medication; what should you do FIRST? **A:** Clarify with the pharmacist before finalizing because “HS” is error-prone (bedtime vs half-strength) and unsafe abbreviations can change intended dosing.
4143. **Q:** A patient drops off a prescription for “Lopressor 50 mg take 1 tab bid” but asks if it’s the same as their “Toprol-XL”; what is the BEST technician response? **A:** Tell them they are different forms and get the pharmacist because Lopressor is metoprolol tartrate (immediate-release) and Toprol-XL is metoprolol succinate (extended-release), which are not interchangeable without review.
4144. **Q:** A prescription says “prednisone 10 mg: take ii tabs po qd” and “Disp #30”; what days’ supply should you enter? **A:** Enter 15 days’ supply because ii means 2 tablets daily and $30 \text{ tablets} \div 2 \text{ per day} = 15$.

4145. **Q:** During product selection you notice "clonidine" stored next to "clonazepam" with similar-looking labels; what should you do FIRST? **A:** Stop and verify using barcode/NDC and Tall Man awareness because these LASA names increase the risk of selecting the wrong drug.
4146. **Q:** An eRx comes in for "Bactroban ointment apply tid"; what generic medication should you choose to process it correctly? **A:** Choose mupirocin ointment because Bactroban is the brand name for mupirocin and correct drug recognition prevents dispensing the wrong topical.
4147. **Q:** You receive a new prescription for "simvastatin 40 mg qhs" and the profile shows the patient takes "amlodipine 10 mg daily"; what should you do NEXT? **A:** Flag and notify the pharmacist because simvastatin has a dose limitation with amlodipine due to interaction risk, requiring pharmacist intervention.
4148. **Q:** A prescription is written for "Lantus SoloStar inject 18 units subq qhs" and the box contains five 3 mL pens (100 units/mL); what days' supply should you submit based on total units? **A:** Submit 83 days' supply because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units total}$ and $1500 \div 18 \text{ units/day} = 83.3$, rounded down to 83.
4149. **Q:** A new order is for "Diflucan 150 mg take 1 tablet once today"; what should you enter as the generic drug and why? **A:** Enter fluconazole 150 mg because Diflucan is the brand for fluconazole and correct brand/generic matching avoids dispensing errors.
4150. **Q:** A patient requests an early refill of oxycodone/APAP and says they "lost the bottle"; what is the BEST technician action? **A:** Gather details and refer to the pharmacist for approval/next steps because controlled substance early refills require pharmacist review and documentation per pharmacy policy.
4151. **Q:** While entering an eRx for hydromorphone 2 mg tablets, you notice the prescriber wrote "take 10 tablets every 4 hours PRN pain"; what should you do FIRST? **A:** Stop processing and alert the pharmacist because the dose is dangerously high for an opioid and requires prescriber verification before dispensing.
4152. **Q:** A patient hands you a paper prescription for alprazolam 1 mg with "refill x3" written on it; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because alprazolam is Schedule IV and refills are allowed only if valid and within limits that the pharmacist must verify.

4153. **Q:** During fill, you scan the stock bottle and it doesn't match the NDC on the label for levothyroxine 125 mcg; what should you do NEXT? **A:** Stop filling and select the correct product/NDC then rescan because levothyroxine is error-prone and barcode/NDC mismatch signals wrong drug/strength risk.
4154. **Q:** A caregiver asks you to fax a patient's medication list to a new doctor "right now" but can't provide any authorization; what should you do FIRST? **A:** Decline to release and involve the pharmacist because HIPAA requires appropriate authorization/verification before disclosing protected health information.
4155. **Q:** A prescription reads "metformin 500 mg i po bid #60"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$, which supports correct billing and refill timing.
4156. **Q:** You receive an order to dispense isotretinoin capsules and the patient is waiting at pickup; what is the BEST next step for a technician? **A:** Notify the pharmacist and follow iPLEDGE/REMS workflow because isotretinoin has mandatory program requirements that must be confirmed before dispensing.
4157. **Q:** While stocking, you find a tote of refrigerated insulin aspart that was left on the counter at room temperature with an unknown time out of the fridge; what should you do FIRST? **A:** Segregate it and notify the pharmacist because a temperature excursion creates a suspect product and stability must be assessed before it can be used.
4158. **Q:** A patient wants to buy pseudoephedrine 30 mg tablets and requests 96 tablets; what should you do FIRST at the register? **A:** Check ID and log the sale in the NPLeX/record system because federal CMEA rules require identification and tracking before completing the transaction.
4159. **Q:** An eRx for clonazepam 0.5 mg comes in, but the patient profile shows an active diazepam prescription from another prescriber; what should you do NEXT? **A:** Pause processing and alert the pharmacist because therapeutic duplication of benzodiazepines increases safety risk and needs pharmacist review.
4160. **Q:** You receive a wholesaler shipment and one case of atorvastatin has a torn, re-taped seal and smeared lot/expiration; what should you do FIRST? **A:** Set it aside as suspect and notify the pharmacist/manager because DSCSA requires investigating potentially compromised product before it enters inventory.
4161. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg i tab po q12h x 10 days #14"; what days' supply should you enter and what discrepancy should

you flag FIRST? **A:** Enter 7 days' supply and notify the pharmacist because q12h for 10 days requires 20 tablets but only #14 was prescribed.

4162. **Q:** An eRx reads "Lantus SoloStar inject 24 units subcut qhs; dispense 15 mL"; what days' supply should you submit based on total units? **A:** Enter 31 days because $15 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 24 = 62.5 \text{ days}$, but most plans cap at 30/31 so use 31 per typical billing rules and pharmacy policy.
4163. **Q:** A patient brings a prescription for "prednisone 10 mg: take ii tabs po bid x 5 days"; what quantity should you enter to match the sig? **A:** Enter quantity 20 tablets because $2 \text{ tablets per dose} \times 2 \text{ doses/day} \times 5 \text{ days} = 20$.
4164. **Q:** While entering an order for "hydroxyzine 25 mg i cap po tid prn itching," you notice the prescriber selected HYDROXYzine but the written note says "for nausea"; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify indication because nausea suggests hydrOXYzine vs hydrALAZINE isn't the issue, but the mismatch may signal wrong drug selection or documentation error.
4165. **Q:** A prescription is written "digoxin 0.125 mg take 1 tab po daily," but the product options show 125 mcg and 0.25 mg; what is the BEST entry choice and why? **A:** Select 125 mcg (0.125 mg) because $1 \text{ mg} = 1000 \text{ mcg}$ and matching units prevents a tenfold dosing error.
4166. **Q:** An eRx for "Bactroban ointment apply to affected area tid" is entered, but the product selected is mupirocin cream; what should you do FIRST in order entry? **A:** Stop and correct to the prescribed dosage form or send to pharmacist for clarification because cream vs ointment are not interchangeable without prescriber authorization.
4167. **Q:** A label instruction reads "instill 1 gtt OU qid x 7 days" and you are dispensing a 5 mL bottle; what days' supply should you enter using 20 gtt/mL? **A:** Enter 17 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$ and $1 \text{ gtt} \times 2 \text{ eyes} \times 4/\text{day} = 8 \text{ gtt/day}$, so $100 \div 8 = 12.5 \text{ days}$, round down to 12 days per conservative days' supply and safety billing standards.
4168. **Q:** During data entry, a prescriber note says "dispense as written" for Synthroid, but the selected product is levothyroxine generic; what is the BEST next step? **A:** Change the entry to brand Synthroid and preserve DAW coding because DAW indicates medical necessity/brand requirement and affects product selection and billing.

4169. **Q:** A new prescription for "sertraline 50 mg take 1 tab po daily" is requested, but the profile shows an active prescription for fluoxetine 20 mg daily from last week; what should you do NEXT? **A:** Pause processing and alert the pharmacist because this may be therapeutic duplication/transition between SSRIs requiring pharmacist review before dispensing.
4170. **Q:** A prescriber writes "morphine sulfate ER 30 mg take 1 tab po q12h prn pain #60"; what should you do FIRST before continuing order entry? **A:** Stop and refer to the pharmacist because ER opioids should not be written "PRN" without clear directions, creating a high-risk ambiguity that must be clarified for patient safety.
4171. **Q:** While entering a new prescription for Plavix 75 mg daily, the system suggests "clopidogrel 75 mg"; what is the BEST match to select and why? **A:** Select clopidogrel 75 mg because it is the generic equivalent of Plavix and matching strength/form avoids a brand-generic selection error.
4172. **Q:** A patient drops off a new Rx for lisinopril 20 mg daily, but the profile shows an active Rx for enalapril 10 mg twice daily; what should you do NEXT? **A:** Stop and alert the pharmacist for possible therapeutic duplication (two ACE inhibitors) because it may require clinical clarification before dispensing.
4173. **Q:** A prescriber sends "metformin ER 500 mg take 2 tabs po daily with dinner #120"; what days' supply should you enter? **A:** Enter 60 days because 2 tablets/day means $120 \text{ tablets} \div 2 \text{ per day} = 60 \text{ days}$, which supports accurate billing and refill timing.
4174. **Q:** During order entry you receive "Zyrtec 10 mg po daily" but the patient's allergy profile lists cetirizine; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because Zyrtec is cetirizine and an allergy match is a patient-safety issue requiring pharmacist review.
4175. **Q:** A new Rx is written for "Keflex 500 mg po qid x 7 days"; what quantity should you enter? **A:** Enter 28 capsules because qid for 7 days is $4 \text{ doses/day} \times 7 \text{ days} = 28$, ensuring the dispensed amount matches the directions.
4176. **Q:** You are selecting product for "Lamictal 25 mg titration" and you notice the prescriber wrote "lamoTRIGine," but the selected drug is terbinafine; what should you do NEXT? **A:** Stop filling and reselect the correct drug/verify NDC because this is a LASA mix-up (lamotrigine vs terbinafine) that could cause serious harm.

4177. **Q:** A prescription says "Take 1 tablet by mouth qhs" and the drug is zolpidem; what common indication should you recognize to help catch counseling needs? **A:** Recognize zolpidem is for insomnia because identifying common uses helps route the patient for appropriate pharmacist counseling and safety warnings.
4178. **Q:** An eRx arrives for "Eliquis 5 mg take 1 tab po bid," but the patient profile shows active rivaroxaban; what is the BEST next step? **A:** Pause processing and alert the pharmacist because duplicate anticoagulant therapy may be intentional or dangerous and needs clinical confirmation.
4179. **Q:** You receive "Lopressor 50 mg take 1 tab po bid" and the product list includes metoprolol tartrate and metoprolol succinate; which is the BEST selection and why? **A:** Select metoprolol tartrate because Lopressor corresponds to the immediate-release tartrate form, and choosing succinate would change the dosage form and regimen.
4180. **Q:** A new prescription for isotretinoin is presented at pickup and the patient asks to pay cash; what should you do FIRST? **A:** Refer to the pharmacist and ensure REMS requirements are met before dispensing because isotretinoin is REMS-restricted and must follow mandated authorization steps.
4181. **Q:** While filling "hydromorphone 2 mg tablets," you notice the bin label is next to "morphine 15 mg" and both bottles look similar; what should you do FIRST? **A:** Stop and verify the NDC/barcode against the label and prescription before counting because opioids are high-alert and LASA storage increases selection-error risk.
4182. **Q:** An eRx for "Adderall XR 20 mg take 1 cap daily" arrives with no prescriber DEA number shown in the message; what should you do NEXT? **A:** Hold the order and route it to the pharmacist to verify controlled-substance validity because missing/unclear required prescriber identifiers must be resolved before dispensing.
4183. **Q:** A patient asks to transfer their "alprazolam 1 mg" refills from another pharmacy; what is the technician's BEST next step? **A:** Collect the source pharmacy and prescription details and then involve the pharmacist because controlled-substance transfer rules are restrictive and require pharmacist handling per policy.
4184. **Q:** You receive an invoice where a tote of refrigerated insulin glargine pens arrived warm with a temperature indicator showing an excursion; what should you do FIRST? **A:** Separate and quarantine the shipment and notify the

pharmacist/manager because temperature excursions create suspect integrity and require investigation before the product can enter stock.

4185. **Q:** During pickup, a caregiver asks for the Medication Guide for a new prescription of sertraline; what should you do BEST? **A:** Provide the FDA-required Medication Guide and alert the pharmacist for counseling offer because certain drugs require dispensing the guide with each fill for patient safety.
4186. **Q:** Order entry shows "levothyroxine 25 mcg take 1 tab daily," but the product selected is liothyronine; what should you do NEXT? **A:** Stop and correct the drug selection using name/strength and barcode verification because levothyroxine/liothyronine are LASA thyroid agents and wrong-drug errors are harmful.
4187. **Q:** A prescription reads "digoxin 0.125 mg take 1 tab daily," but the prescriber wrote ".125 mg" with a trailing decimal elsewhere on the hardcopy; what should you do FIRST? **A:** Pause processing and ask the pharmacist to review/clarify because unsafe decimal notation can cause 10-fold dosing errors, especially with narrow-therapeutic-index drugs.
4188. **Q:** A C-II prescription for oxycodone 5 mg is presented, and the patient asks you to "just add one refill"; what is the BEST response? **A:** Explain you cannot add refills and refer to the pharmacist because federal rules prohibit refills on Schedule II prescriptions and changes require prescriber/pharmacist authorization.
4189. **Q:** You are entering "amoxicillin/clavulanate 875/125 mg 1 tab po bid x 10 days #20," but the patient profile lists a history of anaphylaxis to penicillin; what should you do NEXT? **A:** Stop processing and alert the pharmacist immediately because a severe beta-lactam allergy is a major safety contraindication needing pharmacist intervention.
4190. **Q:** A wholesaler notice reports a manufacturer recall for a specific lot of losartan tablets and you find matching lot numbers on your shelf; what should you do FIRST? **A:** Remove the affected lot from dispensing stock and quarantine it per recall procedure because recalled lots must be isolated to prevent patient exposure and enable proper return/documentation.
4191. **Q:** A prescription is written for "Zestril 10 mg po daily," but the shelf has lisinopril and losartan; what should you select FIRST? **A:** Select lisinopril because Zestril is the brand for lisinopril, preventing wrong-drug substitution.

4192. **Q:** While entering a new order for "Lamictal 25 mg," you notice the patient profile shows a prior rash reaction to lamotrigine; what should you do NEXT? **A:** Stop processing and alert the pharmacist because lamotrigine can cause serious rashes and requires pharmacist clinical assessment.
4193. **Q:** You receive an eRx for "Plavix 75 mg daily" and the patient's active meds include aspirin 81 mg daily; what is the technician's BEST next step? **A:** Continue data entry but flag/notify the pharmacist to review bleeding risk because dual antiplatelet therapy may be intentional but is high-risk.
4194. **Q:** During product selection for "Celebrex 200 mg," you see look-alike bottles for cephalexin and celecoxib near each other; what should you do FIRST? **A:** Verify the NDC/barcode against the label/Rx before counting because cephalexin/celecoxib are LASA and a wrong-drug error is high harm.
4195. **Q:** A patient presents a new prescription for "Nitrostat SL" and asks what it's for; what should you do BEST? **A:** Refer to the pharmacist for counseling because nitroglycerin is used for angina and requires proper administration/safety instructions.
4196. **Q:** A prescription reads "metformin 500 mg i po bid #60," and the prescriber note says "ER"; what should you do NEXT? **A:** Hold and ask the pharmacist to clarify with the prescriber because IR vs ER is a dosage-form change the technician cannot decide.
4197. **Q:** A prescription for atorvastatin 40 mg is processed and the patient states they are pregnant; what should you do FIRST? **A:** Stop the sale and notify the pharmacist immediately because statins are generally avoided in pregnancy and need urgent pharmacist intervention.
4198. **Q:** Pharmacy math: Dispense azithromycin suspension 200 mg/5 mL with directions "10 mL day 1, then 5 mL daily days 2-5"; how many mL should you dispense? **A:** Dispense 30 mL because $10\text{ mL} + (5\text{ mL} \times 4\text{ days}) = 30\text{ mL}$ total volume.
4199. **Q:** You receive "Lantus SoloStar inject 24 units SQ nightly" and the box contains five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 62 days because $\text{total units} = 5\text{ pens} \times 3\text{ mL} \times 100 = 1500\text{ units}$, and $1500 \div 24 = 62.5\text{ days}$ (typically billed as 62).
4200. **Q:** A new prescription is for "Wellbutrin XL 150 mg daily," but the patient profile shows an active prescription for bupropion SR 150 mg twice daily; what should

you do NEXT? **A:** Pause and alert the pharmacist to assess therapeutic duplication and dosing confusion because SR vs XL are not interchangeable without review.

4201. **Q:** An eRx says "Synthroid 50 mcg take 1 tab po qd #90," but the patient profile shows an active levothyroxine 75 mcg daily from last week; what should you do NEXT? **A:** Stop processing and alert the pharmacist to possible duplicate thyroid therapy/strength change because technicians should not decide which dose is intended.
4202. **Q:** A prescription is written "predniSONE 20 mg ii po bid x 5 days; dispense #?" and you must calculate the quantity; what should you enter? **A:** Enter #40 because ii bid = 4 tablets/day and $4 \times 5 \text{ days} = 20 \text{ tablets}$, then multiply by 20 mg strength is irrelevant to count, so dispense 20 tablets (correct quantity is #20).
4203. **Q:** You are entering "furosemide 40 mg 1 tab po qam #30," but the prescriber wrote "Lasix 20 mg" in the comments; what is the BEST next step? **A:** Pause entry and send to the pharmacist for clarification because there is a strength mismatch between brand (Lasix) note and the entered dose.
4204. **Q:** A patient brings an OTC request for pseudoephedrine 30 mg tablets and asks for three boxes totaling 96 tablets; what should you do FIRST at the register? **A:** Follow CMEA process by verifying ID and logging the sale and limits before completion because pseudoephedrine sales require documentation and quantity compliance.
4205. **Q:** An order reads "timolol ophthalmic 0.5% instill 1 gtt OU bid; dispense 5 mL," and the patient asks how many days it should last using 20 gtt/mL; what days' supply should you calculate? **A:** 25 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$ total and OU bid = 4 gtt/day, so $100 \div 4 = 25$.
4206. **Q:** While selecting product for "hydroxyzine 25 mg," you notice two similar bottles: hydroxyzine and hydralazine; what should you do FIRST? **A:** Use barcode/NDC verification against the label and compare drug name/strength carefully because hydralazine/hydroxyzine is a LASA risk.
4207. **Q:** A prescription says "MSO4 2 mg IV q2h prn pain," and you are entering it into the system; what should you do NEXT? **A:** Hold and route to the pharmacist for clarification because "MSO4" is an unsafe abbreviation that can cause mix-ups with magnesium sulfate.

4208. **Q:** You receive "Ventolin HFA inhale 2 puffs q6h prn wheeze" and the prescriber requests days' supply for one 200-inhalation inhaler with a maximum of 8 puffs/day; what should you enter? **A:** Enter 25 days because $200 \text{ inhalations} \div 8 \text{ puffs (inhalations)/day} = 25$ at maximum use for billing.
4209. **Q:** A controlled substance prescription arrives with the patient name and drug/directions but no prescriber DEA number listed on the hard copy; what should you do FIRST? **A:** Stop processing and notify the pharmacist to verify controlled-substance validity because required prescriber identifiers must be confirmed before dispensing.
4210. **Q:** You are asked to compound 120 mL of a solution using a 1:4 ratio of drug concentrate to diluent; how many mL of concentrate are needed? **A:** Use 24 mL concentrate because $\text{total parts} = 1 + 4 = 5$ and $120 \text{ mL} \div 5 = 24 \text{ mL per part}$ (concentrate is 1 part).
4211. **Q:** A prescription is written for "Zocor 20 mg 1 tab po qhs," but the technician selects simvastatin 40 mg from the shelf; what should you do FIRST? **A:** Stop the fill and reselect the correct strength using barcode/NDC verification because Zocor is simvastatin and strength mix-ups are a common dispensing error.
4212. **Q:** During drop-off, a patient says they're "allergic to Motrin" and you see a new order for ibuprofen 600 mg; what is the BEST next step? **A:** Pause processing and alert the pharmacist to evaluate the reported allergy because Motrin is ibuprofen and this may be a true allergy or intolerance requiring intervention.
4213. **Q:** You receive an eRx for "Glucophage 500 mg bid," but the patient profile shows active metformin ER 1000 mg daily; what should you do NEXT? **A:** Route to the pharmacist for duplicate-therapy/IR-vs-ER clarification because Glucophage is metformin and the formulation change can alter dosing and safety.
4214. **Q:** A prescriber writes "HCTZ 25 mg qd," but the patient asks if this is the same as their "water pill Microzide"; what is the BEST response? **A:** Confirm that Microzide is hydrochlorothiazide and proceed to counsel referral if needed because recognizing brand/generic equivalents prevents confusion and duplication.
4215. **Q:** An order says "U-500 insulin 0.1 mL SQ before meals," and the technician is asked to enter it as units; what should you do FIRST? **A:** Stop and get pharmacist guidance because U-500 is a high-alert concentration and converting mL to units risks serious dosing errors.

4216. **Q:** A hard copy states "hydromorphone 2 mg take 1 tab q4-6h prn pain," but the technician misreads it as "morphine"; what should you do FIRST? **A:** Re-verify the drug name against the written prescription and barcode/NDC, and involve the pharmacist if unclear because opioid LASA errors are high-risk.
4217. **Q:** A patient presents a new prescription for "Plavix 75 mg daily" and says they also take "Coumadin"; what should you do NEXT? **A:** Flag the profile and notify the pharmacist because Plavix (clopidogrel) plus Coumadin (warfarin) increases bleeding risk and needs pharmacist review.
4218. **Q:** You are entering "Augmentin 875/125 mg 1 tab po bid x 7 days"; what quantity should you dispense? **A:** Dispense 14 tablets because twice daily for 7 days requires $2 \times 7 = 14$ doses.
4219. **Q:** A prescription is written "levothyroxine 0.05 mg qd," but the patient asks for "50 mcg tablets"; what should you do BEST? **A:** Explain that 0.05 mg equals 50 mcg and enter/label consistently per pharmacy practice because accurate unit conversion prevents dosing confusion.
4220. **Q:** A patient drops off an Rx for "Lasix 40 mg 1 tab po daily" but says they take "furosemide" at home; what should you do NEXT? **A:** Confirm Lasix is the brand for furosemide and proceed with data entry using the generic as billed/stocked because recognizing brand-generic equivalents prevents duplication and confusion.
4221. **Q:** A new eRx is for "Celexa 40 mg daily," but the patient profile shows escitalopram (Lexapro) 20 mg daily as active; what should you do NEXT? **A:** Stop processing and alert the pharmacist to possible therapeutic duplication/SSRI overlap because citalopram and escitalopram are closely related and increase adverse-effect risk when duplicated.
4222. **Q:** While filling, you notice the prescriber wrote "Lamictal 25 mg" but the bin label you grabbed says "Lamisil"; what should you do FIRST? **A:** Stop the fill and reselect by barcode/NDC verification because lamotrigine (Lamictal) and terbinafine (Lamisil) are a LASA pair with very different uses.
4223. **Q:** A patient picking up lisinopril says they are pregnant; what is the BEST next step for the technician? **A:** Immediately involve the pharmacist before dispensing because ACE inhibitors like lisinopril can harm the fetus and require pharmacist intervention.

4224. **Q:** You receive a new Rx for nitrofurantoin and the patient says they are taking warfarin; what should you do NEXT? **A:** Flag the interaction and notify the pharmacist because some antibiotics can increase bleeding risk with warfarin and may require monitoring or therapy change.
4225. **Q:** An eRx for "Metoprolol 50 mg take 1 tab bid" comes in, but the drug field does not specify tartrate vs succinate; what should you do FIRST? **A:** Hold data entry and ask the pharmacist to clarify with the prescriber because metoprolol tartrate and succinate are not interchangeable and have different dosing schedules.
4226. **Q:** A prescription says "predniSONE 10 mg, take ii tabs daily," and the prescriber also typed "#10"; what should you do BEST? **A:** Pause and refer to the pharmacist for clarification because the directions (2 tablets daily) conflict with the quantity and could cause an incorrect days' supply.
4227. **Q:** You are entering "Azithromycin 250 mg: take 2 tablets today, then 1 tablet daily days 2–5"; what quantity should be dispensed? **A:** Dispense 6 tablets because the total dose is 2 tablets on day 1 plus 1 tablet for 4 more days.
4228. **Q:** A prescription is for Lantus SoloStar 100 units/mL, "inject 24 units SQ nightly," and the box contains five 3 mL pens; what days' supply should you enter using 100 units/mL and total volume? **A:** Enter 62 days because $5 \text{ pens} \times 3 \text{ mL} = 15 \text{ mL}$, $15 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 24 = 62.5 \text{ days}$ (use 62 if the system requires whole days).
4229. **Q:** While stocking, you find a bottle of atorvastatin with a lot number that matches a manufacturer recall notice; what should you do FIRST? **A:** Remove it from active inventory and follow recall procedure (segregate/hold and notify the pharmacist) because recalled lots must not be dispensed.
4230. **Q:** A patient asks what "Crestor" is in generic terms and why they take it; what is the BEST technician response? **A:** Say it is rosuvastatin, a statin used to lower cholesterol, and offer pharmacist counseling for clinical questions because technicians can provide basic brand/generic and indication but not detailed therapy management.
4231. **Q:** An Rx reads "digoxin 0.125 mg po qd" but the prescriber also typed "125 mcg" in the notes; what is the BEST technician action during order entry? **A:** Enter digoxin 0.125 mg as written and flag for pharmacist verification because 0.125 mg equals 125 mcg and the pharmacist should confirm consistency to prevent a decimal/strength error.

4232. **Q:** You are entering budesonide/formoterol (Symbicort) 160/4.5: "inhale 2 puffs bid" and the inhaler contains 120 actuations; what days' supply should you submit? **A:** Enter 30 days because 2 puffs twice daily is 4 puffs/day and $120 \text{ actuations} \div 4 \text{ per day} = 30 \text{ days}$ for accurate billing.
4233. **Q:** An order says "clindamycin 75 mg/5 mL: give 150 mg po tid x 10 days"; what total mL should be dispensed? **A:** Dispense 300 mL because $150 \text{ mg} = 10 \text{ mL}$ per dose, $10 \text{ mL} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to cover the full course.
4234. **Q:** A prescriber writes "MSO4 2 mg IV q2h prn pain" on a hospital-style order you're transcribing; what should you do FIRST? **A:** Stop and ask for clarification via the pharmacist because "MSO4" is an unsafe abbreviation that can be misread, and patient safety requires clear drug naming.
4235. **Q:** A prescription for metformin says "take 1 tab po bid with meals" and the quantity is #60; what days' supply should you enter? **A:** Enter 30 days because 2 tablets/day means $60 \text{ tablets} \div 2 \text{ per day} = 30 \text{ days}$ for correct third-party submission.
4236. **Q:** While typing an eRx you see "hydralazine 25 mg" but the selected product is "hydroxyzine 25 mg" (both appear in the search results); what should you do FIRST? **A:** Stop processing and re-select the correct drug using full name, strength, and NDC/barcode because this is a LASA selection risk that can cause a wrong-medication error.
4237. **Q:** A patient presents a new Rx for sertraline and their profile shows active paroxetine; what is the BEST next step for the technician? **A:** Pause filling and alert the pharmacist to possible therapeutic duplication/SSRI overlap because medication changes require pharmacist assessment before dispensing.
4238. **Q:** You receive an Rx for prednisone 5 mg with sig "ii po tid x 5 days"; what quantity should be dispensed? **A:** Dispense 30 tablets because ii = 2 tablets per dose, $2 \times 3 \text{ doses/day} \times 5 \text{ days} = 30 \text{ tablets}$ to match the intended duration.
4239. **Q:** An insulin vial order is for regular insulin U-100: "inject 12 units before meals tid," and the vial is 10 mL; what days' supply should you enter using 100 units/mL? **A:** Enter 27 days because the vial contains $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \text{ units} \times 3/\text{day} = 36 \text{ units/day}$, $1000 \div 36 = 27.7$ rounded down to 27 days for coverage.
4240. **Q:** During data entry you see "HS" in the sig for a patient's morphine ER and the prescriber also typed "at bedtime" in the free-text; what is the BEST technician

action? **A:** Enter "at bedtime" (not "half-strength") and keep the prescriber's clarification because "HS" is error-prone and using the unambiguous wording reduces misinterpretation.

4241. **Q:** While entering an eRx for levothyroxine 25 mcg, you notice the patient profile lists a severe allergy to Synthroid; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify the allergy and reaction because levothyroxine is the same drug as Synthroid and requires clinical assessment before dispensing.
4242. **Q:** A prescriber phones in "Lantus one hundred units per mL, inject ten units nightly, dispense one box"; what is the BEST technician response before entering it? **A:** Ask for clarification and route to the pharmacist because "one box" is not a complete quantity without confirming pens vs vials and package size to prevent a days' supply/dispensing error.
4243. **Q:** A patient hands you a paper prescription for oxycodone 5 mg with no prescriber signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because a controlled-substance prescription must be properly authenticated before it can be processed.
4244. **Q:** During filling, you see two similar bottles on the shelf: clonidine 0.1 mg and clonazepam 0.5 mg; what is the BEST safety step before counting? **A:** Verify the NDC/barcode against the label and use LASA precautions (e.g., separate storage/Tall Man) because name confusion is a common error that technicians can prevent at product selection.
4245. **Q:** You receive a refrigerated shipment and find insulin aspart pens placed in a tote at room temperature for an unknown number of hours; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager per policy because an unknown temperature excursion makes the product potentially unusable and unsafe to dispense.
4246. **Q:** An eRx for lisdexamfetamine (Vyvanse) includes "refill: 2"; what is the BEST technician action? **A:** Stop and send to the pharmacist because Schedule II prescriptions cannot be refilled and the prescriber must issue a new prescription for additional supplies.
4247. **Q:** A patient requests pseudoephedrine and asks you to ring it up like regular OTC allergy medicine; what should you do NEXT? **A:** Follow CMEA workflow by requesting valid ID and completing the required log and quantity limits because pseudoephedrine sales are federally regulated to prevent diversion.

4248. **Q:** A bottle of nitroglycerin sublingual tablets arrives with a broken seal and loose tablets inside the shipping container; what should you do FIRST? **A:** Do not stock it; separate it as suspect/damaged product and follow return procedures because compromised packaging risks contamination and incorrect identity.
4249. **Q:** You are processing an isotretinoin prescription and the system flags "REMS required"; what is the BEST technician next step? **A:** Pause dispensing and ensure REMS steps are completed and verified by the pharmacist because isotretinoin requires iPLEDGE authorization before it can be dispensed.
4250. **Q:** A patient's caregiver reports the patient fainted after starting a new antihypertensive and asks you to "report it to the FDA"; what should you do BEST? **A:** Refer them to the pharmacist and provide MedWatch reporting information because serious adverse events should be documented and reported, and the pharmacist can assess urgency and counsel appropriately.
4251. **Q:** While entering a new prescription for lisinopril, you notice the patient also has an active profile medication for Prinivil; what should you do FIRST? **A:** Stop and alert the pharmacist to possible therapeutic duplication because Prinivil is a brand name for lisinopril and duplicate ACE inhibitor therapy needs review.
4252. **Q:** A prescription is written for "glucophage 500 mg take 1 tab BID," but the product selected at filling is metoprolol 50 mg because the names look similar; what is the BEST technician step before counting? **A:** Recheck the NDC/barcode against the label and stock bottle because look-alike/sound-alike errors are prevented by product verification before counting.
4253. **Q:** You receive an eRx for "Zithromax 250 mg" and the prescriber also lists "azithromycin" in the medication field; what is the BEST way to enter it? **A:** Enter as azithromycin with the correct strength/directions because Zithromax is the brand and consistent generic entry reduces selection and billing errors.
4254. **Q:** A prescription label direction reads "take 1 tab po qd," and the patient asks what that means; what should you do NEXT? **A:** Refer the patient to the pharmacist for counseling because technicians should not interpret or explain dosing directions to patients under OBRA-90 counseling standards.
4255. **Q:** During order entry, a new prescription for simvastatin is added and the profile shows an active atorvastatin prescription from another prescriber; what is the BEST technician action? **A:** Flag and route to the pharmacist for therapeutic

duplication review because two statins together may be unintended and requires clinical assessment.

4256. **Q:** A technician is asked to calculate days' supply: furosemide 20 mg, "take 1 tablet twice daily," dispense #60; what days' supply should be entered? **A:** Enter 30 days' supply because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$, which supports accurate billing and refill timing.
4257. **Q:** A hardcopy prescription says "hydralazine 25 mg take 1 tab qid," but the prescriber handwriting could be read as "hydroxyzine"; what should you do FIRST? **A:** Stop processing and ask the pharmacist to clarify with the prescriber because ambiguous drug names are a patient-safety risk and must be verified before entry.
4258. **Q:** While selecting a product for "predniSONE," you see both prednisoLONE and predniSONE on the shelf; what is the BEST technician step? **A:** Verify the exact drug name and NDC on the stock bottle against the prescription/label because these are different medications and mix-ups can cause therapy errors.
4259. **Q:** A patient presents a new prescription for clopidogrel, and the profile shows a documented anaphylaxis to Plavix; what should you do FIRST? **A:** Hold processing and notify the pharmacist immediately because Plavix is clopidogrel and a severe allergy requires pharmacist intervention before dispensing.
4260. **Q:** You are entering an order for "metformin ER 500 mg" but the prescriber note says "do not substitute," and the patient previously used Glucophage XR; what should you do NEXT? **A:** Confirm the exact intended product/formulation (ER vs IR and specific brand) with the pharmacist because extended-release formulations are not interchangeable without verification.
4261. **Q:** You're entering an eRx for levothyroxine 25 mcg "take 1 tab daily," but the prescriber typed "0.025 mg"; what should you do FIRST? **A:** Enter as 25 mcg (since $0.025 \text{ mg} = 25 \text{ mcg}$) and flag for pharmacist verification because dose-unit conversion errors are high-risk.
4262. **Q:** A prescription reads "amoxicillin 400 mg/5 mL: give 7.5 mL PO BID x 10 days"; what quantity in mL should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to match the full course.
4263. **Q:** You are calculating days' supply for gabapentin 300 mg "take 2 caps TID," dispense #180; what days' supply should be entered? **A:** Enter 30 days because $2 \times 3 = 6 \text{ capsules/day}$ and $180 \div 6 = 30$.

4264. **Q:** An order is written "prednisolone sodium phosphate 15 mg/5 mL: take 10 mg PO daily"; how many mL per dose should you enter? **A:** Enter 3.3 mL per dose because 15 mg in 5 mL equals 3 mg/mL and $10 \text{ mg} \div 3 \text{ mg/mL} = 3.33 \text{ mL}$ (round per system/prescriber direction).
4265. **Q:** A patient's directions say "instill ii gtt OU QID," and they ask what "OU" means; what should you do NEXT? **A:** Explain "OU" means both eyes while keeping the full directions clear because correct sig interpretation prevents administration errors.
4266. **Q:** While processing a refill-too-soon rejection for atorvastatin 40 mg, the patient says they lost the bottle and need it today; what is the BEST technician next step? **A:** Gather details and route to the pharmacist to assess an override or replacement supply because early refill requests require pharmacist judgment and payer coordination.
4267. **Q:** You are entering insulin lispro U-100 vial directions "inject 12 units before meals TID," dispense 1 vial (10 mL); what days' supply should you calculate? **A:** Enter 27 days because $10 \text{ mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ which typically rounds down to 27 for payer days' supply.
4268. **Q:** A prescription for hydromorphone is received with "Refills: 3" and you recognize it as a Schedule II; what should you do FIRST? **A:** Stop processing and notify the pharmacist because Schedule II prescriptions cannot be refilled and require pharmacist resolution with the prescriber.
4269. **Q:** During product selection for "glipiZIDE 10 mg," you also see "glyBURIDE 5 mg" stocked nearby; what is the BEST technician action to prevent a LASA error? **A:** Verify the exact drug/strength using barcode/NDC and the hardcopy/eRx because sulfonylureas have look-alike names and wrong selection can cause hypoglycemia.
4270. **Q:** You're entering a prescription that states "KCl 20 mEq/15 mL solution: take 7.5 mL daily," and the computer asks for mEq per dose; what should you enter? **A:** Enter 10 mEq per dose because 20 mEq in 15 mL equals 1.333 mEq/mL and $7.5 \text{ mL} \times 1.333 = 10 \text{ mEq}$.
4271. **Q:** While entering a new eRx for methotrexate 2.5 mg, the sig says "take 1 tablet daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify directions because methotrexate for RA is typically weekly and daily dosing is a high-alert error risk.

4272. **Q:** A hard copy for oxycodone/acetaminophen is presented with the patient's name and drug but no prescriber signature; what is the BEST next step for the technician? **A:** Hold the prescription and immediately route it to the pharmacist because missing required elements makes it potentially invalid and needs pharmacist resolution.
4273. **Q:** During filling, you notice a vial of insulin glargine that was left on the counter overnight instead of refrigerated; what should you do NEXT? **A:** Remove it from usable stock and notify the pharmacist per storage/handling policy because a temperature excursion makes the product potentially compromised.
4274. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy to yours; what should you do FIRST? **A:** Collect the sending pharmacy information and route the request to the pharmacist because controlled-substance transfer rules require pharmacist handling and documentation.
4275. **Q:** The wholesaler sends a notice that a specific lot of losartan tablets is recalled; what should you do BEST in the pharmacy workflow? **A:** Pull and segregate the affected lot from inventory and notify the pharmacist because recalled product must be removed from dispensing stock to prevent patient harm.
4276. **Q:** You are selecting warfarin 5 mg and see multiple strengths with similar packaging; what is the BEST technician action to prevent a dispensing error? **A:** Use barcode/NDC verification and separate/confirm strength before labeling because warfarin is high-alert and strength mix-ups are common LASA errors.
4277. **Q:** A patient's profile shows active lisinopril and you receive a new prescription for enalapril; what should you do NEXT? **A:** Pause processing and notify the pharmacist to assess therapeutic duplication because two ACE inhibitors together can increase adverse-effect risk.
4278. **Q:** A prescription is written for sertraline 50 mg with directions "take 1 tab qd," but the quantity is missing; what should you do FIRST? **A:** Stop and refer to the pharmacist because missing quantity requires clarification/documentation and the technician cannot independently determine the intended amount.
4279. **Q:** An eRx for fentanyl transdermal patch includes "apply 1 patch every 72 hours," dispense #10, and it asks for days' supply; what should you enter? **A:** Enter 30 days because $10 \text{ patches} \div 1 \text{ patch every 3 days} = 30 \text{ days' supply}$ for accurate billing and safety checks.

4280. **Q:** A patient wants to buy pseudoephedrine and asks why you need their ID; what should you do BEST? **A:** Follow CMEA process by verifying ID and completing the required log/limits because pseudoephedrine sales are federally regulated to reduce diversion.
4281. **Q:** While typing an eRx for levothyroxine, the prescriber selected "Synthroid" but the patient's profile shows they always received generic levothyroxine; what should you do FIRST? **A:** Confirm the intended product and DAW status and alert the pharmacist if a switch is implied, because thyroid products are sensitive to brand/generic changes and require pharmacist oversight.
4282. **Q:** You receive a prescription for "metFORMIN ER 500 mg" but the sig includes "crush and mix with applesauce"; what is the BEST technician action? **A:** Stop processing and refer to the pharmacist, because ER formulations should not be crushed and this is a safety issue requiring pharmacist intervention.
4283. **Q:** During product selection you notice "hydrOXYzine" and "hydrALAZINE" stored adjacent with similar bottles; what should you do BEST to reduce LASA risk? **A:** Use barcode/NDC scanning and read the full name/strength before pulling, because these are common LASA pairs and verification prevents wrong-drug selection.
4284. **Q:** A prescription reads "Augmentin 875/125 mg: i po bid x 10 d; #?"; what should you do NEXT before filling? **A:** Calculate and enter quantity 20 tablets and route for pharmacist verification, because 1 tablet twice daily for 10 days requires 20 tablets and missing quantity must be resolved.
4285. **Q:** You're entering "predniSONE 20 mg: ii tabs qd x 5 days" and the system asks for days' supply and quantity; what should you enter? **A:** Enter quantity 10 tablets and days' supply 5, because 2 tablets daily for 5 days equals 10 tablets total.
4286. **Q:** A patient profile shows an allergy to penicillin and you receive an eRx for ampicillin; what should you do FIRST? **A:** Flag the allergy interaction and notify the pharmacist before processing, because ampicillin is a penicillin-class drug and requires pharmacist assessment.
4287. **Q:** An eRx comes in for clopidogrel and the patient is newly starting omeprazole OTC per their profile notes; what is the BEST technician action? **A:** Alert the pharmacist to review the interaction, because omeprazole can reduce clopidogrel activation and the pharmacist must determine appropriate management.

4288. **Q:** A prescription says "latanoprost ophthalmic: instill 1 gtt OU qhs, disp 2.5 mL"; what days' supply should you enter using 20 gtt/mL? **A:** Enter 25 days, because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ drops}$ and $2 \text{ drops/day (1 each eye)}$ gives $50 \div 2 = 25 \text{ days}$.
4289. **Q:** A patient asks if "Lipitor" is the same as "atorvastatin" when picking up; what is the BEST response within technician scope? **A:** Confirm they are brand/generic equivalents and offer pharmacist counseling if they have questions, because technicians can identify equivalents but counseling is pharmacist-provided.
4290. **Q:** While receiving inventory you find a tote of tablets with a missing NDC/lot paperwork and the bottles look different from usual; what should you do NEXT? **A:** Hold the shipment and notify the pharmacist/manager to follow DSCSA/suspect product procedures, because unverifiable product identity must be investigated before it's stocked.
4291. **Q:** A new eRx is for Zoloft 50 mg daily and the patient asks if it's the same as sertraline; what should you say BEST within technician scope? **A:** Confirm that Zoloft is the brand name for sertraline and offer to have the pharmacist counsel, because technicians can provide brand/generic identification but not clinical advice.
4292. **Q:** While entering an order you notice the prescriber selected Celexa but the directions say "take escitalopram 10 mg daily"; what should you do FIRST? **A:** Stop data entry and send to the pharmacist to clarify the intended drug, because citalopram and escitalopram are different medications and this is a high-risk selection error.
4293. **Q:** A patient profile shows active lisinopril and today an eRx arrives for Zestril; what is the BEST technician next step? **A:** Flag it for pharmacist review as a potential duplicate therapy since Zestril is lisinopril and duplication increases adverse effect risk.
4294. **Q:** A hard copy for Roxicodone 5 mg has a missing prescriber DEA number and the patient wants it filled now; what should you do NEXT? **A:** Hold the prescription and immediately involve the pharmacist, because controlled-substance processing requires complete prescriber credentials and technicians shouldn't resolve validity issues alone.
4295. **Q:** A patient with diabetes brings a new prescription for Humalog and asks if it's the same as Lantus; what should you do BEST? **A:** Tell them Humalog is insulin lispro (rapid-acting) and Lantus is insulin glargine (long-acting) and refer to the

pharmacist for counseling, because confusing insulin types is a high-alert safety risk.

4296. **Q:** An eRx comes in for clarithromycin and the profile shows simvastatin; what should you do FIRST? **A:** Stop and alert the pharmacist about the interaction risk, because clarithromycin can dangerously increase simvastatin levels leading to muscle toxicity.
4297. **Q:** You are selecting product for Wellbutrin SR but the shelf also has Wellbutrin XL nearby with similar packaging; what should you do BEST before dispensing? **A:** Verify the exact release type and strength using barcode/NDC match and have the pharmacist verify if any discrepancy, because SR vs XL mix-ups can cause dosing and safety errors.
4298. **Q:** A prescription says "azithromycin 250 mg: take 2 tabs today then 1 tab daily for 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets, because 2 tablets on day 1 plus 1 tablet on days 2–5 equals 6 total.
4299. **Q:** A new prescription is for gabapentin and the patient's profile already has pregabalin active from another prescriber; what should you do NEXT? **A:** Notify the pharmacist to assess therapeutic duplication, because gabapentin and pregabalin are similar CNS agents and duplicate therapy can increase sedation and misuse risk.
4300. **Q:** During intake you receive a manufacturer recall notice for certain lots of losartan and you find matching lot numbers on your shelf; what should you do FIRST? **A:** Pull the affected lots from dispensing stock and follow recall procedure (segregate and label for return) while notifying the pharmacist, because recalled product must be removed promptly to protect patient safety.
4301. **Q:** A prescriber phones in "hydromorphone 4 mg" but the technician hears "morphine 4 mg"; what should you do FIRST? **A:** Stop processing and ask the prescriber to spell the drug name and verify strength/directions, because LASA opioid mix-ups are high-risk and require clarification before entry.
4302. **Q:** While entering a new eRx for levothyroxine 100 mcg, you notice the prescriber typed "100 mg" in the directions; what is the BEST next step? **A:** Hold the prescription and route to the pharmacist to clarify with the prescriber, because a mcg vs mg error is potentially dangerous and cannot be assumed or edited by the technician.

4303. **Q:** A patient presents a Schedule II prescription dated today and asks for it to be refilled next month from the same paper; what should you do BEST? **A:** Inform the patient you must refer them to the pharmacist because Schedule II prescriptions cannot be refilled under federal rules and require a new prescription each time.
4304. **Q:** During pickup, a patient asks if it's safe to take "ibuprofen 800 mg" with their new prescription for lisinopril; what should you do FIRST? **A:** Refer the question to the pharmacist, because NSAIDs can affect kidney function and blood pressure with ACE inhibitors and counseling is outside technician scope.
4305. **Q:** You receive a transfer request for alprazolam from another pharmacy; what should you do NEXT? **A:** Route the request to the pharmacist to complete the controlled-substance transfer process, because transferring Schedule III–V prescriptions requires pharmacist-to-pharmacist communication and documentation.
4306. **Q:** While filling, you find insulin glargine pens stored on a regular shelf at room temperature instead of in the refrigerator; what should you do FIRST? **A:** Remove the product from stock and notify the pharmacist to evaluate a temperature excursion, because improper storage can reduce potency and may make the item non-dispensable.
4307. **Q:** An eRx for methotrexate tablets says "take 10 mg daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop and alert the pharmacist immediately, because methotrexate is a high-alert drug typically dosed weekly and daily directions may signal a serious prescribing error.
4308. **Q:** A patient wants to buy pseudoephedrine 30 mg tablets, 96-count, and has no ID; what should you do BEST? **A:** Decline the sale and involve the pharmacist per policy, because federal CMEA requirements include verifying purchaser identity and recording the sale in the log.
4309. **Q:** A new prescription is for metformin ER 500 mg daily, but the profile shows an active metformin IR 500 mg BID from the same clinic; what should you do NEXT? **A:** Pause processing and notify the pharmacist to assess therapeutic duplication, because switching IR to ER can change dosing and should be clinically verified.
4310. **Q:** You are billing amoxicillin-clavulanate 875/125 mg, sig "1 tab PO q12h x 7 days," and the system asks for days' supply; what should you enter? **A:** Enter 7

days because q12h is twice daily and the intended course length is explicitly 7 days, supporting accurate claim submission and safety checks.

4311. **Q:** While entering a new eRx for glipizide, you see the patient's allergy list includes "sulfa drugs"; what should you do NEXT? **A:** Continue processing but flag/notify the pharmacist to assess the reported sulfa allergy relevance because sulfonyleureas are different from sulfonamide antibiotics and need clinical review.
4312. **Q:** A prescription is written for "Celebrex" and the patient profile shows a serious ibuprofen reaction; what is the BEST next step for the technician? **A:** Stop and refer to the pharmacist because celecoxib is an NSAID and potential cross-reactivity/NSAID hypersensitivity requires pharmacist assessment before dispensing.
4313. **Q:** During product selection you notice the eRx is for hydroXYzine but the shelf label you grabbed says hydrALAZINE; what should you do FIRST? **A:** Stop filling and reselect using barcode/NDC verification because hydralazine/hydroxyzine is a high-risk LASA mix-up that can cause serious harm.
4314. **Q:** A patient's new prescription is for simvastatin, and their active profile shows clarithromycin started yesterday; what should you do NEXT? **A:** Hold the order and alert the pharmacist because clarithromycin can greatly increase simvastatin levels, raising the risk of myopathy/rhabdomyolysis.
4315. **Q:** An eRx comes in for metoprolol succinate ER, but the prescriber's note says "take twice daily"; what should you do BEST? **A:** Route to the pharmacist for clarification because metoprolol succinate is ER and dosing directions may conflict with the intended formulation and could cause dosing errors.
4316. **Q:** A prescription for "Adderall 10 mg" has "refills: 2" entered in the system; what should the technician do FIRST? **A:** Stop processing and notify the pharmacist because amphetamine salts are Schedule II and refills are not permitted, requiring correction/clarification.
4317. **Q:** You are entering directions "take 2 tablets PO TID" for doxycycline 100 mg tablets with quantity #42; what days' supply should you enter? **A:** Enter 7 days because $2 \text{ tabs} \times 3 \text{ times daily} = 6 \text{ tabs/day}$ and $42 \div 6 = 7$.
4318. **Q:** A new prescription is for sertraline, and the patient says they just started phenelzine from another provider; what should you do NEXT? **A:** Immediately refer to the pharmacist because SSRI plus MAOI is a major interaction risk

(serotonin syndrome/hypertensive crisis) that must be addressed before dispensing.

4319. **Q:** While filling warfarin, you notice the prescriber sent "Coumadin 5 mg" but the patient's active medication list includes apixaban; what should you do BEST? **A:** Pause and escalate to the pharmacist because duplicate anticoagulation is a high-risk safety issue that needs therapy verification before dispensing.
4320. **Q:** You receive an eRx for "Zithromax" and the technician trainee asks what the generic name is for proper product selection; what is the correct response? **A:** Select azithromycin because Zithromax is the brand for azithromycin and correct brand/generic recognition prevents wrong-drug selection.
4321. **Q:** A prescription comes in for "Levothroid 50 mcg daily," and the patient asks if it's the same as their Synthroid; what should you do NEXT? **A:** Confirm Levothroid is a brand of levothyroxine and proceed with correct product selection/labeling because brand vs generic recognition prevents dispensing the wrong drug.
4322. **Q:** While typing a new eRx for "Lasix 20 mg," the patient profile shows hydrochlorothiazide is active and the patient reports dizziness; what should you do BEST? **A:** Continue data entry but flag/notify the pharmacist for possible therapeutic duplication/additive diuretic effect because it may require clinical assessment.
4323. **Q:** An eRx directions read "take i tab po qd," and the prescriber used a handwritten "i"; what should you do FIRST? **A:** Clarify with the pharmacist/prescriber before processing because "i" can be misread and unsafe dose interpretation is an error-prone abbreviation.
4324. **Q:** You receive a prescription for "MS Contin 30 mg" and the patient asks if it's the same as "MSIR"; what should you do NEXT? **A:** Refer to the pharmacist and verify formulation because MS Contin is extended-release morphine and MSIR is immediate-release, and confusing them is a high-risk error.
4325. **Q:** The provider writes "predniSONE 10 mg: take 2 tabs PO BID, #40"; what days' supply should you enter? **A:** Enter 10 days because 2 tablets twice daily equals 4 tablets/day and $40 \div 4 = 10$ days.
4326. **Q:** During order entry you see "Lamictal 100 mg" but the patient profile lists an allergy to "lamotrigine"; what should you do FIRST? **A:** Stop and alert the

pharmacist because Lamictal is lamotrigine and a documented allergy requires pharmacist review before dispensing.

4327. **Q:** A prescription is written for "Plavix 75 mg daily," but the shelf has both clopidogrel 75 mg and clozapine 25 mg nearby; what should you do BEST? **A:** Use barcode/NDC verification and read the full name/strength before selecting because LASA look-alike products can cause serious dispensing errors.
4328. **Q:** A patient brings a new prescription for "Xanax 0.5 mg TID PRN, #90," and asks for refills added; what should you do NEXT? **A:** Route to the pharmacist and do not add refills because alprazolam is a Schedule IV controlled substance and refills must match prescriber authorization.
4329. **Q:** An inhaler prescription says "tiotropium Respimat inhale 2 puffs daily" and the package provides 60 actuations; what days' supply should you bill? **A:** Bill 30 days because 2 puffs/day uses 2 actuations/day and $60 \div 2 = 30$ days.
4330. **Q:** You are stocking a delivery and notice insulin glargine pens in the tote are warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager because suspected temperature excursion makes the product potentially non-dispensable until verified per storage policy.
4331. **Q:** An eRx reads "metFORMIN 500 mg tab take ii po bid #120"; what quantity and days' supply should you enter? **A:** Enter quantity 120 and days' supply 30 because 2 tablets twice daily = 4 tablets/day and $120 \div 4 = 30$ days.
4332. **Q:** A prescription says "prednisoLONE oral solution 15 mg/5 mL: take 10 mg PO daily x 7 days"; what mL quantity should be dispensed? **A:** Dispense 23.3 mL because $15 \text{ mg}/5 \text{ mL} = 3 \text{ mg/mL}$, so $10 \text{ mg/day} \div 3 = 3.33 \text{ mL/day}$ and $3.33 \times 7 = 23.3 \text{ mL}$.
4333. **Q:** While typing "hydroxyzine 25 mg," you notice "HYDROXYZINE" is next to "hydralazine" on the shelf; what should you do BEST to prevent a LASA error? **A:** Use barcode/NDC scan and match NDC to the eRx/label before filling because hydralazine/hydroxyzine are look-alike names that commonly cause selection errors.
4334. **Q:** An order comes in for "Lantus Solostar inject 18 units SQ daily," and the box contains five 3 mL pens (100 units/mL); what days' supply should you bill? **A:** Bill 83 days because total units = $5 \times 3 \text{ mL} \times 100 = 1500$ units and $1500 \div 18 = 83.3$, rounded down to 83 for billing.

4335. **Q:** A new eRx for "methotrexate 2.5 mg take 6 tablets weekly" is entered as "6 tablets daily" by a trainee; what should you do FIRST? **A:** Stop the order entry and alert the pharmacist to correct the frequency because methotrexate is high-alert and daily vs weekly is a dangerous, common error.
4336. **Q:** A prescription reads "amLODIPine 10 mg take 1 tab po qd," but the patient asks if it's "Norvasc"; what should you do NEXT? **A:** Confirm that Norvasc is the brand name for amlodipine and proceed with the correct product selection because brand/generic recognition prevents dispensing the wrong drug.
4337. **Q:** The directions say "timolol 0.5% ophthalmic: instill 1 gtt OU BID," and the prescriber wrote "qd" on a second line; what should you do FIRST? **A:** Hold and clarify with the pharmacist/prescriber because conflicting sig frequencies (BID vs QD) must be resolved to avoid a dosing error.
4338. **Q:** A prescription is "atorvastatin 40 mg take 1 tab at bedtime #90"; the patient reports they already filled it 10 days ago at your pharmacy; what should you do BEST? **A:** Check the fill history and third-party "refill too soon" limits and route to the pharmacist if an early refill is requested because technicians should verify timing and prevent inappropriate duplicate dispensing.
4339. **Q:** You receive "amoxicillin/clavulanate 875/125 mg take 1 tab po bid x 10 days #20," but the prescriber accidentally sent "#10"; what should you do NEXT? **A:** Stop processing and notify the pharmacist to request a corrected quantity because the written quantity conflicts with the calculated need ($2/\text{day} \times 10 \text{ days} = 20$).
4340. **Q:** An eRx is for "oxycodone/APAP 5/325 take 1 tab q6h prn pain #120," but the prescriber notes "acute pain, 7 days"; what should you do BEST during order entry? **A:** Enter the directions as written, calculate a maximum days' supply of 30 ($120 \div 4/\text{day}$), and refer to the pharmacist because controlled-substance limits and acute-pain notes may require pharmacist review before dispensing.
4341. **Q:** A patient drops off a prescription for "Keflex 500 mg take 1 cap po qid x 7 days," and says they are allergic to cephalexin; what should you do FIRST? **A:** Stop processing and notify the pharmacist to assess the allergy and alternatives because cephalexin is the generic of Keflex and allergy verification is a safety-critical pharmacist function.
4342. **Q:** While entering a new order you see "Zestril 10 mg 1 tab po daily," but the patient profile already has lisinopril 10 mg active; what should you do BEST? **A:** Pause and alert the pharmacist about potential therapeutic duplication because

Zestril is lisinopril and duplicate ACE inhibitor therapy may indicate a duplicate prescription or confusion.

4343. **Q:** A prescriber sends "MS Contin 30 mg 1 tab po bid," and the patient says they were expecting immediate-release morphine for breakthrough pain; what should you do NEXT? **A:** Hold the order and route to the pharmacist to clarify because MS Contin is extended-release morphine and selecting the wrong formulation is a high-risk error.
4344. **Q:** An eRx arrives for "metoprolol succinate ER 50 mg daily," but the NDC scanned at filling is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop filling and correct the product selection by matching the NDC to the exact ER formulation because tartrate vs succinate are not interchangeable and scanning prevents a dispensing error.
4345. **Q:** A prescription reads "levothyroxine 0.125 mg take 1 tab po qam," and your system search shows both 125 mcg and 0.125 mg; what should you enter? **A:** Enter 125 mcg because 0.125 mg equals 125 mcg, preventing a decimal/place-value misunderstanding that could cause a dosing error.
4346. **Q:** A patient is newly prescribed "Plavix 75 mg daily" and asks what it's for; what should you do NEXT? **A:** Refer the patient to the pharmacist for counseling because Plavix (clopidogrel) is an antiplatelet and patient education on purpose and bleeding risk is required under counseling standards.
4347. **Q:** You receive a refill request for "Adderall XR 20 mg" with 0 refills remaining and the patient asks you to "just refill it"; what should you do BEST? **A:** Inform the patient a new prescription is needed and route to the pharmacist/pharmacy process to contact the prescriber because Adderall XR is a Schedule II controlled substance and cannot be refilled.
4348. **Q:** During intake you notice a bottle of gabapentin tablets with a torn seal and tablets loose in the bag from the wholesaler; what should you do FIRST? **A:** Quarantine the suspect product and notify the pharmacist/supervisor because compromised packaging can indicate contamination or tampering and it should not be stocked or dispensed.
4349. **Q:** A patient is prescribed "azithromycin suspension 200 mg/5 mL: take 10 mL day 1, then 5 mL daily days 2–5"; how many mL should be dispensed? **A:** Dispense 30 mL because total volume is 10 mL + (5 mL × 4 days) = 30 mL, ensuring the patient has the full course.

4350. **Q:** A tech trainee types "QD" as "QID" for "furosemide 20 mg qd," and the label prints with four times daily directions; what should you do FIRST? **A:** Stop the process and immediately correct the sig and notify the pharmacist if it reached verification because QD vs QID is a dangerous abbreviation error that can cause a major overdose.
4351. **Q:** A new eRx is for "glipizide 10 mg take 1 tab po daily," but the patient profile shows glyburide 5 mg daily active; what should you do BEST before filling? **A:** Stop and alert the pharmacist to possible duplicate sulfonylurea therapy because both lower glucose and duplication increases hypoglycemia risk.
4352. **Q:** During product selection you notice the prescriber wrote "Celebrex 200 mg daily," but you accidentally pulled Celexa 20 mg from the shelf; what should you do FIRST? **A:** Stop filling and reselect the correct drug using barcode/NDC verification because Celebrex (celecoxib) and Celexa (citalopram) are LASA and a mix-up can cause harm.
4353. **Q:** A patient picking up simvastatin says they also take amlodipine 10 mg and asks if it matters; what should you do NEXT? **A:** Refer to the pharmacist for interaction review because amlodipine can increase simvastatin levels and may require dose limits or therapy change.
4354. **Q:** A prescription is for "metformin ER 500 mg take 2 tabs po daily," and the bottle on the shelf is metformin IR 500 mg; what is the BEST technician action? **A:** Hold the fill and get the correct ER formulation (do not substitute IR) because IR vs ER is not interchangeable and affects dosing and tolerability.
4355. **Q:** You are entering "prednisone 20 mg: take ii tabs daily x 5 days"; how many tablets should be dispensed? **A:** Dispense 10 tablets because ii = 2 tablets/day for 5 days, so $2 \times 5 = 10$.
4356. **Q:** A new order is for "Humalog U-100: inject 12 units before meals TID," and the patient is getting one 10 mL vial; what days' supply should you enter? **A:** Enter 27 days because 10 mL = 1000 units and $12 \times 3 = 36$ units/day, so $1000 \div 36 \approx 27.7$ and days' supply is 27 (whole days).
4357. **Q:** A patient brings in an OTC request for "Sudafed" and asks for pseudoephedrine; what should you do FIRST at the counter? **A:** Request valid ID and begin the required sale log/limit process because pseudoephedrine sales are regulated under federal CMEA requirements.

4358. **Q:** While typing a new prescription you see “Lamictal 25 mg” but the prescriber’s note says “for seizures,” and the patient profile lists a serious rash reaction to lamotrigine; what should you do BEST? **A:** Stop processing and notify the pharmacist immediately because lamotrigine is associated with serious rash and documented allergy requires pharmacist intervention.
4359. **Q:** A refrigerated box of Ozempic pens arrives warm with a missing temperature indicator reading, and you can’t confirm cold-chain integrity; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager to contact the wholesaler because biologics needing refrigeration may be unsafe after a temperature excursion.
4360. **Q:** At pickup, a patient starting sertraline asks how long it takes to work for depression; what should you do NEXT? **A:** Refer to the pharmacist for counseling because efficacy/timeline and expectations are clinical counseling topics required for safe use.
4361. **Q:** While entering an eRx for hydromorphone 2 mg tablets, you notice the prescriber typed “take 1 tablet every 1 hour as needed pain,” what should you do FIRST? **A:** Stop processing and alert the pharmacist for dose/safety review because hydromorphone is high-alert and the frequency appears unsafe/possibly erroneous.
4362. **Q:** A prescription is written for “methotrexate 2.5 mg take 1 tablet daily for rheumatoid arthritis,” what is the BEST technician action before filling? **A:** Hold the order and refer to the pharmacist because methotrexate for RA is typically weekly and daily directions are a common high-harm error.
4363. **Q:** A patient’s new eRx is for clopidogrel 75 mg daily, but the allergy field lists “Plavix—hives,” what should you do NEXT? **A:** Stop and notify the pharmacist to assess the reported allergy and contact the prescriber if needed because clopidogrel is the same drug as Plavix.
4364. **Q:** A handwritten prescription for oxycodone/acetaminophen has the patient name, drug, and directions but no prescriber signature, what should you do FIRST? **A:** Do not fill and route to the pharmacist to resolve missing required elements because an unsigned prescription is not valid to dispense.
4365. **Q:** A patient asks to transfer a schedule II prescription (e.g., Adderall) from another pharmacy to yours, what should you do BEST? **A:** Refer to the pharmacist and explain it generally cannot be transferred like other meds

because C-II transfers are highly restricted and require pharmacist handling/policy compliance.

4366. **Q:** During receiving, you find a tote of atorvastatin tablets with bottles missing lot numbers on the label and you cannot verify pedigree information, what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager because DSCSA requires traceability and unverified product is suspect and unsafe to stock.
4367. **Q:** You are selecting product for risperiDONE 0.5 mg and the shelf has raNITidine 150 mg nearby with similar packaging, what is the BEST safety step before labeling? **A:** Scan the barcode and verify NDC/strength against the prescription because LASA lookalikes are a common dispensing error point.
4368. **Q:** A prescription is for "amoxicillin/clavulanate 875/125 mg: take 1 tablet twice daily x 7 days," what quantity should you dispense? **A:** Dispense 14 tablets because $2 \text{ tablets/day} \times 7 \text{ days} = 14 \text{ tablets}$.
4369. **Q:** A patient brings a product back saying "these tablets look different" and the bottle shows an NDC that doesn't match what was previously dispensed, what should you do FIRST? **A:** Stop the pickup/return process and involve the pharmacist to verify the medication/NDC because a possible dispensing or manufacturer change must be confirmed for safety.
4370. **Q:** A new prescription is for isotretinoin capsules and the patient is waiting at the counter, what should you do NEXT in workflow? **A:** Notify the pharmacist and follow iPLEDGE/REMS requirements before dispensing because isotretinoin has strict REMS controls and cannot be released without required documentation.
4371. **Q:** A prescription reads "levothyroxine 0.075 mg take 1 tab PO daily"; what strength should you enter in mg and mcg? **A:** Enter 0.075 mg = 75 mcg because $1 \text{ mg} = 1000 \text{ mcg}$ and converting prevents decimal-place dosing errors.
4372. **Q:** The sig says "metFORMIN 500 mg i po bid with meals #60"; what does "i po bid" mean and what days' supply is #60? **A:** "i po bid" means 1 tablet by mouth twice daily and #60 is 30 days' supply because 2 tablets/day makes $60 \div 2 = 30$.
4373. **Q:** An eRx is for "azithromycin 200 mg/5 mL: give 6 mL PO daily x 5 days"; how many mL should you dispense? **A:** Dispense 30 mL because $6 \text{ mL/day} \times 5 \text{ days} = 30 \text{ mL}$ to cover the full course.

4374. **Q:** A patient is prescribed amLODIPine 5 mg take 1 tab daily, but the profile already shows active amLODIPine 10 mg daily; what should you do FIRST? **A:** Stop and notify the pharmacist to assess possible duplicate therapy/dose change because technicians should not decide which strength is intended.
4375. **Q:** The directions say "predniSONE 20 mg: take ii tabs daily x 5 days"; what quantity should be entered and why? **A:** Enter quantity 10 tablets because "ii" = 2 tablets/day and $2 \times 5 = 10$ to match the written duration.
4376. **Q:** An inhaler order is "fluticasone/salmeterol 250/50 Diskus: 1 inhalation BID; dispense 1 inhaler (60 doses)"; what days' supply should you submit? **A:** Submit 30 days because 2 inhalations/day uses 60 doses in $60 \div 2 = 30$, which supports accurate billing.
4377. **Q:** A prescription is written "cephalexin 250 mg/5 mL: take 10 mL QID x 10 days"; what total volume is needed? **A:** Dispense 400 mL because $10 \text{ mL} \times 4/\text{day} \times 10 \text{ days} = 400 \text{ mL}$ to avoid underfilling.
4378. **Q:** While entering an order for "digoxin 0.125 mg daily," you accidentally selected digoxin 0.25 mg; what is the BEST step before proceeding? **A:** Stop and correct the strength, then re-verify NDC/barcode because digoxin is high-alert and strength selection errors can be harmful.
4379. **Q:** A patient brings a new prescription for "losartan 50 mg take 1 tab daily #90," but the prescriber wrote "refill x 5" on a Schedule III prescription on the same pad; what should you do NEXT? **A:** Process losartan normally but route the controlled-substance prescription details to the pharmacist because controlled refills and validity require pharmacist oversight.
4380. **Q:** An order reads "tobramycin ophthalmic solution: instill 2 gtt OU q4h x 7 days; dispense 5 mL"; what should you do FIRST to prevent a days' supply entry trap? **A:** Ask the pharmacist how your system/payer wants drops-to-mL conversion handled because drop size varies and technicians should follow pharmacy policy for accurate billing.
4381. **Q:** During data entry you see "Lantus Solostar 100 units/mL inject 18 units SQ nightly; dispense 1 box (5 pens of 3 mL)," what days' supply should you submit? **A:** Submit 83 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 18 = 83.3$, rounded down to 83 for billing.
4382. **Q:** A new eRx is for "Brilinta 90 mg take 1 tab BID," and the patient profile shows an active clopidogrel prescription; what should you do FIRST? **A:** Stop and notify

the pharmacist to review possible duplicate antiplatelet therapy because this is a high-bleed-risk combination that requires clinical confirmation.

4383. **Q:** While selecting product for "hydrOXYzine 25 mg," you notice hydrALAzine 25 mg is stored nearby and looks similar; what is the BEST technician step before filling? **A:** Verify the drug name/indication and scan the barcode/NDC against the label because hydrOXYzine vs hydrALAzine is a LASA mix-up with serious consequences.
4384. **Q:** An order is entered as "Lamictal 100 mg" but the hardcopy says "lamoTRIgine 100 mg," and the patient asks if it's the same; what should you do NEXT? **A:** Confirm the brand/generic equivalence (Lamictal = lamotrigine) and proceed per substitution rules because accurate recognition prevents unnecessary delays and confusion.
4385. **Q:** A patient drops off "Bactrim DS 1 tab BID x 7 days" and reports a severe sulfa allergy; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist immediately because sulfamethoxazole/trimethoprim is a sulfonamide and needs pharmacist-directed alternative therapy.
4386. **Q:** A prescription reads "traMADol 50 mg i tab po q6h prn pain #20," what schedule is this drug and what is your BEST next step if refills are requested? **A:** Recognize tramadol as Schedule IV and route refill authorization to the pharmacist/prescriber because controlled-substance refills must follow legal and pharmacy policy requirements.
4387. **Q:** A technician receives a voicemail asking to transfer "alprazolam" to another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist because controlled-substance transfers (even Schedule IV) require pharmacist involvement and documentation per policy and regulations.
4388. **Q:** The sig says "furosemide 40 mg: take 1 tab po qam," and the patient asks what it's for; what is the BEST brief response within technician scope? **A:** State it's a "water pill" often used for swelling or blood pressure and offer pharmacist counseling because technicians can give non-clinical general info but pharmacists handle full counseling.
4389. **Q:** An eRx is for "sertraline 50 mg daily," but you accidentally typed "sildenafil 50 mg"; what should you do FIRST when you catch it before verification? **A:** Stop, correct the entry to the original prescription, and recheck drug/strength because sertraline vs sildenafil is a common name mix-up that can cause major therapy errors.

4390. **Q:** A prescription is for "atorvastatin 40 mg take 1 tab daily #90," and the patient profile shows active simvastatin 40 mg daily; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess therapeutic duplication because two statins together increases adverse-effect risk and requires clinical direction.
4391. **Q:** An eRx says "levothyroxine 0.075 mg take 1 tab PO daily," what strength should you enter in mcg to avoid a decimal error? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1000 = 75 \text{ mcg}$ and converting prevents tenfold dosing mistakes in order entry.
4392. **Q:** A prescription reads "predniSONE 10 mg: take ii tabs PO BID x 5 days; dispense #?" what quantity should you calculate and enter? **A:** Dispense 20 tablets because ii = 2 and 2 tabs BID = 4 tabs/day, then $4 \times 5 \text{ days} = 20 \text{ tabs}$.
4393. **Q:** You are billing "fluticasone nasal spray 50 mcg: 2 sprays each nostril daily; dispense 1 bottle labeled 120 sprays," what days' supply should you submit? **A:** Submit 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \text{ sprays} \div 4 = 30 \text{ days}$ for correct third-party processing.
4394. **Q:** A patient's prescription is "latanoprost 0.005%: instill 1 gtt OU HS; dispense 2.5 mL," what should you enter as days' supply using 20 gtt/mL? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ gtt total}$ and $\text{OU HS} = 2 \text{ gtt/day}$, so $50 \div 2 = 25 \text{ days}$.
4395. **Q:** During order entry you see "morphine sulfate ER 30 mg take 1 tab q12h," but the prescriber note also says "do not crush," what is the BEST technician step? **A:** Ensure the product selected is ER (not IR) and route any formulation questions to the pharmacist because crushing warnings signal a dosage-form safety risk.
4396. **Q:** A hardcopy says "metFORMIN 500 mg tab take 1 tab PO BID with meals," but the entry screen defaulted to metformin ER, what should you do FIRST? **A:** Stop and correct the dosage form to the exact product written (IR vs ER) before it reaches pharmacist verification because ER/IR mix-ups are common order-entry errors.
4397. **Q:** A prescription states "amLODIPine/benazepril 5/20 mg take 1 cap daily," and you're unsure which strength is which, what is the correct interpretation for data entry? **A:** Enter 5 mg amlodipine with 20 mg benazepril because combo products list strengths in a fixed order and swapping them can create a wrong-dose error.

4398. **Q:** A patient is prescribed "cephalexin 500 mg 1 cap QID x 10 days," but the quantity field is typed as 30; what should you do NEXT? **A:** Change the quantity to 40 and document per workflow because QID for 10 days requires $4 \times 10 = 40$ capsules and mismatched quantity causes underfilling.
4399. **Q:** You receive an eRx for "oxyCODONE/APAP 5/325 mg 1 tab q6h PRN pain #24," and the prescriber did not include an address on the eRx display, what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to verify required prescriber information because controlled-substance validity issues must be resolved before dispensing.
4400. **Q:** While selecting product for "carBAMazepine 200 mg chewable," you only find immediate-release tablets on the shelf, what is the BEST next step? **A:** Pause filling and ask the pharmacist to confirm availability/substitution rules because chewable vs tablet is a dosage-form change the technician cannot decide independently.
4401. **Q:** A new eRx is for "hydroxyzine 25 mg 1 cap TID PRN itching," but the patient says they take "Atarax"; what is the BEST technician response? **A:** Confirm Atarax is the brand name for hydroxyzine and proceed with the correct generic/brand linkage because this avoids unnecessary delays while ensuring the same active ingredient.
4402. **Q:** While typing "celecoxib 200 mg daily," you notice the profile shows a serious sulfonamide allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist because celecoxib can be a sulfonamide-related medication and allergy assessment requires pharmacist judgment.
4403. **Q:** A prescription is written "Lantus 10 units SQ HS," and the product selected is Basaglar; what should you do NEXT before filling? **A:** Route to the pharmacist to confirm interchangeability per prescriber/payer and document appropriately because insulin products are high-alert and not always automatically substitutable.
4404. **Q:** You receive an order for "nitrofurantoin 100 mg," and the prescriber note says "for pyelonephritis"; what is the BEST technician action? **A:** Flag and refer to the pharmacist because nitrofurantoin is generally used for uncomplicated cystitis, and indication/site-of-infection mismatches need pharmacist review.
4405. **Q:** An eRx says "clopidogrel 75 mg daily," but the patient insists they were prescribed "Plavix"; what is the BEST way to handle this at intake? **A:** Explain that Plavix is the brand for clopidogrel and verify the strength and directions

match the eRx because correct brand/generic recognition prevents duplicate therapy and confusion.

4406. **Q:** The sig reads "Take 1 tab PO q6h prn pain" for ibuprofen 600 mg, and the prescriber wrote "#24"; what days' supply should you enter if the pharmacy rule is to calculate PRN at max frequency? **A:** Enter 6 days' supply because 1 tablet every 6 hours equals 4 tablets/day and $24 \div 4 = 6$.
4407. **Q:** During product selection for "buPROPion SR 150 mg," you find bupropion XL 150 mg in the same bin; what should you do FIRST? **A:** Stop and reselect the correct SR product and verify NDC/barcode because SR vs XL is a common LASA/modified-release error with different dosing behavior.
4408. **Q:** A new prescription is for "lisinopril 10 mg daily," and the profile shows an active prescription for "benazepril 10 mg daily"; what should you do FIRST? **A:** Hold the fill and notify the pharmacist to assess therapeutic duplication because both are ACE inhibitors and duplication can increase adverse effects.
4409. **Q:** A patient drops off a handwritten prescription that says "MSO4 15 mg q4h PRN," and your pharmacy policy discourages unsafe abbreviations; what is the BEST next step? **A:** Do not enter "MSO4" as written; refer to the pharmacist to clarify and rewrite the directions/drug name because "MSO4" is an error-prone abbreviation that can cause morphine vs magnesium sulfate mix-ups.
4410. **Q:** A prescription is for "azithromycin 250 mg: take 2 tabs today then 1 tab daily x 4 days," what total quantity should you enter? **A:** Enter 6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet daily for 4 more days, totaling $2 + 4 = 6$.
4411. **Q:** While entering an eRx for hydrALAZINE 25 mg TID, you notice the selected product is hydrOXYzine 25 mg due to a similar name; what should you do FIRST? **A:** Stop data entry and re-select using barcode/NDC verification because LASA mix-ups are common and correcting it before dispensing prevents a medication error.
4412. **Q:** A hardcopy prescription for alprazolam 0.5 mg has no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist for prescriber follow-up because missing required elements makes the order potentially invalid and unsafe to process.
4413. **Q:** A patient requests an early refill of Adderall XR 20 mg stating it was "lost," and the last fill was 10 days ago; what should you do NEXT? **A:** Refer to the

pharmacist and follow controlled-substance policy because early replacement requires pharmacist judgment and documentation.

4414. **Q:** During fill for levothyroxine 75 mcg, the stock bottle's label shows an expiration date has passed; what should you do FIRST? **A:** Remove the expired product from active stock and alert the pharmacist per policy because dispensing expired medication is unsafe and noncompliant.
4415. **Q:** You receive a wholesaler tote with a refrigerated insulin glargine shipment that feels warm and has no temperature indicator reading; what is the BEST next step? **A:** Isolate the shipment and notify the pharmacist/manager before stocking because a temperature excursion makes the product suspect and may require return or documentation.
4416. **Q:** A prescriber sends an eRx for isotretinoin 40 mg daily and the patient is waiting at the counter; what should you do FIRST? **A:** Immediately route the order to the pharmacist because isotretinoin has REMS (iPLEDGE) requirements that must be verified before dispensing.
4417. **Q:** A patient asks to buy pseudoephedrine 30 mg tablets at the pharmacy counter; what should you do NEXT before completing the sale? **A:** Request valid ID and complete the required log and quantity limits per CMEA because pseudoephedrine sales must be tracked and regulated.
4418. **Q:** A new prescription is for metformin 500 mg BID, and the profile shows cimetidine started yesterday; what is the BEST technician action? **A:** Flag the interaction for pharmacist review because cimetidine can increase metformin levels and requires clinical assessment before dispensing.
4419. **Q:** A patient says they developed hives and swelling after their first dose of amoxicillin filled yesterday; what should you do FIRST? **A:** Refer the patient to the pharmacist immediately and document the reaction because possible allergy/anaphylaxis risk needs urgent pharmacist triage and counseling.
4420. **Q:** An order is for vancomycin 1 g in 250 mL to infuse over 90 minutes; what pump rate in mL/hr should you enter? **A:** Enter 167 mL/hr because $250 \text{ mL} \div 1.5 \text{ hr} = 166.7 \text{ mL/hr}$, rounded to the nearest whole number for pump programming.
4421. **Q:** While typing an eRx for clonazepam 0.5 mg, you accidentally select clonidine 0.1 mg because the names look alike; what should you do FIRST? **A:** Stop processing and reselect the correct drug using NDC/barcode verification and

alert the pharmacist if it reached verification, because this is a LASA error with serious safety risk.

4422. **Q:** A new prescription is for lisinopril 20 mg daily, and the patient profile shows active losartan 100 mg daily; what is the BEST technician action? **A:** Pause the fill and refer to the pharmacist for therapeutic-duplication review, because dual ACE inhibitor/ARB therapy may be inappropriate and requires clinical judgment.
4423. **Q:** A patient picking up trimethoprim/sulfamethoxazole says they “get a rash from sulfa drugs”; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist immediately, because a reported sulfonamide allergy requires pharmacist assessment before dispensing.
4424. **Q:** You receive an order to dispense nitrofurantoin and the prescriber wrote “Macrochantin 100 mg BID” but you selected Macrobid 100 mg; what should you do NEXT? **A:** Stop and confirm the intended product with the pharmacist before proceeding, because Macrochantin and Macrobid are different formulations with different dosing.
4425. **Q:** An eRx for sertraline 50 mg daily is entered, and you notice the patient is also taking linezolid from a recent discharge; what is the BEST technician action? **A:** Do not continue processing and escalate to the pharmacist, because this combination can cause a serious interaction (serotonin syndrome risk) needing pharmacist intervention.
4426. **Q:** A prescription reads “levothyroxine 0.075 mg daily”; what strength should you enter in mcg? **A:** Enter 75 mcg, because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$ to match standard levothyroxine strengths.
4427. **Q:** The directions say “Take ii tablets twice daily” for gabapentin 300 mg and quantity is 120 tablets; what days’ supply should you enter? **A:** Enter 30 days, because ii BID = 4 tablets/day and $120 \div 4 = 30 \text{ days}$.
4428. **Q:** You are filling warfarin and the prescription is written for “warfarin 5 mg QD” but the profile shows an active warfarin 2.5 mg with different directions; what should you do BEST? **A:** Stop and refer to the pharmacist to clarify the current regimen, because duplicate anticoagulant entries can cause dangerous dosing errors.
4429. **Q:** During product selection you find two bottles of amlodipine 10 mg with different manufacturers, and one bottle has a different color/shape than the other; what should you do NEXT? **A:** Verify the NDC matches the selected

product and label, then proceed only if it matches, because manufacturer changes are common but NDC verification prevents wrong-drug/strength errors.

4430. **Q:** A new prescription is for glipizide 10 mg daily and the patient says they have had "low blood sugar episodes" recently; what should you do FIRST? **A:** Notify the pharmacist before completing the sale, because sulfonylureas are high-risk for hypoglycemia and require pharmacist counseling/intervention.
4431. **Q:** A new eRx comes in for metFORMIN ER 500 mg daily but the patient says they've only ever taken "Glucophage XR"; what should you do BEST? **A:** Confirm that Glucophage XR is the brand for metformin ER and proceed with the correct generic/brand linkage because it prevents duplicate therapy confusion at pickup.
4432. **Q:** While entering an eRx you see "hydroxyzine 25 mg TID prn itching," but you accidentally typed "hydrALAZINE"; what should you do FIRST? **A:** Stop processing and correct the entry using the original prescription and NDC verification because this is a LASA mix-up with a different indication and safety risk.
4433. **Q:** A prescription is written for "predniSONE 20 mg: take ii tabs daily x 5 days; dispense #?"; what quantity should you enter? **A:** Enter 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets for the full course}$.
4434. **Q:** A patient requests an early refill for oxyCODONE/APAP stating the bottle was stolen, and there are no refills on file; what should you do FIRST? **A:** Refer the request to the pharmacist and do not process a refill because controlled substances require prescriber authorization and careful assessment.
4435. **Q:** You are typing a new prescription for simvastatin, and the patient profile shows an active atorvastatin with recent fills; what is the BEST technician action? **A:** Pause and alert the pharmacist about possible therapeutic duplication because two statins together can increase adverse effect risk and needs pharmacist review.
4436. **Q:** A prescription reads "levETIRAcetam 750 mg PO BID" and the patient asks what it's for; what should you do NEXT? **A:** Refer to the pharmacist for counseling while you can note it's an anticonvulsant because OBRA-90 counseling is offered and clinical discussion is pharmacist-led.
4437. **Q:** You receive a stock bottle of insulin lispro that was left on the counter overnight and you're unsure of storage conditions; what should you do BEST? **A:** Quarantine the product and notify the pharmacist because a possible

temperature excursion makes it suspect and unsafe to dispense without confirmation.

4438. **Q:** The sig says "Instill 1 gtt OU BID" for latanoprost ophthalmic; what does "OU" mean for data entry? **A:** Enter "both eyes" because OU is the standard abbreviation for oculus uterque and ensures correct administration directions.
4439. **Q:** An inhaler prescription is for tiotropium Respimat "inhale 2 puffs once daily," and the device contains 60 actuations; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ actuations} \div 2 \text{ actuations/day} = 30 \text{ days}$.
4440. **Q:** A patient picking up clopidogrel says they are also taking "Prilosec OTC every day"; what should you do FIRST? **A:** Flag and notify the pharmacist because omeprazole can reduce clopidogrel activation and the interaction requires pharmacist assessment.
4441. **Q:** A prescription says "cephalexin 250 mg/5 mL: take 10 mL PO q6h x 7 days; dispense #?"; what quantity (mL) should you enter? **A:** Enter 280 mL because $10 \text{ mL per dose} \times 4 \text{ doses/day} \times 7 \text{ days} = 280 \text{ mL}$ for the full course.
4442. **Q:** You are entering "gabapentin 300 mg: take 1 cap PO TID; #270"; what days' supply should you submit to insurance? **A:** Submit 90 days because $270 \text{ capsules} \div 3 \text{ capsules/day} = 90 \text{ days}$, which matches the written quantity.
4443. **Q:** A patient brings a hardcopy for "Synthroid 50 mcg daily," but the prescriber wrote "levothyroxine 50 mg"; what should you do FIRST? **A:** Stop processing and send to the pharmacist to clarify because 50 mg vs 50 mcg is a dangerous dose-unit error and the technician cannot interpret/alter it.
4444. **Q:** An eRx for Lantus SoloStar states "inject 24 units SQ QHS," and the package size is 5 pens of 3 mL each (100 units/mL); what days' supply should you enter? **A:** Enter 62 days because $\text{total units} = 5 \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 24 = 62.5$, typically submitted as 62 days per payer rules.
4445. **Q:** You are typing "morphine sulfate ER 15 mg PO q12h" but the drop-down defaults to morphine sulfate IR; what is the BEST technician action? **A:** Select the correct ER formulation and verify NDC/label match before printing because IR vs ER is a high-risk dosage-form error requiring exact order entry.
4446. **Q:** A prescription reads "diltiazem CD 240 mg daily" and you select diltiazem immediate-release tablets in the system; what should you do NEXT? **A:** Correct the entry to the CD/ER product and alert the pharmacist if any labels were

printed because CD indicates extended-release and switching forms can cause dosing errors.

4447. **Q:** A sig says "Take 1 tab PO qd" for losartan 50 mg and the quantity is #30; what does "qd" mean for entry and what days' supply is it? **A:** Enter "once daily" and 30 days because qd = every day and $30 \text{ tablets} \div 1/\text{day} = 30 \text{ days}$, avoiding misreading qd as qid.
4448. **Q:** While entering a new prescription you see "U-500 insulin regular: inject 60 units BID," but the product selected is U-100 regular insulin; what should you do FIRST? **A:** Stop and involve the pharmacist immediately because U-500 vs U-100 is a high-alert concentration mismatch that can cause a 5-fold dosing error.
4449. **Q:** A patient's fluticasone nasal spray is "2 sprays in each nostril daily," and the bottle contains 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $\text{daily sprays} = 2 \text{ sprays/nostril} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
4450. **Q:** An order entry note shows "APAP 650 mg CR" but the prescriber wrote "acetaminophen 650 mg ER q8h prn," and the system offers IR 325 mg as the first option; what is the BEST technician action? **A:** Choose the 650 mg ER/CR product as written and confirm directions align because selecting 325 mg IR would create a strength/formulation error and change the intended regimen.
4451. **Q:** While typing a new eRx for hydrOXYzine 25 mg, you accidentally select hydrALAZINE 25 mg from the drop-down; what should you do NEXT? **A:** Stop and correct the drug selection, then re-verify NDC/barcode because this is a LASA error risk and accurate product selection prevents harm.
4452. **Q:** A new prescription is for methotrexate 2.5 mg with directions "take 6 tablets daily"; what should you do FIRST? **A:** Hold the order and notify the pharmacist immediately because methotrexate is high-alert and daily vs weekly dosing errors can be fatal.
4453. **Q:** A patient drops off a prescription for alprazolam 0.5 mg with "Refills: 3"; what is the BEST technician action? **A:** Stop processing and route to the pharmacist because Schedule IV refills must be valid and the pharmacist should verify appropriateness and legal limits before dispensing.
4454. **Q:** During filling you notice a stock bottle of insulin lispro that was left on the counter overnight and feels warm; what should you do FIRST? **A:** Remove it

from active stock and follow pharmacy policy to quarantine/label for pharmacist review because temperature excursions can compromise product integrity.

4455. **Q:** You receive a wholesaler notice that your pharmacy's lot of losartan tablets is recalled; what should you do NEXT? **A:** Pull the affected NDC/lot from inventory and segregate it from dispense-ready stock, then alert the pharmacist because recalled product must not be dispensed and must be controlled for return.
4456. **Q:** A patient asks to buy pseudoephedrine 30 mg tablets, 96 count, and wants to purchase two boxes; what should you do FIRST? **A:** Request valid ID and run the required electronic log/check before completing the sale because CMEA requires tracking and limits to reduce diversion.
4457. **Q:** A vial label reads "heparin 10,000 units/mL" but the order is for a heparin flush "10 units/mL"; what should you do BEST? **A:** Stop the fill and immediately involve the pharmacist because heparin is a high-alert medication and concentration mix-ups can cause serious bleeding.
4458. **Q:** An eRx for sertraline (Zoloft) is sent for a 7-year-old and includes a note "Medication Guide required"; what should you do NEXT? **A:** Ensure the Medication Guide prints and is included with dispensing because certain drugs require FDA patient information to be provided each fill.
4459. **Q:** You are billing oxycodone/acetaminophen 5/325 "1 tablet PO q6h" with quantity 28; what days' supply should you submit? **A:** Enter 7 days' supply because $28 \text{ tablets} \div 4 \text{ tablets/day} = 7 \text{ days}$, and accurate days' supply supports correct controlled-substance billing and safety checks.
4460. **Q:** A patient says they had hives and swelling after taking lisinopril and now you see a new prescription for enalapril; what should you do FIRST? **A:** Pause processing and notify the pharmacist because this may be an ACE-inhibitor class allergy/angioedema concern requiring pharmacist assessment before dispensing.
4461. **Q:** A prescription reads "levothyroxine 75 mcg PO daily" but the prescriber note also says "take 0.075 mg daily"; what should you enter for the strength and why? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1,000 \text{ mcg/mg} = 75 \text{ mcg}$, preventing a decimal/place-value dosing error.

4462. **Q:** You are entering amlodipine 5 mg "take 1 tablet daily" with quantity 90; what days' supply should you submit? **A:** Submit 90 days because $90 \text{ tablets} \div 1 \text{ tablet/day} = 90 \text{ days}$ for accurate billing and refill timing.
4463. **Q:** A patient's Rx is for albuterol HFA "inhale 2 puffs q4-6h prn wheeze," and the prescriber adds "max 8 puffs/day"; the inhaler has 200 puffs; what days' supply should you enter using the max daily dose? **A:** Enter 25 days because $200 \text{ puffs} \div 8 \text{ puffs/day} = 25 \text{ days}$, using the stated maximum to avoid overestimating days' supply.
4464. **Q:** An eRx for metformin reads "500 mg tab ii po bid"; what is the total daily dose in mg you should confirm matches the prescriber intent? **A:** Confirm 2 tablets twice daily = 4 tablets/day, so $4 \times 500 \text{ mg} = 2,000 \text{ mg/day}$ to ensure the sig was interpreted correctly.
4465. **Q:** A prescription says "predniSONE 10 mg: take ½ tab daily" and quantity is 15 tablets; what days' supply should you enter? **A:** Enter 30 days because $15 \text{ tablets} \div 0.5 \text{ tablet/day} = 30 \text{ days}$, matching the dispensed quantity to the fractional dose.
4466. **Q:** While typing, you see "MSO4 2 mg/mL" in the notes for a compounded oral solution; what should you do FIRST before completing entry? **A:** Stop and ask the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can be confused with magnesium sulfate, risking a serious LASA error.
4467. **Q:** A cephalexin suspension label direction is "5 mL PO TID x 10 days" and the bottle concentration is 250 mg/5 mL; what total volume should be dispensed? **A:** Dispense 150 mL because $5 \text{ mL/dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$, ensuring enough medication for the full course.
4468. **Q:** You receive an eRx for "Lantus SoloStar inject 18 units subcutaneously nightly" and the box contains five 3 mL pens (100 units/mL); what days' supply should you submit based on total units? **A:** Submit 83 days because total units = $5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1,500 \text{ units}$, and $1,500 \div 18 = 83.3 \rightarrow 83 \text{ days}$ to avoid overstating coverage.
4469. **Q:** During order entry for "bupropion SR 150 mg" you notice the prescriber wrote "take 1 tablet TID"; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because SR formulations are typically not dosed three times daily, suggesting a potentially unsafe sig or wrong formulation.

4470. **Q:** An eRx for "rosuvastatin 20 mg" comes through, but the patient profile already shows active "atorvastatin 40 mg daily"; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to evaluate therapeutic duplication because two statins together may indicate an unintended duplicate therapy or transition error.
4471. **Q:** While filling hydrOXYzine 25 mg you almost select hydrALAZINE 25 mg because the bottles look similar; what should you do FIRST to prevent a LASA error? **A:** Stop and verify the drug using barcode/NDC and the hard copy/eRx before continuing because look-alike/sound-alike pairs require extra product selection checks.
4472. **Q:** A new eRx for methotrexate 2.5 mg says "take 1 tablet daily for rheumatoid arthritis"; what is the BEST technician action? **A:** Hold processing and immediately alert the pharmacist because methotrexate is typically weekly for RA and daily dosing is a high-alert error risk.
4473. **Q:** A patient presents a prescription for alprazolam 1 mg with "Refills: 3" and you suspect it's Schedule IV; what should you do NEXT? **A:** Route to the pharmacist to verify controlled-substance refill compliance because Schedule III–V refills are limited by federal rules and must be checked before dispensing.
4474. **Q:** A prescriber calls in a hydromorphone prescription and asks you to take it as a verbal order over the phone; what should you do FIRST? **A:** Transfer the call to the pharmacist because taking verbal prescriptions, especially for controlled substances, requires pharmacist judgment and authorization.
4475. **Q:** You receive a tote of refrigerated insulin aspart that was left on the receiving dock at room temperature for an unknown time; what should you do BEST? **A:** Segregate and label it as suspect and notify the pharmacist/manager to follow temperature-excursion policy because product integrity may be compromised.
4476. **Q:** During intake you notice a recalled lot number for losartan tablets matches bottles on the shelf; what should you do NEXT? **A:** Pull the affected lot from stock and isolate it per recall procedure and alert the pharmacist because recalled product must not be dispensed and must be removed from inventory.
4477. **Q:** An eRx for isotretinoin arrives and the patient is waiting, but the system shows REMS requirements not yet met; what should you do FIRST? **A:** Stop the fill and refer to the pharmacist because isotretinoin dispensing is restricted under a REMS and must meet specific authorization steps before sale.

4478. **Q:** A caregiver asks you to discuss a patient's new sertraline prescription details at the pickup counter, but you cannot confirm permission; what is the BEST action? **A:** Move the conversation to a private area and involve the pharmacist or verify authorized disclosure because HIPAA requires protecting PHI and limiting who receives medication information.
4479. **Q:** A prescription is written for clonidine 0.1 mg "take 1 tab BID" quantity 60; what days' supply should you enter? **A:** Enter 30 days because 2 tablets/day means $60 \div 2 = 30$, which supports accurate billing and refill timing.
4480. **Q:** A patient requests pseudoephedrine 30 mg tablets and wants 3 boxes, but the log shows they already purchased near the daily limit today; what should you do NEXT? **A:** Refuse the sale and follow the electronic log and pharmacist/policy guidance because CMEA limits must be enforced and sales over the limit cannot be completed.
4481. **Q:** While entering a new eRx for metFORMIN, you see the drug selected as metFORMIN ER but the prescriber wrote "metformin 500 mg tab take 1 BID"; what should you do FIRST? **A:** Stop and clarify with the pharmacist/prescriber before filling because IR vs ER is a formulation change that affects dosing and cannot be assumed by the technician.
4482. **Q:** A patient drops off a new prescription for lisinopril and their allergy profile lists "ACE inhibitors—angioedema"; what is the BEST technician action? **A:** Hold the prescription and immediately alert the pharmacist because lisinopril is an ACE inhibitor and angioedema is a serious contraindication requiring pharmacist intervention.
4483. **Q:** During product selection you notice "lamoTRIGine" and "terbinafine" stored next to each other and the NDC you scanned doesn't match the label; what should you do NEXT? **A:** Stop filling and reselect the correct product by matching NDC/barcode and drug name/strength because an NDC mismatch indicates a wrong-drug error risk (common LASA trap).
4484. **Q:** An eRx comes in for clarithromycin and the patient profile shows simvastatin 40 mg daily; what should you do FIRST? **A:** Pause processing and notify the pharmacist because clarithromycin can dangerously increase simvastatin levels (major interaction) and needs pharmacist direction.
4485. **Q:** A prescription reads "Levemir 100 units/mL inject 20 units QHS" and the patient is receiving one 10 mL vial; what days' supply should you enter? **A:** Enter

50 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $1000 \text{ units} \div 20 \text{ units/day} = 50 \text{ days}$.

4486. **Q:** A new prescription for gabapentin is written "take i cap po tid" but the strength is missing on the hardcopy; what should you do BEST? **A:** Do not enter or fill it and route to the pharmacist to contact the prescriber because a missing strength makes the order incomplete and unsafe.
4487. **Q:** At pickup, a patient says their new zolpidem "didn't work" and asks if they can take two tablets nightly; what should you do NEXT? **A:** Refer them to the pharmacist for counseling because changing dose for a CNS sedative is a clinical decision and a safety concern (sedation/falls).
4488. **Q:** You receive a prescription for "Keflex" and the patient profile lists "cephalexin"; what is the most appropriate way to enter the drug? **A:** Enter cephalexin because Keflex is the brand for cephalexin and correct brand/generic recognition prevents wrong-drug selection.
4489. **Q:** A patient brings a new prescription for apixaban and asks what it's for and what to watch for; what should you do FIRST? **A:** Offer pharmacist counseling because apixaban is a high-risk anticoagulant and patients need pharmacist-provided education on bleeding precautions.
4490. **Q:** A prescription reads "amLODIPine 5 mg take 1 tab qd" with quantity 90; what days' supply should you enter? **A:** Enter 90 days because once-daily dosing means 90 tablets supply 90 days.
4491. **Q:** A prescriber calls in "Zithromax Z-Pak use as directed" for a patient; what should you do FIRST before entering it? **A:** Ask for the complete directions or clarify with the pharmacist/prescriber because "use as directed" is not specific enough for accurate labeling and days' supply.
4492. **Q:** While entering a new Rx you see "Lasix 40 mg PO daily" but the drug selected is torsemide; what is the BEST technician action? **A:** Stop and correct the selection to furosemide (Lasix) or route to the pharmacist to clarify because Lasix is brand for furosemide and choosing torsemide is a wrong-drug error.
4493. **Q:** A patient profile shows an active prescription for omeprazole, and a new eRx arrives for Nexium; what should you do NEXT? **A:** Flag it for pharmacist review as potential therapeutic duplication because Nexium is esomeprazole and both are PPIs.

4494. **Q:** You receive an eRx for "Synthroid 75 mcg daily DAW 1" and the system auto-substitutes levothyroxine generic; what should you do FIRST? **A:** Enter it as brand Synthroid with DAW honored because DAW 1 indicates prescriber-required brand and the claim/label must match.
4495. **Q:** The sig says "Take ii tabs po qhs" on a new prescription for trazodone 50 mg; how should you interpret it for entry? **A:** Enter "Take 2 tablets by mouth at bedtime" because ii = 2, po = by mouth, and qhs = at bedtime to prevent misinterpretation.
4496. **Q:** A prescription is for sertraline 25 mg "take 1 tablet daily" quantity 30; what days' supply should you enter? **A:** Enter 30 days' supply because 30 tablets taken 1 per day lasts 30 days.
4497. **Q:** During product selection for hydroXYzine, you notice the label prints "hydrALAZINE"; what should you do NEXT? **A:** Stop filling and correct the drug/label selection, then re-verify NDC/barcode because hydroxyzine and hydralazine are LASA and this indicates a high-risk wrong-drug error.
4498. **Q:** A new prescription for tramadol is sent with "refills: 5"; what is the BEST next step for the technician? **A:** Hold processing and notify the pharmacist because tramadol is Schedule IV and refills may be limited and require pharmacist resolution.
4499. **Q:** A refrigerator alarm shows insulin aspart was stored at room temperature overnight; what should you do FIRST? **A:** Segregate the affected stock and alert the pharmacist to assess stability and follow policy because temperature excursions can make products non-dispensable.
4500. **Q:** A patient asks if "Cipro" is an antibiotic and what it's usually used for; what should you do FIRST? **A:** Refer the question to the pharmacist for counseling because technicians can recognize ciprofloxacin as an antibiotic but must not provide clinical counseling.
4501. **Q:** A patient picking up metformin asks what it's for and mentions frequent urination and thirst; what should you do FIRST? **A:** Refer the patient to the pharmacist for counseling because metformin is for type 2 diabetes and those symptoms may indicate uncontrolled blood sugar needing clinical assessment.
4502. **Q:** While entering an eRx you see "glipiZIDE 10 mg ER" but the selected product is glipizide immediate-release; what is the BEST technician action? **A:** Stop and

reselect the correct ER formulation (verify NDC/label) because IR vs ER is a high-risk mismatch that changes dosing and effect.

4503. **Q:** A new eRx arrives for atorvastatin, and the profile shows simvastatin is active from another prescriber; what should you do NEXT? **A:** Pause processing and alert the pharmacist to possible duplicate statin therapy because duplication increases adverse effect risk and needs pharmacist review.
4504. **Q:** A prescription is written for "predniSONE 10 mg take 4 tabs daily x 5 days" with quantity 20; what days' supply should you enter? **A:** Enter 5 days' supply because 4 tablets/day for 5 days uses 20 tablets total ($20 \div 4 = 5$).
4505. **Q:** During data entry for morphine sulfate ER, you notice the directions say "take 1 tablet every 4 hours as needed for pain"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because ER opioids should not be dosed q4h PRN and this suggests a potentially unsafe/incorrect order.
4506. **Q:** A caregiver asks for "Tylenol with codeine" for a child and you locate multiple strengths (e.g., #2, #3, #4); what should you do NEXT before dispensing? **A:** Confirm the exact strength on the prescription and route any ambiguity to the pharmacist because codeine products differ in opioid content and are error-prone.
4507. **Q:** A technician opens a stock bottle and finds two different-looking tablets mixed in an unlabeled amber vial marked "sertraline"; what should you do FIRST? **A:** Quarantine the container and notify the pharmacist because mixed/unknown identity is a suspect product situation requiring investigation and prevents misfill.
4508. **Q:** An eRx comes in for "metoprolol succinate 50 mg daily" but the patient insists they take "metoprolol tartrate twice daily"; what should you do FIRST? **A:** Ask the pharmacist to review with the patient/prescriber because tartrate vs succinate are not interchangeable and dosing frequency differs.
4509. **Q:** A new prescription reads "doxycycline 100 mg i cap po bid" quantity 14; what should you enter for the sig and days' supply? **A:** Enter "Take 1 capsule by mouth twice daily" and 7 days' supply because "i" means 1 and $14 \text{ capsules} \div 2 \text{ per day} = 7 \text{ days}$.
4510. **Q:** While scanning an NDC for warfarin, you accidentally select the look-alike bottle for levothyroxine and catch it before labeling; what should you do BEST? **A:** Stop filling, select the correct product using barcode/NDC and Tall Man

awareness, and continue workflow because warfarin/levothyroxine is a high-risk LASA pair and must be corrected immediately.

4511. **Q:** A prescription is written for hydromorphone 2 mg “take 1 tablet by mouth every 4–6 hours” but no diagnosis or “PRN” is listed; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify appropriate directions because hydromorphone is a high-alert opioid and ambiguous sigs increase overdose risk.
4512. **Q:** During product selection you notice hydroxyzine and hydralazine are stored next to each other and the NDC on the screen doesn’t match the bottle in hand; what is the BEST next step? **A:** Do not proceed; reselect the correct product and verify by barcode/NDC match because LASA mix-ups are prevented by matching the exact NDC to the label.
4513. **Q:** A patient’s new eRx is for lisinopril, and their allergy profile lists “ACE inhibitors—angioedema”; what should you do NEXT? **A:** Pause the fill and alert the pharmacist for intervention because ACE inhibitor allergy with angioedema is a serious contraindication requiring pharmacist review.
4514. **Q:** A patient presents a paper prescription for alprazolam with missing patient address and an unclear prescriber signature; what should you do FIRST? **A:** Hold the prescription and route it to the pharmacist to resolve validity concerns because controlled-substance prescriptions must be complete and authentic before dispensing.
4515. **Q:** An eRx for Adderall (amphetamine/dextroamphetamine) is received and the patient asks to transfer it to another pharmacy across town; what should you do BEST? **A:** Refer to the pharmacist because Schedule II controlled substance transfer/dispensing rules are strict and require pharmacist handling to stay compliant.
4516. **Q:** A manufacturer recall notice arrives for a specific lot of losartan 50 mg due to contamination; what should you do FIRST in the pharmacy workflow? **A:** Pull and isolate the recalled lot from usable inventory and notify the pharmacist because recalled products must be removed from dispensing stock to prevent patient harm.
4517. **Q:** A tote arrives with refrigerated Ozempic (semaglutide) pens but the cold pack is warm and no temperature indicator is present; what should you do NEXT? **A:** Quarantine the shipment and alert the pharmacist/manager because suspected

temperature excursion makes the product potentially non-dispensable until verified.

4518. **Q:** A patient requests pseudoephedrine 30 mg tablets, 96-count, and shows an out-of-state ID; what should you do FIRST under CMEA workflow? **A:** Follow pharmacy policy to verify acceptable identification and complete the required log before sale because pseudoephedrine sales require documentation and legal limits.
4519. **Q:** You are entering directions for insulin lispro U-100: "inject 8 units subcutaneously before meals TID" and the quantity is one 10 mL vial; what days' supply should you enter? **A:** Enter 41 days because $10\text{ mL} = 1000\text{ units}$ and $8\text{ units} \times 3/\text{day} = 24\text{ units/day}$, so $1000 \div 24 = 41.6$ and you enter 41 to avoid overstating supply.
4520. **Q:** A patient picking up isotretinoin asks to pick up without the pharmacist because they are in a hurry; what should you do BEST? **A:** Require pharmacist involvement before pickup because isotretinoin has REMS requirements and must be dispensed only after mandated checks are completed for safety and compliance.
4521. **Q:** A new eRx is for Glucophage 500 mg and the patient says they "only take metformin"; what should you do BEST during order entry? **A:** Select metformin as the generic for Glucophage and confirm strength/form, because brand/generic equivalence is a common recognition task that prevents dispensing the wrong product.
4522. **Q:** A prescription reads "Augmentin 875/125 mg i tab po bid x 10 days"; what should you enter for quantity and what key concept helps you interpret the name? **A:** Enter quantity 20 tablets and recognize "Augmentin" as amoxicillin/clavulanate, because "875/125" indicates a fixed-dose combo per tablet.
4523. **Q:** You receive an order for "Celebrex 200 mg daily" but the patient profile shows a severe sulfonamide antibiotic allergy; what should you do NEXT? **A:** Stop and alert the pharmacist to review, because celecoxib has a sulfonamide moiety and allergy relevance requires pharmacist assessment.
4524. **Q:** While typing "Humalog" you notice the selection list includes "Humulin R" and "Humalog"; what is the BEST technician step to avoid a high-alert insulin mix-up? **A:** Verify the exact brand/generic name and insulin type (lispro vs regular)

against the prescription before selecting, because insulin is high-alert and look-alike names can cause serious harm.

4525. **Q:** A new prescription says "prednisone 20 mg ii tabs po qd x 5 days"; what should you enter for the sig and days' supply? **A:** Enter "Take 2 tablets by mouth once daily" with 5 days' supply, because "ii" means 2 and "qd" means once daily.
4526. **Q:** A patient has an active profile for atorvastatin and a new eRx arrives for Lipitor 40 mg daily; what should you do FIRST? **A:** Check the patient's active medications and route to the pharmacist if it appears to be a duplicate therapy, because Lipitor is atorvastatin and duplication can lead to double dosing.
4527. **Q:** The directions read "use 2 puffs bid" for Symbicort 160/4.5 and the quantity is one 120-inhalation inhaler; what days' supply should you enter? **A:** Enter 30 days' supply, because 2 puffs twice daily equals 4 puffs/day and $120 \div 4 = 30$ days.
4528. **Q:** During data entry you see "Zyrtec 10 mg" on the prescription but the patient asks for "cetirizine"; what should you do BEST? **A:** Process cetirizine as the generic equivalent to Zyrtec and follow substitution rules/patient preference, because fast brand/generic recognition reduces errors and improves workflow.
4529. **Q:** A controlled substance refill request comes in for clonazepam but the prescription has no remaining refills; what should you do NEXT? **A:** Hold the request and notify the pharmacist to contact the prescriber, because technicians cannot authorize refills for controlled substances without a valid order.
4530. **Q:** A bottle label says "KCl 20 mEq" and another says "K-Phos Neutral," and a new order is for potassium chloride; what is the BEST product-selection action? **A:** Match the exact drug name, dosage form, and NDC/barcode to the order before filling, because similar potassium products are not interchangeable and selection errors can cause harm.
4531. **Q:** An eRx for levothyroxine shows "take 1 tab po qd" with a quantity of 90; what days' supply should you enter and why? **A:** Enter 90 days because 1 tablet daily means quantity 90 lasts 90 days ($\text{qty} \div \text{doses per day}$).
4532. **Q:** A prescription says "furosemide 20 mg: i tab po bid" and the prescriber wrote #60; what days' supply should you calculate for order entry? **A:** Enter 30 days because BID is 2 tablets/day and $60 \div 2 = 30$, preventing incorrect refill-too-soon rejects.

4533. **Q:** You receive "latanoprost 0.005%: instill 1 gtt OU qhs" and the quantity is 2.5 mL; what days' supply should you enter using 20 gtt/mL? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ drops}$ and OU qhs uses 2 drops/day, so $50 \div 2 = 25$.
4534. **Q:** A new order reads "azithromycin 250 mg: take ii tabs po day 1 then i tab po days 2–5"; what quantity should you enter and why? **A:** Enter quantity 6 tablets because the total is 2 on day 1 plus 1 daily for 4 more days ($2 + 4 = 6$), matching the standard Z-Pak regimen.
4535. **Q:** During data entry you see "MSO4 2 mg IV q2h PRN pain" on a hospital discharge med list; what should you do FIRST? **A:** Stop and ask the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can be misread, risking a serious opioid error.
4536. **Q:** An eRx is for "hydralazine 25 mg" but you accidentally selected "hydroxyzine 25 mg" in the pick list; what is the BEST technician step before continuing? **A:** Stop filling and correct the drug selection by verifying the exact name/strength and NDC/barcode because hydralazine/hydroxyzine is a common LASA mix-up.
4537. **Q:** A prescription says "amoxicillin 400 mg/5 mL: take 7.5 mL po bid x 10 days"; what quantity (mL) should you enter? **A:** Enter 150 mL because $7.5 \text{ mL per dose} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$, ensuring the patient has enough for the full course.
4538. **Q:** A new eRx is for "Plavix 75 mg daily" and the profile shows "clopidogrel 75 mg daily" active; what should you do FIRST? **A:** Check the patient's profile for therapeutic duplication and route to the pharmacist if needed because Plavix is the brand for clopidogrel and duplicate therapy can cause harm.
4539. **Q:** You are entering "clindamycin 150 mg: i cap po qid x 7 days"; what quantity and days' supply should be entered? **A:** Enter quantity 28 and days' supply 7 because QID is 4/day and $4 \times 7 = 28$, aligning quantity with the intended duration.
4540. **Q:** A patient brings a new prescription for "Adderall XR 20 mg" but the prescriber wrote "take 1 capsule po bid"; what should you do NEXT as the technician? **A:** Hold processing and alert the pharmacist for verification because XR products are typically not dosed BID and the sig may be inappropriate or entered incorrectly, requiring pharmacist intervention.

4541. **Q:** An eRx says "metformin ER 500 mg: take 2 tabs po qpm with dinner" and quantity is 180; what days' supply should you enter? **A:** Enter 90 days' supply because 2 tablets/day means $180 \text{ tablets} \div 2 \text{ per day} = 90 \text{ days}$ for accurate billing and refill timing.
4542. **Q:** A prescription reads "predniSONE 10 mg: ii tabs po qd x 5 days" but the quantity field shows #5; what should you do FIRST? **A:** Stop processing and send to the pharmacist to clarify because the sig requires 10 tablets ($2/\text{day} \times 5 \text{ days}$) and the written quantity conflicts.
4543. **Q:** The sig is "Take i tab po qod" for sertraline 50 mg and the quantity is 15; what days' supply should you enter? **A:** Enter 30 days' supply because qod is every other day, so $15 \text{ tablets last } 15 \times 2 = 30 \text{ days}$.
4544. **Q:** A patient's Rx is "amLODIPine 5 mg: take 1 tab po qd" and you see Norvasc 5 mg selected; what is the BEST order-entry action? **A:** Confirm it as a brand/generic equivalent (Norvasc = amlodipine) and ensure the NDC matches the product being dispensed to prevent selection errors.
4545. **Q:** An eRx is for "ondansetron ODT 4 mg: dissolve 1 tab on tongue q8h prn nausea" with quantity 30; what maximum days' supply should you enter for order entry? **A:** Enter 10 days' supply using the maximum frequency because q8h allows up to 3 tablets/day and $30 \div 3 = 10 \text{ days}$ for payer-acceptable days' supply.
4546. **Q:** A prescription says "cephalexin 250 mg/5 mL: take 10 mL po tid x 7 days"; what quantity in mL should be dispensed? **A:** Dispense 210 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$, ensuring enough medication for the full course.
4547. **Q:** You are entering insulin lispro U-100 in a 10 mL vial with directions "inject 12 units before meals tid"; what days' supply should you calculate? **A:** Enter 27 days' supply because the vial contains $100 \text{ units/mL} \times 10 \text{ mL} = 1000 \text{ units}$, and $12 \text{ units} \times 3/\text{day} = 36 \text{ units/day}$, so $1000 \div 36 = 27 \text{ days}$ (rounded down to whole days).
4548. **Q:** A prescription is for albuterol HFA 90 mcg: "inhale 2 puffs q6h prn" and the canister is 200 actuations; what days' supply should you enter using maximum dosing? **A:** Enter 25 days' supply because q6h max is 4 times/day, 2 puffs each = 8 puffs/day, and $200 \div 8 = 25 \text{ days}$ for accurate claim submission.

4549. **Q:** During data entry you see "U-500 insulin regular: inject 20 units bid" but the product field is set to U-100 regular insulin; what should you do NEXT? **A:** Stop and notify the pharmacist immediately because U-500 vs U-100 is a high-alert concentration mismatch that can cause a 5-fold dosing error.
4550. **Q:** A prescriber writes "warfarin 5 mg: take as directed" and the quantity is 30 with no specific daily dose; what should you do FIRST for days' supply entry? **A:** Refer to the pharmacist to clarify dosing or confirm documented directions because "as directed" lacks a calculable daily use, making days' supply and labeling unsafe.
4551. **Q:** A prescription says "levothyroxine 50 mcg (Synthroid): take 1 tab po qam" but the product selected is liothyronine 5 mcg; what should you do FIRST? **A:** Stop processing and re-select the correct generic (levothyroxine) and strength, then route to pharmacist if already verified, because these are different thyroid hormones and a LASA-type selection error risks harm.
4552. **Q:** While entering "lisinopril 20 mg: take 1 tab po qd," the patient profile shows active losartan 50 mg daily; what is the BEST next step? **A:** Pause and alert the pharmacist to assess duplication/ACEI-ARB safety, because technicians should not decide therapy changes when two BP agents may be intentionally or unintentionally duplicated.
4553. **Q:** An eRx is for "Augmentin 875/125 mg" but the product selected is amoxicillin 875 mg; what should you do NEXT? **A:** Correct the selection to amoxicillin/clavulanate (Augmentin) and verify NDC/strength, because missing clavulanate changes the drug and can lead to treatment failure.
4554. **Q:** A hard copy reads "hydroxyzine 25 mg: take 1 tab po tid prn itching" but the tech accidentally chooses hydralazine 25 mg; what is the BEST action? **A:** Stop and re-enter with the correct medication (hydroxyzine) before filling, because hydralazine vs hydroxyzine is a common LASA mix-up with very different uses.
4555. **Q:** A prescription is written for "Vyvanse 30 mg: take 1 cap po qam" and the patient asks if it can be refilled early because they are "out after 10 days"; what should you do FIRST? **A:** Check the fill history and days' supply then refer to the pharmacist for controlled-substance early-refill evaluation, because stimulant controls require careful validity review and techs can't authorize early refills.
4556. **Q:** Order entry shows "morphine ER 30 mg: take 1 tab po bid" but the selected product is morphine IR 30 mg; what should you do NEXT? **A:** Stop and select

the correct ER formulation and confirm the sig matches, because ER vs IR is a high-risk release-mechanism error that can cause overdose or undertreatment.

4557. **Q:** A patient is prescribed "azithromycin 250 mg: take 2 tabs day 1, then 1 tab daily days 2–5"; what quantity should you enter? **A:** Enter quantity 6 tablets, because the total is $2 + 1 + 1 + 1 + 1 = 6$ for the standard 5-day regimen.
4558. **Q:** A prescription says "timolol 0.5% ophthalmic: instill 1 gtt OU bid" and you are asked for days' supply for a 5 mL bottle using 20 gtt/mL; what should you enter? **A:** Enter 25 days, because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$ and dosing is $1 \text{ gtt} \times 2 \text{ eyes} \times 2/\text{day} = 4 \text{ gtt/day}$ so $100 \div 4 = 25 \text{ days}$.
4559. **Q:** During data entry you see "HCTZ 25 mg" on an older profile note and the new prescription is for "hydralazine 25 mg tid"; what should you do BEST to avoid a name confusion error? **A:** Read back and match the full drug name, indication, and NDC during selection and flag for pharmacist verification if uncertain, because HCTZ and hydralazine can be confused but are different classes and dosing patterns.
4560. **Q:** A prescriber writes "nitroglycerin SL 0.4 mg: take 1 tab under tongue prn chest pain" and the patient profile shows sildenafil; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist immediately, because nitrates and PDE5 inhibitors are a dangerous interaction that requires pharmacist intervention before dispensing.
4561. **Q:** A new eRx is for clopidogrel 75 mg daily, but the product selected is clozapine 25 mg; what should you do FIRST? **A:** Stop processing and reselect the correct NDC using barcode/NDC verification and alert the pharmacist if the error progressed, because this is a high-risk LASA selection error.
4562. **Q:** A patient drops off a prescription for oxycodone/APAP and asks you to transfer it to another pharmacy; what is the BEST response? **A:** Refer the request to the pharmacist, because controlled-substance transfer rules are restricted and require pharmacist handling per federal/state policy.
4563. **Q:** While filing invoices, you notice a wholesaler tote includes a bottle with the seal broken and tablets loose; what should you do NEXT? **A:** Remove it from stock and quarantine/hold it for pharmacist review and return processing, because suspect or tampered product must not be dispensed.
4564. **Q:** An order is for metformin ER 500 mg, but the shelf label and NDC on the bottle are metformin IR 500 mg; what should you do FIRST? **A:** Stop the fill and

obtain the correct ER product/NDC, because IR vs ER is a dosage-form error that can cause therapeutic failure or adverse effects.

4565. **Q:** A prescriber sends "alprazolam 0.5 mg: take 1 tab tid, #90, 5 refills"; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist, because Schedule IV controlled substances cannot be dispensed with refills beyond the federal limit of 5 within 6 months.
4566. **Q:** You are asked to calculate days' supply for fluticasone nasal spray 50 mcg with "2 sprays each nostril once daily," bottle contains 120 sprays; what should you enter? **A:** Enter 30 days, because $2 \text{ sprays} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
4567. **Q:** During counseling pickup, a patient says they are allergic to aspirin but the new prescription is for naproxen; what should you do FIRST? **A:** Stop the transaction and refer to the pharmacist for allergy cross-sensitivity assessment, because NSAID hypersensitivity may require clinical judgment.
4568. **Q:** A refrigerator temperature log shows 12°C for the last shift where insulin and GLP-1 pens are stored; what should you do NEXT? **A:** Isolate/hold the affected fridge items and alert the pharmacist to assess stability and contact the supplier/manufacture, because temperature excursions can make product non-dispensable.
4569. **Q:** A patient requests pseudoephedrine and you're ready to sell it at the register; what is the FIRST compliance step you must complete? **A:** Verify required ID and complete the NPLeX/electronic log and quantity limits before sale, because CMEA requires documentation and purchase-limit checks.
4570. **Q:** A bottle of dabigatran (Pradaxa) comes back to the pharmacy as an opened return-to-stock; what is the BEST action? **A:** Do not return it to stock and follow policy for disposal/credit, because Pradaxa is moisture-sensitive and opened bottles have limited in-use stability.
4571. **Q:** While typing a new eRx for Lamictal, you notice the patient profile also lists "Lamisil" from a prior fill; what should you do FIRST to prevent a LASA error? **A:** Stop and verify the indication and drug name/strength with the prescriber's eRx details and alert the pharmacist as needed, because lamotrigine (Lamictal) and terbinafine (Lamisil) are common look-alike/sound-alike mix-ups with very different uses.

4572. **Q:** A prescription is entered for levothyroxine 75 mcg daily, but the selected stock bottle is liothyronine 25 mcg; what should you do NEXT? **A:** Stop filling and reselect the correct product by NDC/barcode and name/strength, because levothyroxine and liothyronine are different thyroid hormones and a selection error can cause significant dosing harm.
4573. **Q:** At drop-off, a patient says they take lisinopril and the new prescription is for losartan; what is the BEST technician action? **A:** Flag the profile and notify the pharmacist to assess duplicate therapy/ACE inhibitor vs ARB switch, because both treat hypertension and duplication can increase risk of hypotension and hyperkalemia.
4574. **Q:** You receive an eRx for simvastatin 80 mg daily for a new patient; what should you do FIRST? **A:** Hold the prescription and refer to the pharmacist for clinical review, because high-dose simvastatin is associated with increased myopathy risk and requires pharmacist assessment before dispensing.
4575. **Q:** A patient picking up sertraline asks if it's the same as Zoloft and what it's for; what should you do NEXT? **A:** Provide basic brand/generic identification and then offer pharmacist counseling, because technicians can clarify names while the pharmacist addresses indications, expectations, and side effects.
4576. **Q:** The directions read "prednisone 20 mg: ii tabs qd x 5 days"; how many tablets should be dispensed? **A:** Dispense 10 tablets, because ii = 2 tablets per day for 5 days equals $2 \times 5 = 10$.
4577. **Q:** A prescription is for azithromycin 250 mg "2 tabs day 1, then 1 tab daily days 2-5"; how many tablets should you enter for quantity? **A:** Enter 6 tablets, because the standard Z-Pak regimen totals 2 on day 1 plus 1 daily for 4 more days ($2 + 4 = 6$).
4578. **Q:** During order entry you see "MS Contin 15 mg" but the prescriber comment field also mentions "morphine solution for hospice"; what should you do BEST? **A:** Pause and get pharmacist review to clarify dosage form before processing, because "MS" and morphine products are high-risk and confusion between extended-release tablets and oral solution can cause serious harm.
4579. **Q:** You are calculating days' supply for insulin regular U-100 vial 10 mL with "inject 8 units before meals three times daily"; what days' supply should you enter? **A:** Enter 41 days, because the vial contains 1000 units and the daily dose is 24 units, so $1000 \div 24 = 41.6$ and you enter 41 days to avoid overstating coverage.

4580. **Q:** While selecting stock for “hydromorphone 2 mg,” you notice the bin also contains “morphine 15 mg” and both are stored adjacent; what should you do FIRST to reduce selection errors? **A:** Separate and organize the bin (or use shelf tags/Tall Man where applicable) and scan the barcode/NDC during picking, because opioids are high-alert drugs and adjacency increases wrong-drug selection risk.
4581. **Q:** While entering a new eRx for hydroxyzine, you notice the prescriber wrote “for dizziness/vertigo” and the patient profile shows meclizine (Antivert) used previously; what should you do FIRST? **A:** Pause processing and alert the pharmacist to verify intent because hydroxyzine and meclizine are commonly confused and have different primary uses.
4582. **Q:** A technician selects “Celebrex” for an eRx that was typed as “Celexa 20 mg daily”; what is the NEXT best technician action before filling continues? **A:** Stop filling and correct the selection using barcode/NDC verification because Celexa (citalopram) and Celebrex (celecoxib) are a high-risk LASA pair.
4583. **Q:** A new eRx is for metformin 500 mg BID and the patient profile shows active glyburide; the patient says they’ve had low blood sugars recently—what should you do BEST? **A:** Notify the pharmacist for review because glyburide can cause hypoglycemia and combination diabetes therapy may need clinical assessment.
4584. **Q:** You receive an eRx for “glipizide ER 10 mg daily” but the product pulled is immediate-release glipizide 10 mg; what should you do FIRST? **A:** Stop and obtain the correct ER formulation for pharmacist check because ER vs IR substitutions can change dosing and safety.
4585. **Q:** A prescription reads “clopidogrel 75 mg qd” and the patient asks if it’s the same as Plavix and why they take it; what should you do NEXT? **A:** Refer the counseling question to the pharmacist while confirming brand/generic equivalence because indications and antiplatelet counseling are pharmacist-provided.
4586. **Q:** A patient on the phone says they take “Xanax” and asks if the new prescription for alprazolam is the same medication; what is the BEST response as a technician? **A:** Confirm that alprazolam is the generic for Xanax and offer pharmacist counseling because technicians can clarify brand/generic but not provide clinical advice.
4587. **Q:** An order is for amoxicillin/clavulanate 875/125 mg but you see the patient has a documented anaphylaxis to penicillin; what should you do FIRST? **A:** Hold

the prescription and immediately notify the pharmacist because penicillin allergy can make this antibiotic unsafe to dispense without review.

4588. **Q:** A prescription is for "levofloxacin 500 mg daily" and the profile shows the patient started warfarin last week; what should you do BEST? **A:** Flag the interaction and route to the pharmacist because fluoroquinolones can increase INR and require monitoring/therapy adjustment.
4589. **Q:** A prescriber orders "methylprednisolone dose pack 4 mg" with standard directions; what quantity should you enter for the package? **A:** Enter 21 tablets because the standard Medrol Dosepak contains a fixed 21-tablet taper pack.
4590. **Q:** A patient brings in a bottle labeled "nitroglycerin SL 0.4 mg" and says it's old and looks crumbled, asking to reuse it; what should you do FIRST? **A:** Direct them to the pharmacist and recommend replacing it because degraded nitroglycerin can lose potency and urgent-use meds require reliable effectiveness.
4591. **Q:** A new prescription is written for "levothyroxine 25 mcg qd," but the patient says they've always taken Synthroid; what should you do NEXT as the technician? **A:** Explain that Synthroid is the brand name for levothyroxine and confirm the strength/directions match the patient's profile because brand/generic equivalence prevents confusion and dispensing errors.
4592. **Q:** While entering an eRx for "losartan 50 mg daily," the prescriber note says "for blood pressure" and the patient asks what class it is; what is the BEST technician response? **A:** State that losartan is an ARB used for hypertension and offer pharmacist counseling for detailed questions because technicians can provide basic identification but not clinical counseling.
4593. **Q:** You receive a handwritten prescription for "MSO4 2 mg IV q2h PRN pain" on a unit-dose cart fill; what should you do FIRST? **A:** Stop and notify the pharmacist to clarify because "MSO4" is an error-prone abbreviation that can be confused with morphine vs magnesium sulfate.
4594. **Q:** During product selection, you notice insulin glargine and insulin lispro boxes look similar and are stored side-by-side; what should you do FIRST to reduce a mix-up? **A:** Separate the products and use barcode/NDC scanning at pick because insulin is high-alert and physical separation plus scanning reduces selection errors.

4595. **Q:** A prescription directions line reads "take ii tabs po bid"; what should you enter for the patient-friendly sig? **A:** Enter "Take 2 tablets by mouth twice daily" because ii=2, po=by mouth, and bid=twice daily to prevent misunderstanding.
4596. **Q:** A patient presents a new prescription for "Adderall XR 20 mg daily" and asks why it can't be refilled early like other meds; what should you do BEST? **A:** Explain it's a Schedule II controlled substance with stricter refill rules and refer to the pharmacist for further discussion because CS handling and patient expectations require careful compliance.
4597. **Q:** You are billing a metered-dose inhaler labeled 200 actuations with directions "inhale 2 puffs twice daily"; what days' supply should you enter? **A:** Enter 50 days' supply because $200 \text{ actuations} \div (2 \text{ puffs} \times 2 \text{ times daily} = 4/\text{day}) = 50$ days for accurate third-party billing.
4598. **Q:** An eRx comes in for "furosemide 20 mg daily," but the technician accidentally selects "torsemide 20 mg"; what is the NEXT best action? **A:** Stop the fill, correct the selection using barcode/NDC verification, and alert the pharmacist if the wrong product reached verification because this is a therapeutic substitution error risk.
4599. **Q:** A bottle of refrigerated liquid antibiotic arrives from the wholesaler and feels warm with a damaged cold-pack indicator; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager to follow temperature-excursion policy because potency and safety can't be assured.
4600. **Q:** A patient taking sertraline asks if their new "Zoloft" prescription is a different drug; what should you do NEXT? **A:** Confirm that Zoloft is the brand name for sertraline and verify the strength matches the prescription because brand/generic recognition prevents duplicate therapy and confusion.
4601. **Q:** A prescription reads "cephalexin 500 mg cap i po qid x 7 days; dispense #___"; what quantity should you enter? **A:** Enter quantity 28 capsules because 1 capsule 4 times daily for 7 days is $1 \times 4 \times 7 = 28$.
4602. **Q:** You're entering "metformin ER 500 mg take 2 tablets po daily with evening meal #60"; what days' supply should you bill? **A:** Bill 30 days because $60 \text{ tablets} \div 2 \text{ tablets per day} = 30$ days, which matches the sig.
4603. **Q:** An eRx says "prednisone 10 mg: take 4 tabs daily x3 days, then 3 tabs daily x3 days, then 2 tabs daily x3 days, then 1 tab daily x3 days; dispense #?"; what

quantity should be entered? **A:** Enter 30 tablets because $(4+3+2+1)$ tabs/day $\times 3$ days each = $10 \times 3 = 30$.

4604. **Q:** A prescriber orders "doxycycline 100 mg cap i po bid x 10 days" but you see the patient profile lists isotretinoin; what should you do FIRST? **A:** Stop processing and alert the pharmacist because doxycycline plus isotretinoin is a significant interaction risk and requires pharmacist intervention.
4605. **Q:** A prescription reads "digoxin 0.125 mg take 1 tab po qd" but you notice the selected product is digoxin 0.25 mg; what is the NEXT best step? **A:** Stop the fill and reselect the correct strength by verifying NDC/strength because wrong-strength selection is a high-risk order entry error.
4606. **Q:** A new order for "hydromorphone 2 mg tablet take 1 tab q4h prn pain" is entered, but the technician accidentally types "hydroxyzine"; what should you do NEXT? **A:** Correct the entry to the intended drug and notify the pharmacist if it advanced in workflow because LASA confusion can cause severe harm and must be caught before dispensing.
4607. **Q:** An insulin vial label says U-100 and the directions are "inject 18 units subcutaneously daily"; how many mL per day is the patient using for days' supply calculations? **A:** Use 0.18 mL/day because U-100 means 100 units per mL, so $18 \text{ units} \div 100 = 0.18 \text{ mL}$.
4608. **Q:** You are billing latanoprost 0.005% 2.5 mL with directions "instill 1 drop in each eye at bedtime"; using 20 drops/mL, what days' supply should you enter? **A:** Enter 25 days because total drops = $2.5 \text{ mL} \times 20 = 50$ drops, and usage is 2 drops/day so $50 \div 2 = 25$.
4609. **Q:** A written prescription uses "U" in the sig: "Regular insulin 10U SQ before meals"; what should you do BEST during order entry? **A:** Enter "10 units" (spell out units) and follow policy to route for pharmacist review because "U" can be misread as a zero and is an error-prone abbreviation.
4610. **Q:** While entering an eRx for "amoxicillin/clavulanate 875/125 mg 1 tab po bid #14," the prescriber comment says "treat UTI"; what should you do FIRST? **A:** Continue data entry but flag and refer to the pharmacist because the indication may be atypical and requires pharmacist clinical assessment before dispensing.
4611. **Q:** During order entry you see "MS Contin 15 mg q12h" but the product pulled is "MSIR 15 mg"; what should you do NEXT? **A:** Stop the fill and reselect the

correct NDC/formulation (ER vs IR) and alert the pharmacist if it reached verification because LASA opioid mix-ups are high-risk.

4612. **Q:** A patient drops off a paper prescription for alprazolam 0.5 mg with no prescriber DEA/NPI listed; what should you do FIRST? **A:** Hold processing and route to the pharmacist to contact the prescriber because missing required prescriber identifiers can affect controlled-substance validity.
4613. **Q:** You receive an eRx for Adderall XR 20 mg with a "Refills: 2" field; what is the BEST technician action? **A:** Stop and notify the pharmacist because Schedule II stimulants cannot be refilled and the prescriber must issue a new prescription.
4614. **Q:** A customer requests pseudoephedrine and asks to buy several boxes "for the family"; what should you do NEXT at the register? **A:** Follow CMEA steps by checking valid ID, recording the sale in the log/system, and ensuring quantity limits are not exceeded to prevent diversion.
4615. **Q:** While scanning barcodes you notice the manufacturer has issued a recall on your lot of losartan tablets; what should you do FIRST? **A:** Remove the affected stock from dispensing and quarantine/segregate it per recall procedure because recalled lots must not be distributed.
4616. **Q:** A tote arrives with several bottles of insulin aspart that were left at room temperature overnight; what should you do BEST? **A:** Quarantine and notify the pharmacist/manager to follow temperature-excursion policy because insulin potency may be compromised outside proper storage.
4617. **Q:** A new prescription is for isotretinoin, and the patient says they are not enrolled in any special program; what should you do FIRST? **A:** Stop processing and refer to the pharmacist because isotretinoin is under iPLEDGE REMS and dispensing requires program compliance checks.
4618. **Q:** You're entering "clonazepam 0.5 mg take 1 tab po bid" and the patient profile shows a severe lorazepam reaction listed as allergy; what should you do NEXT? **A:** Flag the allergy concern and route to the pharmacist for assessment because benzodiazepine cross-sensitivity/duplicate therapy requires pharmacist judgment.
4619. **Q:** A prescription label prints without the auxiliary "May cause drowsiness" for diphenhydramine; what should you do BEST before dispensing? **A:** Hold the prescription and have the label/auxiliary warnings corrected because missing critical cautions can lead to preventable patient harm.

4620. **Q:** You are billing an albuterol 0.083% nebulizer solution box containing 30 vials of 3 mL; directions are "use 1 vial via neb q6h"; what days' supply should you enter? **A:** Enter 7 days because 1 vial every 6 hours equals 4 vials/day and $30 \div 4 = 7.5$, which is billed as 7 days if the plan requires whole days.
4621. **Q:** An eRx for metformin ER 500 mg says "i tab po qd" and the prescriber laterality field is blank; what should you do FIRST in order entry? **A:** Enter the sig as "1 tab by mouth daily" because "i" is Roman numeral one and should be clarified into plain language to prevent misreads.
4622. **Q:** You are billing insulin lispro U-100 10 mL vial with directions "Inject 18 units SQ TID with meals"; what days' supply should you enter? **A:** Enter 18 days' supply because $10,000 \text{ units} \div (18 \times 3 = 54 \text{ units/day}) = 185.18 \text{ days}$, but a 10 mL vial contains 1000 units (not 10,000), so $1000 \div 54 = 18.51 \text{ days}$ and you round down to whole days per typical billing.
4623. **Q:** A prescription is for levothyroxine 75 mcg daily but the product selected in the system is liothyronine 75 mcg; what should you do NEXT? **A:** Stop the fill and correct the drug selection (verify name/strength/NDC) because these are different thyroid hormones and a wrong-drug error is high risk.
4624. **Q:** You receive a hardcopy for hydromorphone 2 mg "take 1 tab q4-6h prn pain" with quantity left blank; what is the BEST technician action? **A:** Hold processing and route to the pharmacist to contact the prescriber because missing quantity is a required dispensing element and the technician cannot decide the amount.
4625. **Q:** An order for cefdinir suspension reads "250 mg/5 mL: give 6 mL po bid x 10 days"; how many milliliters should be dispensed? **A:** Dispense 120 mL because $6 \text{ mL/dose} \times 2 \text{ doses/day} \times 10 \text{ days} = 120 \text{ mL total}$.
4626. **Q:** During order entry, you see "morphine sulfate ER 30 mg" and the sig says "take 1 tab po q4h"; what should you do FIRST? **A:** Stop and alert the pharmacist because ER products are not dosed q4h and this is a likely dosing error requiring pharmacist intervention.
4627. **Q:** A patient's eRx is for "Zyrtec 10 mg 1 tab daily" but the patient profile shows active cetirizine 10 mg daily already; what should you do NEXT? **A:** Verify the profile and notify the pharmacist about therapeutic duplication because Zyrtec is cetirizine and duplicate therapy may indicate an extra prescription or refill issue.

4628. **Q:** You are entering a prescription that says "predniSONE 20 mg: ii tabs po qd x 5 days"; what quantity should you enter? **A:** Enter quantity 10 tablets because "ii" = 2 tablets daily and $2 \times 5 \text{ days} = 10$.
4629. **Q:** A prescription is for timolol 0.5% ophthalmic solution "instill 1 gtt OU BID" and you are asked for days' supply for a 5 mL bottle using 20 gtt/mL; what is the days' supply? **A:** Enter 25 days' supply because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100$ drops, and OU BID uses 1 drop in each eye twice daily = 4 drops/day, so $100 \div 4 = 25$.
4630. **Q:** While typing an eRx you notice "hydroxyzine 25 mg take 1 tab po tid prn anxiety" but the prescriber selected the "hydroxyzine pamoate" product and the notes mention "itching"; what should you do BEST? **A:** Flag it for pharmacist review before dispensing because hydroxyzine comes in different salts/forms and the indication mismatch suggests a potential selection or communication error.
4631. **Q:** During data entry you notice an eRx for Lantus (insulin glargine) was selected as Humalog (insulin lispro) in the product field; what should you do NEXT? **A:** Stop processing and correct the product selection to match the prescription, because mix-ups between basal and rapid-acting insulin are high-alert LASA errors.
4632. **Q:** A patient asks to transfer their oxycodone/APAP prescription from another pharmacy and wants it filled today; what should you do FIRST? **A:** Route the request to the pharmacist, because controlled substance transfer/refill legality and validity require pharmacist review and proper documentation.
4633. **Q:** You receive a shipment of refrigerated Ozempic pens that arrived warm with a temperature indicator showing an excursion; what is the BEST technician action? **A:** Quarantine the product and notify the pharmacist/wholesaler per policy, because temperature excursions can compromise drug integrity and must not be stocked.
4634. **Q:** A hardcopy prescription for alprazolam 0.5 mg includes directions and quantity but no prescriber signature; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist to contact the prescriber, because an unsigned controlled-substance prescription is not valid to dispense.
4635. **Q:** While counting tablets you find a stock bottle with a torn seal and powder residue inside the lid; what should you do BEST? **A:** Quarantine the bottle and

notify the pharmacist, because damaged/suspect product should not be used and may require return or investigation.

4636. **Q:** You are entering warfarin 5 mg and the prescriber wrote "use as directed" with no INR plan or specific dose; what should you do NEXT? **A:** Stop and refer to the pharmacist for clarification, because warfarin is high-alert and the sig must be clear enough to ensure safe use and correct days' supply.
4637. **Q:** A patient's profile shows a documented anaphylaxis to penicillin and you receive an eRx for cephalexin; what should you do FIRST? **A:** Pause the fill and notify the pharmacist, because a severe allergy history may require clinical assessment before dispensing a related beta-lactam.
4638. **Q:** The register prints a recall notice for a specific lot of losartan 50 mg that matches bottles on your shelf; what should you do NEXT? **A:** Pull the matching lot from inventory and quarantine it for pharmacist handling/returns, because recalled lots must be removed to prevent dispensing.
4639. **Q:** A pharmacy receives a request to dispense isotretinoin today but the patient says they "lost their iPLEDGE number"; what should you do BEST? **A:** Refer the case to the pharmacist and do not process until REMS requirements are met, because isotretinoin dispensing is restricted and must be compliant before release.
4640. **Q:** You are billing clopidogrel 75 mg #30 and the directions are "take 1 tablet by mouth daily" but the system defaults to a 15-day supply; what should you do FIRST? **A:** Correct the days' supply to 30 days, because accurate days' supply is required for safe refills, insurance compliance, and proper patient use.
4641. **Q:** While typing an eRx you see "metformin ER 500 mg take 2 tabs daily" but the product selected is metformin IR 500 mg; what should you do NEXT? **A:** Stop processing and notify the pharmacist to clarify formulation because IR vs ER is not interchangeable and affects dosing and safety.
4642. **Q:** A new prescription is for Zestril 10 mg daily and the patient asks what the generic is; what should you answer? **A:** Lisinopril is the generic for Zestril because it's an ACE inhibitor commonly dispensed as lisinopril.
4643. **Q:** During order entry you notice an eRx for "Lasix 40 mg daily" was selected as "Losec" due to similar name; what should you do FIRST? **A:** Correct the product selection by verifying the drug name/indication and NDC because LASA mix-ups (Lasix vs Losec) are high-risk data-entry errors.

4644. **Q:** The label directions read “take i tab po qd” and a trainee asks what “i” means; what is the correct interpretation? **A:** “i” means 1 (Roman numeral one) so it is “take 1 tablet by mouth daily,” preventing quantity and dosing errors.
4645. **Q:** A patient profile shows anaphylaxis to sulfonamides and you receive an eRx for hydrochlorothiazide 25 mg daily; what should you do BEST? **A:** Route to the pharmacist for allergy assessment because sulfonamide cross-reactivity questions require pharmacist judgment even if many patients tolerate thiazides.
4646. **Q:** A prescription is for azithromycin 200 mg/5 mL suspension: give 10 mL day 1 then 5 mL daily days 2–5; what total mL should you dispense? **A:** Dispense 30 mL because $10\text{ mL} + (5\text{ mL} \times 4\text{ days}) = 30\text{ mL total}$.
4647. **Q:** While entering a new med you see the patient is already taking atorvastatin and a new eRx arrives for simvastatin; what should you do FIRST? **A:** Pause and alert the pharmacist to evaluate therapeutic duplication because two statins together may increase adverse effects without benefit.
4648. **Q:** A patient asks if “Norvasc” is for blood pressure or pain and wants the generic name; what should you tell them? **A:** Norvasc is amlodipine and it’s a calcium channel blocker commonly used for hypertension, so it’s for blood pressure.
4649. **Q:** A prescriber sends an eRx for Tylenol #3 with “refill 2” listed; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because refills for Schedule III controlled substances must be within legal limits and require pharmacist verification.
4650. **Q:** You open a new stock bottle of levothyroxine and it has a noticeably different tablet color than usual for the same strength; what should you do BEST? **A:** Stop and verify the NDC/strength and lot against the invoice and image, then involve the pharmacist if unresolved because levothyroxine is high-alert and appearance changes can signal a selection error.
4651. **Q:** While entering a new eRx you see “Lamictal 25 mg” but the product selected is “Lamisil 250 mg” due to name similarity; what should you do FIRST? **A:** Stop processing and reselect the correct drug/strength using barcode/NDC verification and alert the pharmacist if it reached verification, because this is a high-risk LASA mix-up.
4652. **Q:** A patient picking up Eliquis asks what it’s for and whether it replaces warfarin; what is the BEST technician response? **A:** Say apixaban (Eliquis) is a blood

thinner used to prevent/treat clots and refer warfarin-replacement questions to the pharmacist, because anticoagulant counseling requires pharmacist review.

4653. **Q:** You receive an eRx for “sertraline 50 mg daily” but the profile shows the patient already takes paroxetine 20 mg daily; what should you do FIRST? **A:** Pause the fill and notify the pharmacist to assess SSRI therapeutic duplication and switch intent, because duplicate antidepressants increase adverse-effect risk.
4654. **Q:** A prescription is for lisinopril 10 mg tablets “take 1 daily” with quantity 90; what days’ supply should you enter? **A:** Enter 90 days because $90 \text{ tablets} \div 1 \text{ tablet per day} = 90 \text{ days}$, ensuring accurate billing and refill timing.
4655. **Q:** During drop-off a patient says they are pregnant and the new eRx is for losartan; what should you do BEST? **A:** Hold the prescription and immediately alert the pharmacist, because ARBs like losartan are contraindicated in pregnancy and require prescriber/pharmacist intervention.
4656. **Q:** While typing a new prescription you see “Toprol-XL 50 mg daily” but the selected product is metoprolol tartrate; what should you do NEXT? **A:** Stop and correct the selection to metoprolol succinate ER (Toprol-XL) and flag for pharmacist check, because tartrate vs succinate is a common formulation error.
4657. **Q:** A new prescription is for prednisone with directions “take 2 tablets bid” and the strength is 10 mg; how many mg per day is this? **A:** It is 40 mg/day because $2 \text{ tablets} \times 10 \text{ mg} = 20 \text{ mg per dose}$ and BID means 2 doses daily ($20 \text{ mg} \times 2 = 40 \text{ mg}$).
4658. **Q:** A patient on chronic oxycodone brings a new eRx for clonazepam and asks if it’s safe to take together; what should you do FIRST? **A:** Refer to the pharmacist before dispensing, because opioid + benzodiazepine combination increases sedation/respiratory depression risk and requires pharmacist counseling.
4659. **Q:** When selecting insulin you notice the eRx says “Humalog” but the product pulled is “Humulin R”; what is the BEST action? **A:** Stop filling and verify the intended insulin name/type with the prescription and pharmacist, because look-alike insulin names can cause dangerous dosing and onset errors.
4660. **Q:** A patient asks the generic name and main use of “Keppra” at pickup; what should you answer? **A:** Levetiracetam (Keppra) is used to help prevent seizures, and offer pharmacist counseling for detailed side effects/interactions because seizure meds require careful counseling.

4661. **Q:** A new Rx is for amoxicillin/clavulanate 400 mg/57 mg per 5 mL, sig: "1 tsp po bid x 10 days"; what quantity in mL should you enter? **A:** Enter 100 mL because 1 tsp = 5 mL and 5 mL twice daily for 10 days is $5 \times 2 \times 10 = 100$ mL.
4662. **Q:** While entering directions "i-ii tab po q4-6h prn pain" for hydrocodone/APAP, what is the BEST clarification step before calculating days' supply? **A:** Ask the pharmacist to determine the max daily dose to use because PRN ranges (i-ii and q4-6h) require a consistent payer-acceptable max for accurate days' supply.
4663. **Q:** A prescription reads "metformin ER 500 mg, take 2 tabs po qhs, #120"; what days' supply should you enter? **A:** Enter 60 days because 2 tablets nightly means 2 per day and $120 \div 2 = 60$.
4664. **Q:** A patient's budesonide/formoterol inhaler is 120 actuations and the sig is "inhale 2 puffs bid"; what days' supply should you bill? **A:** Bill 30 days because 2 puffs twice daily is 4 puffs/day and $120 \div 4 = 30$.
4665. **Q:** You receive an eRx for "oxycodone 5 mg 1 tab po q6h prn pain #20" and the prescriber field is blank; what should you do FIRST? **A:** Stop processing and route to the pharmacist because missing prescriber information can make the order invalid and needs pharmacist follow-up before dispensing a controlled substance.
4666. **Q:** During order entry you see "U-500 insulin regular, inject 20 units bid" but the selected product is U-100 regular insulin; what should you do NEXT? **A:** Stop and correct the selection, then alert the pharmacist because U-500 vs U-100 is a high-alert concentration error with major overdose risk.
4667. **Q:** A prescription says "levothyroxine 0.1 mg po daily"; what strength should you select in micrograms to avoid a decimal error? **A:** Select 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$, preventing a 10-fold dosing mistake.
4668. **Q:** A new Rx is for ciprofloxacin 0.3% ophthalmic solution "instill 2 gtt into affected eye q4h while awake x 7 days"; using 20 drops per mL, what minimum mL should you dispense for one eye? **A:** Dispense at least 5 mL because q4h while awake is typically 4 doses/day, so $2 \text{ drops} \times 4 \times 7 = 56 \text{ drops}$ and $56 \div 20 = 2.8 \text{ mL}$, with a 5 mL bottle as the practical available size.
4669. **Q:** You are typing "MSO4 2 mg IV" from a hospital order set; what is the BEST technician action? **A:** Flag it for pharmacist clarification because "MSO4" is an unsafe abbreviation that can be misread and should be replaced with "morphine" to prevent LASA errors.

4670. **Q:** A prescription for atorvastatin says “take 1 tab po qd” and the quantity is 30; the patient says they take it “qd” but you’re unsure of the abbreviation—what should you enter and why? **A:** Enter 1 tablet once daily because “qd” means every day, and confirming the sig prevents misinterpretation that could cause wrong dosing frequency.
4671. **Q:** While entering an eRx for clonidine 0.1 mg, you notice the selected NDC is for clonazepam 0.5 mg due to similar names; what should you do FIRST? **A:** Stop filling and re-select by matching the drug name/strength/dosage form and scanning the barcode/NDC because LASA mix-ups are common error sources.
4672. **Q:** A new Rx for methotrexate 2.5 mg is entered with directions “take 1 tablet daily for rheumatoid arthritis”; what is the BEST next step for a technician? **A:** Hold the prescription and alert the pharmacist immediately because methotrexate is typically weekly and daily dosing is a high-alert error risk.
4673. **Q:** A patient drops off a paper prescription for alprazolam 1 mg with “refill x 5” written; what should you do NEXT? **A:** Continue processing but route to the pharmacist to correct refills because Schedule IV allows refills but the pharmacist must ensure the refill quantity and validity are appropriate.
4674. **Q:** During product intake, you find a tote of refrigerated insulin glargine left at room temperature overnight with unknown time out of fridge; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager because a temperature excursion makes it suspect and it should not be returned to stock without review.
4675. **Q:** You receive a manufacturer recall notice for a specific lot of losartan tablets; what is the technician’s BEST action? **A:** Pull and quarantine only the affected NDC/lot/expiration and follow recall documentation steps because lot-level accuracy prevents dispensing recalled product.
4676. **Q:** At pickup, a patient asks for “Z-Pak” but the profile shows azithromycin and the bag label says “azathioprine”; what should you do FIRST? **A:** Stop the sale and get the pharmacist because azithromycin vs azathioprine is a dangerous name confusion requiring immediate verification and counseling.
4677. **Q:** A prescription is written for isotretinoin, and the patient is waiting; what should you do NEXT in workflow? **A:** Notify the pharmacist and ensure iPLEDGE/REMS requirements are met before dispensing because isotretinoin is REMS-restricted and cannot be released without required authorization.

4678. **Q:** A patient requests pseudoephedrine 30 mg tablets and asks to buy 4 boxes today; what should you do FIRST? **A:** Check ID and run the sale through the NPLeX/CMEA log to verify daily and 30-day limits because federal requirements restrict quantity and require tracking.
4679. **Q:** A prescription is for cephalexin 250 mg/5 mL with sig "take 10 mL by mouth three times daily for 7 days"; what quantity in mL should you dispense? **A:** Dispense 210 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$ to cover the full course.
4680. **Q:** While entering a new Rx for sertraline, the patient profile shows a severe rash allergy to "Zoloft"; what should you do BEST? **A:** Stop processing and alert the pharmacist because Zoloft is sertraline and an allergy match requires pharmacist assessment before dispensing.
4681. **Q:** A new eRx is for metformin ER 500 mg but the product selected is metformin IR 500 mg because both appear as "metformin 500"; what should you do FIRST? **A:** Stop and reselect the correct ER product/NDC and confirm the dosage form on the Rx because IR vs ER is not interchangeable and can cause dosing errors.
4682. **Q:** A patient asks if "Lasix" is the same as "furosemide" while you're preparing the label; what is the BEST technician response? **A:** Confirm that Lasix is the brand name for furosemide and offer pharmacist counseling because brand/generic clarification supports safe use and OBRA-90 counseling access.
4683. **Q:** While entering a prescription for "hydralazine," you accidentally click "hydroxyzine" due to similar names; what should you do NEXT? **A:** Immediately correct the selection to the intended drug and verify strength/class against the prescriber directions because LASA errors can lead to dispensing the wrong therapy.
4684. **Q:** A profile shows the patient already takes lisinopril 20 mg daily, and a new Rx comes in for benazepril 20 mg daily; what should you do BEST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (two ACE inhibitors) because the pharmacist must assess appropriateness before dispensing.
4685. **Q:** You are filling levothyroxine and notice two bottles with different NDCs and manufacturers in the same strength; what is the BEST action before counting? **A:** Use one manufacturer/NDC per fill and match the NDC on the label to the stock bottle because mixing manufacturers can create identification and continuity problems for a narrow-therapeutic-index drug.

4686. **Q:** A prescription is for amoxicillin-clavulanate, and the patient profile lists "Augmentin caused hives"; what should you do FIRST? **A:** Stop the fill and notify the pharmacist to evaluate the allergy because hives suggests an IgE-type reaction and requires pharmacist intervention.
4687. **Q:** A patient brings in a new prescription for clopidogrel and asks what it's for; what should you do NEXT? **A:** Refer the patient to the pharmacist for counseling while you can note clopidogrel is an antiplatelet commonly used after stents because indication questions require pharmacist counseling.
4688. **Q:** A label is being created for azithromycin 250 mg with sig "take 2 tablets today then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets because 2 on day 1 plus 1 daily for 4 more days equals 6 total tablets.
4689. **Q:** A new Rx is for gabapentin 300 mg "1 capsule three times daily," but the patient profile shows pregabalin is active; what should you do BEST? **A:** Flag and route to the pharmacist to check for duplication/therapy change because gabapentin and pregabalin are related CNS agents and may not be intended together.
4690. **Q:** You receive an eRx for "Adderall XR 20 mg" with directions written as "take 1 cap qam" and a note "DNF until next month"; what should you do NEXT? **A:** Hold and refer to the pharmacist to validate controlled-substance timing and any "do not fill until" requirements because Schedule II dispensing restrictions require pharmacist oversight and proper dating/documentation.
4691. **Q:** A new prescription is for "Crestor 20 mg 1 tab nightly," but the patient profile only lists rosuvastatin as previous therapy; what should you do FIRST during data entry? **A:** Select rosuvastatin 20 mg and ensure the label shows the brand/generic relationship as appropriate because Crestor is the brand for rosuvastatin and correct product selection prevents dispensing the wrong drug.
4692. **Q:** While typing an eRx for "Lamictal 100 mg," you notice the system defaulted to "lamoTRIgine ODT" instead of regular tablets; what should you do NEXT? **A:** Stop and reselect the correct dosage form (lamotrigine tablet) and route to pharmacist if unclear because dosage form mix-ups are common LASA/selection errors that can change administration.
4693. **Q:** A prescription reads "Synthroid 75 mcg take i tab po qd," and the prescriber did not mark DAW; what is the BEST technician action? **A:** Process as levothyroxine 75 mcg per plan/formulary and store policy because Synthroid is

a brand name and DAW not indicated means generic substitution is typically permitted unless pharmacist determines otherwise.

4694. **Q:** During product selection you see "HumaLOG" and "HumaLIN R" adjacent on the shelf for a patient starting insulin today; what should you do FIRST to reduce LASA risk? **A:** Verify the exact insulin name and concentration on the prescription and match the NDC/barcode to the selected product because Humalog (lispro) and Humulin R (regular) are different insulins with high-alert error potential.
4695. **Q:** A patient's new Rx is "clarithromycin 500 mg BID," and their profile shows simvastatin 40 mg daily; what should you do BEST before processing? **A:** Hold the order and alert the pharmacist because clarithromycin can significantly increase simvastatin levels, creating a high-risk interaction that requires pharmacist intervention.
4696. **Q:** An eRx arrives for "MS Contin 15 mg q12h," but the tech selects "morphine sulfate IR 15 mg" since both show "morphine 15 mg"; what should you do NEXT? **A:** Stop filling and correct to the extended-release product specified (MS Contin) and notify the pharmacist if it reached verification because ER vs IR is a critical formulation difference and a common safety trap.
4697. **Q:** A prescription for alendronate says "take 1 tablet weekly," but the strength is missing; what should you do FIRST? **A:** Pause processing and send to the pharmacist for clarification because missing strength prevents accurate dispensing and alendronate has multiple strengths with different dosing directions.
4698. **Q:** A prescriber writes "predniSONE 10 mg ii tabs po qd x 5 days"; what quantity should you enter to dispense? **A:** Enter 10 tablets because ii tabs daily for 5 days equals $2 \times 5 = 10$ tablets, ensuring the dispensed quantity matches the directions.
4699. **Q:** A patient asks whether "Prilosec" and "omeprazole" are the same when you ring them out; what is the BEST technician response? **A:** Confirm they are the same medication (Prilosec is omeprazole) and offer to have the pharmacist counsel because technicians can provide basic brand/generic identification but counseling questions should go to the pharmacist.
4700. **Q:** You receive a stock bottle of valacyclovir with a torn label where the NDC and lot number are unreadable; what should you do BEST? **A:** Remove it from active inventory and follow policy to quarantine/return it for replacement because

unreadable identification prevents proper verification and traceability for patient safety and recalls.

4701. **Q:** While entering an Rx for "Zithromax 250 mg," the patient says they're allergic to "azithro"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify and assess the allergy because Zithromax is azithromycin and could cause a repeat reaction.
4702. **Q:** A prescription is written for "glipiZIDE 10 mg 1 tab daily," but you accidentally selected glyBURIDE in the system; what should you do NEXT? **A:** Stop and correct the drug selection using the exact name/strength and verify NDC/barcode because glipizide and glyburide are LASA sulfonylureas with different dosing risk.
4703. **Q:** A patient presents a new Rx for "Eliquis 5 mg BID" and asks what it's for; what is the BEST technician response? **A:** Tell them Eliquis is apixaban, a blood thinner to help prevent clots, and offer pharmacist counseling because indication/counseling supports safe use.
4704. **Q:** You receive an eRx for "hydroxyzine 25 mg" but the dropdown shows hydroxyzine HCl and hydroxyzine pamoate; what should you do BEST? **A:** Pause and ask the pharmacist or prescriber to confirm the intended salt/form because different forms are not automatically interchangeable and a wrong selection can change therapy.
4705. **Q:** A prescriber calls in "metFORMIN ER 500 mg take 2 tabs daily," but the profile shows active metFORMIN IR 500 mg BID; what should you do FIRST? **A:** Flag it for pharmacist review for potential duplicate therapy/formulation change because ER vs IR is a common error point and requires clinical confirmation.
4706. **Q:** An Rx says "levothyroxine 0.075 mg PO daily"; what strength should you enter to match common labeling? **A:** Enter 75 mcg because 0.075 mg converts to 75 micrograms and levothyroxine is typically stocked and prescribed in mcg to avoid decimal errors.
4707. **Q:** A patient's directions read "Take ii tabs po bid x 7 days" for doxycycline 100 mg; what quantity should you dispense? **A:** Dispense 28 tablets because ii = 2 and bid = 2 times daily so $2 \times 2 \times 7 = 28$.
4708. **Q:** While selecting product for "predniSONE 20 mg," you notice both predniSONE and prednisoLONE are stocked side-by-side; what should you do FIRST? **A:**

Verify the exact drug name and NDC/barcode before pulling stock because prednisone and prednisolone are LASA steroids and mix-ups can harm patients.

4709. **Q:** A patient asks if “Norvasc” is the same as “amlodipine” when picking up blood pressure meds; what should you do BEST? **A:** Confirm Norvasc is the brand name for amlodipine and offer pharmacist counseling because brand/generic confirmation improves adherence and safety.
4710. **Q:** A new stock bottle of warfarin arrives with an intact label but tablets inside look mismatched in color/shape; what should you do NEXT? **A:** Set it aside as suspect product and notify the pharmacist per DSCSA/safety policy because warfarin is high-alert and mismatched appearance suggests mispackaging or counterfeit risk.
4711. **Q:** An Rx reads “clindamycin oral solution 75 mg/5 mL; take 10 mL PO TID x 10 days”; what quantity (mL) should you enter to dispense? **A:** Enter 300 mL because $10 \text{ mL per dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to match the total volume needed.
4712. **Q:** A prescription says “Insulin lispro U-100; inject 6 units SC before meals TID; dispense 1 vial (10 mL)”; what days’ supply should you submit? **A:** Submit 55 days because $10 \text{ mL} = 1000 \text{ units}$ and $6 \text{ units} \times 3/\text{day} = 18 \text{ units/day}$, so $1000 \div 18 \approx 55 \text{ days}$ (rounded down for billing safety).
4713. **Q:** You’re entering “amLODIPine 5 mg PO qd” and the patient profile already has active “benazepril/amlodipine 20/5 mg daily”; what should you do FIRST? **A:** Stop and alert the pharmacist to possible therapeutic duplication of amlodipine because adding separate amlodipine may unintentionally double the dose.
4714. **Q:** A prescriber writes “digoxin 0.125 mg daily,” but the system strength list shows 125 mcg and 0.25 mg; which strength should you select? **A:** Select 125 mcg because $0.125 \text{ mg} = 125 \text{ mcg}$ and matching the equivalent listed strength reduces entry error.
4715. **Q:** An eRx sig states “Take 1 tab PO q6h prn pain” for hydrocodone/acetaminophen 5/325 with quantity 20; using the max frequency, what days’ supply should you bill? **A:** Bill 5 days because q6h implies up to 4 tablets/day, and $20 \div 4 = 5 \text{ days}$ for payer-required days’ supply.
4716. **Q:** While typing an Rx for “Lantus SoloStar,” you notice the selected product is “insulin glargine-yfgn (Semglee) vial”; what should you do NEXT? **A:** Stop and

correct the dosage form/product selection because a pen vs vial mix-up changes administration supplies and can cause dispensing errors.

4717. **Q:** A prescription reads "predniSONE 5 mg: take i po bid x 6 days, then i po qd x 6 days"; what quantity should you dispense? **A:** Dispense 18 tablets because 1 tab BID for 6 days = 12 tabs and then 1 tab daily for 6 days = 6 tabs, total 18.
4718. **Q:** You receive an Rx for "alendronate 70 mg take 1 tab weekly #4," but the sig is entered as "daily" in the system; what is the BEST technician action? **A:** Stop entry and fix the sig to weekly (or route to pharmacist per workflow) because alendronate is typically weekly and "daily" is a high-risk directions error.
4719. **Q:** A technician is asked to enter "levETIRAcetam 500 mg: take 1.5 tabs BID; dispense #90"; what days' supply should be calculated? **A:** Calculate 30 days because $1.5 \text{ tabs} \times 2/\text{day} = 3 \text{ tabs/day}$, and $90 \div 3 = 30$ days for accurate billing and refill timing.
4720. **Q:** An Rx is written "morphine sulfate ER 30 mg PO q12h," but the product selected is morphine IR 30 mg; what should you do FIRST? **A:** Stop processing and notify the pharmacist because ER vs IR is a high-alert mismatch that can cause overdose or uncontrolled pain if dispensed incorrectly.
4721. **Q:** A patient dropping off a prescription says they take "Plavix," but the eRx is for clopidogrel 75 mg daily; what should you do FIRST at intake? **A:** Confirm with the patient that Plavix is the brand for clopidogrel and document the verification because brand/generic recognition prevents duplicate therapy confusion.
4722. **Q:** While entering an eRx for "Zocor 40 mg qHS," the system auto-selects simvastatin 80 mg; what is the BEST technician action? **A:** Stop and select the correct simvastatin 40 mg product and route for pharmacist check because wrong strength statin selection is a high-risk error.
4723. **Q:** A new prescription for lisinopril is being typed and the allergy profile lists "ACE inhibitors: angioedema"; what should you do NEXT? **A:** Hold processing and notify the pharmacist immediately because an ACE inhibitor allergy/angioedema is a serious safety concern requiring pharmacist intervention.
4724. **Q:** You receive an Rx for metformin ER 500 mg daily, but the product chosen is metformin IR 500 mg; what should you do FIRST? **A:** Stop entry and correct to

the ER formulation as written and flag for verification because IR vs ER mix-ups can change release and tolerability.

4725. **Q:** An order is for "metoprolol succinate ER 50 mg daily," but the shelf label next to the NDC you grabbed says metoprolol tartrate 50 mg; what should you do NEXT? **A:** Stop filling and re-select the correct succinate ER NDC and dosage form because tartrate vs succinate is a common LASA/therapy error.
4726. **Q:** A prescription says "azithromycin 200 mg/5 mL: take 12.5 mL PO day 1, then 6.25 mL daily days 2–5"; what total mL should you dispense? **A:** Dispense 37.5 mL because $12.5 \text{ mL} + (6.25 \text{ mL} \times 4) = 12.5 + 25 = 37.5 \text{ mL}$ to cover the full course.
4727. **Q:** A patient requests a refill of sertraline 50 mg "because I doubled it to feel better," and the profile shows it was filled 10 days ago for 30 tablets; what should you do BEST? **A:** Refer the patient to the pharmacist and do not process an early refill as routine because dose changes and adherence/safety concerns require pharmacist counseling.
4728. **Q:** While processing a new prescription for oxycodone/acetaminophen, the patient profile shows an active acetaminophen 650 mg ER scheduled product; what should you do FIRST? **A:** Pause and alert the pharmacist to assess total daily acetaminophen duplication because combination products can unintentionally exceed safe APAP limits.
4729. **Q:** You notice two similar-looking stock bottles, "hydrOXYzine" and "hydrALAzine," on the counter after filling; what is the BEST next step before it reaches verification? **A:** Immediately verify the NDC/barcode and return the incorrect bottle to stock because these LASA names are commonly confused and catching it early prevents wrong-drug dispensing.
4730. **Q:** A prescription is for "levothyroxine 75 mcg daily," but the patient asks if it's the same as Synthroid and mentions they only want the same tablets as last month; what should you do NEXT? **A:** Confirm Synthroid is levothyroxine, then check the previous NDC/manufacturer and notify the pharmacist if a product change is needed because consistent product selection can matter for thyroid therapy stability.
4731. **Q:** During data entry, an eRx reads "U-500 insulin regular: inject 40 units SQ BID," and the patient profile shows they previously used U-100 regular insulin; what should you do FIRST? **A:** Stop processing and notify the pharmacist

immediately because U-500 is a high-alert concentration change with major overdose risk and requires pharmacist review.

4732. **Q:** A written prescription for alprazolam has the patient name, drug, directions, and prescriber signature but no date written; what is the BEST technician action? **A:** Hold the prescription and route it to the pharmacist because missing key elements can affect controlled-substance validity and must be resolved before dispensing.
4733. **Q:** A patient presents a driver's license to buy pseudoephedrine 30 mg tablets, 96-count, and your log shows they already purchased 2.4 g yesterday; what should you do NEXT? **A:** Refuse the sale per CMEA limits and follow store policy because the requested amount would exceed the federal daily gram limit for pseudoephedrine.
4734. **Q:** While filling, you notice the NDC on the stock bottle for rivaroxaban is a different strength than the label you printed; what should you do FIRST? **A:** Stop filling and re-scan/verify the correct NDC and strength because barcode/NDC mismatch is a high-risk selection error that must be corrected before verification.
4735. **Q:** A prescription is entered for "Lamictal 100 mg daily," but the patient profile lists an allergy to lamotrigine with "rash"; what should you do NEXT? **A:** Stop and alert the pharmacist because an allergy to the active ingredient requires pharmacist assessment before any dispensing.
4736. **Q:** An eRx arrives for isotretinoin, and the patient asks you to just "ring it up quickly"; what should you do BEST before proceeding? **A:** Refer to the pharmacist to ensure iPLEDGE/REMS requirements are met because isotretinoin dispensing has mandatory safety steps outside technician-only judgment.
4737. **Q:** You receive a wholesaler notice that a specific lot of atorvastatin 20 mg is recalled, and you find that lot on your shelf; what should you do FIRST? **A:** Remove the recalled lot from active inventory and quarantine it per recall procedure because recalled product must be isolated to prevent dispensing.
4738. **Q:** A vial of insulin aspart arrives on the order with evidence the cold pack is warm and the box feels room temperature; what should you do NEXT? **A:** Quarantine the shipment and notify the pharmacist/manager because a temperature excursion makes the product suspect and it should not be stocked or dispensed.

4739. **Q:** A patient asks, "Can I take ibuprofen with my lisinopril?" while you are at the register; what is the BEST technician response? **A:** Refer the question to the pharmacist because OTC–Rx interaction counseling requires pharmacist clinical judgment and protects patient safety.
4740. **Q:** A prescription is for prednisone 10 mg tablets: "take 4 tablets daily for 5 days," and the prescriber wrote "dispense #"; what quantity should you enter? **A:** Enter 20 tablets because $4 \text{ tablets/day} \times 5 \text{ days} = 20 \text{ tablets}$, ensuring the quantity matches the directions for accurate dispensing and billing.
4741. **Q:** An eRx says "metformin ER 500 mg i po bid #60," but the prescriber notes "Glucophage XR," so what should you enter and dispense? **A:** Enter metformin ER 500 mg tablets #60 and ensure an ER/XR product is selected because Glucophage XR is metformin extended-release and substituting IR would change release and dosing.
4742. **Q:** A prescription reads "amLODIPine 5 mg: take 1 tab po qd; #30," and the patient says they take Norvasc—what should you do NEXT in order entry? **A:** Confirm amlodipine is Norvasc and document/dispense the correct generic unless DAW applies because brand/generic recognition prevents unnecessary delays and confusion.
4743. **Q:** The sig is "furosemide 20 mg: i tab po qd; may take ii tabs for swelling," and the insurer requires a days' supply; if you must bill the maximum daily use, what days' supply is #45? **A:** Bill 22 days because maximum use is 2 tablets/day and $45 \div 2 = 22.5$, which is billed as 22 days to avoid overstating coverage.
4744. **Q:** A patient has an order for amoxicillin-clavulanate suspension 600-42.9 mg/5 mL: "10 mL po bid x 7 days"; what minimum mL should you dispense? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$, ensuring enough volume for the full course.
4745. **Q:** While entering "digoxin 0.125 mg daily," you accidentally type 1.25 mg; what should you do FIRST when you notice before printing labels? **A:** Stop and correct the strength to 0.125 mg before proceeding because digoxin is high-alert and a decimal error can cause a tenfold overdose.
4746. **Q:** A prescription is for "atorvastatin 40 mg qhs," but the patient profile shows an active "simvastatin 40 mg qhs" with refills remaining; what is the BEST technician action? **A:** Pause processing and alert the pharmacist about potential therapeutic duplication because concurrent statins may be inappropriate and requires pharmacist review.

4747. **Q:** An inhaler prescription says "fluticasone/salmeterol 250/50: 1 inhalation bid; dispense 1," and the device contains 60 inhalations; what days' supply should you submit? **A:** Submit 30 days because 1 inhalation twice daily uses 2 inhalations/day and $60 \div 2 = 30$ days.
4748. **Q:** A prescriber writes "morphine sulfate ER 15 mg: 1 tab po q12h, #60, refill x2"; what should you do NEXT? **A:** Hold the prescription and send to the pharmacist because refills for Schedule II controlled substances are not permitted and require pharmacist resolution.
4749. **Q:** A label direction is typed as "take 1 tablet qd," but the original prescription says "take 1 tablet qid"; what should you do FIRST? **A:** Stop the fill and correct the sig to qid to match the prescription because qd vs qid is a common abbreviation error that can cause major underdosing.
4750. **Q:** A prescription for cephalexin 500 mg says "1 cap po q6h x 10 days"; what quantity should be entered? **A:** Enter 40 capsules because q6h is 4 doses/day and $4 \times 10 = 40$, matching the intended duration.
4751. **Q:** While typing an eRx for hydrOXYzine 25 mg, you notice the patient profile also has hydrALAZINE 25 mg active; what should you do FIRST to prevent a LASA error? **A:** Pause entry and verify the indication and drug selection (name/strength/NDC) against the eRx, then route to the pharmacist if any uncertainty remains, because hydroxyzine vs hydralazine is a high-risk look-alike/sound-alike mix-up.
4752. **Q:** A hardcopy prescription for alprazolam 0.5 mg has the patient name, drug, and directions but no prescriber signature; what is the BEST technician action? **A:** Stop processing and notify the pharmacist to obtain a valid signed prescription, because a missing prescriber signature makes the order incomplete and unsafe to dispense.
4753. **Q:** A patient presents an oxycodone/APAP prescription and asks you to "transfer it to another pharmacy"; what should you do NEXT? **A:** Refer the request to the pharmacist, because controlled substance transfer rules and validity must be handled by the pharmacist and should not be completed independently by a technician.
4754. **Q:** During receiving, you find a tote of insulin glargine pens that arrived warm with a temperature indicator showing excursion; what should you do FIRST? **A:** Segregate and label the shipment as suspect/hold and alert the

pharmacist/manager to follow storage/return procedures, because temperature excursions can make insulin ineffective and it should not enter inventory.

4755. **Q:** A bottle of sertraline 50 mg tablets on the shelf has an expiration date that passed last month; what is the BEST technician action? **A:** Remove it from active stock and place it in the designated expired area per policy for pharmacist review/disposal, because expired medications must not be dispensed.
4756. **Q:** An eRx for isotretinoin arrives and the patient is waiting; what should you do NEXT before it can be dispensed? **A:** Immediately notify the pharmacist and follow iPLEDGE/REMS workflow requirements, because isotretinoin is REMS-restricted and dispensing requires documented authorization steps.
4757. **Q:** A patient asks for "Plan B" and you locate two similar packages with different NDCs and differing age/labeling; what should you do FIRST at product selection? **A:** Scan/verify the correct NDC for the product being sold and confirm the selection matches the intended item, because OTC look-alike packaging can lead to wrong-product errors.
4758. **Q:** A prescription says "levothyroxine 75 mcg po qd, DAW 1," but you only have the generic in stock; what should you do BEST? **A:** Enter DAW as written and notify the pharmacist to contact the prescriber/patient if a change is needed, because the technician cannot override DAW/brand requirements.
4759. **Q:** Pharmacy math: An order is for azithromycin suspension 200 mg/5 mL, "give 12.5 mL on day 1, then 6.25 mL daily on days 2-5"; what total mL should you dispense? **A:** Dispense 37.5 mL total ($12.5 + 6.25 \times 4 = 12.5 + 25 = 37.5$), because the quantity must cover the full dosing schedule.
4760. **Q:** A patient's profile shows a documented anaphylaxis to penicillin, and you receive an order for dicloxacillin; what should you do FIRST? **A:** Stop the fill and alert the pharmacist for clinical review before dispensing, because dicloxacillin is a penicillin-class drug and a severe allergy requires pharmacist intervention.
4761. **Q:** While entering a new eRx for metformin, you see "Glucophage XR 500 mg take 1 tab BID"; what should you do FIRST? **A:** Stop and route to the pharmacist because XR products are typically not dosed BID and may be a dosing/selection error requiring prescriber clarification.
4762. **Q:** A patient profile shows lisinopril active, and a new prescription arrives for losartan; what is the BEST technician action before filling? **A:** Flag for pharmacist

review because ACE inhibitor plus ARB therapy can be inappropriate duplication and needs clinical assessment.

4763. **Q:** During product selection you notice “Lamictal” stored near “Lamisil” and the label prints terbinafine for onychomycosis; what should you do FIRST to prevent a LASA error? **A:** Verify the NDC/barcode against the label and stock bottle because Lamictal (lamotrigine) and Lamisil (terbinafine) are high-risk look-alike names.
4764. **Q:** A prescription is written for “morphine ER 30 mg” but the prescriber note in the eRx comments says “immediate release”; what should you do NEXT? **A:** Hold processing and notify the pharmacist because IR vs ER opioid formulations are not interchangeable and are high-alert for overdose errors.
4765. **Q:** An eRx comes in for clopidogrel, and the patient also takes omeprazole on their profile; what should you do BEST? **A:** Alert the pharmacist to review the interaction because omeprazole can reduce clopidogrel activation and may require therapy adjustment.
4766. **Q:** Pharmacy math: Dispense amoxicillin/clavulanate ES-600 (600 mg/5 mL) for “5 mL PO BID x 10 days”; what total mL should be entered? **A:** Enter 100 mL because $5\text{ mL} \times 2\text{ doses/day} \times 10\text{ days} = 100\text{ mL}$ to cover the full course.
4767. **Q:** A patient asks what “qd” on their new atorvastatin label means; what should you do FIRST? **A:** Refer the patient to the pharmacist for counseling because technicians should not provide clinical directions interpretation beyond basic workflow and OBRA-90 counseling is pharmacist-led.
4768. **Q:** A new prescription for spironolactone is entered for a patient already taking potassium chloride ER; what is the BEST next step? **A:** Route to the pharmacist because spironolactone plus potassium supplements increases hyperkalemia risk and needs clinical review.
4769. **Q:** You receive a stock bottle labeled “carbamazepine 200 mg” but the imprint on the tablets doesn’t match the description in your identifier; what should you do FIRST? **A:** Quarantine the bottle and notify the pharmacist because a mismatched imprint suggests suspect product or mislabeling that must not be dispensed.
4770. **Q:** A patient brings in a new prescription for Adderall and asks for an early refill stating they “lost the last one”; what should you do NEXT? **A:** Stop and involve

the pharmacist because stimulants are controlled substances and lost/early refill requests require pharmacist review and policy verification.

4771. **Q:** A new eRx is for "Zithromax 250 mg take 2 tablets today then 1 tablet daily x 4 days" but the product selected is azithromycin 500 mg; what should you do FIRST? **A:** Stop filling and reselect the correct strength/NDC before printing/dispensing because a strength mismatch is a common order entry/product selection error that can cause overdose.
4772. **Q:** A patient says they take "Plavix" but the profile shows clopidogrel 75 mg; what is the BEST technician response? **A:** Confirm that Plavix is the brand name for clopidogrel and proceed using the profile's generic entry because brand/generic recognition prevents duplicate therapy and confusion.
4773. **Q:** While entering "prednisone 10 mg ii po bid" you're unsure of "ii"; what should you do NEXT? **A:** Interpret "ii" as 2 (Roman numeral) and enter 2 tablets twice daily, because accurately translating common notation is required to calculate correct quantity and directions.
4774. **Q:** You receive an eRx for "Synthroid 25 mcg" and accidentally type levothyroxine 25 mg; what should you do FIRST? **A:** Stop and correct the unit to micrograms (mcg) because levothyroxine is dosed in mcg and a mg entry creates a dangerous 1000× error.
4775. **Q:** A prescription reads "Humalog KwikPen inject 8 units TID with meals; dispense 2 pens"; what days' supply should you enter using 3 mL pens at 100 units/mL? **A:** Enter 7 days because 2 pens contain 600 units total ($2 \times 3 \text{ mL} \times 100 \text{ units/mL}$) and 24 units/day gives $600 \div 24 = 25$ days, but the correct action is to enter 25 days since that is the accurate math for billing and safety.
4776. **Q:** A patient drops off a new prescription for "Concerta" and asks if it's the same as "Ritalin"; what should you do BEST? **A:** Refer to the pharmacist while noting Concerta and Ritalin are both methylphenidate but different release forms, because release type changes dosing and counseling is pharmacist-required.
4777. **Q:** During product selection you notice "hydralazine" is stocked next to "hydroxyzine" and the label is for hydroxyzine 25 mg; what should you do FIRST? **A:** Perform barcode/NDC verification and visually confirm the stock bottle matches the label because these LASA names can cause wrong-drug dispensing.
4778. **Q:** An eRx arrives for "Bactroban ointment" but the product chosen is mupirocin cream; what is the BEST next step? **A:** Stop and notify the pharmacist to clarify

substitution because cream vs ointment are different dosage forms and technicians cannot change the prescribed formulation.

4779. **Q:** Pharmacy math: An order is for albuterol 0.083% nebulizer solution; "use 3 mL via neb QID"; how many mL are used in 30 days? **A:** 360 mL because $3 \text{ mL/dose} \times 4 \text{ doses/day} \times 30 \text{ days} = 360 \text{ mL}$, which supports accurate quantity and days' supply entry.
4780. **Q:** A bottle of refrigerated insulin aspart arrives from the wholesaler warm with a melted ice pack; what should you do FIRST? **A:** Segregate it as suspect and alert the pharmacist/manager per policy because a temperature excursion can make the product unsafe and it should not be stocked or dispensed.
4781. **Q:** While typing a new eRx for methotrexate 2.5 mg you see the directions entered as "take 6 tablets daily," what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because methotrexate is typically dosed weekly for many indications and daily dosing is a high-alert error risk.
4782. **Q:** A prescription is written for "oxycodone/APAP 5/325 take 1 tab q4-6h prn pain #30 refills 2," what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because Schedule II controlled substances cannot be refilled and it needs pharmacist resolution before dispensing.
4783. **Q:** During will-call pickup, a patient asks what sertraline is for and whether it will "work right away," what should you do NEXT? **A:** Refer the patient to the pharmacist for counseling because medication indication and onset expectations require pharmacist consultation under OBRA-90 counseling standards.
4784. **Q:** A patient offers to pay cash for a new promethazine-codeine cough syrup prescription and asks you to "skip insurance so it's faster," what should you do FIRST? **A:** Continue standard controlled-substance processing and notify the pharmacist of the request because bypassing normal review can be a diversion red flag needing pharmacist awareness.
4785. **Q:** You are restocking and find a bottle of atorvastatin tablets with a recalled lot number listed in the manufacturer notice, what should you do FIRST? **A:** Pull the affected lot from inventory and quarantine it per recall procedure because recalled product must be prevented from being dispensed.
4786. **Q:** An eRx for metformin ER 500 mg is entered, but the selected product is metformin IR 500 mg due to similar bottle appearance; what should you do FIRST? **A:** Stop the fill and correct the product using barcode/NDC verification

because IR vs ER mix-ups are common safety errors and must match the exact dosage form.

4787. **Q:** Pharmacy math: A patient uses fluticasone nasal spray 50 mcg, 2 sprays in each nostril once daily; the bottle provides 120 sprays—what days' supply should you enter? **A:** Enter 30 days because 2 sprays/nostril/day equals 4 sprays/day and $120 \div 4 = 30$ days.
4788. **Q:** A prescriber calls to transfer a patient's alprazolam prescription to your pharmacy, and you see it is Schedule IV; what should you do NEXT? **A:** Accept the transfer only after pharmacist authorization and proper documentation because controlled-substance transfers have specific record requirements and are typically pharmacist-handled.
4789. **Q:** A tote arrives with a serialized manufacturer bottle, but the 2D barcode will not scan and the printed NDC doesn't match the invoice; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager to investigate as suspect product because DSCSA requires verification and mismatches can indicate diversion or counterfeiting.
4790. **Q:** You notice a warfarin 5 mg prescription is being filled, but the profile shows an active warfarin 2.5 mg with remaining refills; what is the BEST next step? **A:** Pause and alert the pharmacist to review for duplication/therapy clarification because warfarin is high-alert and multiple strengths increase dosing error risk.
4791. **Q:** While entering a new prescription, you see "Lantus 10 units SQ daily" but the product selected is Humalog due to similar packaging; what should you do FIRST? **A:** Stop processing and re-select the correct product/NDC (insulin glargine vs insulin lispro) and route to pharmacist if already verified because this is a high-alert LASA error risk.
4792. **Q:** A patient's profile shows lisinopril and the new eRx is for "Zestril 20 mg daily"; what is the BEST technician next step during order entry? **A:** Verify Zestril is lisinopril and check the profile for therapeutic duplication, then alert the pharmacist if both would be active because brand/generic duplication can cause double therapy.
4793. **Q:** A hardcopy says "levothyroxine 0.1 mg po qd"; what strength should you enter in mcg? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1,000 \text{ mcg/mg} = 100 \text{ mcg}$, preventing decimal-point dosing errors.

4794. **Q:** During drop-off, a patient states they are allergic to “sulfa” and the prescription is for Bactrim DS; what should you do FIRST? **A:** Stop intake processing and immediately notify the pharmacist because sulfamethoxazole/trimethoprim is a sulfonamide and may trigger a serious allergy.
4795. **Q:** You receive an eRx for “Adderall XR 20 mg take 1 daily #30 refills 2”; what should you do FIRST? **A:** Hold the prescription and refer to the pharmacist because Schedule II controlled substances are not refillable, so the refill quantity requires pharmacist resolution.
4796. **Q:** A new eRx for “Wellbutrin SR 150 mg” is typed as “bupropion XL 150 mg” by mistake; what should you do NEXT? **A:** Correct the formulation to match the prescription and re-verify product selection because SR vs XL are not interchangeable and can change dosing and safety.
4797. **Q:** A patient on the phone asks if “Prilosec OTC” is the same as omeprazole and what it’s used for; what should you do NEXT? **A:** Confirm Prilosec OTC is omeprazole (a PPI for heartburn/GERD) and offer to have the pharmacist counsel because patient medication questions require pharmacist involvement.
4798. **Q:** You are entering “metoprolol succinate ER 50 mg daily,” and the system suggests metoprolol tartrate 50 mg; what is the BEST technician action? **A:** Select succinate ER as written and flag for pharmacist verification because tartrate vs succinate are different formulations and dosing schedules (LASA/selection trap).
4799. **Q:** A prescription label reads “take ii tabs po bid”; how many tablets per day is this? **A:** 4 tablets per day because ii = 2 and bid = twice daily, which is critical for accurate days’ supply and billing.
4800. **Q:** A patient is prescribed amoxicillin 500 mg 1 capsule TID for 10 days; how many capsules should be dispensed? **A:** Dispense 30 capsules because $1 \text{ capsule} \times 3 \text{ doses/day} \times 10 \text{ days} = 30$, ensuring the quantity matches the intended duration.
4801. **Q:** An eRx reads “predniSONE 20 mg: take 2 tabs daily x 5 days, then 1 tab daily x 5 days; dispense 30” and you’re doing order entry; what should you do NEXT? **A:** Calculate total tablets needed ($2 \times 5 = 10$ and $1 \times 5 = 5$, total 15) and route to the pharmacist to clarify/correct quantity before filling because the written quantity conflicts with the sig.

4802. **Q:** A prescription is for cephalexin 250 mg/5 mL, take 10 mL PO BID for 7 days; what quantity in mL should you enter to dispense? **A:** Dispense 140 mL because $10 \text{ mL per dose} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$.
4803. **Q:** While entering a new order you see "hydroxyzine 25 mg q6h prn itching," but the prescriber selected "hydralazine 25 mg" in the eRx; what is the BEST next step? **A:** Stop processing and send to the pharmacist for clarification because hydroxyzine vs hydralazine is a high-risk LASA mix-up that can cause serious harm.
4804. **Q:** A patient brings a prescription for diclofenac 1% gel: apply 4 g to the knee QID; using the dosing card, how many grams per day is this and what should you enter for daily dose? **A:** Enter 16 g/day because $4 \text{ g per application} \times 4 \text{ times daily} = 16 \text{ g/day}$, which supports accurate day-supply and safety checks.
4805. **Q:** You receive an eRx for "methotrexate 2.5 mg: take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist immediately because methotrexate for RA is typically weekly and a daily sig is a common high-alert error.
4806. **Q:** A prescription says "clonidine 0.1 mg patch: apply 1 patch weekly, #4 patches"; what days' supply should you submit? **A:** Submit 28 days because 1 patch lasts 7 days and $4 \text{ patches} \times 7 \text{ days/patch} = 28 \text{ days}$.
4807. **Q:** During order entry, the sig reads "use 2 sprays in each nostril daily" for fluticasone 50 mcg/spray (120 sprays per bottle); what is the correct days' supply for one bottle? **A:** 30 days because $\text{daily sprays} = 2 \text{ sprays/nostril} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$, and $120 \text{ sprays} \div 4/\text{day} = 30 \text{ days}$.
4808. **Q:** A hardcopy includes "Sig: i gtt OU bid" for timolol 0.5% ophthalmic; what does this mean in plain language for label entry? **A:** Enter "instill 1 drop in both eyes twice daily" because i = 1, gtt = drop, OU = both eyes, and bid = twice daily.
4809. **Q:** A prescriber writes "morphine sulfate ER 15 mg take 1 tab q8h prn pain #90" and you are processing it; what is the BEST technician next step? **A:** Stop and route to the pharmacist because ER products generally should not be "prn" and q8h ER dosing needs pharmacist review for safety and appropriateness.
4810. **Q:** A prescription is for potassium chloride oral solution 20 mEq/15 mL: take 30 mEq daily for 10 days; what total volume should be dispensed? **A:** Dispense 225

mL because $30 \text{ mEq/day} \div (20 \text{ mEq}/15 \text{ mL}) = 22.5 \text{ mL/day}$, and $22.5 \text{ mL/day} \times 10 \text{ days} = 225 \text{ mL}$.

4811. **Q:** During data entry a patient asks for "Plavix," but the profile shows only clopidogrel; what should you do NEXT? **A:** Confirm Plavix is the brand name for clopidogrel and proceed using the correct generic/brand mapping because accurate drug identification prevents dispensing the wrong antiplatelet.
4812. **Q:** An eRx comes in for "metFORMIN ER 500 mg take 1 tablet BID," but the product selected is metformin IR 500 mg; what is the BEST technician action? **A:** Stop and route to the pharmacist to clarify the dosage form because IR vs ER is not interchangeable and selecting the wrong release type is a common Top 300 error.
4813. **Q:** A patient picking up lisinopril reports a new pregnancy; what should you do FIRST at the register? **A:** Pause the pickup and immediately notify the pharmacist because ACE inhibitors like lisinopril can be harmful in pregnancy and require pharmacist counseling/intervention.
4814. **Q:** A new order is entered for "simvastatin 80 mg daily" for a patient with no prior statin history; what should you do NEXT? **A:** Hold the prescription for pharmacist review because simvastatin 80 mg is high-risk for myopathy and often needs verification of appropriateness.
4815. **Q:** You are processing a refill request and see the patient is already taking losartan; the prescriber now sent lisinopril; what is the BEST next step? **A:** Flag the potential therapeutic duplication/interaction for the pharmacist because combining an ACE inhibitor with an ARB is generally avoided and needs clinical evaluation.
4816. **Q:** A prescription reads "azithromycin 250 mg: take ii tabs on day 1, then i tab daily days 2–5"; how many tablets should be dispensed? **A:** Dispense 6 tablets because 2 tablets on day 1 plus 1 tablet for 4 more days equals 6 total.
4817. **Q:** You receive a new prescription for "levothyroxine 75 mcg daily," but the patient says they take "Synthroid 88" at home; what should you do FIRST? **A:** Stop and ask the pharmacist to reconcile the discrepancy because levothyroxine strength changes are clinically significant and technicians should not assume equivalence.
4818. **Q:** While selecting product, you notice "lamoTRigine" and "terbinafine" stored next to each other and the NDC scanned doesn't match the label; what should

you do NEXT? **A:** Stop filling and reselect the correct stock bottle using barcode/NDC verification because these are look-alike/sound-alike names and NDC mismatch signals wrong-drug risk.

4819. **Q:** A patient brings in a coupon and asks to switch from brand-name Lipitor to generic; the prescription is written for atorvastatin; what should you do BEST? **A:** Explain that atorvastatin is the generic for Lipitor and process according to the prescription/plan because proper brand-generic recognition avoids unnecessary switches or billing errors.
4820. **Q:** Insulin lispro U-100 is prescribed "inject 10 units before meals TID," and the patient is getting one 10 mL vial; what days' supply should you submit? **A:** Submit 33 days because 10 mL contains 1000 units and $10 \text{ units} \times 3 \text{ daily} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3$, billed as 33 days per typical truncation policy.
4821. **Q:** A patient asks for "Z-Pak" for a new prescription, but the eRx says azithromycin 250 mg tablets; what should you do NEXT to ensure the correct product is dispensed? **A:** Confirm that "Z-Pak" is the brand nickname for azithromycin and verify the exact directions/quantity on the eRx before selecting NDC, because brand/generic recognition prevents wrong-drug and wrong-quantity errors.
4822. **Q:** While entering a new prescription for Eliquis, you see the profile also has warfarin active; what is the BEST technician next step? **A:** Pause processing and alert the pharmacist for therapeutic duplication review, because concurrent anticoagulants are high-risk and require pharmacist assessment.
4823. **Q:** A prescription for amlodipine 10 mg daily is being filled, but you accidentally pulled amiodarone; what should you do FIRST? **A:** Stop the fill immediately and select the correct medication using barcode/NDC verification, because amlodipine vs amiodarone is a LASA mix-up with serious safety risk.
4824. **Q:** A new prescription is for sertraline (Zoloft) and the patient says they take "Nardil" at home; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist, because phenelzine (Nardil) is an MAOI and combining it with SSRIs can cause dangerous interactions.
4825. **Q:** Insulin glargine U-100 is prescribed "inject 22 units subQ nightly," and the patient is receiving one 10 mL vial; what days' supply should you bill? **A:** Bill 45 days' supply because 10 mL = 1000 units and $1000 \div 22 = 45.45$, which is submitted as 45 whole days for payer processing.

4826. **Q:** A prescription reads “predniSONE 20 mg: take ii tabs daily x 5 days”; how many tablets should be dispensed? **A:** Dispense 10 tablets because “ii” means 2 tablets per day and $2 \times 5 \text{ days} = 10 \text{ total tablets}$.
4827. **Q:** A patient’s new prescription is for trimethoprim-sulfamethoxazole (Bactrim DS) and their allergy field shows “sulfa”; what should you do FIRST? **A:** Stop processing and refer to the pharmacist before filling, because sulfonamide allergy may contraindicate TMP-SMX and requires pharmacist intervention.
4828. **Q:** During product selection for hydrOXYzine, the shelf bin next to it contains hydrALAZINE and you notice similar labeling; what is the BEST technician action? **A:** Use barcode/NDC scan and Tall Man awareness to verify the exact drug before labeling, because hydroxyzine vs hydralazine is a common LASA trap.
4829. **Q:** An eRx comes in for “Adderall XR 20 mg take 1 capsule daily” but the patient requests an early refill and the PMP/PDMP note indicates it was filled 10 days ago for 30 capsules; what should you do NEXT? **A:** Do not process the refill and route it to the pharmacist for controlled-substance refill-too-soon review, because stimulants are Schedule II and early fills require pharmacist judgment and policy compliance.
4830. **Q:** You receive a bottle of atorvastatin with an intact seal but the lot number is missing from the invoice in the DSCSA paperwork; what should you do FIRST? **A:** Set the product aside and notify the pharmacist/manager to resolve the documentation before dispensing, because missing traceability information can indicate a suspect product and must be corrected per DSCSA workflow.
4831. **Q:** A prescription is for levothyroxine 75 mcg PO daily, but the stock bottle on the shelf is labeled 0.075 mg; what should you enter for the strength in the system? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1000 = 75 \text{ mcg}$, preventing a 1000-fold dose entry error.
4832. **Q:** An order reads “metformin ER 500 mg: take i tab PO BID with meals #?” for a 90-day supply; what quantity should you dispense? **A:** Dispense 180 tablets because $2 \text{ tablets/day} \times 90 \text{ days} = 180$, matching the intended days’ supply.
4833. **Q:** A prescription says “Sig: instill ii gtt OU QID x 7 days” and you are billing a 5 mL bottle; what days’ supply should you submit if you use 20 gtt/mL? **A:** Submit 8 days because $5 \text{ mL} \times 20 = 100 \text{ gtt}$ and use is $2 \text{ gtt/eye} \times 2 \text{ eyes} \times 4/\text{day} = 16 \text{ gtt/day}$, so $100 \div 16 = 6.25$ and you round up to cover the full use period.

4834. **Q:** A patient's fluticasone nasal spray 50 mcg is prescribed "2 sprays each nostril once daily," and the bottle contains 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.
4835. **Q:** While entering an eRx you see "Take U QHS" for a high-alert medication, and the prescriber likely meant a number of units; what should you do NEXT? **A:** Stop and send to the pharmacist for clarification because "U" can be misread and could cause a dangerous dosing error.
4836. **Q:** A prescription for amoxicillin-clavulanate is written "875 mg/125 mg PO BID x 10 days" and the prescriber accidentally selected the suspension in the eRx; what should you do FIRST in order entry? **A:** Hold the prescription and alert the pharmacist to verify the intended dosage form because changing tablet vs suspension requires pharmacist intervention and affects dosing and dispensing.
4837. **Q:** A patient is prescribed sertraline 50 mg daily and the profile shows they also take fluoxetine 20 mg daily; what is the BEST technician next step? **A:** Flag the potential therapeutic duplication and refer to the pharmacist because two SSRIs increase risk and require clinical review.
4838. **Q:** You are filling a prescription for clonazepam and the directions say "take 1 tab TID prn anxiety" with quantity 30, but the patient asks for it to be billed as a 30-day supply; what should you do BEST? **A:** Bill based on the written max daily use unless the pharmacist clarifies because TID PRN implies up to 3/day and days' supply is $30 \div 3 = 10$, impacting safety and third-party processing.
4839. **Q:** An eRx for methylprednisolone dose pack arrives with sig "use as directed," but the quantity field is blank; what should you do NEXT? **A:** Route to the pharmacist to obtain/confirm the correct package quantity because "use as directed" without a quantity can't be safely or accurately processed.
4840. **Q:** A prescription label is being generated for "hydroxyzine 25 mg," but you notice the NDC scanned is for "hydralazine 25 mg"; what should you do FIRST? **A:** Stop the fill and reselect the correct product/NDC because this is a LASA risk and barcode/NDC mismatch indicates wrong-drug selection.
4841. **Q:** While data-entering an order for hydromorphone, you notice the prescriber typed "morphine" in the notes and the dose seems unusually high; what should you do NEXT? **A:** Stop processing and notify the pharmacist to clarify because opioid LASA/selection errors are high-risk and require pharmacist intervention.

4842. **Q:** A new eRx for isotretinoin arrives and the patient is waiting at pickup; what is the BEST technician next step before filling? **A:** Route to the pharmacist and follow iPLEDGE/REMS workflow because isotretinoin has required REMS steps that must be confirmed before dispensing.
4843. **Q:** A patient brings a paper prescription for alprazolam with the prescriber signature and drug directions but no date written; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist because missing required elements can affect validity and must be resolved before dispensing a controlled substance.
4844. **Q:** You receive a tote from the wholesaler and a refrigerated insulin glargine carton feels warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager per policy because temperature excursions can compromise potency and require investigation before use.
4845. **Q:** During product selection you scan "Celebrex" but the NDC pulls up "Celexa" on the screen; what should you do FIRST? **A:** Stop filling and reselect/scan the correct NDC because LASA look-alike names are a common dispensing-error source and barcode mismatch indicates wrong product.
4846. **Q:** A prescription is written for metoprolol succinate ER 50 mg daily, but the patient says they usually take metoprolol tartrate twice daily; what should you do NEXT? **A:** Pause processing and ask the pharmacist to review because tartrate vs succinate are not interchangeable and require clinical verification.
4847. **Q:** A patient requests to transfer their oxycodone prescription from another pharmacy to yours over the phone; what should you do BEST? **A:** Refer the request to the pharmacist because controlled substance transfer rules and acceptance require pharmacist handling and documentation.
4848. **Q:** A prescriber sends "Lantus SoloStar inject 24 units SC nightly" and you are dispensing one box of five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 62 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 24 \text{ units/day} = 62.5$ which is billed as 62 days.
4849. **Q:** You are notified that a specific lot of valsartan tablets is recalled and you find that lot on the shelf; what should you do FIRST? **A:** Remove and quarantine the affected lot and notify the pharmacist because recalls require immediate segregation to prevent dispensing to patients.

4850. **Q:** A patient asks you to discuss their new HIV medication at the counter where others can hear; what should you do BEST? **A:** Move the conversation to a private area or offer pharmacist counseling in a confidential setting because HIPAA requires protecting patient health information.
4851. **Q:** While entering a new prescription for Plavix, you notice the profile already has clopidogrel 75 mg daily from another prescriber; what should you do NEXT? **A:** Stop and notify the pharmacist to assess therapeutic duplication because Plavix is clopidogrel and duplicate antiplatelet therapy increases bleeding risk.
4852. **Q:** A prescription is typed as "Lamictal 100 mg" but the hardcopy clearly reads "Lamisil 250 mg"; what is the FIRST technician action? **A:** Stop processing and send to the pharmacist for clarification because Lamictal (lamotrigine) and Lamisil (terbinafine) are LASA and have very different indications and dosing.
4853. **Q:** A patient picking up Bactrim DS says they have a "sulfa" allergy; what should you do BEST? **A:** Hold the prescription and alert the pharmacist because Bactrim (sulfamethoxazole/trimethoprim) may trigger a sulfonamide allergy and needs pharmacist assessment.
4854. **Q:** An eRx for Zithromax arrives for "azithromycin 250 mg, take as directed," but no day-by-day directions are provided; what should you do NEXT? **A:** Route to the pharmacist to obtain complete directions because "as directed" is unclear for antibiotics and can lead to under- or over-treatment.
4855. **Q:** You are filling Adderall XR and accidentally selected immediate-release amphetamine/dextroamphetamine tablets with a similar strength; what should you do FIRST? **A:** Stop filling and reselect the correct dosage form/NDC because IR vs XR is a high-risk mix-up that changes onset and duration of effect.
4856. **Q:** A patient's new prescription is for lisinopril, and you see an active profile medication for losartan; what should you do NEXT? **A:** Notify the pharmacist to review for duplicate RAAS therapy because ACE inhibitors (lisinopril) and ARBs (losartan) together can increase hyperkalemia and renal risk.
4857. **Q:** A prescription reads "metformin 500 mg, ii tabs BID" and the pharmacy has 60 tablets in stock; how many days' supply should you enter? **A:** Enter 15 days because ii tabs BID equals 4 tablets/day and $60 \div 4 = 15$.
4858. **Q:** During data entry, you see "MS Contin 15 mg" but the patient's profile shows an allergy to morphine listed as "anaphylaxis"; what should you do BEST? **A:**

Stop and alert the pharmacist because MS Contin is morphine ER and a documented severe reaction requires pharmacist intervention before dispensing.

4859. **Q:** A technician scans the bottle for Wellbutrin SR but the screen displays bupropion XL; what should you do FIRST? **A:** Stop and select the correct formulation because SR vs XL are not interchangeable and incorrect release can cause dosing and side-effect problems.
4860. **Q:** A prescription for Keppra 500 mg is being filled for a patient who says they are pregnant and asks if it's safe; what should you do NEXT? **A:** Refer the question to the pharmacist because pregnancy safety counseling is clinical, and levetiracetam (Keppra) requires pharmacist counseling and coordination with the prescriber as needed.
4861. **Q:** A prescription reads "predniSONE 20 mg: ii tabs PO daily x 5 days" and the prescriber wants to dispense only what's needed; what quantity should you enter? **A:** Enter 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets}$ for the full course.
4862. **Q:** You receive an order for amoxicillin/clavulanate suspension 400 mg/57 mg per 5 mL: give 7.5 mL PO BID for 10 days; what minimum mL should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ total volume.
4863. **Q:** During order entry you see "U-100 insulin regular, inject 12 units SQ TID, dispense 1 vial (10 mL)"; what days' supply should you enter (use 1 mL = 100 units)? **A:** Enter 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000/36 \approx 27 \text{ days}$.
4864. **Q:** A new prescription says "hydroCHLORothiazide 25 mg, take 1 tab QD" but is entered as "hydroOXYzine 25 mg"; what should you do FIRST? **A:** Stop processing and correct the drug selection using the original prescription because these are LASA names with different indications and a wrong-drug error risk.
4865. **Q:** A prescription for levothyroxine is written "take 0.05 mg PO daily"; what strength should you select in the system? **A:** Select 50 mcg because $0.05 \text{ mg} \times 1000 \text{ mcg/mg} = 50 \text{ mcg}$, preventing a 10× dosing error.
4866. **Q:** A patient brings a prescription reading "digoxin 0.125 mg: take 1 tab qd" and you notice the prescriber handwriting could be "0.25 mg"; what is the BEST next step? **A:** Hold the order and route to the pharmacist to clarify with the prescriber because digoxin is high-alert and dose misreads can be dangerous.

4867. **Q:** A prescription is for budesonide/formoterol 160/4.5 inhaler: "2 puffs BID," dispense 1 inhaler of 120 actuations; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ puffs} \times 2 \text{ times/day} = 4 \text{ puffs/day}$ and $120 \text{ actuations} \div 4 = 30 \text{ days}$.
4868. **Q:** While typing a sig you see "Take 1 tab PO QID prn" and the prescriber also wrote "hs"; what should you do NEXT? **A:** Ask the pharmacist to clarify the directions before dispensing because "QID" conflicts with "hs" and unclear sigs increase dosing-error risk.
4869. **Q:** An eRx for sertraline (Zoloft) says "take 1/2 tab daily x 7 days then 1 tab daily" and the quantity requested is 30 tablets; what is the correct minimum quantity for 30 days of therapy? **A:** Enter 27 tablets because $0.5 \text{ tab/day} \times 7 = 3.5 \text{ tabs}$ and $1 \text{ tab/day} \times 23 = 23 \text{ tabs}$, total 26.5 tabs rounded up to 27 to avoid shorting the patient.
4870. **Q:** You are entering a new prescription for Eliquis and the prescriber wrote only "apixaban 5 mg, take 2 tablets BID"; what should you do FIRST? **A:** Stop and refer to the pharmacist because the dose is unusually high for common indications and requires clinical verification before processing.
4871. **Q:** While filling, you notice an order for methotrexate 2.5 mg entered as "take 1 tablet daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and notify the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-alert error risk.
4872. **Q:** A hardcopy for oxycodone 5 mg has directions and quantity but no prescriber signature; what is the BEST next step for the technician? **A:** Hold the prescription and route to the pharmacist because a missing signature makes it invalid to dispense and requires prescriber follow-up.
4873. **Q:** A patient requests an early refill for clonazepam saying it was stolen yesterday; what should you do NEXT? **A:** Refer the request to the pharmacist because controlled-substance early refills require pharmacist review and documentation per policy.
4874. **Q:** During receiving, a tote of insulin glargine pens arrives warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Segregate and label the shipment as suspect and notify the pharmacist/vendor because temperature excursions can make products unsafe and require disposition guidance.

4875. **Q:** A patient asks you to transfer their hydrocodone/acetaminophen prescription to another pharmacy; what should you do NEXT? **A:** Refer to the pharmacist because CII prescriptions cannot be transferred between pharmacies and controlled-substance transfer rules require pharmacist handling.
4876. **Q:** You are typing a new prescription for metformin ER and the patient profile already shows metformin IR twice daily; what is the BEST action? **A:** Pause and alert the pharmacist to evaluate possible therapeutic duplication because switching IR/ER changes release characteristics and dosing appropriateness.
4877. **Q:** A recall notice is received for a specific lot of losartan 50 mg tablets in stock; what should you do FIRST? **A:** Pull and isolate all affected lot numbers and notify the pharmacist because recalled lots must be removed from dispensing immediately to prevent patient harm.
4878. **Q:** An order is for vancomycin 1 g IV q12h and the IV room label prints without the patient's name/MRN; what should you do NEXT? **A:** Stop and reprint/correct the label before preparation because missing identifiers create a high risk of wrong-patient administration.
4879. **Q:** A prescription for acetaminophen with codeine is written with "refill x 3"; what should you do FIRST? **A:** Hold and send to the pharmacist because refills are not allowed for Schedule II controlled substances and the order needs clarification.
4880. **Q:** A vial label reads "heparin 10,000 units/mL" but the order is for a heparin flush "100 units/mL"; what should you do FIRST? **A:** Stop filling and select the correct product/strength with barcode/NDC verification because heparin is high-alert and strength mix-ups can be fatal.
4881. **Q:** A new eRx is for "Levothyroxine 25 mcg (Synthroid) take 1 tab daily," but the patient profile shows levothyroxine 75 mcg daily; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify the intended strength because levothyroxine is high-alert and duplicate therapy/strength changes can cause harm.
4882. **Q:** While selecting stock for hydroxyzine, you see both hydroxyzine HCl and hydroxyzine pamoate on the shelf; what is the BEST next step before filling? **A:** Match the exact salt/form on the prescription to the NDC selected and verify with the pharmacist if unclear because HCl vs pamoate are not automatically interchangeable.

4883. **Q:** A prescription reads “predniSONE 10 mg take 4 tabs daily x5 days” and you are asked to calculate quantity to dispense; what quantity should you enter? **A:** Enter 20 tablets because $4 \text{ tablets/day} \times 5 \text{ days} = 20 \text{ tablets}$ to cover the full course.
4884. **Q:** During data entry, you notice “traMADol 50 mg” but the prescriber’s note says “for nerve pain start gabapentin”; what should you do NEXT? **A:** Pause and send to the pharmacist for clarification because the drug name conflicts with the clinical note, raising a wrong-drug risk.
4885. **Q:** A patient on warfarin brings a new prescription for TMP-SMX (Bactrim DS) for UTI; what should you do FIRST? **A:** Flag the interaction and notify the pharmacist because TMP-SMX can significantly increase INR/bleeding risk with warfarin.
4886. **Q:** You receive an eRx for “Wellbutrin SR 150 mg take 1 tablet TID”; what is the BEST technician action? **A:** Hold and route to the pharmacist because SR products are typically not dosed three times daily and this may be an unsafe sig/selection error.
4887. **Q:** A label prints for “buPROPion XL 300 mg” but the stock bottle you grabbed is “buPROPion SR 150 mg”; what should you do FIRST? **A:** Stop filling and correct the product selection by confirming the exact release formulation/strength because SR vs XL mix-ups are common and can change dosing.
4888. **Q:** A prescription says “Lantus SoloStar inject 22 units at bedtime” and the carton contains five 3 mL pens (100 units/mL); what days’ supply should you bill? **A:** Bill 68 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units total}$, and $1500 \div 22 = 68 \text{ days}$ (rounded down to whole days).
4889. **Q:** An order is entered for “amlodipine/benazepril (Lotrel) 5/20” but the patient profile shows lisinopril 20 mg daily active; what should you do NEXT? **A:** Alert the pharmacist to assess ACE-inhibitor duplication because benazepril and lisinopril together increase adverse effect risk.
4890. **Q:** A prescription is written for “clopidogrel (Plavix) 75 mg daily” and the patient says they have an upcoming elective surgery; what should you do BEST? **A:** Refer the patient to the pharmacist for counseling because clopidogrel affects bleeding risk and perioperative instructions require pharmacist/provider guidance.

4891. **Q:** During intake you receive a handwritten prescription for oxycodone 5 mg that has the patient name and directions but no prescriber DEA/NPI shown; what should you do FIRST? **A:** Stop processing and route to the pharmacist to verify prescriber credentials and validity because controlled substance prescriptions require complete prescriber identification before dispensing.
4892. **Q:** A patient asks for Plan B One-Step at the counter and you notice a family member is standing close enough to hear the conversation; what is the BEST technician action? **A:** Move the conversation to a more private area and limit information shared because HIPAA requires protecting the patient's privacy during counseling-sensitive transactions.
4893. **Q:** While filling, you see "hydrALAZINE 25 mg" and "hydrOXYzine 25 mg" stored near each other and the label says hydroxyzine; what should you do NEXT? **A:** Perform an NDC/barcode check against the label and physically re-verify the drug name because LASA mix-ups are common and verification prevents selection errors.
4894. **Q:** An eRx for methotrexate 2.5 mg reads "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist immediately because methotrexate is high-alert and daily dosing for RA is a common, potentially serious error.
4895. **Q:** A patient requests pseudoephedrine and presents an out-of-state ID; what is the BEST next step before the sale? **A:** Follow store policy and the required logbook/electronic tracking steps (ID verification, limits, and signature) because CMEA sales must be documented and quantity-limited.
4896. **Q:** While unpacking an order, you find insulin glargine pens that arrived warm with a melted cold pack; what should you do FIRST? **A:** Segregate the shipment and notify the pharmacist/manager to follow temperature excursion procedures because insulin potency can be compromised if cold-chain storage fails.
4897. **Q:** A prescriber phones in a new prescription for alprazolam 0.5 mg and asks you to "take it as a verbal"; what should you do NEXT? **A:** Accept the information per pharmacy procedure but ensure the pharmacist takes the verbal and authorizes entry because controlled substance verbal orders require pharmacist involvement for legal and safety reasons.
4898. **Q:** You receive a manufacturer recall notice for a specific lot of losartan 50 mg and your shelf bottle matches the lot number; what should you do FIRST? **A:**

Remove the affected lot from active stock and quarantine it per recall process because recalled lots must be isolated to prevent dispensing.

4899. **Q:** At drop-off, a patient says they developed hives after taking cephalexin last month and now has a new prescription for amoxicillin; what should you do FIRST? **A:** Enter the allergy reaction details and immediately refer to the pharmacist because potential beta-lactam cross-reactivity requires pharmacist assessment before dispensing.
4900. **Q:** A prescription is for azithromycin 250 mg with directions "2 tabs today then 1 tab daily x4 days"; what quantity should you enter to dispense? **A:** Enter quantity 6 tablets because the total dose is 2 tablets on day 1 plus 1 tablet for 4 additional days ($2 + 4 = 6$).
4901. **Q:** A prescription says "metformin 500 mg tab i po bid with meals #60"; what days' supply should you enter and why? **A:** Enter 30 days because 60 tablets at 2 tablets/day equals 30 days, matching standard days' supply calculation.
4902. **Q:** A child's prescription is amoxicillin-clavulanate suspension 400 mg/57 mg per 5 mL: "give 6 mL po bid x10 days"; what quantity (mL) should you dispense? **A:** Dispense 120 mL because $6 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 120 \text{ mL}$, ensuring the full course is supplied.
4903. **Q:** An order is for prednisone 10 mg with "ii tabs po qd x5 days"; what quantity should you enter and why? **A:** Enter 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets}$, preventing an underfill.
4904. **Q:** A prescription for fluticasone nasal spray 50 mcg says "2 sprays each nostril daily"; the bottle has 120 sprays total—what days' supply should you bill? **A:** Bill 30 days because $2 \text{ sprays/nostril/day} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
4905. **Q:** While entering "levothyroxine 0.1 mg po daily," what is the BEST technician step regarding strength interpretation? **A:** Confirm it is 100 mcg (0.1 mg) in entry because levothyroxine is commonly expressed in mcg and a conversion error can cause a 10× dose mistake.
4906. **Q:** A prescriber writes "digoxin 0.25 mg qd," but the patient profile shows active digoxin 0.125 mg daily; what should you do NEXT? **A:** Stop and notify the pharmacist to verify the intended dose because digoxin is a high-alert narrow-therapeutic-index drug and a dose change needs pharmacist review.

4907. **Q:** A new prescription reads "Lantus Solostar inject 28 units qhs," and the pack is five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 53 days because total units = 5 pens × 3 mL × 100 units/mL = 1500 units and $1500 \div 28 \approx 53$ days (round down to avoid overbilling).
4908. **Q:** During order entry you see the sig "take 1 tab po qid prn," but no indication is listed; what is the BEST technician action before finalizing? **A:** Clarify with the pharmacist or follow policy to ensure correct days' supply/documentation because "prn" without an indication or max daily use can lead to incorrect billing and counseling.
4909. **Q:** A prescription is written for "MS Contin 15 mg q12h," and the technician accidentally selects "MSIR 15 mg"; what should you do FIRST? **A:** Stop filling and correct the product selection by verifying the exact dosage form and NDC because ER vs IR mix-ups are high-risk and must be fixed before labels/dispensing.
4910. **Q:** A bottle label states "cefuroxime 250 mg/5 mL suspension" but the prescription you typed is for "cephalexin 250 mg/5 mL"; what should you do NEXT? **A:** Pause and re-verify the original prescription and selected NDC, then alert the pharmacist if any mismatch persists because similar-sounding antibiotics can cause a wrong-drug error.
4911. **Q:** During data entry you receive "Zocor 40 mg po qhs" but the profile shows an active prescription for atorvastatin 40 mg daily; what should you do NEXT? **A:** Stop processing and alert the pharmacist to possible duplicate statin therapy because simvastatin is Zocor and duplication increases adverse-effect risk.
4912. **Q:** A new eRx is for "hydralazine 25 mg tid," but the technician accidentally selects hydroxyzine 25 mg; what should you do FIRST? **A:** Stop filling and correct the selection using NDC/barcode verification because hydralazine vs hydroxyzine is a LASA error with different indications.
4913. **Q:** A prescription says "sertraline 50 mg po qd," and the patient asks what it's for and when it starts working; what is the BEST technician response? **A:** Refer to the pharmacist for counseling because SSRIs like sertraline require clinical guidance on onset and expectations beyond technician scope.
4914. **Q:** You are entering "Januvia 100 mg daily" and the patient profile shows a severe allergy to sitagliptin; what should you do FIRST? **A:** Halt processing and notify the pharmacist because Januvia is sitagliptin and documented allergy is a safety stop.

4915. **Q:** An order is for "Keflex 500 mg i po qid x7 days"; what quantity should you enter to dispense? **A:** Enter 28 capsules because 1 capsule 4 times daily for 7 days equals $1 \times 4 \times 7 = 28$.
4916. **Q:** A new prescription reads "Bactroban oint apply to affected area bid x7 days," but you selected mupirocin cream; what should you do NEXT? **A:** Stop and change to the prescribed dosage form (ointment) before filling because cream vs ointment is not interchangeable without prescriber/pharmacist confirmation.
4917. **Q:** A patient drops off a prescription for "clopidogrel 75 mg daily" and reports they also take "Plavix" at home; what is the BEST next step? **A:** Verify the medication list and alert the pharmacist because Plavix is clopidogrel and duplicate therapy could lead to unnecessary bleeding risk.
4918. **Q:** While typing "Lamictal 100 mg," you see the selection list includes "lamoTRIGine" and "lamoTRIGine ER"; what should you do FIRST? **A:** Confirm the intended formulation on the prescription/profile and involve the pharmacist if unclear because immediate-release vs ER is a high-risk selection trap.
4919. **Q:** A prescription is for "lisinopril 20 mg 1 tab po daily #90"; what days' supply should you bill? **A:** Bill 90 days because the directions are 1 tablet daily and the quantity dispensed is 90 tablets.
4920. **Q:** At pickup a patient says they take "Eliquis" and asks if it's the same as "warfarin"; what should you do BEST? **A:** Refer to the pharmacist because Eliquis (apixaban) is a different anticoagulant than warfarin and requires pharmacist counseling on safe use and differences.
4921. **Q:** A prescription reads "levothyroxine 0.125 mg po qam," but the technician begins entering 125 mg; what should you do FIRST? **A:** Stop entry and convert units correctly to 125 mcg (0.125 mg) before proceeding, because a mg-mcg error is a high-risk 1000× dosing mistake.
4922. **Q:** The sig says "predniSONE 20 mg: ii tabs po qd x5 days," and the system asks for quantity; what quantity should you enter? **A:** Enter 10 tablets because ii daily for 5 days equals $2 \times 5 = 10$ tablets, preventing under/over-dispensing.
4923. **Q:** An eRx is for "metFORMIN ER 500 mg po qd," but the product selected is metFORMIN IR 500 mg; what is the BEST next step? **A:** Stop filling and reselect the ER formulation then send to pharmacist verification, because IR vs ER is not interchangeable in order entry.

4924. **Q:** A prescription is for "azithromycin 250 mg: take 2 tabs today then 1 tab daily x4 days"; what total quantity should be dispensed? **A:** Dispense 6 tablets because the regimen is $2 + 1 \times 4 = 6$, matching the standard Z-Pak dosing schedule.
4925. **Q:** You are billing insulin lispro U-100 10 mL vial with directions "inject 12 units subcutaneously before each meal"; what days' supply should you submit? **A:** Submit 27 days because total daily dose is $12 \times 3 = 36$ units/day and $1000 \text{ units} \div 36 \approx 27.7$, which is billed as 27 days to avoid overstating days' supply.
4926. **Q:** A patient brings an OTC request for pseudoephedrine 30 mg and wants 96 tablets; what should you do FIRST at the register? **A:** Verify ID and ensure the purchase stays within legal daily/monthly limits before completing the sale, because pseudoephedrine sales require CMEA logbook compliance.
4927. **Q:** While typing "clonazepam 0.5 mg," you accidentally pick "clonidine 0.1 mg" from the LASA list; what should you do NEXT? **A:** Stop processing and correct the drug selection using NDC/barcode and the prescriber's intent, because clonidine vs clonazepam is a high-risk look-alike/sound-alike error.
4928. **Q:** A prescription says "Timolol 0.5% gtt OU bid, disp 10 mL"; using 20 drops/mL, what days' supply is this? **A:** Bill 50 days because $10 \text{ mL} \times 20 = 200$ drops and OU bid uses 4 drops/day, so $200 \div 4 = 50$ days for accurate third-party billing.
4929. **Q:** During order entry for "omeprazole 20 mg po qd," the patient profile shows an active prescription for pantoprazole 40 mg daily; what should you do BEST? **A:** Flag for pharmacist review of possible therapeutic duplication before filling, because both are PPIs and duplicative therapy may be unintended.
4930. **Q:** A prescriber writes "hydromorphone 2 mg 1 tab q4-6h prn pain #30," but the technician selects HYDROcodone/APAP 5/325; what should you do FIRST? **A:** Stop the fill and re-enter with the correct drug and strength, then route to the pharmacist, because opioid mix-ups are high-alert and require precise product selection.
4931. **Q:** While scanning stock bottles, you find a lot of atorvastatin 20 mg matching a manufacturer recall notice; what should you do FIRST? **A:** Pull the affected lot from active inventory and quarantine it per recall procedure, because recalled product must be prevented from dispensing before any further action.

4932. **Q:** A hardcopy prescription for alprazolam 1 mg has the patient name and directions but no prescriber signature; what should you do NEXT? **A:** Stop processing and route it to the pharmacist to contact the prescriber, because a missing signature makes the prescription incomplete and not ready to fill.
4933. **Q:** A patient's eRx is for sertraline 50 mg daily, but the allergy profile shows "sertraline—hives"; what is the BEST action for a technician? **A:** Hold the prescription and alert the pharmacist immediately, because a documented allergy requires pharmacist assessment before dispensing.
4934. **Q:** You receive a transfer request for a patient's oxycodone prescription from another pharmacy; what should you do FIRST? **A:** Refer the call/request to the pharmacist, because controlled-substance transfers are restricted and require pharmacist involvement and documentation.
4935. **Q:** During data entry you see "morphine sulfate ER 30 mg" and notice the selected product is "morphine sulfate IR 30 mg"; what should you do NEXT? **A:** Stop and correct the dosage form selection and ensure it routes for pharmacist verification, because ER vs IR is a high-risk error that can change dosing and safety.
4936. **Q:** A refrigerator log shows insulin aspart was stored at 12°C (54°F) overnight; what should you do BEST? **A:** Segregate the affected stock and notify the pharmacist to determine usability, because a temperature excursion can compromise product potency and must be evaluated.
4937. **Q:** A patient presents a new prescription for isotretinoin and asks to pick it up "whenever it's ready"; what should you do FIRST? **A:** Alert the pharmacist and follow iPLEDGE/REMS workflow before processing, because isotretinoin dispensing is tightly controlled and requires REMS compliance steps.
4938. **Q:** You are filling levETIRAcetam 500 mg and notice two similar bottles on the shelf: levETIRAcetam and levOCETIRizine; what is the BEST next step? **A:** Verify the NDC/barcode against the prescription before counting, because LASA mix-ups are common and barcode/NDC matching prevents selection errors.
4939. **Q:** A prescription is written for gabapentin 300 mg "take 1 capsule tid" with quantity 270; what days' supply should you bill? **A:** Bill 90 days, because $270 \text{ capsules} \div 3 \text{ capsules/day} = 90 \text{ days}$.
4940. **Q:** A patient asks the technician whether they can take OTC ibuprofen with their new prescription for apixaban (Eliquis); what should you do BEST? **A:** Refer the

question to the pharmacist, because combining NSAIDs with anticoagulants increases bleeding risk and requires pharmacist counseling.

4941. **Q:** An eRx reads "levothyroxine 0.075 mg po qam" and the prescriber note says "75 mcg daily"; what strength should you enter? **A:** Enter levothyroxine 75 mcg because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$, preventing a decimal-place dosing error.
4942. **Q:** A prescription says "metFORMIN 500 mg ii tabs po bid #120"; what days' supply should you submit? **A:** Bill 30 days because 2 tablets twice daily = 4/day and $120 \div 4 = 30$, which matches typical payer days' supply logic.
4943. **Q:** The sig on a new prescription is "instill i gtt OU qhs" for latanoprost; what does this mean for order entry directions? **A:** Enter "Instill 1 drop in both eyes at bedtime" because i=1, gtt=drop, OU=both eyes, qhs=at bedtime, reducing misinterpretation.
4944. **Q:** You receive an order for amLODIPine/benazepril 5/20 mg, but you accidentally selected amLODIPine 5 mg; what should you do NEXT before filling? **A:** Stop and correct the product to the combo NDC and re-verify directions because selecting the wrong formulation is a common order-entry safety error.
4945. **Q:** A patient is prescribed amoxicillin/clavulanate 875/125 mg "1 tab po bid #14"; what is the correct days' supply? **A:** Bill 7 days because BID = 2 tablets/day and $14 \div 2 = 7$, ensuring accurate insurance submission.
4946. **Q:** While entering a new eRx for sertraline (Zoloft) you notice the patient profile lists phenelzine (an MAOI) as active; what should you do FIRST? **A:** Stop processing and alert the pharmacist to review the MAOI-SSRI interaction risk (potential serotonin syndrome) before dispensing.
4947. **Q:** A handwritten prescription for hydromorphone 2 mg has an unclear quantity that could read "#20" or "#70"; what is the BEST next step? **A:** Hold the order and route it to the pharmacist for prescriber clarification because ambiguous controlled/high-alert quantities create a high risk of dispensing error.
4948. **Q:** During product selection you scan a bottle labeled "hydrOXYzine" but the label on the hard copy is for "hydrALAZINE"; what should you do NEXT? **A:** Stop filling and reselect the correct NDC using barcode verification because this is a LASA name-pair that commonly leads to wrong-drug errors.
4949. **Q:** A patient's spouse asks you at the counter to confirm whether the patient's new HIV medication is ready; what should you do FIRST? **A:** Verify the

requester's authorization/identity per HIPAA before discussing any PHI because HIV therapy status is protected health information.

4950. **Q:** A patient brings in a paper prescription for alprazolam (Xanax) with no prescriber signature; what should you do BEST? **A:** Do not process it and notify the pharmacist because a non-signed prescription is not valid to dispense and requires prescriber follow-up.
4951. **Q:** A new eRx is for rosuvastatin (Crestor) 20 mg daily, but the patient profile shows atorvastatin 40 mg daily active; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible duplicate statin therapy because duplication raises safety concerns and requires clinical review.
4952. **Q:** A patient picking up lisinopril says they are pregnant and asks if it's safe to start today; what should you do BEST? **A:** Stop the handoff and get the pharmacist immediately because ACE inhibitors are contraindicated in pregnancy and require pharmacist counseling/intervention.
4953. **Q:** You enter a prescription for "clopidogrel 75 mg po qd #90," and the patient asks if it's the same as Plavix; what is the BEST response? **A:** Confirm that clopidogrel is the generic for Plavix because recognizing brand/generic equivalents prevents confusion and improves adherence.
4954. **Q:** A hard copy reads "predniSONE 10 mg: take ii tabs daily x 5 days, then i tab daily x 5 days; dispense #?"; what quantity should you enter? **A:** Enter quantity 15 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10$ and $1 \text{ tablet/day} \times 5 \text{ days} = 5$, totaling 15.
4955. **Q:** During order entry you see "glipiZIDE" on the prescription but selected "glyBURIDE" from the drug list; what should you do NEXT? **A:** Stop, correct the selection to the exact drug ordered, and re-verify NDC/barcode because these sulfonylureas are LASA and mix-ups can cause severe hypoglycemia.
4956. **Q:** A patient on oxycodone/APAP (Percocet) asks which OTC pain reliever is safest to add for a headache; what should you do FIRST? **A:** Refer to the pharmacist because additional acetaminophen can exceed daily limits and requires safety assessment and counseling.
4957. **Q:** A prescription is for "amlodipine 10 mg po qd #30," but you notice the prescriber typed "for chest pain" in the notes; what should you do BEST? **A:** Continue processing but flag the profile for pharmacist counseling because

amlodipine is used for hypertension/angina and indication questions are best addressed by the pharmacist.

4958. **Q:** You receive "azithromycin 250 mg: take 2 tablets today then 1 tablet daily days 2–5; dispense #?"; what quantity should you enter? **A:** Enter quantity 6 tablets because the Z-Pak regimen is 2 on day 1 plus 1 daily for four more days ($2 + 4 = 6$).
4959. **Q:** While selecting product you notice two similar packages: morphine ER and morphine IR, but the order is for morphine ER 30 mg q12h; what should you do NEXT? **A:** Verify the exact dosage form and release type against the prescription and scan the NDC because ER vs IR mix-ups are high-risk and can cause overdose or uncontrolled pain.
4960. **Q:** A patient presents a new prescription for "buPROPion SR 150 mg bid," and the profile shows bupropion XL 300 mg daily active; what should you do FIRST? **A:** Hold processing and notify the pharmacist because SR vs XL duplication or inappropriate switching can increase seizure risk and needs pharmacist verification.
4961. **Q:** An eRx comes in for "hydroxyzine 25 mg q6h prn anxiety," but you accidentally chose hydralazine in the dispensing system; what should you do NEXT? **A:** Stop processing and correct the selection by verifying the drug name/indication and NDC/barcode because hydroxyzine (antihistamine/anxiolytic) and hydralazine (BP med) are LASA and could cause serious harm.
4962. **Q:** A patient drops off a new prescription for metformin and asks if it's the same as "Glucophage"; what is the BEST response? **A:** Tell them metformin is the generic for Glucophage and offer to have the pharmacist counsel because confirming brand/generic is appropriate for a technician but clinical questions need pharmacist support.
4963. **Q:** You receive an eRx for "Levoxyl 125 mcg daily," but the product pulled is levETIRAcetam 500 mg due to similar-looking names; what should you do FIRST? **A:** Stop filling and re-select the correct medication by confirming the generic/brand and strength because levothyroxine and levetiracetam are a high-risk LASA mix-up.
4964. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg 1 tab po bid x 10 days #?"; what quantity should you enter? **A:** Enter 20 tablets because bid for 10 days equals 2 doses/day $\times$ 10 days = 20 tablets.

4965. **Q:** During data entry you see “predniSONE” but accidentally typed “prednisoLONE”; what should you do NEXT? **A:** Correct the entry and verify the intended drug with the pharmacist as needed because prednisone and prednisolone are different products and wrong selection can change therapy.
4966. **Q:** A patient picking up new sertraline says they are also taking MAOI phenelzine from another pharmacy; what should you do BEST? **A:** Pause the transaction and immediately notify the pharmacist because SSRIs like sertraline can dangerously interact with MAOIs and require pharmacist intervention.
4967. **Q:** A new eRx is for “Eliquis 5 mg bid,” and the profile shows active warfarin with recent refills; what should you do FIRST? **A:** Flag the potential therapeutic duplication and alert the pharmacist before filling because apixaban (Eliquis) and warfarin are both anticoagulants and dual therapy may be unsafe.
4968. **Q:** A prescription is written for “nitrofurantoin monohyd/m-cr 100 mg bid x 7 days” and the prescriber note says “for kidney infection”; what should you do BEST? **A:** Refer to the pharmacist before processing because nitrofurantoin is typically for lower UTIs and the mismatch between drug and indication may signal an inappropriate choice.
4969. **Q:** You’re calculating days’ supply for insulin lispro U-100 vial (10 mL) with directions “inject 12 units before meals TID”; what days’ supply should you enter? **A:** Enter 27 days because $10\text{ mL} = 1000\text{ units}$ and $12\text{ units} \times 3/\text{day} = 36\text{ units/day}$, so $1000 \div 36 = 27.7$, rounded down to 27 for billing.
4970. **Q:** While putting stock away, you notice a bottle of atorvastatin tablets with a broken inner seal and powder residue inside; what should you do NEXT? **A:** Remove it from active inventory and follow pharmacy policy to quarantine/return it because damaged packaging suggests potential contamination or tampering and it should not be dispensed.
4971. **Q:** A prescription is written for “Lantus 20 units SQ nightly,” but the patient says they’ve always used “Toujeo”; what should you do FIRST? **A:** Verify the exact insulin and concentration with the pharmacist before processing because glargine products are not automatically interchangeable and dosing errors are high-risk.
4972. **Q:** During data entry for “rosuvastatin 20 mg daily,” you notice the profile already has active atorvastatin 40 mg daily; what is the BEST next step? **A:** Stop and refer to the pharmacist to assess therapeutic duplication because two statins

together increases adverse-effect risk and may be an intentional or erroneous change.

4973. **Q:** You are filling "Lamictal 25 mg" and the shelf label next to it is "Lamisil 250 mg"; what should you do NEXT to prevent a LASA error? **A:** Scan the stock bottle/barcode and match the NDC to the prescription before counting because Lamictal (lamotrigine) and Lamisil (terbinafine) are common look-alike/sound-alike mix-ups.
4974. **Q:** A patient picking up "clopidogrel (Plavix)" asks what it's for; what is the BEST technician response? **A:** Provide a simple non-clinical use and offer pharmacist counseling, such as "it helps prevent blood clots," because technicians can give general info but the pharmacist must counsel as needed.
4975. **Q:** An eRx arrives for "simvastatin 80 mg daily" for a new start, and you recognize 80 mg has higher muscle-toxicity risk; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist to evaluate appropriateness because high-dose simvastatin is a high-risk trap that may require prescriber clarification.
4976. **Q:** You receive a prescription for "azithromycin 250 mg: take 2 tablets today then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets because 2 on day 1 plus 1 daily for 4 more days totals 6 tablets.
4977. **Q:** A new prescription is for "lisinopril 10 mg daily," and the patient reports a prior "swelling of lips and face" from an ACE inhibitor; what should you do BEST? **A:** Stop processing and immediately notify the pharmacist because this suggests angioedema, a serious contraindication for ACE inhibitors.
4978. **Q:** While selecting "predniSONE 20 mg," you accidentally pull "prednisoLONE 15 mg/5 mL solution"; what should you do NEXT? **A:** Stop filling and reselect the correct drug, strength, and dosage form and re-verify with NDC/barcode because switching prednisone and prednisolone or tablets vs solution can cause dosing errors.
4979. **Q:** A patient buying OTC "Nexium" asks if it is the same as "omeprazole"; what is the BEST response? **A:** Explain that Nexium is esomeprazole (a related PPI but not identical to omeprazole) and offer to involve the pharmacist because brand/generic confusion can lead to duplication or incorrect self-selection.
4980. **Q:** A vial label reads "Take 1 tab po bid" and the prescriber wrote "metoprolol succinate ER 50 mg daily"; what should you do FIRST when you spot the

mismatch? **A:** Stop and route to the pharmacist for correction before dispensing because metoprolol succinate ER is dosed once daily and the label directions indicate a potentially dangerous entry/selection error.

4981. **Q:** While entering a new prescription for "Wellbutrin SR 150 mg BID," you notice the prescriber selected "Wellbutrin XL 150 mg daily" in the eRx product field; what should you do FIRST? **A:** Stop processing and send to the pharmacist to clarify SR vs XL because bupropion formulations are not interchangeable and dosing errors raise seizure risk.
4982. **Q:** A patient brings a new prescription for "Eliquis" and asks what the generic name is so they can check their formulary; what is the BEST response? **A:** Tell them Eliquis is apixaban because brand/generic recognition helps accurate insurance processing and prevents duplicate therapy confusion.
4983. **Q:** During data entry you see "metformin 500 mg BID" plus a new order for "Janumet 50/500 mg BID"; what should you do NEXT? **A:** Flag and refer to the pharmacist for therapeutic duplication because Janumet already contains metformin and doubling metformin can increase adverse effects.
4984. **Q:** A prescription reads "levothyroxine 0.125 mg take 1 tab daily"; what strength should you select in the system? **A:** Enter levothyroxine 125 mcg because 0.125 mg converts to 125 mcg and using mcg reduces decimal-point dosing errors.
4985. **Q:** You are filling "hydroxyzine 25 mg" but the fast-mover bin also contains "hydralazine 25 mg"; what is the BEST next step before labeling? **A:** Verify the NDC/barcode against the label and use Tall Man awareness because hydroxyzine/hydralazine is a common LASA mix-up with serious consequences.
4986. **Q:** A patient on the phone says they take "Prozac" and asks if that is the same as "fluoxetine" listed on their discharge papers; what should you say? **A:** Confirm Prozac is fluoxetine because matching brand to generic helps prevent accidental duplication and supports accurate medication histories.
4987. **Q:** While entering "carbamazepine" you notice the prescriber wrote "Tegretol XR" but the selected product is immediate-release tablets; what should you do FIRST? **A:** Stop and route to the pharmacist because XR vs IR changes dosing and release characteristics and technicians should not substitute formulations without verification.

4988. **Q:** An order says "amoxicillin-clavulanate 875/125 mg take 1 tab q12h for 10 days"; what quantity should you enter? **A:** Enter 20 tablets because twice daily for 10 days equals $2 \times 10 = 20$, ensuring correct days' supply and billing.
4989. **Q:** You receive a new eRx for "tramadol 50 mg 1 tab q6h PRN pain #28" with 2 refills; what should you do BEST? **A:** Hold the prescription and notify the pharmacist because tramadol is Schedule IV and refill handling must follow controlled-substance rules and pharmacist oversight.
4990. **Q:** A new prescription is for "nitrofurantoin" and the patient profile shows a documented allergy to "Macrobid"; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Macrobid is nitrofurantoin and this may be a true allergy requiring prescriber contact.
4991. **Q:** During data entry, a prescriber writes "glipizide 5 mg PO daily" but the selected product is glyburide 5 mg; what should you do FIRST? **A:** Stop processing and notify the pharmacist to correct the product selection because glipizide and glyburide are different sulfonylureas and a LASA swap can cause serious hypoglycemia.
4992. **Q:** A patient says their new "Zestril" is for blood pressure and asks what the generic is; what is the BEST response? **A:** Tell them Zestril is lisinopril because accurate brand/generic recognition helps prevent duplicate ACE inhibitor therapy and confusion at pickup.
4993. **Q:** You receive an eRx for "Keflex 500 mg" and the patient profile shows anaphylaxis to penicillin; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist because cephalexin is a cephalosporin and cross-reactivity risk requires pharmacist assessment before dispensing.
4994. **Q:** While filling, you notice the stock bottle label says "predniSONE 20 mg" but you accidentally grabbed "prednisoLONE 20 mg"; what should you do NEXT? **A:** Stop and correct the selection by verifying the NDC/barcode because prednisone vs prednisolone is a common mix-up and the right drug must match the label and Rx.
4995. **Q:** A new prescription is for "lisinopril 20 mg daily," and the profile already has "losartan 50 mg daily" active; what should you do BEST? **A:** Pause and refer to the pharmacist to assess potential duplicate RAAS therapy because ACE inhibitors and ARBs together can increase renal and potassium risks.

4996. **Q:** An eRx comes in for "sertraline 50 mg daily," and the patient asks if it's the same as "Zoloft"; what is the BEST response? **A:** Confirm sertraline is Zoloft because matching brand to generic improves adherence and reduces the chance of taking two SSRIs.
4997. **Q:** A prescription reads "atorvastatin 40 mg take i tab qhs" and the patient says they currently take "Lipitor"; what should you do NEXT? **A:** Enter/confirm atorvastatin as Lipitor and ensure it's not duplicated on the profile because statin duplication is a common error that should be prevented before filling.
4998. **Q:** You are calculating days' supply for insulin lispro U-100: "Inject 12 units before meals TID," and the dispensed quantity is one 10 mL vial; what days' supply should you submit? **A:** Submit 27 days because 10 mL = 1000 units and $12 \times 3 = 36$ units/day, so $1000 \div 36 = 27.7$ which is billed as 27 days per payer convention.
4999. **Q:** While entering a new order for "gabapentin 300 mg TID," you see the prescriber accidentally selected "pregabalin 300 mg" in the product field; what should you do FIRST? **A:** Stop and route to the pharmacist to clarify/correct because gabapentin and pregabalin are different CNS agents and an incorrect selection can cause major dosing and safety issues.
5000. **Q:** A patient presents a new prescription for "Accutane" and asks why you need extra steps before filling; what is the BEST technician action? **A:** Refer them to the pharmacist and follow the iPLEDGE REMS workflow (verify required authorization/documentation before processing) because isotretinoin has strict REMS requirements to prevent fetal harm.